

Rainbow Child. (RAINBOW)

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Call: May 2023

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y 22, 2023

National Stock Exchange of India Limited

Exchange Plaza, Plot No. C/1, G Block,

Bandra Kurla Complex, Bandra (E),

Mumbai – 400 051.

Symbol: RAINBOW BSE Limited

Corporate Relationship Department,

Phiroze Jeejeebhoy Towers,

Dalal Street, Mumbai – 400001.

Scrip Code: 543524

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ub: Transcript of the Investor call - Financial Results f or Q4 & FY23.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015 .

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lease find enclosed the Transcript of the Investor call held on May 15, 2023, with regard to Financial Results of the Company for Q4 & FY23.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

<https://www.rainbowhospitals.in/investor-relations/reports/Analyst-Concall-Transcript-Q4-FY-23.pdf>

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e request you to kindly take the same on record.

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hanking You,

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ours faithfully,

For

Rainbow Children's Medicare Limited

Ashish Kapil

Company Secretary and Compliance Officer

Encl.: As Above

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Rainbow Children's Medicare Limited

Q4 FY '23 Earnings Conference Call

May 15, 2023

Call Duration • 1 hour and 18 minutes

Management Speakers • Dr. Ramesh Kancharla – Chairman and Managing Director

• Mr. Sanjeev Sukumaran – Group Chief Operating Officer

• Mr. R. Gowrisankar – Chief Financial Officer

• Mr. Saurabh Bhandari – Group Business Analyst

Participants who asked questions • Damayanti Kerai, HSBC

• Bansil Desai, JP Morgan

• Pritesh Chheda, Lucky Investments

• Arpit Shah, Stallion Asset

• Alankar Garude, Kotak Securities

• Prashant Kutty, Sundaram Mutual Fund

• Dhavan Shah, Alfaccurate Advisors

• Aditya Khandelwal, SIMPL

• Aneesh Deora, Nomura

• Karthik Narayan, SCP

• Yogesh Tiwari, Arihant Capital

Page 2 of 29 Siddharth Rangnekar: Welcome everyone to the earnings conference call of Rainbow Children's Medicare Limited, to discuss the financial performance for the fourth quarter and full year ended March 31, 2023. We have with us Dr. Ramesh Kancharla, Chairman and Managing Director, Mr. Sanjeev Sukumaran, Group Chief Operating Officer, Mr. R. Gowrisankar, Chief Financial Officer and Mr. Saurabh Bhandari, Group Business Analyst.

Before we commence, I would like to share that some of the statements made on today's call could be forward-looking in nature and may involve certain risks and uncertainties. A detailed statement in this regard is available in the Q 4 FY '23 results presentation that, is posted on the company's website and uploaded on the stock exchange site.

I would now like to invite Dr. Ramesh to make his opening comments. Over to you, sir.

Dr. Ramesh Kancharla: Thanks, Siddharth. Good morning everyone. It gives me immense pleasure to welcome you all to the earnings call for the fourth quarter and for the full year ' 2023. It 's been one full year since we had our IPO and when I lo ok back at the last year 's journey, few things stand out. Rainbow remains the only listed pediatric hospital chain in the English- speaking world. Consequently, the listed universe has no peer comparison for the financial analyst s as well as investors.

Ther efore, ever since the IPO in May '22, we had to assiduously engage with the investors both in India and overseas and explain the various building blocks of our business model, our key differentiators , significant growth opportunity and its potential. Our e fforts are bearing fruit and there is a perceptibly, a better comprehension of our business model among the investor and the analyst community. I must say that I have really enjoyed the journey of evangelizing children's healthcare and its business potenti al.

Once again, to reiterate our clinical model, Pediatric services under the Rainbow brand includes newborn and pediatric intensive care services, pediatric mult

i -specialty care,

pediatric quaternary care, organ transplantation. Birthright by Rainbow is a n integrated obstetric vertical, which includes normal and complex obstetric care, multi -disciplinary fetal care, perinatal genetics and fertility care along with gynecological services.

Rainbow Children's Hospital is built on a strong fundamental of multi -disciplinary approach with a full -time consultant engagement model , commitment of 24x7 service, in a child -centric environment. We run India's largest academic training program for the pediatrics and the pediatric super specialties, in the private health care sector, offering

Page 3 of 29 training in DNB post -graduate training program as well as fellowship programs in various specialties including intensive care services as well as pediatric super specialties.

Historically, the strong momentum of the second quarter and third quarter tapers down in the fourth quarter with the examination season and beginning of the summer holiday vacations. However, this time the strong momentum witnessed in the second quarter and third quarter continued into the fourth quarter across al l the key operating metrics, including outpatient footfalls , inpatient volumes and occupancy. I'm pleased to inform you that the company has delivered robust quarterly performance, led by high patient footfalls across all hospitals. The revenues for the Q4 FY '23 was INR . 316 crores, which is a growth of 49.2% compared to the INR . 212 crores, which is in Q4 FY '22. The EBITDA for Q4 FY'23 was INR . 98 crores, which is a growth of 103% compared to the INR 48.1 crores in Q4 FY'22. And the PAT for Q4 FY '23 was INR. 53.8 crores, which is a growth of 339% compared to the INR . 12.2 crores in Q4 FY ' 22.

The occupancy for the quarter was 58.8%, which is significantly higher compared to 39.6%, in the corresponding quarter of the last financial year. The occupancy wa s higher compared to previous quarter occupancy of 57.1%. This is a result of continued momentum of Q3 and Q4 with the various illnesses, especially viral illnesses, in the community, which is leading to kind of increased footfalls as well as inpatient adm issions. In particular, Adenovirus was the most common cause of pneumonia during the season, where the children required admission for longer periods of time.

Now, coming to our addition of beds and expansion:

- We have recently added 100 beds hospital in F inancial District Hyderabad, which commenced operations on 1st of March 2023. W e are seeing a good traction of outpatients as well as inpatients at this hospital .

- We are going to add 270 beds in the current financial year across various geographies in Hyd erabad , Bangalore and Chennai.

- o In Central city Hyderabad, which is Himayat Nagar, we are coming up with a 60-bed Spoke Hospital .

- o A new block adjacent to the existing Hyder nagar hospital , is being built for the growth opportunity, because we have been run ning extremely busy in that hospital and requires additional beds. So that's going to come very soon.

Page 4 of 29 o Anna Nagar , Chennai is coming up with 80 beds spoke hospital .
o A Brownfield 80 -bed Spoke Hospital is coming in Bangalore at Sarjapur area.

o An additional block with an outpatient department and an IVF facility at Rainbow Children's Hospital, LB Nagar, a Spoke, to enhance the patient facilities at the existing hospital and also cater to the future growth at this hospital. We are adding additional space in this hospital to allow it to yield some more beds to cater to the demand.

- These hospitals are expected to come into operation during the second half of the current financial year.
- There are another 160 beds , which are going to come in next 18 months to 20 months' time, which are a 100- bed facility in Rajahmundry, which is one of the important cities in Andhra Pradesh, and a Spoke Hospital at Hennur Cross Road in Bangalore city of 60 beds. These projects are likely to come in about 18 months to 20 months' time. The work is in progress.

• Recently the company participated in an E -Auction held by Haryana Shehri Vikas Prad hikan (HSVP), which is the government site for hospitals . So, we won the two bids. One is in sector 44 with 2.32 acres of land, and another one is sector 56 ~ 1.23 acres of land. On these land parcels , we are going to build Greenfield hospitals. The sector 44 land parcel is very close to Huda City Centre and close to FMRI, we're going to build a 300 -bed facility, which will be a children's hospital with focus on the tertiary and quaternary care services for children. This hospital will be kind of a referral center for multi -specialty, pediatrics and perinatal care across Gurgaon and northern states, as well as international patients.

• The Spoke Hospital of 100 beds will be built in sector 56, which is very close to the Golf Course Road, and this is for the rapidly growing affluent population of Gurgaon, in the Golf Course Road as well as Golf Course Extension Road. This hospital will be primarily offering 24x7 emergency services for children and as well as women, large outpatients, obstetrics and pediatrics inpatient services and the level 3 NICU.

With the expansion plan highlighted now, Rainbow Group is comfortably placed to add 1,000 beds as envisioned in the business plan and as outlined during the IPO and investor

Page 5 of 29 meetings earlier. We are focusing now on the execution of these projects in a timely manner.

Coming to clinical excellence:

- We have crossed an important milestone of a million outpatients across the group. Perhaps we have actually done 1.2 million footfalls of outpatients in the last financial year.
- Dr. Nageswara Rao Koneti, is the Director of Rainbow Children's Heart Institute. He is a leading cardiologist. He received a patent for a device named "KONAR - MF" . This device is utilized to close holes in the heart for children with cardiac defects. This is being used across 60 countries. We are very proud of him for coming up with a cost -effective device.
- Despite a large patient inflow , our doctors had published 100 research papers in the last one year. It gives me immense satisfaction to see such an academic interest among our doctors.
- We have successfully completed 20 liver transplants and five kidney transplants with excellent results. After 20 liver transplants, we have only lost one child and 19 of them actually gone home with a successful liver transplant. All five of them, the renal transplants have done very well and got discharged.

I take this opportunity to thank all my doctor colleagues and the paramedical staff, who have put in an untiring effort to deal with such large volumes and achieving excellent outcomes.

With a well -laid business plan, our main priorities are now to strive harder to deliver robust clinical outcomes with an excellent patient care, strengthen our hubs with multi -specialty and quaternary care and expand our Hub and Spoke model in Bangalore and Chennai .

Before I pass on the mic to our CFO for business updates, I would like to take this opportunity to welcome Mr. Sanjeev Sukumaran, who has joined us, as a Group Chief Operating Officer effective from April 15, 2023. Sanjeev has more than 25 years of experience doing various senior manager roles across various industries. We are very glad to have him with us.

Page 6 of 29 I now request Mr. Sanjeev Sukumaran to introduce himself to the audience.

Sanjeev Sukumaran: Thank you Dr. Ramesh. Good afternoon, everyone. I am extremely pleased to have been

given this opportunity by the Chairperson and the board and I really look forward to contributing to the success of this organization as we move forward in this ever-exciting journey. I wish to bring in the 25 years of experience that I have in various industries and

work closely with the Board and the Chairperson and the senior leadership team over here, to continue to grow the business as well as to continue to bring in excellence in clinical as well as operations. And I look forward to your continued support too. Thank you. And I hand it over to Mr. Gowrisankar now.

R. Gowrisankar: Thank you Sanjeev. Good afternoon. I would like to thank you all for taking your time and joining our earnings update call. I will now share some insights on our financial performance during the period under review.

- Our quarterly performance, the revenue for Q4, stood at INR . 316 crores and has grown by 49.2% compared to the corresponding quarter of the last financial year.
- EBITDA for Q4 FY ' 23 stood at INR . 98 crores and has grown by 103.62% compared to the corresponding quarter of the last financial year. EBITDA margins are at 30.91% in the current quarter as against 22.66% in the corresponding quarter last year. Expansion in EBITDA margin is on account of improved business and better operating leverage.
- PAT for Q4 FY '23 stood at INR . 53.86 crores and has grown by 339.34% compared to the corresponding quarter of the last financial year. PAT margins are 17% in the current quarter and as against 6%, in the corresponding quarter last year.
- Our OP and IP volume for the current quarter has grown by 48% and 57% over the corresponding period of FY ' 22.
- We have recorded 59% occupancy during the quarter. Our Matured hospital has witnessed 67% occupancy and New hospital has witnessed 41% occupancy during fourth quarter.
- Our return on capital employed and return on equity stands at 24.61% and 25.36% on a full -year basis for FY ' 23.

Page 7 of 29 • Our payor mix between cash and the insurance stands at 52% and 48%. 52% is cash and 48% is credit.

- During Q4, the company has incurred about INR . 35 crores as capital expenditure towards new projects, medical equipment and other fixed assets.

With this, I conclude my remark. We can now open the call for your valuable questions and suggestions. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session. The first question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: Hi, good afternoon and thank you for the opportunity. My first question is on your occupancy. So, the fourth quarter obviously was very strong. As Dr. Ramesh mentioned the momentum from viral fever, etcetera, continued. So, going ahead, how should we look at occupancy, especially for the mature hospitals? Say like we don't see such exceptional cases coming due to some seasonal viral fever, etcetera.

Dr. Ramesh Kancharla : Yes, mature hospitals will continue to clock the similar occupancies. And what happens for us is that the quarter one is generally muted, in the sense of occupancy because children are on holidays in the summer season. And we do , kind of , more of surgical work in summer season. As we move into the second quarters and third quarters, occupancy really picks up. So , a mature hospital shouldn't be a big problem for occupancies because they're already kind of well established. They will have a good traction of patients across the specialties as well as the pediatrics and obstetrics as well.

Damayanti Kerai: Mature should be sustaining 60% plus occupancy rate. Obviously, it could be better, but 60% plus is something we can definitely look at.

Dr. Ramesh Kancharla : Yes, of course . we've clocked around 67% occupancy across mature hospital (in the Q4), which is pretty good occupancy. At Rainbow , we do not do as much revenue from other things. So, it's purely a private insurance and the cash patients. That 's where it's very difficult to kind of look at our occupancies and compare it to adult hospitals.

Damayanti Kerai: Sure. My second question is your average revenue per operating bed. So, in FY '23,

ARPOB grew around 4% year -on-year, after adjusting for COVID vaccine benefit in the previous year. So, this looks a bit lesser than earlier, we talked about sustaining growth in high single digits. So how should we look at this number ahead? And also, if I

Page 8 of 29 look at your press release, ARPOB for new hospital looks better than mature hospitals in fourth quarter and full year. So, can you please explain this?

Dr. Ramesh Kancharla : If you discount the COVID vaccines from the previous year of ' 22 and look at FY23 we have clocked ARPOB ~48,000 plus. I think we achieved about 4%, growth (YoY) . So that's what

actually we had guided ~4% to 5% growth of ARPOB year -on-year.

When you look at the new hospitals, the ARPOB is about Rs. 49,000+ . Because what happens in new hospitals is that they kind of, are focused on very small areas of business with more deliveries and more of ICUs, less occupancies that skews more towards the higher ARPOB. When you get occupancy levels increased, as hospitals may choose that, the ARPOB gets moderated.

Damayanti Kerai: Okay. My last question is on your FCF yield. So obviously on the EBITDA level, you have been delivering one of the best margins across listed hospitals. But if I look at the FCF yield, my calculation suggests, it should be around 1%. So that looks like, very less for the kind of margin which you make on your business. So, can you explain it and how should we look at FCF yield going ahead as some of your key units reach maturity?

Dr. Ramesh Kancharla : I'm very sorry. I did not understand the question.

R. Gowrisankar: It's about what, yield on, you're talking the free cash flow yield ?

Damayanti Kerai: Yes, free cash flow. Yield is coming around 1% for FY '23, despite having such healthy margins. So, if you can explain it like, why it should be so low and how should we look at, in coming period?

R. Gowrisankar : I think, we will take this question later, Damayanti, if you don't mind.

Damayanti Kerai: Okay, sure. Thank you. I'll get back to you.

R. Gowrisankar : We need a little more understanding on this. We'll take it up later. Thank you.

Moderator: Thank you. Ladies and gentlemen, a reminder to you, if you wish to ask any questions, please enter star and one. The next question is from the line of Banshi Desai from JP Morgan. Please go ahead.

Banshi Desai: Yes, good afternoon everyone. Thank you for taking my questions and congratulations on a good set of numbers again. Sir, my question is on the Gurugram acquisition that we have announced. So, this is a Greenfield expansion. This is very different from our previous

Page 9 of 29 strategy, where we have expanded on a very asset light manner. So, what are your thoughts in terms of acquiring these lands out there? And did you evaluate having lease hold properties instead of going for outright acquisition?

Dr. Ramesh Kancharla : Yes, Ms. Banshi, we have actually been evaluating for a fairly long period. And there is probably nothing in Gurgaon with the kind of rental costs . And also, that effective utilization of the space, for what rent you pay is very low : as almost 40% of the rental space will be invisible because of loading. So therefore, when we worked out, (we realized) it's probably better to buy the land and build a Greenfield project. We have cash on the balance sheet, so we kind of are taking that as an advantage. So, we got the land parcels in the key areas which we wanted.

We may take a year, a year and a half extra to do the Greenfield project. But that actually positions us better to build modern hospitals in places like Gurgaon, where we envisage to build truly a referral hospital for multi -specialty care for children in Gurgaon of 300 beds. And also Spoke hospital in a rapidly growing Golf Course and Golf Course Extension Road . I think I'm quite happy with what, how it has panned out. Of course, it takes

some time to

do this project. I'm quite happy about the way things are progressing.

Banshi Desai: Okay. And given the system Haryana government, do we need to reserve beds here for economically weaker section? Or are there any such terms of agreement with the government?

Dr. Ramesh Kancharla : Like most of the hospitals in Gurgaon, we have got a similar clause. If there's a specialty hospital , then 10% of the beds are to be allocated for the economically weaker section under CGHS rates for super specialty hospitals, and for general hospitals, they will have kind of allocate 20% beds . Since I'm going to build a super specialty children's hospital, I think I can negotiate on that (10% beds to be allocated) .

R. Gowrisankar : It's not totally free beds, Banshi. They will pay you on our tariff, there is a discounted tariff. They get about 20% discount. On that rate, they'll pay for the super specialty hospital.

Banshi Desai: Okay. And on those rates, are you still able to recover some of your fixed costs?

R. Gowrisankar : We can do that. So, it's unlike the Delhi DDA, where you have to give totally free, 25% of OP and the 10% of IP beds are totally free. Here it is not so, actually. So, we can definitely, we can get a better rate. The super specialty hospitals are given a better rate by the Haryana government.

Page 10 of 29 Banshi Desai: Okay. And the funding for this project will be largely met through internal accruals?

R. Gowrisankar : Yes. In fact, we have allocated some money from IPO as well. But we can meet everything through internal accruals.

Banshi Desai: Okay . So, you highlighted actually, INR . 450 crores of investments for this asset over the next three, three and a half years. I'm assuming, INR . 160 crores odd out of this would be purely for the land. So, the balance amount would be, it basically translates into a capex per bed of INR . 70,00,000 or so. Is that correct?

Dr. Ramesh Kancharla : We need to see how (CAPEX/Bed pans out) , probably once we have designs and those things , we will probably have a better idea about capex for the bed. I would see it probably closer to the kind of INR. 90,00,000 to INR . 1,00,00,000 for the bed. Because if we are building beds for future as a Greenfield project, obviously, Gurgaon has to be high-end hospital.

Bansi Desai: Yes. So, these INR . 90,00,000 is basically inclusive of land cost, are you saying?

Dr. Ramesh Kancharla : Exclusive of land cost. Probably it's going to be about INR1.2 crores to INR1.25 crores. Including land, I am envisaging it.

Bansi Desai: Okay. So sir, in that case, how should we view the break -even timelines? I understand, Gurgaon as a market is definitely, better paying market. So ARPOBs also will be equally good compared to the rest of the regions. But just in terms of break -even timelines, should that differ given, this is the capex?

Dr. Ramesh Kancharla : I think what we need to, we would be working out is that, how are we positioning this hospital? And what is the kind of construct of this hospital in terms of if it is a normal children's hospital with intensive care services, you don't need 300 beds. Okay. So, what I am envisaging is to see this as a super -specialty children's hospital with all the pediatric specialties, large intensive care services, and quaternary care services to start in Gurgaon. So, there is a significant international opportunity for children to be treated in this hospital as well. So, when you look at the opportunity and when we are building the hospital for today, probably we need to establish a hospital as a super -specialty. It's almost like a multi -specialty hospital capex. It won't differ. If I am doing a children's hospital or Spoke hospital, the n definitely capex will come down significantly. When I am looking at a hospital of that stature, its capex almost like a multi -specialty hospital.

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viously , now it all depends on the kind of teams which I am going to bring in, kind of facilities what I am going to provide and also what kind of treatment I am going to offer. So that's how it's going to pan out.

Obviously, with my experience of 20 years of building children's health care, what we have in Hyderabad today, if I position myself in Gurgaon as a referral center, obviously that's required to build a 300 -bed hospital in Gurgaon. So, this is what background statistics I have done. So, another hospital which is about 100 -bed hospital, which is going to be like a children's hospital with maternity, ICUs and those things.

Moderator: Thank you. The next question is from the line of Pritesh Chheda from Lucky Investments. Please go ahead.

Pritesh Chheda: Sir, we quite didn't understand this business model change. Earlier we used to have INR. 50,00,000 a bed as the capex and we have a combination of specialty and Spoke hospital today in our targeted regions or core regions of Chennai, Bangalore and Hyderabad. How will this Gurgaon investment be different from it and why is it different? And how will the ARPOB change in this kind of investment?

Dr. Ramesh Kancharla : So, obviously what I'm looking at Gurgaon is not as a micro market. I see Gurgaon as a Hub for the NCR plus North. So, when I'm presenting myself with the 20 years of experience in children's healthcare, we do need to build something which is of that reputation, credibility of providing a comprehensive care for children. So therefore, it definitely differs from our routine children's hospitals which we have a capex as a Brownfield project. It's a Greenfield project and it's going to be a high capex project. It's going to be a multi -specialty hospital which is almost like your capex is going to be like a super specialty adult hospital . So that's why it's going to be different. ARPOBs will definitely be closer to the multi -specialty hospitals . And the surgical work and the specialty work is going to be closer to multi -specialty hospitals. This is what I envisage in Gurgaon Hub hospital.

Pritesh Chheda: You have a Hyderabad Hub hospital also, right?

Dr. Ramesh Kancharla : Yes.

Pritesh Chheda: Right, and you would have a Bangalore Hub hospital as well. So there what has been your capex per bed?

Dr. Ramesh Kancharla : My Hyderabad Hub hospital my capex was about INR . 70,00,000 per bed which is a Brownfield project, which has been a kind of semi -warm shelf. So, for my Chennai or

Page 12 of 29 Bangalore hospital was kind of a Hub hospital but we continue to put a capex as we were kind of adding more specialties in Bangalore. So, in Gurgaon, I'm going to do everything at one go, which is why it is a different hospital.

Pritesh Chheda: So, if you had to put a Greenfield today, let's say in Bangalore or Chennai or Hyderabad, what it would have come up to? And for a Greenfield like or for a specialty center like 300 beds of Gurgaon, how many Spokes can you put then surrounding that hospital?

Dr. Ramesh Kancharla : You mean to say that, with that money, how many Spokes can be done?

Pritesh Chheda: No, not with that money, that 300 bed hospital can support how many Spokes?

Dr. Ramesh Kancharla : See Gurgaon is a small area as of kind of say. This is, where we are going to be in right next to Huda City Center. That's the central point. From there, we can drive anywhere about in

15 minutes time. The Spoke, which is going to come in closer to the Golf Course Extension Road, in the Golf Course Road. So that will cater for the rapidly growing Golf Course Extension Road and Golf Course Road area. So, we will have a sufficient number of beds in Gurgaon for a longer period of time.

Pritesh Chheda: And how much does the super-specialty now cost, if you had to set up a Greenfield in, let's say Hyderabad or Bangalore, if you had to set up here?

Dr. Ramesh Kanchar

la : If I had to set up the super-specialty hospital, it will cost the same. Probably, it may cost little more because the land cost here, if I have to buy a private land, it will probably cost INR. 20,00,000 more (per bed) .

R. Gowrisankar: So excluding land, it will cost about, say INR. 80,00,000. So, the current Hyderabad as well as Bangalore Hub hospital, we have set up now five years and seven years back actually.

So that time, it cost us INR . 70,00,000, INR. 60,00,000 per bed and then you add upon inflation also. And then this hospital, it will come in next one or two years because you need to take permission for everything before construction. So obviously you will end up somewhere about close to INR . 80,00,000 -INR. 90,00,000 per bed actually excluding land.

Pritesh Chheda: Okay. Sir on the ARPOB side, you didn't answer whether the specialty hospital in Gurgaon will have ARPOB. You said, it will be closer to a normal specialty hospital? So there, we have range. So Max Healthcare is at INR . 60,000 ARPOB. But someone like KIMS is at INR. 25,000 -INR. 30,000 ARPOB. So, which one, we should take? We should take INR . 60,000 ARPOB for you?

Page 13 of 29 Dr. Ramesh Kancharla : Well, we right now, we are in INR . 48,000 ARPOB. So, we are not very low -end hospitals.

We are high -end hospitals. We are not like KIMS and Narayana (in terms of ARPOB) . We are closer to Apollo's and Max (in terms of ARPOB) . Max is kind of a dominant under NCR hospital. It's a city -based hospital. So, lot of surgical work being done. So, which is why their ARPOBs are high. So, as a children's hospital, which we like to position ourselves to be, not on the ARPOBs, we want to kind of offer for children of, what adult is getting in a multi -specialty hospital, for a child. This is a s a children's hospital group, what we would like to embark in NCR.

Pritesh Chheda: So, it will be higher than your current ARPOB of INR . 48,000?

Dr. Ramesh Kancharla : Yes. I would think so, it's too early for me to guess, that . far.

Pritesh Chheda: And lastly, sir, this ARPOB which you have reported in FY'23, based on the capacity that we have, how much increase can you see further on your existing setup of 16 hospitals , that you have? Because you are a bit seasonal. So, there are two quarters, where the season is very high. So, you have higher occupancy and a couple of quarters with lower occupancy. So, what is the blended annual number that we should put it?

Dr. Ramesh Kancharla : I'll tell you what. See, last year, I've guided that, we would do a kind of 18% to 20% of a top line operating revenue. And I will say hereafter , we'll still be high teens for our growth, this current year also. But I would like to also emphasize on one fact that, we are adding almost 270 beds. Plus, we recently added 100 beds, plus 50 beds, 150 beds. A total of 430

new beds are going to be there. Of course, there are going to be existing areas, where our reputation is at the highest level, Hyderabad, Bangalore and Chennai. But still, when you have so many new beds, your growth to p line growth will be fine. There will be some degree of moderation on the margins. So still, we'll see that we are in a kind of an area, which is not a new geography.

Pritesh Chheda: Sir, my question was, that on the 1,650 bed occupancy at 55%, right? For a FY '23, this 55% can go to what number? Can you reach 60% plus?

Dr. Ramesh Kancharla : Well, if we stop expanding, then it could go to 65% also. But again you need to add beds for opportunity and also build our business and footprint.

Pritesh Chheda: And what is the risk of a price cap, if any, by government on pediatrics hospital? We have seen that happening in non- pediatrics, on certain elective procedures. But do you ever think of a regulation by any chance in pediatrics?

Page 14 of 29 Dr. Ramesh Kancharla : Bit unlikely because pediatrics is an

emergency -based hospital and it is not a packages hospital, where you've got cardiac, renal, and those things. Our package billing is very -very small, which is why we are probably a better off in that. But the government is doing this all the time, DPCO drugs. This year also, the government has put quite a significant number of drugs (under price control) . So, the government is doing its own way and insurance companies are doing their own way. Well, still, we continue to have to do business. That's life.

Moderator: Thank you. The next question is from the line of our Arpit Shah from Stallion Asset. Please go ahead.

Arpit Shah: Hello. Yes. I just wanted to understand the sequential jump in other expenses, this quarter.

R. Gowrisankar : Yes. The other expenses, between the two quarters , December to March, so it has increased by INR . 16 crores. So, this is, due to the expanded business, we have taken a little more of ECL provision of about INR . 2 crores. And then, there are kind of bad debts and the written -off which, we have not done in other quarters. There is INR . 1 crores write -off. Plus, we have, incurred some expenses for our JCI, repair and maintenance ., At our Banjara Hills flagship hospital , we are going for JCI accreditation. And then, we have done a lot of repair maintenance in the Q4 because we will just prepare the hospitals for managing the next season. And there are a couple of other business promotion expense we have incurred. That's where it's about INR . 16 crores.

Arpit Shah: Q4 always will be having a higher other expense number on our realization , right?

R. Gowrisankar : Yes. So, we do a little more of R&M in this Q4.

Dr. Ramesh Kancharla: So that is the time, we get a little bit of free time normally or it's kind of a push for us to kind of get ready for hospitals for the next year.

R. Gowrisankar: Also, we have opened a couple of hospitals in the last quarter. So like it's Sholinganallur as well as Financial District . So, some of the expenses are there actually, the running and maintaining the hospital, hospital maintenance. And then, we do a little more of marketing for those hospitals that has come in this section.

Arpit Shah: Good. And sir, If you can break up the number of beds addition every year. Let's just FY '24, FY '25 FY'26. What would be the number of bed additions?

Page 15 of 29 Dr. Ramesh Kancharla : I've already explained to you in the presentation. So, what's going to come is that there

will be about 270 beds are going to come in this current financial year. So, we have already added about 150 beds in the last financial year. Another 160 beds are going to come in next 18 months' time. These are all going to come. Most of the hospitals are going to come in the Southern part of India. Our, the 400 beds (in Gurugram) , which probably will take about three years to 3.5 years' time. We are really fast in execution, but we'll have to take that kind of time for a Greenfield in Gurgaon, in three years to 3.5 years' time. So that's the landscape at the moment. But we always try to identify some buildings in the surrounding areas. We are negotiating some of the cities also in the neighborhood, like regional Spokes like in Nellore and Coimbatore. But when they come into the kind of reality, I will speak to you.

Arpit Shah: Good. And the insurance price as which were supposed to be effective, Q4 for Hyderabad. So, they were effective for how many months this quarter?

Saurabh Bhandari : So, they have been effective only for last 15 days of last quarter. The entire price hike will be there in this current year.

Arpit Shah: If you can just quantify that number for everyone. The price hike which would come for FY '24?

Saurabh Bhandari : So, for the next year, if you look at our Hyderabad insurance business should contribute about INR .15 crores to INR. 20 crores ,

close to that.

Arpit Shah: Got it. And for the Gurugram project where we are targeting about INR . 450 crores of investment, and it will be ready by FY '26. What kind of ROC or what kind of payback we are targeting over there? Because it is a very different kind of investment that we have seen other than what we have done before. So, what kind of payback? Because if you see our ROCs have been closer to 25% or so. What kind of ROC you are targeting in this new Greenfield project?

Dr. Ramesh Kancharla : I think the payback of such a high capex hospital in the industry works about 7 years to 8 years.

Arpit Shah: And that is what you are targeting? Is that something?

R. Gowrisankar : Yes, it may take (same 7 -8 years) because it is a capex intensive project. So, we may do it in about eight years' time, we should do it. We should achieve it.

Page 16 of 29 Dr. Ramesh Kancharla : Yes, seven years to eight years.

Arpit Shah: Got it. Any guidance on the revenue and the margin front for FY '24?

Dr. Ramesh Kancharla : I do not think any Greenfield project payback period will be five years or so. It is impossible because even multi -specialty, I have been talking to my peer groups and those things. About seven years to eight years is a super performance for the payback for Greenfield projects. More than 200 bed hospitals. The smaller hospitals may be different but larger hospitals will take some time.

Arpit Shah: Got it. Any revenue or margin guidance for FY '24?

Dr. Ramesh Kancharla : I think as I earlier told you, we have guided being the first year , I was very clear that I needed to give a guidance. We kind of guided markets kind of INR . 350 crores of EBITDA, INR. 1,100 crores of top line which we have executed very well. We have exceeded that.

So, for the current financial year, we did put it as a business plan during the IPO time and during the initial investor calls about INR420 crores of EBITDA. So, I think what is important is the top lines are not going to be a big problem because we would achieve it. Growth will be, in the kind of high teens.

We will have to look at, how the margins are going to be because this is the first time, we are going to have almost like 400 plus new beds, or new hospitals, coming into the city. But I am a little optimistic because quite a few numbers of beds are built for the demand's sake rather than kind of a future opportunity. So, let's see how it goes. It's going to be difficult for me to do a quarter-on-quarter guidance because it doesn't work that way. If I give you something I don't want to be wrong. Year-on-year it's a multi-year business honestly. It's a multi-year business and it's a going story. It's a huge opportunity as a children's hospital to build in this country.

When you look at the developed countries to us where we are, we are nowhere. So, Rainbow is one which has been building it. I think an opportunity for me is to build this model strong. It's a multi-year opportunity and people are going to be happy with their returns on a longer-term basis.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Securities. Please go ahead.

Alankar Garude: Hi, good afternoon, everyone. Sir, would you agree that incidents of viral infections in the paediatric segment was a bit higher than usual in FY '23?

Page 17 of 29 Dr. Ramesh Kancharla : Oh, definitely Mr. Alankar. Because what has happened was that children have been almost like a homebound for nearly two years' time. What has happened was when they have come out COVID has never been a big problem for the children. COVID perhaps didn't really bother too much, except few children had some cardiac issues and other problems.

Majority of them have not gone through complications like adults including death and those things. Because the longer periods they have been kept in the houses and routine seasonal illnesses

s have not occurred to them. So, they have not built immunity sufficiently to deal with the routine viral infections. Normally Adenovirus is a simple, trivial, seasonal infection. So, this comes as a kind of surprise across the globe. We do see an Adenovirus infection every season. In the western world, what they have seen is about five times increase in adenovirus infections leading to admissions, sickness, all those things. I would think that in India, it is much bigger. There is no statistical data on that. It is a much more manifold increase in the Adenovirus infections of which children have been very sick. Actually, some of them requiring intensive care and they presented with the pneumonia. So, which is why you see in the third quarter we really struggled for beds and also how patients have been chock-a-block with not only Adenovirus but with various other virus. Most of them (virus), normally are trivial, but their presentation was very pronounced this year. Because of staying at home for longer periods and they have not acquired the normal immunity what they were supposed to get year-on-year.

Alankar Garude: Understood sir. So, in that case what is giving us confidence that we will be able to grow over the 61% occupancy reported in FY '23 in the mature hospitals?

Dr. Ramesh Kancharla : Yes of course.

Alankar Garude: So just wanted to understand the reason sir. So, on that relatively elevated base of higher footfalls in FY '23 due to acute infections, viral infection. What is giving us that confidence sir that we will continue to grow occupancies in the mature hospitals?

Dr. Ramesh Kancharla : Now at 61% let me be honest with you that we struggled for the beds. We really struggled for the beds. What annoys me is that now when patients come, I won't be able to give a bed and that's not good because in a children's hospital it's a very emotional state. If somebody needs an admission, I'm not able to accommodate them, it is not right actually. So therefore, this is business one has to accept it. If I can clock up occupancies of 65% in a steady state in a mature hospital, I would do the revenues or EBITDA as much as other

Page 18 of 29 hospitals because I don't have a government business. I don't have people sitting in the hospitals for longer periods our ALOS are low.

The moment the child gets better, the moment someone delivers, they are ready to go home the moment they recover. So, this is a different business model and we do need to build a capacity for the opportunity and also to have a greater number of patients treated.

This is how I look at a children's hospital. So please don't ever compare children's hospital occupancies with adult hospitals.

Alankar Garude: Okay. So, the second question is if you look at our experience of lease versus Greenfield till now except Vizag all our hospitals have been leased. Now given the upcoming Greenfield one in Gurgaon. Can you take us through your experience in Vizag over the past three years, four years and how would you compare it to your expansion across the other leased facilities?

Dr. Ramesh Kancharla : Vizag is a small city. I'm not looking at Gurgaon, as a Gurgaon city of 50,00,000- 60,00,000 population or 30,00,000 population or whatever it is. I'm looking at Gurgaon as a kind of a Hub, medical Hub for the northern India and also for international. This is how I'm trying to position this hospital for children's healthcare. So, if we look at the number of beds in Gurgaon is huge already. And there are going to be many more beds, many more thousands of beds are going to come into Gurgaon because in the landscape of NCR, if you look at it, Gurgaon is probably a place for healthcare destination. That's what every healthcare leader thinks about it.

There's Max and Apollo, Medanta and Fortis who thinks about it. So, I think I completely agree to that because it's the proximity of Gurgaon to

any northern Indian states. And still, unlike southern India, even today the NCR hospitals are focused to offer tertiary care and advanced care for North. People don't come from Bangalore to Hyderabad, Hyderabad to Chennai for any treatment because they're well developed. But people come from all over the northern states, capital cities or rest of the cities come to Delhi only, for all advanced treatment. So therefore, we need to look at the Delhi opportunity, NCR opportunity as a northern India opportunity of six or seven states plus international. So, this is how, I conceptualized this thought process to say that if we are going to be, we need to be there in a place where the largest healthcare Hub is : Gurgaon. So therefore, we need to be there to kind of define children's healthcare.

Page 19 of 29 Alankar Garude: Would it be fair to assume that despite this Gurgaon announcement, we would continue to actively scout for facilities in Delhi city and Noida?

Dr. Ramesh Kancharla : I think of Delhi proper; I probably won't be kind of looking at any Greenfield, it's very unlikely. And Noida, of course, it won't be as big as this one. Noida would probably require about 100- 125 bed children's hospital. And this will be the Hub hospital for the entire Delhi NCR.

Alankar Garude: Fair enough, sir. And sir, sorry, just one last question with your permission. Is there any change in seasonality patterns, over the years? We have data only for the last three years, but if you look at the drop in quarter 4 margins for the previous two fiscal versus FY'23, clearly, fourth quarter FY'23 has been bit of an outlier and you explain the reason for the same. But just, from a longer trend standpoint, wanted to understand, have you seen over the years any change in seasonality across quarters for our business?

Dr. Ramesh Kancharla : I think right from 2002, we have been seeing it. We have seen the variations of seasonality's, particularly, when we had our Dengue outbreaks in the 2002 to 2007, 2009. And we have seen seasonality getting tweaked. Again, we have seen swine -flu going up to the summer and the seasonality got shifted. So, it all depends on that, the nature and rains and various other factors, which actually influences that, the abnormal pattern of the viruses and also the infections prevailing in the community.

Moderator: Thank you. The next question is from the line of Prashant Kutty from Sundaram Mutual Fund. Please go ahead.

Prashant Kutty: Yes, thank you for the opportunity and congrats on a good set of numbers. So first question is about the occupancy part of it. While you highlighted that, on the matured beds, you typically would be doing about 60% -65%. Please correct me, if I'm wrong, but even in a relatively, let's say a muted quarter, which typically Q4 is, we still have managed to deliver very good occupancy numbers. You highlighted that, there would be some one-off because viral infections are high and all. But incrementally, can you actually assume 65% to 67%, being a norm for the matured hospitals?

Dr. Ramesh Kancharla : So, you would like to know the 67%, for the matured hospitals, without being in the seasons, right?

Prashant Kutty: Yes, exactly, without being in the season?

Page 20 of 29 Dr. Ramesh Kancharla : Yes. Okay. Well, I see the thing is that, clocking over 60% occupancy is more important.

Whether it is 4%, 5% delta is always going to be debatable. The question is that knowing what kind of case mix you have, in the hospital is more important. Sometimes, we had about 55% occupancies. We had done bigger revenues than actually at 65% occupancy. See, sometimes we may have a huge season and your occupancies are very high. But some, when your beds are not occupied with intensive care services, then the revenue is very, very, miniscule. So, it does depend on the case mix and also the sickness. And these are the things, determine

the overall revenue generation in a children's hospital.

Prashant Kutty: But suffice to say that, when you're talking about, at least you're looking for at least above 60% plus kind of a number for the matured hospitals? Be it any point of time, be it a season or non-season point of time?

Dr. Ramesh Kancharla : Yes, matured hospitals will clock 60% plus occupancy.

Prashant Kutty: Understood. Got it. So, another point was that, again, the reference point was typically the fourth quarter of a year. It usually used to be slightly more muted compared to the second quarters and the third quarters. This quarter, however, some seems to be a bit of an aberration. You have done really good numbers and both top line as well as an EBITDA front. Should one take this as a more bit of a norm in terms of, from a reference point, or again, are there any one-off elements over here? Because as compared to our Q4 FY '21 or Q4 FY'22, I understand those had some impact of COVID and all, but because this is a quarter, where there was no such thing, is that a far more normalized quarter for us?

Dr. Ramesh Kancharla : Yes, well, I wouldn't take any quarter as a reference point. I would take an overall and a year as a reference point. The reason is year-to-year is, I can fairly talk about it because it's going to be impossible in an emergency-based hospital to talk about the consistency of quarter-on-quarter. They can keep shifting to one quarter to another quarter. But overall, in a year, we can kind of envisage to say that this is what, we can do.

As an earlier gentleman asked me a question, that's what, it says. Our growth trajectory will continue to be high teens towards the 18%, 19%. And the revenue, and we will have to kind of foresee, which quarters are going to be higher, which quarter are going to be moderated. It's almost like guessing. I'm sorry, I wouldn't like to do this. I'm a doctor.

Prashant Kutty: Sir, I get that point. But the reason for asking this question, typically, if you look at it, overall, the EBITDA numbers look to be far more superior. And obviously, it's a function of the matured hospitals doing well. But again, a point to that, is the newer hospitals, we've

Page 21 of 29 not seen too much of change in the occupancies over there. I understand that those are relatively new and will take their time. But are we seeing acceleration in that happening, where the occupancies could kind of jump up faster over there? I mean, have the paybacks for these Spokes, which you recently added, which you're going to add, have they been kind of coming off?

Dr. Ramesh Kancharla : Oh, definitely. See, what happens is in our business. For example, in Hyderabad. So, Hyderabad is kind of a matured market for us. I'm doing more and more beds in Hyderabad for demand. So, I wouldn't have any problems. For me, if I started a hospital in Hyderabad, I wouldn't worry about cash burn. It will definitely not burn the cash. So when I do a hospital in geographies such as Hyderabad, with a newer or still we're building our reputation like Chennai and those things, it may take a year, when taken Spoke to third Spoke, it'll get better. Fourth to fifth Spoke, it'll get even better. So that is overall reputation of your hospital or brand or how strong are your clinical outcomes. These all things are going to matter.

So, today we are sitting in Hyderabad. I know, your concerns. We're almost getting closer to 1,000 beds in Hyderabad by end of the year, almost 940 - 950 beds. So, I never imagined that we're going to do so many beds, but I continue to do it because there's a need and there's demand. So, we've gone to Bangalore. Yes, we're going to get to 500 beds and still opportunity is there. So, there's a lot of beds are there to build a business in those things in Bangalore. So, Chennai, we clocked very well in the Hub hospital, second years, third years with an extremely

well. That's why, we have added kind of one more Spoke and with one more Spoke is going to come this year.

So, it all depends on how we have placed ourselves to do business. What is the reputation of the hospital? What are the medical doctors whom we have? These are all the determining factors, the hospital to do well and then the payback period will reduce significantly. So, Brownfield price, naturally, you take much less for that period than Greenfield Project. So, this is how, I look at our business. So, when we have kind of such a strong business in South, obviously, Delhi, when I'm going there, places like Gurgaon, where everyone is bullish about, being a healthcare Hub or healthcare capital, definitely, I need to be there because as a pioneer in children's healthcare.

Prashant Kutty: Which means that, in the next year or so, even the newer hospitals or in the next two years, should see an increase or should see an acceleration in the occupancy numbers?

Page 22 of 29 Dr. Ramesh Kancharla : Yes, that's what I expect to do.

Prashant Kutty: Sure, definitely. And one last point sir, in terms of margins, it's been a very good year in terms of margins as well. You did highlight that, you're putting up almost about 400 beds odd between the one, which already added 150 beds and then another 250 beds, in the next year. Given, what you Spoke about in the occupancy front, should one assume that margins really shouldn't correct much compared to, what you would have earlier thought?

Dr. Ramesh Kancharla : When you have so many beds been added, there will be some degree of margin pressure will be there.

Prashant Kutty: Sure, fair point, But I'm just asking, earlier you were probably working from a 30%, 31% kind of a number. You already clocked in about 34%, so I'm just asking from that

perspective?

Dr. Ramesh Kancharla : Yes, I'm sure there will be some moderation .

R. Gowrisankar: We always say the pre -INDAS of 25% because whenever we expanded earlier also, we have seen that not clocking at that rate. So , the last two years, the bed addition was less that's why it has gone to th at 34%, 35% So we are saying that it will be 30% post -INDAS . It's doable considering that you have a bucket of good number of matured hospital and then you are adding new addition so 30% is doable.

Moderator: Thank you. The next question is from the line o f Dhavan Shah from Alfaccurate Advisors.

Please go ahead.

Dhavan Shah: Yes, thanks for the opportunity, sir. So my question is related to the cost of medical consumables I look at, the Y -o-Y business, it's been down by around

20-odd% and what is the other cost is more or less up by 25% so just wanted to understand your thoughts on this part because I think the inpatient volume growth was roughly 25 odd % so how should we calculate these two heads if you can help on this thing?

R. Gowrisankar: The consumabl e is always about 14%, 15% actually. So last year, we have done COVID vaccination, which is a less margin business compared to hospital business so that year was about 20% , now it has come to normal because of normal cy in hospital business . So, you can take, going forward in normal circumstances it will be, about 14%, 15%.

Dhavan Shah: Okay. And what about the other cost?

Page 23 of 29 R. Gowrisankar : Other cost it ranges about, 7% to 8%. It will not be more than that actually. S ee again, what is there now because of th at the professional fee is grouped separately. So obviously it's about 6%, 7% and then including the bed addition everything and it gives us an opportunity for a better operating leverage also it can come down if you don't add it will come down. Otherwise, it stands there.

Dhavan Shah: No, but if even the professional fee to the doctor is already excluded in the financial statement. But still if I do the math of 7%, 8% on the INR . 1,174 crores, it comes to around INR. 82 crores odd. So, i

t's more than that around INR . 200 crores is the other cost for the year.

R. Gowrisankar: No, doctor its 22%. You got clarification or still you want to know ?

Dhavan Shah: No. So basically, as you mentioned that the other cost should be 7% to 8% of sales excluding the profe ssional fee . So, professional fee line item is already apart from other expenditure. So, if I do the math other cost comes to around 17% odd for the FY'23 as against the 7% to 8% you are highlighting. So, is there anything I'm missing over here?

R. Gowrisa nkar: No, you are not missing anything actually. It's about, I think, INR . 202 crores. Yes, it's about 17%. I think, yes, that's right only.

Dhavan Shah: So how should we assume? Because I think the other cost for the year has been increased so any fixed c ost into this part?

R. Gowrisankar: We have added two more hospitals in this financial year late in the last quarter and there is an increase in marketing expenses as well and increase in no. of repair and maintenance. So that's where you will see compared to FY '22 to FY '23 on a full year basis, there is an increase actually.

Dhavan Shah: Okay. So, on yearly basis, I think the advertisement and the repair maintenance cost should be how much?

R. Gowrisankar: On an average, we spend about between 2% to 3% on the marketing and business promotion expenses on the top line.

Dhavan Shah: And the repair and maintenance cost?

R. Gowrisankar: So that will be about 2.5%.

Page 24 of 29 Dhavan Shah: Okay. And the second question is about the break -even timing. So would like to understand about the new hospital that we are adding roughly 400 beds odd in the next two years to three years. So, what is the normal break -even timing for new ones? And any EBITDA per bed number for the mature versus the new hospital that yo u can give? And how do you see the mix going forward in the next two years to three years? Because right now around 70% of the overall bed is from the mature one and given that we are adding 400 beds, 500 beds, so how do you see the overall mix going forwa rd?

Dr. Ramesh Kancharla : I think it will become 50% -50% because we are adding a significant number of beds this year. So, it will kind of become 50% : 50% mature and new hospital beds . I need to see that how many are going joining a mature group also, some of the hospitals may join. So because of the bed additions and those things, the maturing hospitals will little bit change to last two years, because of the COVID year, we have not expanded much. And last year, 150 beds were added. So therefore, we don't see much of in the maturing beds. I think this year will change significantly to the maturing hospitals number.

Saurabh Bhandari : Only one hospital. Only Madhukar Rainbow Children's Hospital will move.

Dr. Ramesh Kancharla : Only one hospital. So, there will be more on the new hospital's growth. In terms of, you asked other thing is about break-even points and those things. Hyderabad, we do kind of break-even in the first year itself. It's not a problem for us. In the Chennai and Bangalore will take one and half year, 12 months to 18 months' time. The break-even depends on the location, size of the hospital and those things. That's how we look at it, our break-evens.

Dhavan Shah: And the EBITDA per bed for a mature versus new hospital?

Saurabh Bhandari : No, we don't calculate the EBITDA per bed level.

Moderator: Thank you. The next question is from the line of Aditya Khandelwal from SIMPL. Please go ahead.

Aditya Khandelwal: Yes, hi sir. Thanks for the opportunity. Sir, in your matured assets, if we look at the occupancy in the Hyderabad cluster, we were at around 65% based on the DRHP. But for Bangalore or Chennai, we were at around 50%. So just wanted to know has the occupancy ratio for Bangalore and Chennai improved?

Dr. Ramesh Kancharla : They have improved overall,

and we don't do actually the city-wise cluster, we look at the matured and the maturing hospitals. So, hospital to hospital, we don't really do it city to city, it becomes too complex for us to do. Therefore, they have definitely increased

Page 25 of 29 otherwise we wouldn't have clocked about 67% occupancy. (in Q4 FY24 for matured hospitals)

Aditya Khandelwal: Right. And in our matured hospitals, you said our occupancy is around 60% to 65%. So just wanted to understand if the future revenue growth for the company would come only from setting up new centres, or there is scope for growth to come from matured centres as well?

Dr. Ramesh Kancharla : No, these come from both, because matured hospitals will continue to grow in terms of the occupancy, price mix, case mix, definitely, they will continue to grow what our experience is that we continue to grow in the matured hospitals also in terms of revenue.

Aditya Khandelwal: Okay. And as per my calculation, our inpatient volume for our new centres which was in the range of 3,800 to 3,900 in the last two quarters, has increased to 4,300 to 4,400 this quarter. So, has this increase majorly come from our new centres open in Chennai and Hyderabad or our existing centres have also seen an increase?

Dr. Ramesh Kancharla : No, across all the new centres. Because of the viral epidemic it is global actually, it's not about city to city, we have seen across the country and across the globe we have seen these viral infections in children.

Aditya Khandelwal: Right. Okay. And our inpatient realization, increase will primarily come from two factors, inflation, and patient mix. We just wanted to understand what kind of increase can we expect from both these factors going forward?

R. Gowrisankar: The price increase, we are saying on an average, our ARPOB will increase by about 7% to 8% year-on-year. So, you can take about, so put together 7% to 8%. This includes the case mix and inflation.

Moderator: Thank you. The next question is from the line of Aneesh Deora from Nomura. Please go ahead.

Aneesh Deora: Yes, thanks. So for the Gurgaon facility, it's a completely, it's a newer geography than South India, where you're currently dominant. So, do you foresee any change in the doctor engagement model that you would have for the Gurgaon facility? So, in South India, you have the doctor at a full-time basis, 24x7 availability kind of a thing. So do you think, that would be possible in the Gurgaon region or are you looking at any other doctor engagement model for that geography?

Page 26 of 29 Dr. Ramesh Kancharla : Children's hospitals, if you want to drive across the world, it is an institutional model. What I've told are the fundamentals. So, you may actually have a different engagement model, but it has to be a full-time commitment doctor. So that is a gold standard for children's hospitals. We would work towards that. Maybe price points may be different from doctor's payment expectations. But the children's hospital demands full-time doctors to deliver quality clinical results.

Aneesh Deora: Understood. So probably you are indicating that the doctor's payments in the Gurgaon region could be higher than what the average would be, in South India currently?

Dr. Ramesh Kancharla : Yes, your price points will be higher. Your doctor's costs will be higher. We need to be kind of clear on that.

Moderator: Thank you. The next question is from the line of Kartik Narayan from SCP India Advisors. Please go ahead.

Kartik Narayan: Firstly, congratulations not just on the numbers, but also the clinical results. It's fantastic. Most of my questions were answered. I have a couple of follow-ups. One is with respect to the 48% growth that you've seen this quarter. If you had to break that down into price increase and volume increase, how much of that came from prices, either through insurance or cash price

ice increases, and how much from volume, that would be helpful.

Dr. Ramesh Kancharla : I've done the calculations. I've definitely, it is a more of the volume driven as well as the kind of the case mix. First, I can broadly tell you that, we've done a lot of intensive care work and a lot of sick children. So, more of volume and also the case mix than price.

Kartik Narayan: Understood. And if I were to look at the full year, you mentioned earlier that, it's better to look at the full year, the 20% year-on-year growth. Would you say it's similar? Is it mostly related to volume versus price?

Dr. Ramesh Kancharla : Yes, we have always delivered. In the pre-IPO, the last 10 years' time, we've delivered it. I hope to do that. So, yes, of course, we have done it. Still, we've always been saying there's a high teen to 20. So, the growth, I don't think that there will be any problem for that.

Saurabh Bhandari : Just to add, if you look at except COVID, our ARPOB has increased on an annual basis by 4%. If you break down the 20% growth, 4% has come from tariff and case mix and the rest has come from volume mix.

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Kartik Narayan: Understood. That's helpful. And one question related to the full year numbers. So, if I look at FY '22 versus FY '23, rather a strong 20% year-on-year growth. But the doctor costs have, the professional fees to doctors was around 34%. So, has there been any change in terms of the way we have engaging with the doctors in the past year?

Dr. Ramesh Kancharla : I think because of a lot of new doctors being added to the pool, new doctors and new centers have come because our doctor engagement model is full-time. So, when you start a new center, our doctor costs go significantly higher. What we always see is that doctor costs should be between 22% to 24%. Sometimes, it can go up to 25% also.

Kartik Narayan: Understood. So, 25% is perhaps the benchmark that you aspire to maintain.

Dr. Ramesh Kancharla : I would get worried, if it is beyond that.

Kartik Narayan: Understood. And no, I think that's all I had. Again, congratulations on the results for clinical and financial.

Dr. Ramesh Kancharla : Thank you.

Moderator: Thank you. The next question is from the line of Yogesh Tiwari from Arihant Capital. Please go ahead.

Yogesh Tiwari: Thank you, sir, for taking my question. I had certain questions on the balance sheet. So, last year we had this goodwill of about INR . 30 million, which we do not have now. So, if you can share some details on the same.

R. Gowrisankar: On the consol, the goodwill was created long back, when we merged one of our subsidiary way back in 2014 - 2015. So that calls for the writing of goodwill. So, we have taken it and written it off. It's not a very material amount. It's about INR2 crores .

Yogesh Tiwari: So, sir, going forward, there will not be, we can assume, there will be no, nothing in terms of goodwill going forward?

R. Gowrisankar: No, there is nothing. No intangible declared goodwill in the books.

Yogesh Tiwari: Sure, sir. And sir, other thing on the intangible. So, we have some intangible assets jumped from INR. 1 crores to INR . 4 crores. And there are some under development from INR. 1 crores to INR . 2 crores. So, how do we look at this? Like what would be under development and other intangible assets? And the jump in the same?

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R. Gowrisankar: We are implementing new HIS. So, it's all IT related. Some of the software, what we are implementing. And those are all, we are kind of upgrading our HIS, after seven years, So, that related to that we bought some of the assets actually.

Yogesh Tiwari: Sure, sir. So, just for the modeling purpose, would it remain in the same range for FY '24?

R. Gowrisankar: So, in FY '24, we may incur another about, INR . 2 -3 crores towards this actually. To complete that entire implementation and

the another the new software, whatever we do, the BI tool, all those things. Another INR . 2 -3 crores will be spent on this.

Yogesh Tiwari: So, like INR. 2 crores, INR . 3 crores, some more under asset, intangible asset under development, it might include, is correct?

R. Gowrisankar: Yes, current FY, you will see the addition of INR . 2 -3 crores.

Yogesh Tiwari: Okay. Thank you very much. Sir, just last one question. On the other financial assets, it has increased by about INR. 55 crores to INR. 234 crores. So, if you can share the spike in the other financial assets?

R. Gowrisankar: No, it's actually the fixed deposits, what we have. So, it's based on the reclassification we have done. So, as per that, any deposit, which has a maturity of more than 12 months has to go to the other financial asset.

Moderator: Thank you. As no further questions, I would now like to hand the conference over to management for closing comments.

Dr. Ramesh Kancharla: So, thank you very much to all the analyst community for patiently listening, and we continue to engage. Of course, if there are any questions, you may reach out to our investor relations team. So, thank you very much, CDR and all the analysts and investors.

Thank you.

R Gowrisankar : Thank you very much.

Saurabh Bhandari : Thank you.

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Moderator: Thank you. Ladies and gentlemen, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy. Please also note that this document has been edited without changing much of the content, for enhancing the clarity of the discussion. No unpublished price sensitive information was shared/discussed on the call.

Call: Aug 2023

August 16, 2023

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051. Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001. Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q1 FY24.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on August 8, 2023, with regard to Financial Results of the Company for Q1 FY24.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

<https://d2sn5i18drfi94.cloudfront.net/investor-relations/reports/Analyst-Concall-Transcript-Q1-FY24.pdf>

We request you to kindly take the same on record.

Thanking You,

Yours faithfully,

For Rainbow Children's Medicare Limited

Ashish Kapil
Company Secretary and Compliance Officer
Encl.: As above.
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"Rainbow Children's Medicare Limited
Q1 FY2024 Earnings Conference Call"

August 08, 2023

MANAGEMENT : D R. RAMESH KANCHARLA – CHAIRMAN AND MANAGING
DIRECTOR – RAINBOW CHILDREN 'S MEDICARE LIMITED
MR. SANJEEV SUKUMARAN – GROUP CHIEF OPERATING
OFFICER - RAINBOW CHILDREN 'S MEDICARE LIMITED
MR. VIKAS MAHESHWARI - CHIEF FINANCIAL OFFICER -
RAINBOW CHILDREN 'S MEDICARE LIMITED
MR. SAURABH BHANDARI – GROUP BUSINESS ANALYST -
RAINBOW CHILDREN 'S MEDICARE LIMITED

Rainbow Children's Medicare Limited
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Page 2 of 16 Moderator: Ladies and gentlemen, good day and welcome to Rainbow Children's Medicare Limited Q1 FY2024 earnings conference call hosted by IIF L Securities Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then 0" on your touchtone phone. Please note that this conference is being recorded. I would now like to hand the conference over to Mr. Rahul Jeewani from IIFL Securities Ltd. Thank you and over to you Sir!

Rahul Jeewani : Good afternoon, everyone. I am Rahul from IIFL Institutional Equities. I welcome you all to the quarter one morning conference call of Rainbow Children's Hospitals. From Rainbow, we have with us today Dr. Ramesh Kancharla – Chairman and Managing Director, Mr. Sanjeev Sukumaran, Group Chief Operating Officer, Mr. Vikas Maheshwari, Chief Financial Officer, and Mr. Saurabh Bhandari – Group Business Analyst. Over to you sir for your opening comments.

Dr. Ramesh Kancharla: Thank you Rahul. Good afternoon ladies and gentlemen. This quarter has been marked by significant achievements and growth reflecting our commitment to excellence in paediatric healthcare and strategic expansion. Firstly, I would like to tell with immense pride, as Rainbow Children's Hospital, our hub hospital in Hyderabad was awarded prestigious Gold Seal of Quality approval by the Joint Commission International (JCI). This recognition underscores our dedication to delivering world class healthcare services to children and stands as a testimony to our relentless pursuit of quality. This makes us the only paediatric hospital in India to receive this accreditation.

Our financial performance for the quarter under fiscal year highlights our continued growth trajectory despite the seasonally weak nature of the Q1. We have witnessed a commendable 21% increase in the revenue driven by higher delivery numbers, paediatric surgical cases and the addition of 155 beds in our 2 units. Our commitment to enhancing patient care has led to these impressive numbers with the r

evenue for Q1 FY2024 growing by 21% to Rs.287.1 Crores. Our EBITDA has grown by 7% to Rs.87.74 Crores and that PAT has also seen 7% increase reaching Rs.41.5 Crores. Well the overall occupancy rate for the quarter was approximately 41%. It is worth noting that this figure reflects the growth phase of our recently commissioned hospitals as we continue to expand our footprint, we are confident that thes e new units will contribu te significantly to our future growth.

Expansion Plan:

■ Our expansion plan for the current fiscal is clear: with a focus on adding 270 beds across various locations. We are about to commence a new outpatient block with a modern IVF facility adjoining the LB Nagar spoke hospital in Hyderabad city. The expansion in Hyderabad central city, Himaya t Nagar with 60 beds and an additional Rainbow Children's Medicare Limited

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Page 3 of 16 block in Hydernagar with 50 beds in addition to the existing 78 beds are expected to be operational by second-half of the financial year.

■ Our new hospital in Anna Nagar, Chennai of ~80 beds and Sarjapura, Bengaluru ~80 beds is also expected to commence operati ons in the fourth quarter of the current financial year.

■ Other spoke hospital in Hennur, Bengaluru is expected to commence operations in about 15 months' time, work is in progress.

■ The construction of Regional Green Field Spoke Hospital in Rajahmundry in Andhra Pradesh of 100 beds is progressing well, is currently at the basement stage.

■ We have completed our 100% payment to HSVP (Haryana Shahri Vikas Pradhikaran), for 2 land parcels in Gurugram and received the allotment letter. We are in the process of taking over the possession of the land.

■ We recently signed an asset light 125 bed Gr eenfield hospital at a strategic location at Avinashi Road in Coimbatore.

When it comes to clinical excellence, we normal ly do a quite significant number of long-distance transports. Last year we did about 1000 plus longer distant transports and off late we are witnessing significant number of calls coming to us for the air transports. We have transported in

this quarter, the airlift to typically children and newborns from Goa, Raipur, Nagpur and Vishakhapatnam to our hub hospitals. The annual Paediatric Cardiac Surgical Results, has been on par with international standards with mortality of 1.5%, which includes 40% of the complex cardiac surgeries. Our focus remains offering high standards of multidisciplinary paediatric health specialty care with comprehensive services.

I also take this opportunity to extend a warm welcome to Mr. Vikas Maheshwari, our Chief Financial Officer who brings over 25 years of valuable experience in the Corporate Finance, MMA, Strategy, Treasury Management, his expertise will undoubtedly contribute to our financial growth and strategic initiatives. With these remarks, I would now like to hand over floor to Mr. Vikas Maheshwari who will provide us with a detailed financial update.

Thank you once again for joining us today. We look forward to your questions and insights as we move forward. Thank you.

Vikas Maheshwari : Thank you sir. Ladies and gentlemen, good afternoon to each one of you. It is indeed a pleasure to address you today and provide an overview of Rainbow Hospitals financial performance for the first quarter of FY2024. I am Vikas Maheshwari and I recently joined as a Chief Financial Officer. I am excited to be part of the Rainbow Hospital and contribute to our continued success. Let me now delve into our financial performance of the quarter one of FY2024.

■ Our revenue for the first quarter stood at Rs.287.2 Crores reflecting an impressive growth of 21% when compared to the corresponding quarter of the previous financial year. Rainbow Children's Medicare Limited

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Page 4 of 16 ■ Our EBITDA for quarter first FY2024 amounted to Rs.87.7 Crores, marking a 7% growth compared to the same

period last year. However, the EBITA margin for the current quarter was 30.5%, which is slightly lower than the 34.6% in the corresponding quarter of the previous year. This marginal impact on the EBITDA can be attributed to below key points.

- o As CMD, Dr. Ramesh has spoken earlier that we have opened two units recently which is financial district, Hyderabad and at OMR, Chennai, so that has impacted the margin slightly.

- o Other things which has impacted the EBITDA is the onboarding of the senior leadership at both the corporate and the unit levels coupled with the impact of RSU cost.

- o We have also increased our expense for the branding and the promotional expenses, which is aimed at supporting our new units and the future growth of the company

- o We have also carried out few necessary repairs and maintenance across our hospitals to ensure that they remain ready for the upcoming seasons.

■ The profit after tax for the first quarter FY2024 is Rs.41.5 Crores indicating a 7% growth in comparison to the corresponding quarter of the last financial year.

■ In terms of the operational performance, both outpatient and inpatient volumes witnessed commendable growth with increase of 11.6 and 13.9% respectively when compared to the corresponding period in FY2022.

■ The overall occupancy rate for the quarter stood at 40.9%, while mature hospitals exhibited a robust occupancy rate of 47.6%. Our newly commissioned hospitals recorded an occupancy rate of 28.1% during quarter first FY2024. Newly commissioned hospitals occupancies lower rate has been impacted by hospitals which we have opened recently. We expect as these hospitals grow and walk towards maturity, the occupancy level of the new hospitals will also reflect as we have reflecting in the matured hospitals.

■ Our return on capital employed and return on equity for the first quarter FY2024 was 7.2% and 3.8% respectively. These are non annualised numbers.

■ Our pair mix remains very robust and was very balanced with 48% of the revenue coming from the insurance and the balance 52% coming from the cash patients.

■ Furthermore, delighted to note that our international business now constitutes 3% of our total business compared to the earlier figure of 2%.

■ I am pleased to inform that our company's balance sheet remains robust with a substantial cash and cash equivalent position of Rs.474.2 Crores as of June 30th of this year. This strong financial position will be instrumental in supporting our ongoing

capital expenditure plan as outlined by Chairman and Managing Director. Given our current cash flow and anticipated internal accruals in the coming years, we remain confident in our ability to complete all capital expenditures through internal accruals.

■ During the quarter, the company has invested approximately Rs.187 Crores in the capital expenditure out of which the majority of the amount has gone towards the purchase of the land in Gurugram. (which is roughly Rs. 142 Crores)

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Page 5 of 16 With these insights, I conclude my financial update. I now invite your questions and

suggestions. Thank you.

Moderator : Thank you very much. Ladies and gentlemen, we will now begin the question and answer session. We take the question from the line of Nitesh Dutt from Burman Capital. Please go ahead.

Nitesh Dutt : Hi, thanks for opportunity. My first question is what is the impact on EBITDA due to the 58 beds operationalized this quarter and also the cost that we must be incurring for the new hospitals that are going to come in?

Vikas Maheshwari: Can you please repeat this question again. There was some disturbance in between.

Nitesh Dutt : I was saying what is the impact on EBITDA due to the 58 beds which were operationalized this quarter and also the new hospitals which will be coming in the second-half of this financial year.

Vikas Maheshwari: So as far as the new units which

we have opened, delighted to tell you that the Financial District, Hyderabad, has done very well and it is EBITDA positive. Since it is in Hyderabad and built for existing demand, in three months itself it has become EBITDA positive. Obviously as the new units get open till time they reach the maturity, there will be a some impact on the EBITDA, but we remain confident that in the next one year to 18 months' time they will be reaching to 20%, 25% EBITDA. So quantifying based on the new unit which is coming up in operational what will be the impact on the EBITDA will be slightly difficult, but obviously there will be some impact on the EBITDA, the quantification of that is little difficult, but we remain confident whatever EBITDA we have shown in this quarter for the two units has got operational on year on year basis. we remain confident that the trajectory of the EBITDA should be the same.

Nitesh Dutt : Understood but if we exclude 58 beds which were operationalized, what could have been our EBITDA, so basically considering only the existing beds?

Vikas Maheshwari: So, it should have been better Mohit. We do not quantify and give these figures separately, but delighted to again tell that both the units which we have operationalized which is OMR, Chennai and Financial District, Hyderabad combined together, both are EBITDA positive though at the very nominal level, but it will further grow.

Nitesh Dutt : Alright, thanks. Second question is what is the reason for fall in occupancy of new hospitals. So even if we exclude the capacity addition, basically the new hospital, there seems to be a fall in occupancy.

Dr. Ramesh Kancharla: The new hospitals will take its time to build up. We just came out of Q1. In Q1, even the most matured hospitals also would not cross more than 50% that is because that we are children's hospital. It is the seasonally weaker quarter as it is a holiday season. Most of the families tend to go out of the cities and also this is literally a healthy period that is why we see a kind of a Rainbow Children's Medicare Limited

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Page 6 of 16 significant drop in the occupancy levels. Traditionally, it has always been like that for Q1. What actually happens in Q1 is that we do a lot of surgical volumes.

Nitesh Dutt : Occupancy that you are expecting from mature hospitals for the full year?

Dr. Ramesh Kancharla: See we clocked about 60% plus in the last year in the matured hospitals and the maturing hospitals were about ~45% occupancies last year. We have about 270 beds coming in this year for operations, that is almost like ~30% of the current capacity beds. Once they get added, even with that we probably kind of a look at that annualized occupancy probably about 50%. Matured hospitals should be closer to 60% occupancy for full year

Nitesh Dutt : Sure that is helpful. I have a question on the Madhukar consolidation also. So if we exclude Madhukar revenue this quarter what could have been our value by revenue growth and our ARPOB for the quarter?

Vikas Maheshwari: The revenue contribution is roughly Rs.50 Crores from the Madhukar, where we provide the medical services and OP services which we run. So that revenue remained or more or less flat with the marginal impact not much of the impact.

Nitesh Dutt : But in the corresponding quarter of previous years, this revenue, the entire revenue must not have been recognized, right only the some percentage of that must have been recognized.

Vikas Maheshwari: So, that remains same Mohit. There is no change in the arrangement. So that arrangement continues to be same. There is no change in the data and the figures are comparable versus last year.

Nitesh Dutt : Understood and just last question on our expenses side. There have been steep increases in employee expenses both Y-o-Y and Q-o-Q. So just want to understand the reason for this and is it normalised going forward, what is the normalised expense go

ing forward and similarly for operations and maintenance expenses these are also up by 120 bps Y-o-Y. So for these also are there any one off this quarter or will these kind of expenses continue?

Dr. Ramesh Kancharla : Mr. Sanjeev will answer this question to you. Thanks.

Sanjeev Sukumaran : Thank you very much for your question. As sir mentioned there are some newer hospitals that have onboarded, which means that we employ more human resources. So, an additional expense that you see are definitely because of that. The repair and maintenance had to go up usually as an investment from the organization to continue to improve the infrastructure, because Q1 seasonally a low quarter, we have enough time in our hands to actually invest in improving our

infrastructure expenses. So those are largely why you see these line items showing up. Also, the on boarding of doctors in these new hospitals as well as in our existing facilities is also a contribution to the employee and doctor cost.

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Page 7 of 16 Nitesh Dutt : Got it but as percentage of revenue, will it continue to be elevated in the upcoming quarters because we are adding new capacity as well?

Vikas Maheshwari: Mohit, as we are heading towards the busy season which is September and December quarter, we expect the staff cost or the employee costs which we have shown remain more or less same and as the revenue grows up as a percentage, these costs will get moderated.

Nitesh Dutt : Understood. Thank you. I will get back in the queue.

Vikas Maheshwari: And also Mohit, I will encourage you to go through our PPT which is the page #18 which shows the breakup of our mature hospital and the new hospital on the occupancy basis. On the new hospital though the percentage of the occupancy has come down but as a number of the beds

occupancy it has still remained the higher than the last year. So please just go through with that

and if after that any clarification is required , we are available after this conference call.

Nitesh Dutt : Sure. Thank you.

Moderator : Thank you. The next question is from the line of Alankar Garude from Kotak Securities. Please go ahead.

Alankar Garude : Hi. Good afternoon everyone. So what was our paediatric maternity revenue mix in this quarter?

Dr. Ramesh Kancharla: At least a 33% is obstetrics revenue.

Alankar Garude : Okay. So I mean just wanted to understand if apart from the higher surgical mix which you mentioned in this quarter and as well as the higher maternity mix, was there any other reason which show the strong sequential growth in our ARPOB in the newer hospitals?

Dr. Ramesh Kancharla: I think what has happened is Alankar is that routine paediatrics is down significantly, and neonatal business is also down, hence the overall occupancy is low. Both obstetrics and gynecology surgeries as well as the paediatrics surgery are extremely short ALOS. So overall our ALOS have come down to 2.5 days and this the lowest ALOS that we have seen. Also, for most of these surgical cases length of stay is less, maximum they stay either one to one and a half

days. So that push up the ARPOB higher and that is what we have witnessed. There were not many long stay patients.

Paediatric is one which is a kind of the low average revenue per day that

is completely not there in this season because of the people moving out and holidays and also season is not the one which to get admitted . So these are the things which kind of play out to

increase ARPOB. We have done some transplant work and those things, cardiac surgeries happened. These are all the combination of those things actually made our ARPOB higher.

Alankar Garude : Understood, Sir. And my second question is on Madhukar, I think Vikas just mentioned about Rs.50 Crores of number which was broadly flat on the year on year basis. So just from a broader perspective, how is the hospital doing, I mean is it now in dou

ble digit EBITDA margin at the

overall level, consol level and how do you see the traction going forward?

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Page 8 of 16 Vikas Maheshwari: So, you are talking about this Madhukar revenue growth, right?

Alankar Garude : Yes, Yes, I mean not just the revenues which we recognize, but on an overall basis as well in general how the hospital is doing broad sense on the margins trajectory.

Dr. Ramesh Kancharla : Yes, Alankar I will take this now. Last year we have done decent growth at Madhukar and what we are witnessing now the case mix is changing the surgical component is increasing, ICU started doing very well. We do highest number of deliveries in entire South Delhi. I think it is

kind of looking much, much better. Now, what is important, is how do we actually take it further

to build (pediatric) specialties. That is something we are working on that.

There the international business, Delhi attracts more international business . We are focusing to

build a lot more international business because we have a long term plan for Gurugram. The

major initiative which we have taken now, in the current year is that we will invest and also do outreach clinics in East Africa and also in their neighborhood countries and also some of the

Russian countries so this is something which we are trying to build and have started doing it. I think some early results are coming up. We expect this year international business should be 4%

of overall business. so we are looking at very positively to build more business from outside - the longer distances and also the neighboring countries.

Alankar Garude : That's helpful sir. I have more. I will come back in the queue. Thank you.

Dr. Ramesh Kancharla : Thank you.

Moderator : Thank you. The next question is from the line of Banshi Desai from JP Morgan. Please go ahead.

Banshi Desai : Thank you. Sir my question is on our expansion plans. we have added Coimbatore to be added in

FY2025, so can just share more about this region, what is the potential, what are your plans in

terms of longer term expansion, do you plan to do a hub and spoke here and why do we do greenfield again in this particular asset?

Dr. Ramesh Kancharla : Okay. So Banshi we just kind of a signed up for the Greenfield in Coimbatore. We have seen

enough buildings for the brownfield project. We could not really find a suitable building at a right location, because location is very important in Coimbatore. Avinashi Road is one area which is very popular and accessible to entire Coimbatore and also for people coming from outside. When it comes to Coimbatore it's a big medical hub. Among tier 2 cities, probably is one of the most advanced multispecialty space. So therefore there is a space and we have evaluated it. There is only one GKNM and one trust hospital in Coimbatore. I think the Coimbatore is probably one of the important healthcare destinations for all the part of affordable class of people in that region. So we are pretty convinced about Coimbatore as a city to go in.

Vikas Maheshwari: Also Bansi to add what sir has just told if we look at whole Tamil Nadu, it is the second largest populated city in whole Tamil Nadu. So after the Chennai, this is the second largest city and Rainbow Children's Medicare Limited

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Page 9 of 16 medically, obviously the requirement for the dedicated mother and child is needed, so it makes lot of strategic sense also to be present there.

Bansi Desai : Okay, what was the investment here , or should it be in line with what you have said in the past?

Dr. Ramesh Kancharla : Yes, I think the capex is probably going to be about ~70 lakhs per bed that's what we're looking at. So that's probably outer limit that is what I expect, considering the specialties what we are going to build at this hospital. So, the outer limit is about 65 to 70 lakhs.

Bansi Desai : Ok in terms of ramp up therefore should

do one assume Coimbatore will also have the ramp in line with what we have seen in Chennai?

Dr. Ramesh Kancharla : I think I am pretty optimistic about Coimbatore because no. one, there is no shortage of doctors availability No. 2, there is no shortage of specialist doctors and No. 3 there is need and necessity for a pediatric hospital. Further, I know quite a number of people in Coimbatore because Coimbatore is very linked to Andhra. Most of business people in Coimbatore are actually Andhra people. When I talk to people about the need for a pediatric hospital they are pretty optimistic about the need of a children hospitals, and also the peripheries whether it is Tiruppur, Erode, Palghat it is a huge area and they are very affordable.

Bansi Desai : This is helpful Sir. Thank you.

Dr. Ramesh Kancharla : Thanks.

Moderator : Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai : Hi, thank you for the opportunity. My first question earlier you used to give revenue split between mature and new units. I could not see this time, so can you specify that?

Saurabh Bhandari: Damayanti it is basically a derivative figure only. We have given the number of occupied beds, we have also given the occupancy and ARPOB. if you were multiplying this, you will get the revenue and split between matured and new hospitals. This is available on slide number 18, Damayanti.

Damayanti Kerai : Yes. And my second question is ARPOB for 14% growth year on year. Earlier you like indicated 5 to 6% annual growth year, so can you just split this 14% interest drivers like what is the major contributors etc., and related question is in the new units we have seen very strong 30% year on year increase in ARPOB. If you can explain that also.

Vikas Maheshwari : So Damayanti let me take this. So ARPOB increase on the consol basis 14% - is function of what is the ALOS. So if you look at the ALOS has come down by 7%. So the balance explains the ARPP growth which basically is case mix, complex mix which we have taken. So that is the ARPOB increase.

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Page 10 of 16 As far as your second question in terms of new units, ARPOB increase, obviously this new units since they are making the trajectory towards the mature we have more flexibility towards the price increase. Second the more complex works come to the hospital as the hospital keeps increasing their journey to the maturing when name and fame of the hospital, doctors is spread over and you get a more complex work so that also explain partially about the ARPOB increase though the 16% ALOS adjustment is there, but 14% type you can say is the complex work plus the price change which we have undertaken.

Damayanti Kerai : Price is also a major contributor here for new units, right?

Vikas Maheshwari : Sorry Damayanti?

Damayanti Kerai : You know I am saying for new units price hike is also a major contributor here.

Vikas Maheshwari : Yes, so the price hike plus the complexity of work which we do there is a combination of both, right. So as the new units march towards the maturity the name and doctors may etc., it just spreads over. So initially whenever you open the hospital you will get the basic type of work. As the hospital matures the more complex work keeps coming. So that is what is explained. So 16% is the ALOS change and the balance is the case mixed plus your price change.

Damayanti Kerai : Okay, so for annual consol basis, how should we look at ARPOB now. Earlier you said somewhere mid-single digit growth looks sustainable. So are you going to revive this up?

Vikas Maheshwari : So if the ALOS remains at the same level which we have shown you should assume it is fair to see that it should be the 5% to 6% growth of the ARPOB.

Damayanti Kerai : Okay.

Vikas Maheshwari: This is the very reasonable level of ARPOB right? Yes, we should be there.
Damayanti Kerai

: My last question is one of your multispecialty peer who is based Andhra, Telangana spoke about stepping up their effort in mother and child care segment. So do you think in your core market you will be seeing higher competition from such players like who are stepping up efforts there?

Dr. Ramesh Kancharla: No, we always had some competition, but competition made us better actually over a period of time. Of course, now with awareness more people are trying to start children's hospitals, few chains are also coming up. We will have to see how it is going to be. I am not too worried about it because we are not a mother and baby or mother and child hospital, we are a paediatric multispecialty, our segment is very different. We dominantly offer paediatric to sick kids and more intensive care, more specialty game. We also do other things (quaternary care) and that will definitely protect us.

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Page 11 of 16 Earlier some children hospitals were started they become our referral centers, maybe I think we will have to see that how to convert them as a kind of referral centers for tertiary and quaternary care.

I think any growth and development in healthcare, we should welcome them, expect a positive competition. The pie is big and definitely for people as when they are disciplined, the healthy competition is good for healthcare.

Damayanti Kerai : You are not much worried on competition which might come on the talent side right. If there is more demand for paediatric specialist etc., you remain confident about retaining your best doctor or how do you see that?

Dr. Ramesh Kancharla: On doctors, the thing is that the doctors who work with us, they are on a very different engagement model. Their clinical aspirations are very different, so most of them, the specialty doctors, would not go and drive the mother and child hospitals. It is the newborn specialist, the ones who actually go, but there is enough supply of newborn specialist now a days. It is not a problem (to recruit a newborn specialist).

None of these mother and child hospitals can actually build up pediatric intensive care because this model is lot more complex and intensive care takes a long time to build. The mother and child is very simple model, which includes basic paediatric care, baby care, and maternity care, so it is a very simple model and easily replicable. But Rainbow is not about mother and child, it is a multispecialty paediatric hospital and also offers comprehensive perinatal care, so we have a very different play in that space.

Damayanti Kerai : Okay sir. Thank you for your response. I will get back in the queue.

Moderator : Thank you. The next question is from the line of Rahul Jeewani from IIFL Securities Ltd. Please go ahead.

Rahul Jeewani : Hi sir. Sir this 14% ARPOB growth which we saw during the quarter. Can you quantify what would have been the contribution of insurance in the Hyderabad market if that dictate during the quarter?

Vikas Maheshwari: Hi Rahul. So the 14% ARPOB growth, some adjustment you have to do for the ALOS which is lower by 7%, so that impact of ALOS, once that is adjusted, the balance 7-8% you can assume through tariff; off which roughly 2 % to 3% from the insurance.

Rahul Jeewani : Okay. So out of the 7%, 2-3% was insurance price division and rest would have been cash increases or better case mix.

Vikas Maheshwari: Yes.

Rahul Jeewani : But sir, the better case mix was reflected in ALOS, which is driving your lower ALOS.

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Page 12 of 16 Vikas Maheshwari Yes. So, some price hike which we have taken from the first April this year, so that also contributing towards that.

Rahul Jeewani : Okay and what was the quantum of price increase on the cash business?

Vikas Maheshwari : Between various services there has been changes, on an average it would be roughly 5%.

Rahul Jeewani

ani : Ok, sure and sir on these two hospitals which you commissioned, you said that financial district Hyderabad was able to achieve EBITDA break even within a three month period so if you can quantify a similar timeline for OMR Chennai?

Vikas Maheshwari : OMR Chennai as per the trajectory, this coming September quarter or the December quarter it should be EBITDA positive.

Rahul Jeewani : Okay, so then Chennai would have taken almost 12 to 15 month's period to achieve EBITDA breakeven.

Vikas Maheshwari : This hospital at OMR, Chennai started in September last year is currently very marginal negative EBITDA. So, by September or by December when the seasons picks up, we remain very confident that it will be EBITDA positive.

Dr. Ramesh Kancharla: What happens in Hyderabad is different. We do the beds for the demand. We are opening two more units now, which will add another 120 beds in Hyderabad in second half of this financial

year. 100 beds came up 3 to 4 months ago (Financial District, Hyderabad). This all totals up to 220 beds, that have come up. All these beds are for demand and future needs of the Rainbow, so when we add beds for demand, your EBITDA is still positive. If you have got an established clientele already, your doctor pool is also kind of get redistributed between the facilities, it is much easier gain for us. When you go to any other city, it is where we are doing a spoke to kind of cover the vital part of the city. We will see definitely based on the spoke, it will take 12 to 18 months' time. That is the kind of general guidance we give that the spoke in Bengaluru or Chennai will take about 12 to 18 months' time.

Rahul Jeewani : Sure sir and sir given that we are adding 270 incremental beds this year, out of which roughly 60% would be outside of the core Hyderabad market so if we will look at your overall EBITDA margin profile playing out this year?

Dr. Ramesh Kancharla: There will be some pressure on EBITDA, but I don't think we need to appeal to headwinds because one unit is coming Bengaluru, one is coming in Chennai. I would not see the huge challenge in Bengaluru and Chennai, Also both these hospitals are going to come towards the end of the year. So overall for the year's trajectory, I would not see a kind of a major challenges. Of course, adding beds definitely moderates the occupancies number one, second it will definitely have some impact in the EBITDA but I do not think it will be a very significant.

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Page 13 of 16 Vikas Maheshwari: The quantification of that is little difficult, Rahul. So there will be some impact, but how much that we have to see as we progress.

Rahul Jeewani : Sure sir and one last question from me before I join back the queue, so Vikas, can we just reconfirm the net cash and the capex numbers which you called out, so I think you said 470 Crores of net cash on balance sheet so in that after the 190 Crores of capex which we did during the quarter?

Vikas Maheshwari: That is correct, so as on 30th June we had the cash balance of Rs.474 Crores that is correct understanding.

Rahul Jeewani : And this is after accounting for 187 Crores of capex division?

Vikas Maheshwari: Correct.

Rahul Jeewani : Then the cash generation seems to be a very high number because end of last year, our net cash position was somewhere around Rs.390 Crores and despite this incremental capex our cash on book has gone up, so can you comment in terms of what led to this strong cash generation in the first quarter?

Vikas Maheshwari: This 474 Rahul is inclusive of the IPO fund which is ~130 Crores. You are counting that along with this.?

Rahul Jeewani : Okay sure, got it. Thank you.

Vikas Maheshwari: Net of that will be roughly 354 Crores which will be internal cash accruals and 120 is the IPO funds.

Rahul Jeewani Sure thank you.

Moderator : Thank you. The next question is from the line of Harsh Bhatia

from Bandhan MFI. Please go ahead.

Harsh Bhatia : Quick clarifications. One in terms of the price hike you mentioned 2% to 3% price hike in the insurance part and 5 to 6% in the cash business. Is it the right number?

Vikas Maheshwari : No, so what we have told is the 5% is the cash increase which has happened in the tariff which we have taken on a blended basis on an average this is the increase. The tariff increase for the insurance was for the Hyderabad cluster only and the impact was on the revenue was roughly 2% so that is what we were telling.

Harsh Bhatia : Okay pointing to one of the previous comments made during the call with the 110 beds that are already added and the 270 beds that will be added did we mean that we should be building in a

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Page 14 of 16 blended occupancy of 50% for the entire, that is what I could understand from the previous comments?

Dr. Ramesh Kancharla: Yes, that is what we are aiming at kind of annual average bed occupancy for this year probably kind of aiming at about 50% including our new units.

Harsh Bhatia : Okay sir. Alright Thank you.

Moderator : Thank you. The next question is from the line of Bino Pathiparampil from Elara Capital, please go ahead.

Bino Pathiparampil : Just few clarifications. What is the timeline for this Coimbatore hospital?

Vikas Maheshwari : Coimbatore, since it is a greenfield hospital and as MD sir has told that warm shell given by the builder, we are expecting, should be operational sometime in FY2025-2026 which is two years from here.

Bino Pathiparampil : Two years, thank you.

Moderator : The next question is from the line of Rishabh Tiwari from Allegro Capital. Please go ahead.

Rishabh Tiwari : Hi just wanted to confirm the number of beds or the capacity that you are planning for Coimbatore?

Vikas Maheshwari : 125 beds Rishabh.

Rishabh Tiwari : Okay. Thank you.

Moderator : Mr. Tiwari you may go ahead with your question sir.

Rishabh Tiwari : Yes, thanks. That is all.

Moderator : Thank you. The next question is from the line of Rahul Jeewani from IIFL Securities Ltd. Please go ahead.

Rahul Jeewani : Yes. Sir can also comment in terms of how our profitability has been performing in the Delhi NCR market, and any update with respect to the intercorporate loans which we had extended to Madhukar in terms of getting that money back?

Vikas Maheshwari : Rahul so Delhi we are doing a lot of work there and we plan the profitability in the overall period it will grow. As far as Madhukar advances is concerned during the last quarter we have got roughly Rs. 1.5 Crores back from them. So as on date the whole exposure outstanding including interest is roughly Rs.42 Crores with Madhukar and we remain confident that the unit is doing very well and we will recover this amount over a period of time. So there is no cause of worry for us on this.

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Page 15 of 16 Rahul Jeewani : Okay. So 1.5 Crores we have got last quarter and outstanding is 42 Crores?

Vikas Maheshwari : Yes, this is excluding the security deposit which we have given. The 42 Crores is the loan and plus interest which is accrued.

Rahul Jeewani : Okay sure sir and sir with respect to let say your capex Q1 obviously saw our number on account of this payment for the Gurugram land parcel but how are you looking at capex plan for the rest.

Vikas Maheshwari : Great question Rahul, so our spending should be something like this, including Gurugram we are just building roughly 930 beds (over next 4 years) for which the overall cost can be roughly between 800 to 900 Crores out of that 474 Crores cash is lying with us and this capex is spread over the next four years, including this year or the balance of this year and we remain confident with the cash generation which we are doing every year plus the cash balance with us, we will still be remain a cash positive company

with completion of all the capex plan which we have announced so far.

Rahul Jeewani : But sir our annual CAPEX, excluding the Gurugram land parcels continue to be around 170 to 175 Crores kind of an annual number or you think that one number will be on the higher side?

Vikas Maheshwari : As we progress Rahul, as the capex plan is taking steam and we are opening four hospitals this year and next year also we have scheduled two hospitals and onward. So the capex spend will be slightly higher. We expect roughly 200 to 225 Crores to be spent next year.

Rahul Jeewani : Ok. Sure sir. That is it.

Moderator : Thank you. The next question is from the line of Rishabh Tiwari from Allegro Capital Advisors. Please go ahead.

Rishabh Tiwari : Just a follow up on the capex run rate out of the 200 to 225 Crores that we are planning, how much of it would be routine capex, would it be around 170, 180?

Dr. Ramesh Kancharla : No routine capex is not so high for us. It is between 20 to 25 Crores for full year. It is not high for us.

Rishabh Tiwari : So this is for the project?

Dr. Ramesh Kancharla : Correct.

Moderator : Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference back to the management for closing comments. Thank you and overall to you all.

Dr. Ramesh Kancharla : Thank you very much ladies and gentlemen. I think very insightful questions and as we have said we remain very positive for the year with a lot of project execution and as well as the great operations. We are happy to answer any of your queries. Please connect with our Investor

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Page 16 of 16 Relations, information for which is available on the deck and CFO Mr. Vikas Maheshwari and Saurabh Bhandari both of them will answer your questions.

Thank you very much once again. Hope to see you soon. Thank you.

Moderator : Thank you. On behalf of IIFL Securities Limited, we conclude today's conference. Thank you all for joining and you can disconnect your lines.

Call: Nov 2023

November 7 , 2023

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.

Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.

Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q2 & H1 FY24.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and
Disclosure Requirements) Regulations 2015 .

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on October 31 , 2023, with regard
to Financial Results of the Company for Q2 & H1 FY24 .

The said Transcript is also available on the website of the Company and can be accessed
through the following link:

Analyst -Concall -Transcript -Q2-H1-FY24.pdf (d2sn5i18drfi94.cloudfront.net)

We request you to kindly take the same on record.

Thanking You,

Yours faithfully,

For Rainbow Children's Medicare Limited
Ashish Kapil
Company Secretary and Compliance Officer
Encl.: As above

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"Rainbow Children's Medicare Limited Q2 FY2024
Earnings Conference Call "

October 31, 2023

Management Speakers • Dr. Ramesh Kancharla – Chairman and Managing Director

- Mr. Sanjeev Sukumaran – Group Chief Operating Officer
 - Mr. Vikas Maheshwari – Group Chief Financial Officer
 - Mr. Saurabh Bhandari – Group Business Analyst
- Coordinator Mr. Rahul Jeewani – IIFL Securities Limited

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Moderator : Ladies and gentlemen , good day and welcome to Rainbow Children's Medicare Limited
Q2 FY '24 Earnings Conference Call hosted by IIFL Securities.

As a reminder , all participant lines will be in the listen -only mode and there will be an
opportunity for you to ask questions after the presentation concludes . Should you need assistance
during the conference call, please signal an operator by pressing ' * ' and then '0' on your
touchtone phone. Please note that this conference is being recorded .

I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited . Please go
ahead, sir.

Rahul Jeewani: Good morning, everyone. I am Rahul from IIFL Institutional Equities , and I welcome you to the
2nd Quarter Earnings Conference Call of Rainbow Hospitals .

From Rainbow , we have with us Dr. Ramesh Kancharla – Chairman and Managing Director,
Mr. Sanjeev Sukumaran, Group COO, Mr. Vikas Maheshwari, Group CFO, and Mr. Saurabh
Bhandari – Group Business Analyst.

Over to you sir, for your opening comments.

Ramesh Kancharla: Thank you, Rahul . Good morning, everyone. , I am delighted to extend a warm welcome to this
Earnings Call for the 2nd Quarter and the First Half of the current Financial Year . I trust

everyone enjoyed the recent festival celebrations .

Historically , the 2nd Quarter is characterized by height ened occupancy rates , a surge in patient visits and a favorable financial performance. This year too , the 2nd Quarter displayed positive trends compared to the first quarter . However , certain environmental factors such as the delayed onset of the monsoon and a marked variation in seasonal rainfall , especially pronounced in the southern regions of India , influenced our performance . August was notably drier , a rarity we haven't seen in a century , impacting the typical seasonal pattern and pre valence of various illnesses . Despite these headwinds , we have maintained a consistent and positive trajectory in both our operational and financial indicators .

Against this back drop, I would like to delve into the “ Financial Metrics” for the 2nd Quarter:

In Q2 FY '24, our revenue registered a growth of 6.3% amounting to Rs. 332.7 crores .

Similarly, our EBITDA saw an increase of 7.3% reaching 117.6 crores and the PAT , showing a growth of 2.7% reaching 63.2 crores .

For the quarter under review , our occupancy rate stood at 51.4% . Highlighting specifics , our Matur ed Hospitals recorded an occupancy of 60% , which was lower due to delayed onset of season . Additionally , our new hospitals reported an occupancy rate of 35% indicative of the nascent stage of their growth trajectory .

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Coming to Expansion Plans :

After a relatively quiet first half of the year , we are now gearing up for significant expansion in H2 with the plans to add 270 beds . We are launching three new hospitals across Hyderabad , Bengaluru and Chennai , which will further strengthen our Hub and Spoke model in these cities .

- We have recently inaugurated a modern outpatient block featuring a contemporary IVF facility adjacent to our LB Nagar spoke hospital in Hyderabad . This initiative aims to elevate patient experience and support future growth at this hospital.
- Our new hospital strategically located in Central Hyderabad with 60 beds is expected to launch clinical operations on 14th November aligning with our foundational anniversary .
- Another 50-bed additional block at Hydernagar, Hyderabad branch, Hyderabad spoke is expected to be operational by December .
- Our new spoke hospitals in Anna Nagar, Chennai and Sarjapur, Bangalore each with a capacity of 80 beds are in the concluding stages of their construction. They are slated

to become operational in Q4 of the current financial year.

This encapsulates our project roadmap for the ongoing fiscal year.

- The rest of the projects which is the spoke hospital at H ennur, Bangalore of 60 beds and the Regional Green Field Spoke Hospital at Rajahmundry in Andhra Pradesh of 100 beds are progressing well , are expected to commence operations in the next financial year.
- In the last quarter , we completed 100% payment to HSVP for two land parcels in Gurgaon and received the allotment later for these land parcels. Efforts are underway in collaboration with the government entities to secure possession of these land parcels and to go ahead with the registration process .

We maintain a confident outlook envisioning that this expansion will strengthen our presence in the growing cities of Hyderabad , Bengaluru , Chennai and the national capital region .

Beyond our growth plans , I would like to shed light on some significant achievements that underscore our dedication to pediatric healthcare excellence:

- We are proud to announce that our OMR Hospital in Chennai , which began its operation in September of the previous year , has been awarded the NABH accreditation . With this addition , 12 of our hospitals have garnered the NABH accreditation.
- Our commitment to advancing quaternary care has shown tangible results in the first half of the fiscal year. We successfully completed approximately 450 cardi ac surgeries

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and procedures along with 35 transplants encompassing liver , kidney , and bone marrow transplants .

- In a landmark achievement , we orchestrated the airlift of an 18-month-old infant suffering from severe influenza, pneumonia with lung failure from the Goa Medical College to Rainbow Children's Hospital , Hyderabad . This is perhaps the first air transport of a child on ECMO therapy in India . This child recovered after 29 days of

ECMO therapy and ready to be discharged . This news was extensively covered by the Indian media couple of days ago.

- I would like to present another great example of multi -disciplinary teamwork where pediatric intensive care doctors , pediatric hepatology doctors , infectious disease consultants , and the liver transplant team work ed together to give you a new lease of life for a 4-month-old child with 4 kilograms . Coming from Darjeeling, this baby was suffering from a multi -organ sepsis following a liver surgery , the baby was rescued initially in PICU and later underwent liver trans plantation . This baby was discharged very healthy without any morbidities .

- Rainbow Children's Hospital successfully organized a conference of international and national level of pediatric gastroenterology, hepatology and nutrition ISPGHAN and in association with Commonwealth Association of Pediatric Gastroenterology and Nutrition CAPGAN between 26th and 29th of October 2023 in Hyderabad . The event was addressed by national and international faculty and over 300 delegates attended the conference fr om all over the world . The team Rainbow including Dr . Prashant Bachina, the Senior Gastroenterologist and Dr . Adarsh Kancharla, one of our promoters , jointly organized the conference. This was well recognized and appreciated the outstanding work carried out by team Rainbow.

- The Company remains focused on growing opportunities in the medical tourism . Having recently signed a MoU with the government of Tanzania in the presence of Honorable President of Tanzania in New Delhi , the MoU focuses on health , co-operation between two countries , and with this MoU, Rainbow expects to receive significant number of patients for the specialty pediatric problems , especially for bone marrow transplantation , liver transplantations , and complex pediatric surgeries .

- We have underta

ken a pilot project focusing on Six Sigma in supply chain management . The objective is to refine inventory management practices and drive down supply chain -related costs .

- Recognizing the imperative of leadership in the pivotal phase , we have onboarded seasoned professionals in diverse domains compassing senior level leaderships in learning and development, projects and engineering , and core operations . Fortifying Rainbow Children's Medicare Limited
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our leadership structure is paramount in our vision of sustainable scaling and organizational growth.

With this, I would like to pass the mi ke to our CFO – Mr. Vikas Maheshwari to take you through the Financial Update.

Thank you once again for joining us today . We look forward to your questions and insights as we move forward . Thank you.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you , and thanks for attending this teleconference. I am happy to brief the “Financial Performance and Key Developments ” of Rainbow Hospitals for the 2nd Quarter and the First Half of FY '24:

- Our operating revenue for the quarter stood at Rs. 332.7 crores reflecting a growth of 6.3% when compared to the corresponding quarter of the previous financial year. For H1 , our revenue stood at Rs. 619.8 crores reflecting a growth of 12.7% when compared to H1 of the previous financial year.

- Our EBITDA for the 2nd Quarter amounted to Rs. 117.6 crores marking a 7.3% growth compared to the same period last year . For H1, our EBITDA stood at Rs. 205.3 crores reflecting a growth of 7.1% when compared to H1 of the previous financial year. Our EBITDA margins improved for the current quarter to 35.3% , while for the H1 our EBITDA margin stood at 33.1% .

- The profit after tax for the quarter is 63.2 crores marking a growth of 2.7% in comparison to the corresponding quarter of the last financial year. The increased depreciation expense to the tune of approximately 4 crores year -on-year basis mainly towards the new units led to the lower PAT growth in this quarter . For the H1 , our PAT stood at Rs. 105 crores reflecting a growth of 4.4% when compared to the H1 of the previous financial year.

- In terms of the operational performance, Both outpatient and inpatient volumes witnessed a minor decline of 2% and 1% respectively when compared to the corresponding period in the last financial year. Dr. Ramesh has already highlighted the reasons for the same in the opening remarks. Deliveries which are independent of the seasons grew by 4% compared to the corresponding period of the last year.

- Our return on capital employed and return on equity for the 2nd Quarter was 9.7 % and 5.6% on non-annualized basis respectively. For the H1, the ROCE and ROE are at 16.1% and 9.3% on a non-annualized basis respectively. It is expected to go up as the new units commence operations.

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- Our patient mix continued to remain robust and balanced with 50% of the revenue coming from the insurance and the balance 50% coming from the cash patients. For the H1, the patient mix stood at 51% cash and 49% insurance.

- Furthermore, I am delighted to note that our international business now constitutes roughly 4% of our total business for the 2nd Quarter FY '24 compared to the earlier figure of 2%.

- I am pleased to inform you that our Company's balance sheet remains robust with net cash position of Rs. 472 crores as of 30th September of this year, and this will support our ongoing capital expenditure plan as outlined by Dr. Ramesh, Chairman and Managing Director. Given our current cash flow and the anticipated internal accruals in the coming years, we remain confident in our ability to complete all planned capital expenditures through internal

accruals without any debt financing. During the quarter, the Company invested approximately Rs. 55 crores in the capital expenditures.

With these insights, I conclude my financial update. I now invite questions and suggestions from the participants. Thank you very much.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Nitesh Dutt from Burman Capital. Please go ahead.

Nitesh Dutt: My first question is to improve my understanding of your business model. So, Rainbow's EBITDA margins versus other listed multi-specialty hospitals are significantly higher despite lower occupancies and similar in-range ARPOB. Rainbow's EBITDA is 30% plus typically, and other multi-specialty hospitals are usually at early 20s level. So, I just want to understand if there is a structural difference in our cost structure due to the pediatric focus business model, which allows us to make high EBITDA margin?

Ramesh Kancharla: Obviously, the Rainbow business model is quite different from the adult hospitals. The Rainbow operates on 1. one is that the (setting is children-centric) in a children's hospital, 2. the outpatient business is significant. About 30% of revenue comes from the outpatient services, whereas in the multi-specialty it is about 16% to 18% of revenue.

Second important thing is that our average length of stay is significantly lower, which varies between 2.6 to 2.8 days. The adult hospitals ALOS could be north of four days. And also, when you look at kind of the case mix, we have obstetrics which are kind of a short stay and higher revenue generation and also intensive care services, 30% of beds are in intensive care services which again generates higher revenue.

What actually don't do much of per day revenue generation is routine seasonal pediatric patients are the ones too, but they are bulk of the patients in seasons. So, therefore, when you see in the current quarter, our ARPOBs are pretty high and that is because of the seasonal business is much

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less because of a delayed monsoon, which is why our ARPOBs are pretty high. Still we manage to maintain the industry leading EBITDA.

Nitesh Dutt: That is clear. ARPOB angle is clear, but is there any specific difference in cost structure as well, for example, cost of consumables or employee cost savings versus other hospitals?

Ramesh Kancharla: The professional cost is the same. We also pay about 23% - 24% and the HR cost is more or less similar. Our consumable cost is about 4-5% lower. In adult health care, you have a lot of stents,

devices all those things (high end consumables). We don't have much of those devices, stents because we don't do kind of implants and devices and those stuff. So, there is about 5% difference in the supply chain cost.

Nitesh Dutt: I have couple of more questions specifically for this quarter. So, first is on occupancy levels. You mentioned delayed onset of rainfall and seasonal deficit, right? But is there any other reason? Because occupancies are drastically lower. So, 51% is even lower than Q4 of FY '23. So, that is one. And second, any guidance for Q3 and full-year occupancy numbers from your side?

Ramesh Kancharla: Occupancy is definitely lower. We are still discovering what is the post-COVID pattern. During the COVID, the occupancy was quite muted. Post-COVID during last one, one-and-a-half year, the occupancy has been very heightened and there is pronounced illness pattern. Now, this year what we have seen is that we have not come back to the normal pattern of the year because of the delayed rains and all the seasonal variations. We are still kind of not very sure how the rest of the year H2 is going to go.

W

We expect our occupancies to be around 50%. That is blended including the new hospitals which have come into operations. From the last year to this year about 11% of the beds been added in the two hospitals (OMR, Chennai and Financial District, Hyderabad). We need to take into this significant fact into consideration. We are going to add some more beds, about 16% of the existing bed capacity this year. With all these additions of beds, if we could reach 50% of occupancies, we would be pretty happy with that.

Nitesh Dutt: And sir, on ARPOB also, ARPOB has significantly increased by 16%. So, any factors that have led to a better case mix? And is it sustainable? How sustainable is this improvement? Also, same question. Any guidance on ARPOB growth on full-year basis?

Ramesh Kancharla: We expect the ARPOB to grow about 5% - 6% (on an annualized basis). When you have a lower season definitely the case mix will be different, as we don't have much impact of seasonal business. That actually gives a better ARPOB because what remains is high cash generating business. And also, case mix is changing year-on-year. So, that is a significant factor. And some price escalation is a second thing.

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If you look at the international business it was 2% last year, this year it is over 4%. So, these are all the macros to have better ARPOB. But however, we do expect when the season picks up, the ARPOB to get moderated because of the volumes. When we do overall better business, the ARPOBs will get moderated.

Nitesh Dutt: Last question from my side is on the expenses front. So, both our gross margin and OpEx as a percentage of revenue are down Y-o-Y and our employee expenses have reduced sequentially even at an absolute level. So, what are the reasons? And how do you see these expenses moving in Q3 and Q4 with the commissioning of new hospitals?

Ramesh Kancharla: Obviously the employee cost will go up when you add new hospitals because our model is a full-time doctor and also, we employ significant number of people to train them to be positioned for the future business.

But at a group level what happens is that when you don't have a huge seasonal business we also try to work around to moderate or optimize the cost. We take the handle off on the attrition and also there won't be any kind of additional payments in terms of overtime and those things would get moderated.

This is something which is based on how many new beds are going to come, what is your existing business? Is it running at high occupancies? Sometimes we do kind of transfer our employees from one organization to newer organizations. We do this math all the times to optimize our overall HR cost.

Moderator: Thank you very much. The next question is from the line of Girish Bakhru from OrbiMed. Please go ahead.

Girish Bakhru: Actually, just again we wanted to reconcile this gross margin expansion despite fall in occupancies and ARPOB. Why is gross margin higher?

Vikas Maheshwari: You are comparing it on year-on-year basis or quarter-on-quarter?

Girish Bakhru: Quarter-on-quarter.

Vikas Maheshwari: Quarter-on-quarter, definitely there is a lot of change because if you look at in the quarter first, the occupancies were in the range of 41%, and this quarter occupancy is slightly higher. So, one the operating leverage which has kicked in. All the other expenses if you look at as a percentage of revenue has improved because of the operating leverage. There was a question on the staff cost quarter one call, which Dr. Ramesh, MD and Chairman has already explained you.

One more reason which was there is that in the quarter first the sequential HR cost has come down because in the quarter first, there were some one-off payments also because

use of the senior leadership hiring which we have paid for . (In this quarter) this (one off) amount was not there.
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And then obviously , since the quarter second occupancy was not as per the full expectations , so there was some less OT payments and the escalation of the staff cost. So, that was the reason . Otherwise , if you look at all the expenses in absolute terms has definitely gone up because of the two units Sholinganallur , Chennai and the Financial District, Hyderabad has opened up . So, mainly is the game of the operating leverage.

Girish Bakhrui: And just if you look at this overall mix in the revenue which you were mentioning before , 30% is from outpatient . There is a high ICU critical beds contribution . Where would be the surgical mix? How much , let's say , revenue would be from surgical procedures ?

Ramesh Kancharla: Surgeries are far less in the Children 's Hospital . Children's Hospital are dominantly medical , if you take the obstetrics separately. Let's put it this way , if you have to look at our surgical numbers at group level it would be around 8,000 to 9,000 surgeries annually . Children 's Hospital is mainly a domain of medical , lots of intensive care and specialty patients .

Surgeries are mainly from specialty side like cardiac or pediatric surgery . We do have all the specialties like orthopedic surgery but pediatrics is not wear and tear . Pediatric is either acute or genetic. Hence, it's not comparable with multi -specialty .

Girish Bakhrui: I understand because last couple of calls you were discussing this , the ARPOB is highest in your quaternary procedures , which I would assume would include surgery , and obstetrics would come at the similar higher level, and then secondary and OPD would come lower . So, the sensitivity of the ARPOB, is it higher to surgical procedures or delivery ?

Ramesh Kancharla: The quaternary care is the complex neurosurgeries , liver transplants , cardiac surgeries , renal transplants , BMT s and related intensive care along with these patients . This quaternary care, is a growing segment in the Rainbow Group. All quaternary care is fairly nascent. It's about four , five years old. The growth of quaternary care is fairly impressive .

Girish Bakhrui: So, this change in ARPOB, is it sensitive more to the deliveries as of now ? Is the sensitivity highest there ? I am just trying to figure out what exactly.

Ramesh Kancharla: I think it's a combination of factors . It is to an extent the deliveries , to an extent the complex surgeries , to an extent it is international business and to an extent it is pricing. I don 't think there is any one dominant factor among them . It is a combination of these stuff.

Girish Bakhrui: And just lastly , can you share of these 755 doctors , how many are surgeons ?

Ramesh Kancharla: The surgeons will be about 10% to 15%. 10% maybe outer limit.

Moderator: Thank you. The next question is from the line of Rishi Modi from Marcellus Investment s Managers . Please go ahead.

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Rishi Modi: So, first, just a bookkeeping question. The loans that you have given to Madhukar Hospital, how much is still pending for us to receive from that hospital ? I think FY '23 we had around 40 crores remaining to receive. So, Q2, how much is left?

Vikas Maheshwari: Rs. 3 crores in this quarter we have received . The amount has got reduced to that extent . That is the update on this Madhukar one.

Rishi Modi: 3 crores that we have received in this quarter?

Vikas Maheshwari: Yeah .

Rishi Modi: In Q2 or half year?

Vikas Maheshwari: This is for the quarter second which we are updating . We are not worried about this Madhukar loan which we have given . That unit is doing

well and we have learned a lot about the North

Market , how does it operate and we are pretty confident that whatever the money we have given over a period of time we will be able to recover it. It is just time and the money which we have invested on that , and we are pretty sure that this money will get recovered over a period of time .

There is not too much of the worry for us right now .

Rishi Modi: Second , on the international business front , so we are at 4% of revenue. Is there any target that we are targeting over say the next three to five years ? And how different is the ARPOB for international patients versus domestic patients , if you could give some understanding out there ?

Ramesh Kancharla: It's a very early phase now . We started our international business post pandemic actually . We are trying to two things in international business . One is that, so far, no Children's Hospitals has been presented to the international patients or international doctors . So, our approach is that the people need to know what is Rainbow Children's Hospital and its capabilities .

Number two is influence more at the doctor level , have associations and agreements at the country level , the government level , foundations level and other levels.

Number three is to try to get other agencies . So, work at a multipronged approach and try to see

that maximum coverage over one-two years' time frame .

So far there are some of the countries which are kind of very promising basis the number of the patients what they are sending and all the feedback what we are getting . So maybe by next year we will have a better idea about this , how we can actually position ourselves, what is the expectations on international business .

On other hand, what we are trying to do is to establish quaternary care in the Bangalore and Delhi also . All these things are going to be important to drive the international business to our Rainbow Children's Medicare Limited

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network group hospitals . Right now, Hyderabad is a dominant player followed by Delhi . So, we just need to see that balance between all of them .

In the recent concluded conference, which is the Pediatric Gastroenterology Conference of the Commonwealth and Indian Society , lots of doctors have come from Africa , Middle East and Indian neighborhood countries like Bangladesh and Russia. They have all seen and appreciated what we are doing . They were quite surprised to see that the Children's Hospital is doing all the quaternary care procedures .

It is the journey which we have embarked on and it will take some time for us to strengthen our position and to really have proper estimates . And so far, it's pretty encouraging . I think we have guided about 3% to 4% . I think the last two quarters we have been doing 4% , which is pretty encouraging .

We just signed an MOU with the Tanzanian government ; the Health Minister came and visited the hospital recently and I signed M oU in presence of the Honorable President of Tanzania. They are quite happy to send their children to Rainbow Network for all the tertiary care and quaternary care.

Rishi Modi: And if you could just give an understanding of how different is the ARPOB? Is there any multiple on that difference versus our Company average ?

Ramesh Kancharla: I think it's definitely above the Company average, but in the initial phase we want to show the results . My aim is to see that the results are better when the treatment (patients are treated) is done in Children's Hospital . First, I probably want to bat for that than the ARPOB.

Rishi Modi: Finally , you mentioned that you all hired some senior employees in this quarter . Could you just give us a brief understanding on who all have you all hired in this quarter on the operations front as well as on the medical side

of things ? And what are they bringing to the table ? What are their cares ? How are you all planning to use their capabilities ?

Ramesh Kancharla: We have hired all the senior management in the last 6 months ' time. Now this is one of the important things because we got a large CAPEX and significant bed expansion . So, we need very seasoned projects head to drive the projects on time . That's one position which we have onboarded . And also, as we increase the number of units , I think learning and development is one very important area, because this is a very service driven , the Children's Health Care . So, a lot of focus and emphasis goes on that . So, we are trying to work on that .

And also, even the sales and marketing space, because Rainbow has always been very , very organically grown , I think sales and marketing space is something which we are trying to build .

Mr. Sanjeev Sukumaran has actually come from a very seasoned background of sales and marketing and operations background . So, he is trying to work very seriously on that (sales and marketing). so that is something we are going to have a big leverage on that as we move forward Rainbow Children's Medicare Limited

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with increased number of beds as we are strengthening our hub and spoke model in next two, three years ' time.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities . Please go ahead.

Alankar Garude: Sir, first question . Were there any specific cities wherein the year -on-year impact on occupancies was more pronounced in this quarter ? Or was the impact widespread for us ?

Ramesh Kancharla: No, Alankar, to be specific, actually , we have seen this across the group. I think we have seen that to an extent a little more in Chennai, to an extent in Hyderabad and Andhra also. I can feel that there is some degree of seasonal variation . What we have seen is that the last year season lasted till March and April onwards became kind of quietened up, and then we had delayed monsoon. So, that is good for the society that healthy period continues to go on till August and middle of September . So, what we had is about kind of some degree of seasonal business only last five weeks of this quarter .

So, that's why we see that the occupancy plummeted to the extent to the tune of 8% in matured hospitals , about 11% to 11.5% in the growing , the new spokes . That's seen across the

southern parts . Maybe Bangalore is slightly better than compared to the Chennai, Andhra and Hyderabad .

Alankar Garude: Sir, the second question is , given the big expansion phase over the next six months or so and even beyond that , but more focused on the next six months , possibly , this is the highest ever in the Company 's history . Are there any meaningful execution risk which you would like to highlight here?

Ramesh Kancharla: I think the execution challenges may be there, but risks may not be there . The challenge, I mean, always will be when you commence four or five hospitals in four, five months ' time. There are a lot of executional challenges , which I am sure with a large team now , we should be able to troubleshoot .

One of the important thing is , in Hyderabad there is no challenge because enough number of people are there and Hyderabad when we do expansions , we always do it for demand. the last expansion at Financial District is doing very well , and it's posting significant positive EBITDA. So, therefore, when we do in Central City or additional block in Hyderabad , these are all for the demand .

So, Chennai , when you look at it, we want to expand the hub and spoke model . I would say that it still takes 12 to 18 months for break even in Chennai. On execution, since we are already there with the two units , I am sure

we will be able to orchestrate.
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Bangalore, we have already have three units and adding one more unit is not going to be a big challenge. The area where we are going to operate in Bangalore is a pretty promising part of Bangalore which is Sarjapur , Bellandur area. lot of doctors are interested to work with us . it's in a way for the demand in Bangalore also. So, that's why we are kind of fairly confident about though it is 270 beds, we should be able to kind of manage well.

Vikas Maheshwari: Alankar, if you look at the breakup of the beds which are coming up (270 beds) , out of that , 110 beds are coming in Hyderabad which is our forte which is roughly 40% of the beds coming into the Hyderabad . So, we are not worried about that .

And more importantly , these two units are almost ready Central Hyderabad and Hyderabad . Central Hyderabad is going to open in next 15 days' time or 14 days to be very precise . Hyderabad , which is just an extension of the existing unit and will be open by December . So, we are not worried about that at all.

And with this , you know, we will be reaching the capacity in Hyderabad to roughly 950 beds , which is sizable for the pediatrics and the mother and child focused hospital . And as being told by MD , in Bengaluru , we have already three hospitals , adding one more for the growth, and Tamil Nadu 80 beds. And those are in also good shape . So, that should get opened by March. The execution risk more or less what was anticipated is on the timeline etc., I think we are meeting that and pretty confident that this will get opened on the timelines which is being stated .

Moderator: Thank you. The next question is from the line of Bansil Desai from J .P. Morgan . Please go ahead.

Bansil Desai: My question is on margins in this quarter . So, despite sharp decline in occupancies , we have reported margin of 35% plus . I understand there was a benefit of case mix , also cost control , which would have driven these margins . But just trying to understand , had this quarter been a normal quarter , would our margins be any different ? Just trying to understand , you know, is case mix a bigger driver of margins or occupancy for us?

Ramesh Kancharla: I think the margins always with the case mix rather than occupancy . I think what happens is beyond critical occupancy , the additional case mix will kick in more operating efficiencies because you have the same doctor , same HR , same system everything else. That's what typically happens in the seasons in quarter two and quarter three when you are doing well on top of it and season adds to it , so that will kind of push lots of outpatients . That generates a lot of revenue and also the seasonal business . It's like icing on the cake. So, that gives a kind of a more of operating leverage and ARPOBs will get moderated , but you will have additional revenues will kick in.

Bansil Desai: And sir, how has the occupancy trended for the month of October for us ? We have heard , you know, there has been rise in Dengue cases etc. So, have you seen any improved occupancy in this particular month ?

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Ramesh Kancharla: So, definitely , there has been an improvement in the last two months ' time and from October onwards , September onwards . So, there is an increase in occupancies . There is some city-to-city variation . Some places I think some of the adults are pretty sick and still few adolescents are also pretty sick, but mostly the smaller children under five years , they are not getting very critical requiring intensive care which is good.

Bansi Desai: That's good to hear. And my second question is on occupancy . So, for the newer beds , we have reported 35% occupancy . So, if

you were to adjust for the new beds which got commissioned in the last four quarters , what would have been the occupancies like for that cluster ?

Ramesh Kancharla: It would be 40% plus . Just over 40%.

Vikas Maheshwari: let me add . So, this is a lower occupancy we see as an opportunity . So, that shows the Delta which we have got for the growth, and which can drive the revenue and the EBITDA in the coming quarters as we ramp up the occupancy at these new units .

Moderator: Thank you. The next question is from Mr . Rahul Jeewani who is a coordinator for the call from IIFL Securities Limited . Please go ahead.

Rahul Jeewani: Sir, would it be possible for you to quantify the contribution of the seasonal business this year versus last year in terms of how much did it contribute to you?

Ramesh Kancharla: It varies from year to year . I think , you know, let's put it this way. It is probably about 12% to 15%, I would say.

Rahul Jeewani: So, then this year , maybe the contribution was lower than this 12% to 15% usual , right ?

Ramesh Kancharla: Yeah .

Rahul Jeewani: And sir, you talked about , let's say , you are targeting break even in the Chennai market for the new hospitals from between a 12 to 18 month period. Can you similarly talk about what would be your targeted timelines for break even in Bangalore , which is again a home market for us and then maybe in some of these tier two, tier three markets like Rajahmundry, which you are planning to enter into next year ?

Ramesh Kancharla: I would expect Rajahmundry to do breakeven in two calendar years' time and the Bangalore would be the same 12 to 18 months' time .

Rahul Jeewani: And sir , Vikas indicated that the Madhukar Hospital has been doing well. So, any operational parameters which you can share of Madhukar in terms of what kind of EBITDA margins the hospital is currently running at ? And I think we were trying to convert the Rosewalk Hospital into a normal birthing center versus their previous positioning of being a premium birthing Rainbow Children's Medicare Limited
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center . So, any updates with respect to your plans for the Delhi NCR market in terms of existing hospitals ?

Vikas Maheshwari: So, Rahul , as far as the Delhi is concerned , we are happy to brief that the performance has improved a lot. Since we provide only the medical services there, so we don't track their operating metrics so closely as we track for all our 16 hospitals , but it has improved over a period of time , and we have seen it is a cash positive and they have been able to repay our loans which we have given for the operational performance etc. As far as the Rosewalk is concern ed, I will request Dr . Ramesh to brief on that .

Ramesh Kancharla: Rosewalk there is no bleed from the operational point of view . It's breaking even . So, what we are trying to see is it's a small hospital, number one . So, as a group , we never believed in this boutique businesses or we kind of not really been big fan of a small , small hospitals because you wouldn 't be able to kind of display your strength and your ability to offer the patients the large care.

So, this has been a problem of Rosewalk like many other boutique centers in the country . So, therefore, we are constantly trying to see that remove that the luxury brand, but unfortunately , it's not getting removed . So, the people still in the minds of the Rosewalk and say that , you know, this is the luxury hospital brand . We have recruited some full -time doctors and even the doctor's concerns are okay , this is luxury hospital . So, I may not be able to see large number of patients . So, I can't stop my clinic to depend on completely Rosewalk . So, there multiple things are there. I will give the mi ke to Sanjeev Sukumaran. He is fully into Rosewalk now. So, he should be abl

e

to give you a little better outlook of Rose walk. One second , Rahul.

Sanjeev Sukumaran: I think it is a pertinent question, Rahul . So, couple of things that you spoke about . Do we convert this into any other birth ing center ? And Dr. Rames h did touch upon that , but I think it's important for us to also know and understand that Rosewalk is in the southern part of Delhi , the South Delhi . So, there is a demand for what we call as a boutique center , and though we try to come back to a normal birthing center , there has always been a consistent demand on this . What we have been doing over the last two quarters is trying to beef up a team of marketing so that we start to position our brand in a much more compulsive manner . We are starting to see some results on that . We have hired a few full-time doctors , which are very , very prominent in the Southern Delhi space. So, we are starting to see leverage of that, and we are building those . We are building the portfolio of doctors in that area. And I am quite positive that over the next few quarters , we will start to see an uptake in what Rosewalk contributes.

Rahul Jeewani: And sir, just one last request from my end . Can you please start disclosing the payor mix and

case mix data on a quarterly basis ? Because that would be helpful to us in terms of analyzing the numbers and given that most of your peers do disclose that data , so that would be very helpful .
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Vikas Maheshwari: We will do that, Rahul . The payor mix, obviously , we are covering under our opening remarks , we will include into our presentation , and we will also look to explore the possibility of putting up the case mix . Because the case mix which we put up in our matrix in the Children 's Hospital is slightly different . We will discuss it offline with you and the others to see how best we can disclose that .

Moderator: Thank you. The next question is from the line of Sande ep Jain from Baroda BNP Paribas Mutual Funds . Please go ahead.

Sandeep Jain: Most of the questions have been answered . Only one question on new hospital and ALOS and all. So, the advantage what we have got in the ARPOB is the reduction in the ALOS and a couple of other things , but which is largely happening in the new hospitals , right ? If I look at the year - on-year basis , the new hospital ALOS has declined from 3.67 to 2.9, and as well as the occupancy also. So, how we are looking at it means in terms of if I look at occupancy , ALOS going ahead or in terms of the growth overall going ahead , what kind of ALOS or what kind of occupancy we are targeting in the new hospital with the new bed and all so that I can get a more sense on the ARPOB? That's the first question, and there is one follow -up.

Vikas Maheshwari: So, as far as the new units are concerned, you know, as the y mature and whatever the matured hospitals are showing the ARPOB, ALOS and occupancy , we believe that that will also be our new hospitals which is not matured less than 5 years which we are classifying will display the same type of characteristics plus minus 6 months , one year here and there, but they will reach there. So, we see this is an opportunity for the growth which will come in the coming quarters from the new hospitals .

Sandeep Jain: And up to the desired level of occupancy of somewhere around 50%, 55% kind of thing , how much time we can take?

Vikas Maheshwari: So, from the date of the operation of the hospital , it takes four to five years ' time, and obviously , we will reach there.

Sandeep Jain: And by the end of this year , we have somewhere around 100 new beds are coming . Is my this thing , calculation is right ?

Vikas Maheshwari: So, 270 beds we are opening in the next five months , and which will get commissioned by 31st March

Sandeep Jain: 270 new beds , fine.

Ramesh Kancharla: Yeah, 270 new beds.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities . Please go ahead.

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Alankar Garude: Sorry , I got dropped off earlier . Just one final question . If we look at Bangalore as a market for us, we have been present for about eight years now . So, in terms of the margin trajectory in this cluster , would it be fair to say that over a period of time , the margins in this cluster would largely be in sync with our margins in the Hyderabad market or would you like to say that Hyderabad being a home market will always have that advantage for us compared to any other geography we enter into ?

Ramesh Kancharla: Yeah , I think the margins in Bangalore have been fairly good and as the hospitals are maturing , we can see that one of the hospitals has almost reached to the Hyderabad level , but I will tell you, Hyderabad always will have an advantage, the home advantage will always be there in terms of your pricing power , in terms of case mix, maturity . All these things play up. It's not only with Rainbow , it's for every hospital chain, home ground of every group displays that strength , and I think Rainbow also displays the same strength .

But that doesn't mean that Bangalore will be significantly lower . For what I think , we would continue to be industry leading in Bangalore as we move forward in next couple of years .

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC . Please go ahead.

Damayanti Kerai: So, most of the questions have been answered . I have two clarifications . First, are you done with tariff hikes for all the clusters which you intended for this year , or something is yet to be done ?

Saurabh Bhandari : Yes, Damayanti, we have done the negotiations

Ramesh Kancharla: We have done most of the negotiations and all the tariffs have been negotiated with the TPA .

Damayanti Kerai: So, all been done and then maybe we will see in next cycle only , but for time being it's all negotiated as you said ?

Ramesh Kancharla: Yeah , most of them are closed actually . Hyderabad was pending when we spoke last time. That's

also been closed .

Damayanti Kerai: My second question is on ARPOB again . So, 16% year-on-year growth, can you broadly help understand like what is the contribution here from price hike and what kind of contribution came from case mix so that, I guess , we have understanding about the drivers better ?

Ramesh Kancharla: Damayanti, we have to look at kind of yearly ARPOB. That's what we have guided earlier about 5% to 6% yearly ARPOB growth , but what we are seeing at the moment is that the 2nd Quarter to now is 16% ARPOB growth . This is largely because one is that we don't have seasonal business . Number two is our case mix . Number three is our kind of price optimization . Another reason is about the international business . All are put together I think we had a significant growth in ARPOB .

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Suppose if this quarter had a significant season about another 10% to 12% of occupancy increase, that would have got ARPOB optimized significantly , to somewhere around 52,000 or 51,000. That usually happens year -on-year to us . So, we can talk about ARPOB on a year -on-year basis rather than one quarter comparison.

Damayanti Kerai: So, sir, this annualized 5% to 6% increase in ARPOB, will it be mostly driven by the price hike part or other factors like mix, and all will also play important role?

Ramesh Kancharla: We expect significant to be in the case mix . Factor number one is the case mix . Factor number two is the international, and number three is price hikes .

Vi

kas Maheshwari: And also, Damayanti, if you can go through the presentation slide number 23 , if you look at the most of the ARPOB growth as a percentage if you look at has come from the new hospital s, right ? So, the new hospital ARPOB for the H1 has grew by 30% . So, that shows that those hospital s have started gaining the pricing power plus the clinical mix is going up on those hospital s. So, that also helps .

Moderator: Thank you. The next question is from the line of Dheeresh Pathak from White Oak . Please go ahead.

Dheeresh Pathak: Sir, I was asking that last year the OMR and Financial District, the newer spokes that we had opened , their occupancy and their break even if you can just talk to that?

Ramesh Kancharla: we don't give a unit wise break -up. I can give you an overall broader perspective. OMR I think by end of the year , we will have break even . So, Financial District, I think we are already break even . This year, by end of the year , we will post positive EBITDA.

Dheeresh Pathak: And just on this reclassification , so Rainbow Children's Hospital , this is Madhukar, right ? This has been now mature.

Ramesh Kancharla: Madhukar, yes.

Dheeresh Pathak: So, that has also then impacted the ARPOB of the newer and the mature, right ? Because you don't count the bed capacity , but you show the revenue , right ? So, that's why I am seeing lower ARPOB in new bucket versus and slightly higher ARPOB in the mature market . Is that also one of the reasons ?

Ramesh Kancharla: Yeah .

Vikas Maheshwari: Yeah , that is one of the reasons . And Dheeresh, this will continue to happen because the sizable hospitals which are under five years has not matured , will move to the mature . So, that plus minus will continue to happen . But what is more important to see at the group level , what type

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of matrix we are showing , and all the new units , how they are progressing to the matrix to achieve the matured hospitals ' matrix. So, I think we are focused more on that.

Moderator: Thank you. The next question is from the line of Nikhil from SIMPL. Please go ahead.

Nikhil: Few questions . One is, sir, in a previous question , you mentioned that even though in Bangalore we are now 8 years old , the profitability matrix for the Bangalore cluster will remain , is currently below the profitability matrix Hyderabad market has.

But if I extend it over next five years and the whole idea of our , the whole proposition , and I could be wrong here, is that we are able to take market share from smaller clinics or from smaller independent practitioners , which means that even in markets like Bangalore where there is a lot of incoming crowd , our brand should be able to gain a premium over the pricing which would be prevalent .

So, when do you see that a market becomes good enough for us to improve profitability ? Because eventually , the profitability for Bangalore or Hyderabad , any other cluster should not be too different as our brand keeps gaining scale unlike , so that should be the proposition, right ? Or am I understanding something wrong here ?

Ramesh Kancharla: Well , what happens is we are in eighth year of operations in Bangalore . So, with Hyderabad , we

are about 20 years of operations , right ? So, you see, it's purely a maturity and your pricing strategy , your overall business mix and also the number of the operating metrics are all very different . You can look at even adult hospitals which are home -based . All other hospitals will display the same strength because of sheer maturity , and also your cost structures also , revenue generation dominates over the cost structures and post very impressive returns . So, that is the home ground effect , number one .
That doesn't mean that other cities don't do well .

So, I would say in the next 5 years we will be a dominant player in Bangalore. There is no doubt about it because when you look at your hiring , your inclusion of the doctors , what is today versus what was 10 years ago , it's very different , right ? Isn't it?

If I want to hire a doctor , if I want to hire a manager , today from now to future is very different . Cost structures have changed . So, which is why you may do a top line very well . You may have a good EBITDA. You may have a very well -run hospital , but still that original hospital where you started commenced , that will never match that . So, I think that's my understanding of it. But one thing I am very sure is that any home base of any healthcare chain displays the same pattern . That doesn't mean that other cities people have not done well . People have done very well in other cities ,Fortis has done very well in Bangalore. So, that's not a question about it . So, we will do very well in Bangalore, Chennai , all those domains . But that kind of a comparison, I
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don't know , it's how do you compare with that . It is always home ground and you have control on what's happening at micro level . That's what I would say.

Nikhil: But just one question here. See, in a multi-specialty , there are multiple areas where a hospital can focus on, say, oncology , or someone could be very strong on cardio, while in a Children Hospital , what I understand is that those permutation combinations of multiple specialties is limited because children don't have all those . So, very rare issues might be happening , but on an average the infection pattern or the disease pattern would be similar across geographies . So, eventually , because there are not many permutations and combinations which can come on the specialty side, the treatment or the treatment focus for a children would be similar whether it's in Hyderabad or in Bangalore . And as the brand or the presence of the brand gets felt more and more , the footfalls would come inherently for the Company . We should be able to pull the footfalls .

So, I am just trying to understand , for a specialty I understand that home market makes a big difference, and we have seen across chains , but for our kind of a model , would the home market versus a Bangalore or a Chennai or any new market , eventually , it should even out, or it should converge to a similar profitability .

Ramesh Kancharla: Yeah , we can talk on the subject for a fairly long time because it's based on multiple parameters . You see, for example , at Hyderabad , someone comes from 250 kilometers just to show us, if the child is all right. From Adilabad they come to Rainbow just to show and check that their child is okay because they know , they have heard about Rainbow is the best children's hospital . So, this is a network . This is an effect of longevity and also brand which is penetrating not only in the city , into the kind of a people of the rural . So, there are many factors that will be there in that. The city factors are to an extent , but in the larger picture , the multiple factors work around . And also, what is important in children's healthcare is the destination . So, Children Hospital is a destination . It is not like adult hospitals . If 10 doctors moves , Patients also moves along with them . In Children's Hospital , it's a destination effect . So, and so hospital is still good. It is a destination for everything . People just come to you because it's a destination rather than a kind of a hospital . With few doctors moving around , they won't be able to take patients . So, which is why the maturity of the hospital is an impression , gives a lot of positive impression among the public , and not only in the city , in the peripheries across the state

and beyond state borders . It's a large subject , we can discuss.

Nikhil: Sure. Moving on to the second question , this is more a, see, one clarification first. So, you said on the newer hospitals , we are at 40% . So, the older hospitals which were not added in last , say, one year , our occupancies are around 40% . Is that the number you mentioned earlier ?

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Ramesh Kancharla: No, the matured hospitals which are old, they have kind of we had about 60% occupancy versus 68% occupancy in the last year . Without the addition of any beds, there is an 8% drop in occupancy . That is because the season did not kick in . This is what I was trying to say.

Nikhil: No, my question was on the newer hospitals . So, if I average out , say, June 22 and September 22, last year first half versus current first half , we have added on an average from we were at 448, 500 to now 600 or a round 130 beds have been added . So, on the older beds of average count of 320 , we are at 40% is what the understanding .

Ramesh Kancharla: Yeah, exactly. That's true.

Moderator: Thank you. As there are no further questions from the participants , I now hand the conference over to the management for the closing comments .

Vikas Maheshwari: Thank you everybody and participating on the conference call . We will look forward to receive any other further questions if the participants have. They can send. They can contact Mr . Saurabh Bhandari or myself . The email ID s and the coordinates is already given in our presentations . Look forward to meeting all of you in the coming quarters . Thank you very much .

Moderator: Thank you. On behalf of IIFL Securities Limited , that concludes this conference. Thank you for joining us , and you may now disconnect your lines .

This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy. Please also note that this document has been edited without changing much of the content, for enhancing the clarity of the discussion. No unpublished price sensitive information was shared/ discussed on the call.

Call: Feb 2024

February 1, 2024

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051. Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001. Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q3 & 9M FY24.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on January 25, 2024, with regard to Financial Results of the Company for Q3 & 9M FY24.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

<https://d2sn5i18drfi94.cloudfront.net/investor-relations/reports/Analyst-Concall-Transcript-Q3-9M-FY24.pdf>

We request you to kindly take the same on record.

Thanking You,

Yours faithfully,

For Rainbow Children's Medicare Limited

Ashish Kapil
Company Secretary and Compliance Officer
Encl.: As above.
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"Rainbow Children's Medicare Limited
Q3 FY '24 Earnings Conference Call"
January 25, 2024

MANAGEMENT : D R. RAMESH KANCHARLA – CHAIRMAN AND
MANAGING DIRECTOR
MR. SANJEEV SUKUMARAN – GROUP CHIEF
OPERATING OFFICER
MR. VIKAS MAHESHWARI – CHIEF FINANCIAL
OFFICER
MR. SAURABH BHANDARI – GROUP BUSINESS
ANALYST

M
MODERATOR : M R. RAHUL JEEWANI – IIFL SECURITIES LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to Rainbow Children's Medicare Limited Q3 FY '24 Earnings Conference Call, hosted by IIFL Securities Limited.

As a reminder, all participant lines will be in the listen-only mode. And there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Thank you and over to you, sir.

Rahul Jeewani: Good morning, everyone. I am Rahul from IIFL Institutional Equities. I welcome you all to the third quarter earnings conference call of Rainbow Hospitals. From Rainbow we have with us today, Dr. Ramesh Kancharla, Chairman and Managing Director, Mr. Sanjeev Sukumaran, Group Chief Operating Officer, Mr. Vikas Maheshwari, Group Chief Financial Officer, and Mr. Saurabh Bhandari, Group Business Analyst.

I will now hand over the call to the Rainbow Management Team for their opening comments.

Over to you, sir.

Dr. Ramesh Kancharla: Thank you, Rahul, and a very happy new year to everyone. I hope you all had a splendid new year celebration. It is my pleasure to welcome you to our earnings call for the third quarter and nine months of the current financial year.

On operations front, the current quarter performance is relatively slower, because of various reasons like delayed onset of monsoon and inconsistent rainfall. As such, both the second and third quarter performances have been relatively lower compared to the seasonally strong Q2, Q3 historically. Although the third quarter mirrored the previous one, our proactive measures taken in the second quarter to streamline costs have paid off.

However, the specialty services, which includes the paediatrics super-specialty services, obstetrics and tertiary care services continued to do well through the quarter. We encountered disruptions due to cyclone Michaung, which affected operations for about 10 days to 15 days in

Tamil Nadu and Andhra Pradesh, impacting our facilities in Chennai, Vijayawada, and Vishakhapatnam. Despite these environmental headwinds, our company has achieved its highest Revenue and EBITDA numbers, demonstrating our operational and financial resilience.

Turning to the financial specifics for the third quarter, we have witnessed a positive growth across the board. Our revenues increased by 9.6%, reaching INR335.9 crores. EBITDA grew by 10.6%, amounting to INR118.1 crores and PAT rose by 7.4%, reaching INR62.5 crores. For the quarter-under review, our occupancy rates stood at 50.8%.

Highlighting the specifics, our mature hospitals recorded an occupancy of 57.6% and our new hospitals reported an occupancy rate of 37.8%, indicative of the nascent stage of their growth trajectory and also an effect of cyclone at these new hospitals in Tamil Nadu and Andhra Pradesh.

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Moving to expansion plans,

■ I'm happy to share that we commenced operations at our 8th hospital in Central City, Hyderabad, with 60 beds on 4th January, 2024. With this addition, Rainbow has a bed strength of 890, comprising of one hub and seven spokes in Hyderabad City. This will further strengthen our hub and spoke network in Hyderabad.

■ Our new spoke hospitals in Anna Nagar (~80 beds) and Sarjapur, Bangalore (~90 beds), are in concluding stages and will be inaugurated very soon, in the next few weeks' time.

■ The project work for the additional block at Hydernagar, 50 beds in Hyderabad, is in concluding stages, we'll commence operations within this quarter.

■ With these additions, we are on course to add 280 beds in the current financial year as budgeted.

■ Project work at

the spoke hospital at Hennur, Bangalore, (~60 beds), and a regional greenfield project of 100 beds at Rajahmundry, Andhra Pradesh is progressing well and is expected to commence operations towards the end of the next financial year.

■ We have signed up for a new greenfield hospital in Coimbatore, of 125 beds. The builder has received the permissions from the government agencies, and the project work will soon commence, probably in February.

■ The status on NCR land parcels remains the same as that of the last quarter. We completed 100% payment to HSVP, and land parcels, for two land parcels in Gurgaon, and received the allotment letter for these land parcels. Efforts are underway in collaboration with the government entities to secure possession of these parcels.

Simultaneously, we are working actively with the architects in planning and designing of these two hospitals.

We maintain a confident outlook and envision that these expansions will strengthen our presence in growing cities of Hyderabad, Bengaluru, Chennai, and the National Capital Region. Beyond our growth plans, I would like to shed some light on significant achievements that underscore our dedication to periodic healthcare excellence.

■ Our commitment to advancing quaternary care has shown tangible results. In the first nine months of this fiscal year, we successfully completed ~685 cardiac surgeries and procedures along with 55 transplants including liver, kidney, and bone marrow transplants.

■ I would like to present a few clinical scenarios which demonstrate our clinical excellence. A six-month-old infant was air lifted from Raipur to our hub hospital in Bengaluru. This baby was suffering from severe renal failure and high blood pressure as a result of Hemolytic Uremic Syndrome (HUS). It's a rare medical complication of E. coli toxins post-diarrheal episodes. Our team promptly responded to the parents' request to airlift the infant and was taken in pediatric intensive care under the supervision of a pediatric nephrologist in Bengaluru. This baby was severely unwell and required ventilation, number of plasma exchanges and dialysis. The baby recovered fully in two weeks' time and was discharged back to Raipur.

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■ In another clinical scenario, a 10-year-old child presented with an episode of left-sided seizures. This child was evaluated by a periodic neurologist and an MRI demonstrated an arteriovenous malformation with a blood clot. After discussing with the parents, the patient underwent immediate neurosurgical examination of the arteriovenous malformations. Post-operatively, the child recovered very well without any neurological deficiencies and discharged home in four days' time.

■ In another clinical scenario, a four-year-old child was referred from another pediatric hospital in Hyderabad with severe breathing difficulties with significant stridor for two weeks' duration. Since this child was in and out of the hospital as an inpatient and was not improving, referred to Rainbow Children's Hospital for specialty care. This child was initially evaluated for various causes of respiratory distress and was suspected to have an obstructive upper airway. CT angiogram demonstrated a pulmonary sling, (pulmonary artery compressing the windpipe). The cardiac surgeon at our Cardiac Institute performed re-implantation surgery to connect the left pulmonary artery to the main pulmonary artery with an end-to-side anastomosis. Post-surgery, the child improved significantly from breathing difficulties as well as stridor. With these cases, I want to emphasize the need of multidisciplinary care for paediatrics under one roof to deliver excellent clinical outcomes.

The company has also taken a noble initiative of performing health checkups of marginalized people affected by Cyclone Michaung in Chennai and surrounding areas.

With this, I would

like to pass the mic to our Group CFO, Mr. Vikas Maheshwari to take you through the financial update. Thank you once again for joining us today. We look forward to your questions and insights as we move forward. Thank you very much.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you and thanks for attending this teleconference.

I'm happy to brief the financial performance and the key developments of Rainbow Hospital for the third quarter and the first nine months of financial FY '24.

■ Our operating revenue for the quarter stood at INR335.9 crores, reflecting a growth of 9.6% when compared to the corresponding quarter of the previous financial year. For the first nine months, our revenue stood at INR955.8 crores, reflecting a growth of 11.6% when compared to nine months of the previous financial year.

■ Our EBITDA for the third quarter amounted to INR118 crores, marking a 10.6% growth compared to the same period last year. For the nine months, our EBITDA stood at INR323.3 crores, reflecting a growth of 8.4% when compared to the first nine months of the previous financial year.

■ The EBITDA margins remain steady for the current quarter at 35.1%, while for the nine months our EBITDA margin is 33.8%.

■ The profit after tax for the quarter is INR62.6 crores, marking a growth of 7.4% in comparison to the corresponding quarter of the last financial year. The increased depreciation expense to the tune of ~ INR4.5 crores is mainly towards the new unit impacted PAT growth. For the nine months, our PAT stood at INR167.2 crores,

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reflecting a growth of 5.5% when compared to nine months of the previous financial year.

■ In terms of the operational performance, both outpatient and inpatient volumes for the quarter remained steady with a growth of 1% respectively when compared to the corresponding period of the last financial year. The reasons are already well explained by Dr. Ramesh, CMD. Deliveries which are independent of the season grew by 3% compared to the corresponding period of the last year.

■ Our payor mix continued to remain robust and balanced for the quarter, where 50.4% of the revenue coming from the insurance and the balance 49.6% coming from the cash patients. For the nine months, the payor mix stands at 50.4% cash and 49.6% as insurance.

■ Furthermore, our international business was approximately 3% of our total business for the third quarter and approximately 3.4% for the first nine months.

■ I am pleased to inform that our company's balance sheet remains very robust with a net cash position of INR495 crores as of 31st December and will support our ongoing capital expenditure plan as outlined by Dr. Ramesh, Chairman and Managing Director. Given our current cash flow and anticipated internal accruals in the coming years, we remain confident in our ability to complete all planned capital expenditures through internal accruals without any debt financing.

■ During the quarter, the company has invested approximately INR60 crores in the capital expenditure.

With these insights, I conclude my financial update. I now invite questions and suggestions from the participants. Thank you very much.

Moderator: Thank you very much, sir. We will now begin the question-and-answer session. The first question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Hi, good morning. Thank you for the opportunity. Sir, first question. While occupancy has been significantly lower this quarter on a year-on-year basis, our margins have been fairly resilient. One reason can be the strong ARPOB growth and lower ALOS. Are there any other reasons which led to margins remaining relatively steady in this quarter?

Dr. Ramesh Kancharla: Thank you, Alankar. normally Q3 will definitely be very strong on the seasonal businesses. But because there is no season at

all due to various reasons of untimely rains and also shortfall of rains and other things. Let's put it this way, it's fairly healthy year.

We do see this kind of year following a very busy year, fairly quiet and a healthy year. Business went on very well in tertiary care, super-specialty paediatrics, obstetrics and also the quaternary care which is growing significantly with the transplant business all those things. So, these are all the high ARPOB business. When you have the almost flat or lower seasonal business, your ARPOB automatically goes up, which we have been witnessing in the current quarter and also the previous quarter. So, that's actually has pushed up the ARPOB. As you said, the relatively lower ALOS plus the superior case mix are the two things which have actually pushed up the ARPOB. In a normal scenario, we would see moderation of the ARPOBs in the second and third quarter because of the seasonal business, which has not happened this time.

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Alankar Garude: Understood, sir. Anything on the cost side which you can highlight or basically it was business as usual in this quarter on the cost side?

Dr. Ramesh Kancharla: We have been disciplined because we sensed it in the second quarter itself that the year is likely to be a kind of healthy. Now, what happens is by the time we are in August and getting into September, we will know exactly how to forecast. As I said earlier in the call, we have been very disciplined with the cost and also various other measures taken to stick to the expenditure budget.

Alankar Garude: Understood, sir. And my second question is quite a few hospital companies have highlighted about higher cost of construction and equipment in the last few years. Can you update us on how the per bed capex requirements for us across Greenfield, Brownfield and if you can break that up across hubs and spokes? How have these capex requirements per bed changed in the last few years?

Dr. Ramesh Kancharla: I think we have not done the Greenfield hospitals in the last one and a half year time. The hospitals that have come up in the last one year is Financial district and the hospital at Himayat Nagar in Hyderabad. It has gone up definitely. We have not computed the total project yet completely. Definitely it has gone up. I would guess that the costs would have probably gone up

by 20%. This is my sense. Maybe I am not sure about the Greenfield project whether it has gone up even further because of cement and steel. The Brownfield project my expectation is between 15% to 20% run up on the cost because everything has gone up including the lighting fixtures, electrical, all the MEPs, cement, steel, everything has gone up.

Alankar Garude: Understood, sir. I have more, I will come back in the queue, sir. Thank you.

Dr. Ramesh Kancharla: Thank you.

Moderator: Thank you. The next question is from the line of Nitesh Dutt from Burman Capital. Please go ahead.

Nitesh Dutt: Hi, thank you for the opportunity. I have a question regarding our occupancy levels this quarter.

These are some of our newer hospitals cannibalizing some of our mature existing hospitals and also is there an impact of increased competition for some of our hospitals and due to which the occupancy is lower?

Dr. Ramesh Kancharla: Yes. I think definitely there will cannibalization within the urban spoke model as there will be an extent of the patient movement happens because of the proximity between the two hospitals. People choose to go one of the hospitals. That happens but we recover within nine to 10 months because of adding new patients to us. This has been a scenario we have been witnessing for a long time. Second thing is that yes, competition in this sense definitely there is an increased awareness of children's health care. A lot of smaller children's hospitals not the size of Rainbow but smaller between 15 beds- 30 beds children's

hospitals or doctors grouping together and doing some hospitals - this has definitely increased. As we move forward hospitals like ours will do more of complex work and also some of the patients will go to the nearby hospitals. May be this will
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become a little more organized Earlier they were going to the nursing homes in future, they might go to paediatrics hospitals.

Second important thing is the reference from the districts obviously for neonatal cases. Every year about 250 neonatologists comes out from the training in the country and a lot of people (neonatologists) are moving to districts. Hence more trained persons are now available so that will start doing some degree of intensification in the references from the districts, and will reduce some of the borderline things, like babies who are 1.5 kilos or 1.2 kilos, people who are less than 1 kilos tend to come to the main city. This will happen over a period of time. The market size is also increasing and awareness is increasing. This is a balance between supply versus demand.

Nitesh Dutt: Understood. Thanks for that. Second question, is it possible for you to provide occupancy data or at least occupancy trends by city? Lastly, what is the expected normalised occupancy levels going forward if you have a view there?

Dr. Ramesh Kancharla: Okay. So if we go back and look at it, last year we have done clocked about an average of 55.6% occupancy and this year occupancies have obviously come down because of the seasonality and the healthy periods, long healthy periods, those are the things. What we see is, at the Group level we look at occupancy ~55% at matured state. Plus, we are adding beds constantly. Each year we are adding about 150 beds. Perhaps, this year almost 280 beds are going to be added. This is almost like 20%-25% of the existing bed capacity. That will dilute the occupancy.

I think what we aim and look at is that mature hospitals occupancy of north of 60% and overall consolidated occupancy is about 55%. We would do extremely well in terms of providing quality care as well as financial performance.

Because unlike multi-specialties we have differential areas in the children's hospitals and also we cannot keep them in areas which we are not suppose like -- an ICU is very restricted. Intensive cares are restricted. Isolation wards are restricted. We can't keep the children mixed with the women. There are a lot of other restrictions, which is why occupancy levels of 55% at group level is pretty good to achieve to post the best of the results.

Nitesh Dutt: Got it. Sir, can you provide any city level occupancy trends for example what happen in Hyderabad, Bangalore, and Chennai etcetera this quarter?

Dr. Ramesh Kancharla: We looked at it actually, what is the best way of presenting ourselves as a Group. We are constantly adding more beds in urban spoke models. What we felt is, it's a matured and maturing hospitals is probably a right way rather than taking city view and approach which kind of each city is different, so that how we stuck to it. We have tried to look at it but I think this probably gives you a better forecast for the future.

Nitesh Dutt: Sure. Thanks for the answers. I'll get back in the queue.

Moderator: Thank you. The next question is from the line of Rishi Mody from Marcellus Investment Managers. Please go ahead.

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Rishi Mody: I just wanted to get an understanding from the ARPOB growth. How much is the impact of mix change versus price hikes?

Dr. Ramesh Kancharla: Price hike we have taken at the beginning of the year. As discussed earlier, as the year panned out, ARPOB has been driven more by quality of case mix and the shorter ALOS. The less seasonal business pushed up the ARPOB probably by 5% or 6% more. Usually what happens is that if we look

at the occupancy is almost 11% down.

That moderation of occupancy, which is dominantly a seasonal business, definitely pushed up ARPOB relatively higher. This is more of a balancing which we have seen, our ARPOB increased. There's no significant price hikes being taken in between. It's more of a quality of business. It's specialty, paediatrics and surgical and quaternary care and intensive care. All these things have pushed the ARPOB higher as in a paediatrics hospital, these are high ARPOB business.

Rishi Mody: Understood. Secondly, just wanted to get an understanding with 220 beds coming in Q4. How much of the fixed cost will get upfronted and then how will the occupancy rates pan out for these 220 beds over the year? If you can give an understanding of that.

Dr. Ramesh Kancharla: Sorry, would you mind repeating the question?

Rishi Mody: Yes. Basically, what I wanted to understand is that you're coming up with 220 beds in Q4 this year. How much of a fixed cost upfronting will happen on account of these beds? Secondly, how will the occupancy rates pan out for these bed additions over the year?

Vikas Maheshwari: Rishi, this is a good question. As we prepare for the 220 beds opening, what is important is to look at the breakup of these hospitals. In Hydernagar, which is just an extension and we're opening the new beds for meeting the new requirements and the demand, basically. We don't think any cost pressures to come on that.

As far as another hospital which is Himayat Nagar, which is in Hyderabad, is build for demand and we don't expect much of the fixed cost or operational losses to come on that. We expect these two units which are opening in Hyderabad, consisting roughly, 110 beds should do well.

In a very short period, they should break even. As far as the opening of the Sarjapur and Anna Nagar is concerned, we have always guided that these markets will take 12 months to 15 months' time for the breakeven. We believe that we should be on those tracks.

Rishi Mody: Okay. Got it. So, if I would get a per-bed fixed cost versus per-bed variable cost for you guys, how much -- how would you split your cost between fixed and variable?

Vikas Maheshwari: See, in hospitals, if you bifurcate, it's all operational costs. If you look at doctors, paramedics, staff, nursing cost, this is the major cost. And then there's the pharmacy and consumable which is more or less a variable cost. Some part of the doctor's cost is fixed cost. Some part of the nursing cost is fixed cost. So, on an average, if they achieve INR3 crores of the revenue per month, we should be there at breakeven there, including all fixed cost and the variable cost.

Rishi Mody: Okay, INR3 crores, INR4 crores revenue per month.

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Vikas Maheshwari: Yes, INR3 crores, INR3.5 crores, we should be breakeven, yes.

Rishi Mody: Okay, got it. And finally, your ALOS has kind of had a step down. So just wanted to understand, have you done anything on the clinical side to reduce the ALOS and improve the capacity utilization for existing beds?

Vikas Maheshwari: It's actually more or less an issue of what type of cases coming to the hospital and what is our operational efficiency. How fast we can treat the kid faster, recover the kid faster, or the mother gets recovered faster, and we can discharge that. It's a question of little bit of operational efficiency also. I think this ALOS, what we are seeing is 2.58 for the nine months. I think the range should be between 2 to 3 days for ALOS. We should budget depending on season-to-season.

Rishi Mody: Right. Operational steps that have been taken to improve the ALOS so that understanding of how you all are thinking about it?

Dr. Ramesh Kancharla: In a hospital like us with the children and women, absolutely the patient discharge is going to be very, very important because this all are the people, who are very active.

When a child is in the recovering phase itself we plan for a discharge because of anticipated recovery by next day. Same is with the young mother.

So very, very different healthcare scenario because in the children and women hospitals because people tend to or want to or request for discharge very early which we also promote, which is why our ALOS of kind of been 2.6 to 2.7, the best of the scenario, 2.5 to the 2.7 between those two. This has been trend in the last six, seven, ten months. I don't think it would increase, it would go beyond 3, unless we kind of start looking at very long-term patients of post-surgical

or even the transplants nowadays, we are discharging them on the 12th day, 14th day. So that's how the children are.

Rishi Mody: Okay, got it. Thank you. That's it from my end.

Moderator: Thank you. The next question is from the line of Prashant Kutty from Sundaram Mutual Fund. Please go ahead.

Prashant Kutty: Hi, thank you for the opportunity, sir. Just one question over here in terms of, you can highlight about the occupancy being lower, but just to understand a little more about the mature hospitals.

Any reason as to why the occupancy kind of goes down so sharply, typically in a quarter like this, which is considered to be a seasonal quarter? Is it only the reason of floods, which your kind of highlighted, which kind of impacted the occupancy? Anything else you can read into it?

Dr. Ramesh Kancharla: Well, in the children's hospital, even the matured hospitals actually get affected by seasonal variations. Why it happens is that in a season, even intensive care gets busier, and the complications will increase, so the specialties get improved, and also surgical work goes up also.

This is the same scenario you see in multi-specialty also. Remember, when the season comes, like in the past, dengue or COVID, the increase in the occupancy will happen across the board.

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Not only kind of end up with the wards and other things, but also increased footfall for the inpatients and outpatients. It's kind of a multi-dimensional season.

Prashant Kutty: Sure, sure, understood. Also, secondly, in terms of your capacity expansion plans, could you probably kind of just outlay in terms of is there any change happening on that front? You could probably also speak a little bit more on the daily part of it, because that is one part that you're very confident about in the year 2025. So any changes happening from that perspective?

Saurabh Bhandari: Capex plan, sir. Any change in the capex plan?

Dr. Ramesh Kancharla: I think that in terms of the capex, as we were discussing earlier about it, cost of the beds have definitely gone up. However, our capex plan, in terms of the addition of beds, remains very intact. Whatever is the proposed expansions in the newer geographies, like our regional hospital in Rajahmundry, regional hospital in Coimbatore, which is going to be started, it will commence project work in the next month. And there's a hospital getting built in Hennur, Bangalore.

In Delhi, two hospitals are in the kind of pre-project phase, and we've gone for the architectural designs. We have still got a few things to sort out with the government before registering. And meanwhile, parallelly we are working with architects.

Prashant Kutty: Sure. And last point, sir, in terms of the, obviously the ARPOB, I'm going to just want to check with you. Are there any insurance-related hikes which can probably help the pricing going forward? Just want to check, in any of the places, any insurance-related benefits which might probably help in terms of the pricing?

Dr. Ramesh Kancharla: See, the most of the insurance tie-ups have been done in the last and in the current financial year time. So they've all signed up for two years. So it's very unlikely to see that any price hikes to the insurance

in this current year and in the coming year. So, the new hospitals, if we can get the insurance tie-ups quickly, that will be a bonus for us. So we're working really hard in various existing clusters in Hyderabad, Bangalore, Chennai.

We are pursuing because insurance companies to list these new hospitals, to empanel these hospitals sooner than later. Otherwise they usually take about six months to nine months' time.

We are actually trying; we are successful in Hyderabad. We want to see that happens in Bangalore and Chennai as well.

Prashant Kutty: So, suffice to say that the pricing will largely be driven by your mix, or case mix itself incrementally, is it at least for the next one or two years to come?

Dr. Ramesh Kancharla: Yes, sure.

Prashant Kutty: Thank you so much, and all the very best to you.

Moderator: Thank you. The next question is from the line of Himanshu Binani from Anand Rath. Please go ahead.

Himanshu Binani: Hi, so good day everyone, and thank you for taking my question. So, I have just two questions.

Number one on continuing with the previous participant's question on the capex per bed. So can

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you please provide us with the absolute capex per bed number for both the spoke and the hub hospitals? And if at all, there is any sort of like variation in case of the Greenfield and the Brownfield projects?

Vikas Maheshwari: So, as far as the capex is concerned, whatever we have guided, there is not much of the change in the hub and spoke models. Whatever we have guided based on the inflationary trend, which

we have seen in terms of furniture, chairs, medical equipment, and other ancillary equipment, whatever the pressures we have seen is the pricing of roughly 15% to 20%. So, as on date, as we speak, the hub and spoke, particularly the project which we have announced, they should cost between INR65 lakhs to INR75 lakhs per bed.

Himanshu Binani: Okay. And so, there is any sort of like variation in the spoke and the hub or the cost is the same?

Vikas Maheshwari: Pardon, sorry, your voice is very blurred actually.

Moderator: Sorry, your voice is muffled. If you're using your Bluetooth, I will request you to kindly remove it for some time to only ask questions.

Dr. Ramesh Kancharla: Definitely the hub hospital will be a little more expensive because of the equipment, various things. Same time, because of the size, the number of beds also get increased. So, that optimizes the Capex per bed. Still, we always take about 15% of the higher capex for the hub hospital.

Himanshu Binani: Got it, sir. And so, the second question was, pardon me for this question, basically due to my limited understanding of the sector. So, what I can see is that the average length of stay that has come down while the ARPOB has increased. And one of the factors for the increase in ARPOB was a better case mix. So, what I wanted to get a sense is that if the cases are complex or complicated, then that would result into a better ARPOB growth. Then how come the average length of stay has come down? So, maybe if you can give a sense.

Dr. Ramesh Kancharla: You know, the average length of stay, when you look at the obstetrics, it's extremely low, obstetrics, which is one of the best performing this year. Obstetrics, the average length of stay is less than two. The second important thing is the surgical side also length of stay, we have condensed it from previous year to now. The paediatrics surgical, which the volumes have increased significantly, most of them have done minimal access surgery. So, there are also low. And the other thing, even the transplant work, which we have been doing, which used to go for three weeks to four weeks staying in the hospital, it's come down to almost 11 days to 13 days in the recent past. Some of the babies, which comes to us for the critical illnesses, they get treated and also get tra

nsferred to the district sharing hospital. Because we have a lot of shared care hospitals, which send patients to us for the critical part of it. We send them back for the healing phase. All these things put together is one side.

Another one is that when you don't have much of seasonal component of that, that will get kind of moderated. That will actually give us an extra push to increase ARPOB. So, these are the combinations.

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Price increase, we have taken the last year.. So, I think the influence is already been reflected. If we go back on our earnings calls last year, we had a very impressive price hikes for Bangalore, Chennai, as well as for Hyderabad. So, that's really given us a kind of a, probably a kind of a pricing advantage, which will continue for this year and the next financial year also.

Himanshu Binani: Got it, sir. And sir, can you again quantify on the quantum of price hike which was taken on a consolidated basis and what is the frequency at which we have been taking price hikes?

Vikas Maheshwari: So, it's linked with the inflationary link. It should be between 6% to 7% on an average.

Himanshu Binani: This we take on a yearly basis?

Vikas Maheshwari: Yes, depending upon the situation we also react to the competition pricing. We benchmark and study that what the other hospitals are doing. And based on that, we take a decision on that. But yes, it should be linked with more or less on the inflationary impact.

Himanshu Binani: Got it. Thank you.

Moderator: Thank you. The next question is from the line of Karan Gupta from Varanium Capital. Please go ahead.

Karan Gupta: The overall industry is expanding and all the players are getting now aggressively expanding in terms of bed capacity. So, do you have any figure of, any data related to the doctors, right? On an average, how many doctors or paediatricians are getting graduated? So, just wanted to understand the set-up of the supply side of, or the availability of the doctors in this segment.

Dr. Ramesh Kancharla: See, we have specialty in children's hospital. So, for general supply, it may not be much, because for someone to become a consultant in our hospital, so we actually look at more of European standards, they have to be at least five years post-MD to be a consultant in the paediatrics or to be a paediatrician in Rainbow. We do train a lot of people within our system as well. Our focus of taking the paediatricians is very, very less. There's a lot more supply there for us, than having difficulty in recruiting them (paediatrician).

Where our interest and focus is intensive care group, with the neonatologist and paediatrics intensive care, under paediatrics, super specialties like neurology, nephrology, gastroenterology. This is where we look at the demand and supply. Things are much better now than what it used to be, 15 years ago, in the initial days. There are a lot of training centres, including ours, like there are three or four centres in the country for training paediatric neurologists, we are one of the centres.

So, we do actually produce significant number of specialty doctors, of which we always try to retain about 30% within the system. And we recruit people, significant numbers from outside the country, where they are trained, because they have done paediatrics degree in this country have gone out to UK or Canada for specialty training and after the specialty training want to come back, we give them opportunity to join us.

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Karan Gupta: Okay, so, if you need a permanent doctor, you don't work on the contract basis, or maybe, I mean, kind of case-to-case basis?

Dr. Ramesh Kancharla: No, Rainbow is absolutely a fulltime doctor engagement for long term career and this is how we have positioned ourselves. We do not have too much of moonlighting, short-term

term contracts, or

even partial contracts. Also we do not encourage (part time contracts), in the mainstream paediatrics and paediatrics specialty areas.

Karan Gupta: Okay. Because why I'm asking this question, is when everybody is expecting and expanding in terms of bed capacity, right? So, and doctors' availability is less. I think some of the reports saying that only one doctor is on over 100 patients, or maybe 1,000 patients. So, is it right? I mean, because this will, if you extrapolate this thing over the years, this will maybe impact on margin, or maybe ARPOB will reduce over the time, because supply constants will be there, in terms of doctors, and you are expanding in terms of bed capacity, so? There will be gap of...

Dr. Ramesh Kancharla: I'm not sure what you're trying to link. You know, if there's one number of doctors, who would reduce, I mean, I'm not sure, I'm not able to really understand your question.

Saurabh Bhandari: if there are more hospitals, more expansions, more beds, but availability of doctors is less compared to expansion.

Dr. Ramesh Kancharla: I think see, this is one of the reasons and at Rainbow, I mean, with the balance sheet we have, cash on the balance sheet, I could add 500 beds a year for next five years' time, but I'm not doing it. So, we are limiting about 200 beds, 250 beds a year. One of the reasons, is it's not only about the doctors, this is a full-time model, and also for the clients, the digitally native population, the young people, the children, they are the people, they need to buy your model. You need to demonstrate the ability to provide a high-quality care for the community.

Then they start looking at us as a fellow. This is a process which takes about two years to three years' time. In a new city it will take about three years' time. Therefore, it's very important that we can look at all the factors of execution; which isn't only project execution, but it's a medical execution of building the business. It is an organic business which we are to build. Unlike multi-specialty, there's no wear and tear problem, which are readily available. So, this is more of an emergency-based specialty, which is driven very strongly on the shoulders of the doctors, who are very well-qualified. It's not the number of doctors, it's one of what kind of quality and skill set they have is more important.

Karan Gupta: Okay. And second question is related to, what's the payback period in hub and spoke model, as you said, and what's the traction you are seeing from other mother and care hospitals, that they are referring their patients to you guys? Or what's the steps you are taking to increase this traction?

Vikas Maheshwari: I think there's a lot of voice noise here, I think coming nearer to your phone or something like that some voice disturbance there. Could not understand your question.

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Moderator: So, Mr. Gupta, may we request you to kindly re-join the queue for follow-up questions, please?

There are other participants are waiting for their turn. If you could, please. Thank you, sir. We'll take the next question from the line of Kunal Dhamesha from Macquarie Group. Please go ahead.

Kunal Dhamesha: Yes, thank you for the opportunity. So, first one, on the business model optimization. So, as you have alluded, that for mother and childcare, the optimum utilization or occupancy could be between 55% to 60%, given the requirement of NICU isolation, etcetera. But let's say, when we are coming up with new hospitals, are we seeing any optimization as to probably increase the NICU bed, or something which could lead to probably higher occupancy in those newer hospitals?

Dr. Ramesh Kancharla: Yes, definitely, lot of qualified doctors coming in, and there are definitely going to be more hospitals. Not probably children's hospitals, more in mother and child segments due to low capex and al

so easily replicable, more of extensions. Those are things that will keep coming up. They will definitely reduce number of referrals for example, the over and above on e-kilo babies. But

these hospitals will not be able to provide care for very extreme prematurity, the babies who are less than one-kilo, babies who are complex. They require larger systems.

So, in a way, in a short run, actually what we see is that there may be decrease in number of referrals, but in the long term, what happens is more complex will come to you, because when you position yourself as a kind of tertiary, quaternary care hospital, then the awareness becomes more, and even the people becomes more aware of what is to be done, where to go, which hospital to go. They choose hospital more wisely.

The market will increase in size. All these things happen, it's the dynamics. This is how we have seen a multispecialty has grown in large cities in the country, where four, five groups have kind of built a huge number of beds in all the four, five cities. Market increases significantly with awareness, and also the affordability going up, insurance segment going up. These are all the tailwinds which are going to push overall. It cannot be just one Rainbow Hospital covering any of our cities.

Multiple number of players will definitely come up. There will be some competition.

Eventually, what's important is, who provides the best care, better clinical outcomes, and good results, they stay with the premium. That will be well regarded and respected. That's what we have seen in Hyderabad. There are number of Children's hospitals in Hyderabad. Not only now, for the last seven, eight years, because of the resounding success of Rainbow Children's Hospital. But we continue to dominate and do well because people know that which is the hospital to go to when they become sick beyond a point.

Kunal Dhamesha: But, sir, my question is more on optimizing the model itself in a way that right now our optimal capacity or occupancy we are expecting is 55%, 60%. Would you be designing a new hospital in such a way that we can go up to 70%, 75% probably changing the mix of you know?

Dr. Ramesh Kancharla: Yes, you're right. For example, in matured markets like Hyderabad, we have increased the capacity in number of beds. We increased capacity of beds in Secunderabad from 50 to 100 a couple of years ago. We are increasing some beds in Hydernagar, which is an additional block Rainbow Children's Medicare Limited

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we are building next to the existing block. So, there are two ways about it. Do we do a very large hospital from the beginning? And it's going to be a huge capex. And also, as a spoke hospital, there are some limitations, some certain geographies. Certain geographies may need more beds. That will, time will tell us what kind of a call is we need to take. We can't do a spoke hospital, every spoke hospital, 100 -plus beds. But, at the same time, when there is a need and requirement, we can kind of ramp up. This is fine balance which we need to take at the right time.

Kunal Dhamesha: Sure, sir. And second question is on the differential between the rates between, let's say, two channels that we have, cash-paying and insurance channels. I just want to understand, for mother and childcare procedures, what's the differential for the patient? One is cash-paying patient, and another is insurance-paying patient. What could be the rate differential for us?

Dr. Ramesh Kancharla: I think, see, in the paediatric scenario, we are not a mother and child hospital, we are a predominantly paediatrics hospital, and we believe that maternity is a part of the larger format of paediatrics hospitals. This is how we position ourselves. Mother and child hospitals are small and their capability is very limited in many things.

So, generally, an insurance company with a tie-up with us, our cash and insurance tariff

is more

or less the same for the medical conditions, critical care, and everything. However, there is some packaging, which is done mainly in the maternity side, for the deliveries and those things. Definitely within insurance, especially the GIPSA, they have moderated significantly what they're paying (maternity side packaging). That's definitely there, I can't recall how much percentage-wise difference is there. The tariff is significantly different for GIPSA patients to other patients (maternity side packaging).

What we are trying to do is, to provide better facilities for the patients who pay cash. Obviously, we need to do that. So, they'll be sitting in the deluxe rooms, these people will be sitting in the single private rooms. We need to provide quality and also deliverables for people who are paying more. At the same time, insurance, we have to respect, because it's a large segment and they've got the ability to negotiate with you because of the large size of the patient pool.

Kunal Dhamesha: Sure, sir, thank you.

Moderator: Thank you. The next question is from the line of Charul Agrawal from Bank of America. Please go ahead.

Charul Agrawal: Hi, thank you for taking my question. My question is regarding the Hyderabad region. So, we have already reached around a thousand beds in the region. So, I wanted to understand what is the kind of headroom you see for Rainbow in that region, and how do you determine how much capacity the region can hold?

Dr. Ramesh Kancharla: Well, we've always built for demand and need in Hyderabad. The city has grown significantly in the last, 10 years' time. So, we have identified more geographies within the city. As we identify more geographies, which are becoming more of the micro-markets, then we do hospitals in those areas. I personally believe at this point of the time; we probably have got enough beds in
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Hyderabad city for the next three years' time. At the same time, I would not hesitate to add more beds if there's a demand and there's a further market there. Because this is one city where the Rainbow is super-strong, everyone knows about Rainbow. People would love to come to Rainbow Hospital. A lot of times, actually, my diligence basically comes from the patient side. If more-and-more people wanted a Rainbow Hospital over there (a particular micro market), we will set up the hospital over there. Depending on the micro-market, we see the size of the hospital. Is it 50 beds would be enough, or 75 beds are required. I think we have enough beds for now, and for next couple of years.

Charul Agrawal: I got it. Thank you, sir. That was my only question.

Dr. Ramesh Kancharla: Thank you.

Moderator: Thank you. The next question is from the line of Namit Arora from In Growth Capital. Please go ahead.

Namit Arora: Thank you for the opportunity. So, my question was around the Tier 2 opportunity. Clearly, you're very strong in the metro, and there is further demand. But from a longer-term view, let's say five-year or a 10-year view, are you looking at any Tier 2 locations strategically, given that you have all the clinical excellence and domain expertise, and there may be demand for you even in the Tier 2 cities? Thank you.

Dr. Ramesh Kancharla: Certainly, sir. We've been there in Vijayawada for the last 10-15 years' time, and we have one hospital in Vizag. These are not small hospitals, actually. They are regional hospitals, which are kind of 100-130 beds. We've done in Vijayawada about 135 beds, which is one of the very well-performing hospitals for us, with a huge reputation. Now, we have done in Vishakhapatnam two or three years ago. That's again doing well.

Now, the plan, we are doing one hospital in Rajahmundry, which is about 100 beds, in the project phase, and we have signed up in Coimbatore to do one hospital. These are all significant cities within Tier 2,

which we would like to do and have plans to connect the south strongly between Hyderabad, Chennai, and Bangalore. There are a number of Tier 2 cities out there.

Eventually, in three four years, we'll be connecting every 200 kilometres. There will be a Rainbow Hospital (every 200 KMs in targeted cities in AP and TN). to include larger population, and to facilitate the emergencies to the regional spokes to hub hospitals. That gives us a lot of leverage in terms of expertise and also transporting the sick children and babies. This is the plan.

Namit Arora: Got it. Thank you for your very detailed thoughts. Very helpful. Sir, I have just one more question. Is there a focus on attracting international patients as well, given that, you have a really excellent, and whether it's, let's say, Southeast Asia, or Africa, or Middle East, is that a market that you want to target for international medical tourism into India, or do you think the domestic opportunities that you are more focused on? Thank you.

Dr. Ramesh Kancharla: Domestic opportunities, obviously, is very large. Internationally, of course, there is an opportunity, if you look at the international, like Indian neighbourhood, or Africa, parts of
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Middle East, some of the Russian countries, a significant number of these countries are dependent on Indian healthcare. It's going to be similar with the paediatrics also.

Even now, paediatrics patients are coming to various hospitals, they're piggybacking to adult and coming to the multi-speciality hospital. We started international business in the last couple years, post-COVID, and this year, we are expecting to do about 3.5% to 4% of our top line from the international. We actually have a separate team working on it and we have been directly having MOUs with the governments.

Recently, we had done a MOU with Tanzania, and we have already got some governmental tie-ups with Oman and some other African countries, and done a tie-up with Zanzibar last week. We are actively working on International business. This is one of the things like adult healthcare, where eventually, for the children's healthcare, when we are able to build quaternary care more-and-more, the whole of developing world will come around to us.

Namit Arora: Got it, thank you very much, sir, and all the very best to the entire team. Thank you.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional

Equities. Please go ahead.

Alankar Garude: Yes, thank you again. Sir, can you comment on the progress of the Madhukar Hospital?

Vikas Maheshwari: Yes, Alankar, that's a great question. The hospital remains on the steady note as we discussed last quarter. What we have done in the Madhukar is that there is change of leadership. We have brought a new leadership and we expect them to drive the revenue and profitability from here. So that may take some three months to six months' time and all the right ingredients are already there now. We expect it to improve from here, Alankar.

Alankar Garude: So would it be fair to say that whatever challenges we have faced in the past, say, three years, four years, a large part of that has already been addressed, and it's just a matter of time. I mean, we are awaiting results from some of those initiatives?

Vikas Maheshwari: Yes, that's correct. So right ingredients are already put up. The doctor team, nursing team, management team, everything is set up there now. And it's a fresh leadership team. I think you will see the results in three months to six months' time, Alankar.

Alankar Garude: And by that if you can quantify that in terms of margins, how much time would it take, in your view, to reach the margin levels of, say, Bangalore or Chennai for this particular hospital?

Dr. Ramesh Kancharla: Alankar, this is Dr. Ramesh here, it would never, ever reach the Hyderabad or B

angalore levels

because this is not a hospital where we have incurred the capex. (This was low Capex, high rental model). We have done the hospitals but these beds are not included in any of our presentations or deck as we do medical management. This is not a capex heavy bed for us. We are only doing management of these beds. For 140 beds, our initial investment was only about INR40-45 crores.

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Second important thing is that we have engaged with the owner on a high rental basis. The structure is slightly different. Thus, huge cost goes as rentals. Third important thing is it's a DDA property where we have to offer some free beds to the government, The free treatment is going up significantly, about INR60 lakhs-70 lakhs a month.

Fourth, the minimum wages are pretty high in Delhi. Being a medical hospital in a Delhi with a high rental cost and the high HR and high doctor cost, that's all about it (high cost structure). If you look at some other group of hospitals, which I won't go on to name, have also got similar structure. Eventually, my dream is to do about 15% to 18% of EBITDA. I would be super happy if I do 15% of EBITDA at Madhukar Hospital.

What Madhukar has actually taught me, I'm in the South Delhi, number one, Number two, I understood the Delhi market, whatever, all the things. So, it's given a kind of a huge experience for me to move forward, which is why we are doing more hospitals in Gurgaon, spotting workplace in Noida.

That's where we are supposed to be. We can expect our operating models to go like Hyderabad or Bangalore in those areas (surrounding areas of Hyderabad and Bangalore), but not in the South Delhi or Central Delhi. Because of the structural cost issues and not being our hospital brownfield or greenfield. These are the issues which we have been addressing. The top lines, you can make it grow to an extent, but bottom line is always difficult because of the cost structures.

Alankar Garude: Fair enough, sir. And that 15% to 18% which you mentioned is at the unit level. That is not the Rainbows share...

Dr. Ramesh Kancharla: The unit level, that's confined to the Madhukar. Madhukar does about 15% of EBITA, which is pretty good because the low-capex beds being in South Delhi, maintaining that is pretty good. It adds value being in the South Delhi.

Alankar Garude: What's the outstanding amount now, sir? And how much have we paid in this quarter?

Vikas Maheshwari: So, more or less, remains the same, Alankar. It's roughly INR30 crores, which remain outstanding there. We are not much worried about that, as Dr. Ramesh has explained. We are there for some strategic reasons and get the learning. And some amount is also required for the working capital operations, etcetera, there. So, I expect this, the whole loan outstanding, which is there, it will take some time to nullify but we are not worried about that amount which we have invested there.

Alankar Garude: I understood. And maybe one last question. See, based on the data which you provide in the presentation on the mature and new hospital split, it appears that mature hospitals have grown by about 12% top line in nine months. But unfortunately, this data is not like-to-like, and hence, it's difficult to know the exact SSG for the mature hospitals.

Now, I mean, assuming the mature hospitals or the SSG data suggests about 11% to 12% top line growth in nine months, just wanted to understand whether the growth rates would vary

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significantly across, say, Hyderabad, Vijayawada, as well as Bangalore, which fall under the mature leg for us?

Vikas Maheshwari: No, so in Hyderabad, now we will have some maturing hospital, right? And then Bangalore, Chennai, both are maturing. So, see, what is this like-to-like comparison you're not able to do, because few hospitals

keeps moving from the threshold of the five years to the new limit, right?

So, that may be not giving the right picture to you. You can separately engage with Saurabh, who is handling our IR. He can give the correct mix, like-to-like, if you want to compare. He can give you some flavour on that.

Alankar Garude: Sure.

Moderator: Thank you very much. Alankar, sir, you can connect offline with the management, please. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for their closing remarks. Over to you, sir.

Vikas Maheshwari: Thank you all the participants for active participation and giving us the insight and the question-and-answer, which we have been able to answer. If any of the participant feels that they have not got the chance or their question remains not answered fully, they can connect us separately.

The contact phone numbers and the email IDs are already given in our presentation. So, they can connect separately. We will be happy to provide all the clarifications.

With this, I conclude the conference. I thank again all the participants.

Moderator: Thank you very much, sir. Thank you, members of the management. Ladies and gentlemen, on behalf of IIFL Securities Limited, that concludes this conference. We thank you for joining us and you may now disconnect your lines. Thank you.

This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy. Please also note that this document has been edited without changing much of the content, for enhancing the clarity of the discussion. No unpublished price sensitive information was shared/discussed on the call

Call: May 2024

May 27 , 2024

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.
Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.
Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q4 & FY24.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015 .

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on May 21, 2024, with regard to Financial Results of the Company for Q4 & FY24.

The said Transcript is also available on the website of the Company and can be accessed through the following link :

Analyst -Concall -Transcript -Q4-FY2024.pdf (d2sn5i18drfi94.cloudfront.net)

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited

Ashish Kapil
Company Secretary and Compliance Officer
Encl.: As above

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"Rainbow Children's Medicare Limited
Q4 FY '24 Earnings Conference Call "
May 21, 2024

Call Duration • 1 hour and 4 minutes

Management Speakers • Dr. Ramesh Kancharla – Chairman and Managing Director

• Mr. Sanjeev Sukumaran – Group Chief Operating Officer

• Mr. Vikas Maheshwari – Group Chief Financial Officer

• Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions • Ms. Banshi Desai , JP Morgan

• Ms. Damayanti Kerai, HSBC Securities and Capital Markets

• Mr. Nitesh Dutt, Burman Capital

• Mr. Karan Gupta, Varanium Capital

• Mr. Abhay Sahukar, Individual investor

• Ms. Dhara Patwa, SMIFS Limited

• Mr. Alankar Garude, Kotak Institutional Equities

• Mr. Harsh, Bandhan AMC
Moderator • Mr. Rahul Jeewani - IIFL Securities Limited

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Moderator: Ladies and gentlemen, good day and welcome to Rainbow Children's Medicare Limited Q4 FY24 Earnings Conference Call hosted by IIFL Securities Limited. As a reminder, all participants' lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch-tone phone.

Please note that this conference is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Thank you and over to you, sir.

Rahul Jeewani : Good morning, everyone. I am Rahul from IIFL Institutional Equities. I welcome you all to the fourth quarter earnings conference call of Rainbow Hospitals. From Rainbow, we have with us today Dr. Ramesh Kancharla, Chairman and Managing Director, Mr. Sanjeev Sukumaran, Group Chief Operating Officer, Mr. Vikas Maheshwari, Group CFO, and Mr. Surabh Bhandari, Head Investor Relations & Group Business Analyst. Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla : Thank you, Rahul. Good morning, everyone. I'm delighted to extend a warm welcome to the earnings call for the fourth quarter and the financial year 23-24. As I reflect on the quarter and the year, the last quarter witnessed the highest revenue and significant addition of the new beds across the group. However, the year witnessed flat trend without any seasonal uptick in

occupancy in Q2 and Q3, which normally are our seasonally strong quarters. The lower occupancies were a result of delayed monsoon, lower seasonal illness-based admissions, cyclone effect in Chennai and parts of Andhra Pradesh.

Despite these headwinds, we did well for the quarter and the year as a whole, both operationally and financially. There were so many milestones that we achieved during this quarter and the year.

- Firstly, we commissioned 280 beds in the last quarter across three new spoke hospitals, one each in Hyderabad, Bangalore and Chennai and also added a new block to the existing hospital at Hydernagar in Hyderabad City. Apart from these hospitals, we opened an outpatient clinic in Hennur, Bangalore City. This is the first time that we had such a large capacity getting commissioned in a short span of time. Our projects and operations team have meticulously executed on ground to ensure the projects were commissioned on time. With these expansions, Rainbow now operates 2,000+ beds including the managed beds at Delhi.

- The second highlight is I'm extremely happy to share the Rainbow Children's Hospital flagship facility in Bangalore was awarded the prestigious Gold Seal of Quality approval by the Joint Commission International, JCI. This recognition underscores our dedication to delivering world-class healthcare services to children. This makes us the only pediatric hospital chain in the country to have three hospitals accredited by JCI : the flagship facility hospitals in Hyderabad, Bangalore and the Kondapur fertility centre in Hyderabad.

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- We have expanded our IVF services across the group from three centers who were providing IVF services last year. Now we have about 11 centers offering IVF services. Fertility services have further strengthened our Birthright by Rainbow offering. We plan to add more IVF services, wherever possible in our existing hospitals and also in the upcoming new hospitals as well.

- During the quarter, the company recorded its highest ever revenue. The key operating metrics during the quarter have been resilient despite all the capex and challenges during the year. We have done well on maintaining the financial discipline. As I stated in the previous call, we sensed the probability of

having a muted season and have kept sharp focus on all the cost line items to optimize the cost.

With these, I would like to dwell into the financial metrics for the fourth quarter. In Q4 FY24, our revenue registered a growth of 7.6%, amounting to 341.1 crores. Similarly, our

EBITDA saw an increase of 7.7 %, reaching to 105.5 crores. And the PAT was down by 5.2% to 51.14 on account of higher depreciation and finance cost.

For the quarter under review, our occupancy rates stood at 48.5 %, highlighting specifics our mature hospitals recorded an occupancy of 59.2% . Additionally, our new hospitals reported an occupancy of 32.1% , which includes the newly opened hospitals in Hyderabad, Bangalore, and Chennai.

Coming to upcoming projects update :

- The spoke Hospital at Hennur , Bangalore of 60 beds and the regional spoke Hospital, Rajahmundry in Andhra Pradesh of 100 beds, are progressing well, and expected to commence operations in Q4 FY25.

- For the regional spoke Hospital in Coimbatore the project work has just begun. It is expected to take about 24 months for completion.

- We have now received procession letters for both land parcels in Gurgaon, and working to get the conveyance read done probably in a couple of weeks' time.

Simultaneously, we are in the process of finalizing the architectural designs and expect to do the ground breaking to happen in about three to four months' time.

Beyond our growth plans, I would like to shed light on some significant achievements that underscore our dedication to pediatric healthcare excellence.

- Our commitment to advancing coronary care has shown tangible results. In the last fiscal year, we successfully completed approximately 1,000 + cardiac surgeries, including cardiac cath procedures, along with 75 transplants, encompassing liver, kidney, and bone marrow.

- I would like to dwell on a couple of clinical cases, which were challenging ones. The five-year-old child was referred to Rainbow Children's Hospital, Banjara Hills, with a fulminant liver failure and a severe renal dysfunction. This child was ventilated with a liver support measure and the haemodialysis . We have evaluated for an urgent liver transplantation for a fulminant liver failure, but the donor, child's mother , there was Rainbow Children's Medicare Limited

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blood group mismatch, which poses serious problems during the transplant, that was mitigated with plasmapheresis and rituximab, which is an immunosuppressive agent, to prevent the rejection of graft -versus -host disease. The liver transplant was performed under great supervision by multiple teams. The child was very critical, and the preoperative and postoperative periods are very turbulent. However, the liver started functioning in a few days' time. That started improving the renal function as well and the overall general condition. The child was in ICU for 20 days and was discharged after six weeks very healthy.

- In another case, the 16 -year -old boy from Nairobi, Kenya, came to our Heart Institute with a pumping heart, rather bouncing left chest after a blunt abdominal injury while playing football. The child underwent a stenotomy for left ventricular aneurysmal repair in Nairobi 10 years ago. Once again, after the abdominal blunt injury, he developed aneurysm again in the same place. Therefore, our cardiologists have taken a unique approach of closing the aneurysm through the catheterization. A VSD device was deployed angiographically across the mouth of the aneurysm, and the position was confirmed by an angiogram. Post -procedure, the pumping heart, the bouncing heart completely disappeared with a normal cardiac function. This boy was discharged in 48 hours' time.

With these cases, I want to emphasize the need of a multidisciplinary approach for testing in quaternary care patients for better outcomes.

In conclusion, the year

was challenging but very eventful. However, I am optimistic and hopeful

as we move to the next year. We have put in a lot of hard work to strengthen our operations, added leadership teams in corporate sales , projects, nursing, and learning and development. This foundation will help us to achieve growth in the coming year.

As we celebrate our silver jubilee year of providing world -class children's health care in India, I extend my heartfelt gratitude to our doctors, nurses, and the entire Rainbow family, along with the valued stakeholders and young parents who have contributed to the incredible Rainbow journey.

With this, I would like to pass the mic to our group CFO, Mr. Vikas Maheshwari, to take you through the financial updates. Thank you once again for joining us today.

We look forward to your questions and insights as we move forward. Thank you.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you and thanks for attending this teleconference.

I am happy to brief the financial performance and key developments of Rainbow Hospitals for the fourth quarter and FY 2024.

- Our operating revenue for the quarter stood at INR341.1 crores, reflecting a growth of 7.6% when compared to the corresponding quarter of the previous financial year. For the full year, our revenue stood at INR1,297 crores, reflecting a growth of 10.5% when compared to the previous financial year.

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- Our EBITDA for the quarter amounted to INR105.5 crores, marking a growth of 7.7% compared to the same period last year. For the full year, our EBITDA stood at INR428.9 crores, reflecting a growth of 8.2% when compared to the previous financial year.
- The EBITDA margins for the current quarter were 30.9%, while for the full year, our EBITDA margin was 33.1%.
- The profit after tax for the quarter is INR51.1 crores, with a degrowth of 5.2% in comparison to the corresponding quarter of the last financial year. The increased depreciation and the interest expense to the tune of INR7.9 crores, mainly towards the new units impacted PAT growth. For the full year, our PAT stood at INR218.3 crores, reflecting a growth of 2.8% when compared to the previous financial year. For the full year, the impact of depreciation and interest costs on account of new units was INR12.5 crores.
- In terms of operational performance, the outpatient and inpatient growth was 3% and 1% respectively for the current year when compared to the last financial year.
- The deliveries grew by 7% compared to the last year. For the year, the ROC and ROE are 29.6% and 18.8% respectively. It is expected to go up as the new units commence operation and deliver healthy growth.
- Our payor mix continued to remain robust and balanced for the quarter, where 53.3% of the revenue coming from the insurance and the balance 46.7% coming from the cash patients. For the full year, the payor mix stands at 49.4% cash patients and 50.6% from the insurance segment.
- Our international business was 2.5% of our total business for the fourth quarter and 3.4% for the full year. Our total international business for the year was approximately INR44 crores.
- I am pleased to inform that our company's balance sheet remains robust with net cash position of INR515.9 crores as of 31st March of this year and will support our ongoing capital expenditure plan as outlined by Dr. Ramesh, Chairman and Managing Director. Given our current cash flow and anticipated internal accruals in the coming year, we remain confident in our ability to complete all planned capital expenditures through internal accruals without any debt financing.
- For the quarter, the company has invested approximately INR50 crores in the capital expenditure.
- I am pleased to share that the Board has recommended the final dividend of Rs.3 for the year.

With these insights, I conclude my financial

update. I now invite questions and suggestions from the participants. Thank you very much. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Banshi from JP Morgan. Please go ahead.

Banshi: Hi. Thank you for taking my question. My first question is on the IP and OP volume growth that we have seen this year. We understand this year was seasonally softer for us. But despite that, the volume growth of 1% and 3% both respectively for IP and OP seems very low. Is there any
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other reason besides the seasonality? And how should we think about a steady state volume growth for our business?

Dr. Ramesh Kancharla : Thank you, Ms. Banshi. The last year was kind of a year, we rarely see. This kind of flattish year. Usually, during the Q2 and Q3, we do see about an average of 6% uptick in the occupancies, resultant revenues as well, in both the quarters. That's completely not seen in the last year, which is unusual.

What I have experienced in the last 25 years of my journey is: we do see that there is a period of three-four years run, and then there's a pause for a year time, and then it gets settled after that. So apart from these, some of the kind of seasonal plus the local factors like the floods and cyclones, we haven't seen any other factors that really impacted the admissions and occupancy.

Banshi: Okay, so assuming this year to be a normal year in terms of monsoons, should we then assume like a good mid-single-digit kind of growth in IP volume?

Dr. Ramesh Kancharla : Yes. If we go back and see, we've done the revenues fairly well for the year and business has come from the obstetrics , intensive care services and specialty services . If we foresee, that the season is going to be normal with monsoons coming at the right time and overall there are no other significant factors . I don't foresee it (any significant factors) because of the elections everything is done.

I would expect this year is going to come back to good occupancies and volumes, and on top of it, last year we've been pretty busy with building up the capacity . This year , single most focus is to build the business.

Bansi: Understood, thanks. And my second question is on IVF expansion that we've announced. We've done 11 hospitals now where we are providing the services. There's significant ramp up this particular year. So just wanted to understand, this category has always remained quite promising. So what prompted us to be aggressive this particular year?

Are there any changes in regulation that has come through, and how should we think about this opportunity say over the next four to five years, how meaningful can it be for Rainbow ? And

what kind of invest ments will be required for you to grow this business?

Dr. Ramesh Kancharla : Okay. As we are building more and more spoke hospitals in Hyderabad, Bangalore, Chennai and Andhra Pradesh, we have seen is that our obstetrics volumes , delivery and outpatient numbers are going up significantly. So, the IVF will naturally grow within the system.

The patients have a lot of trust in us in terms of the woman's care. IVF addi tion at this point of the time, we thought and we deliberated and it started to make sense. We have our own captive population who are coming to Rainbow and also the people who are expecting to have a very ethical IVF. Look at all the centres this is what we have built.

We have not done any standalone IVF centre. They'v e all been built in the existing hospitals.

This is the approach we want to take, because we have 19 centers. We would try to do or add IVF centre wherever the opportunity is good. We would like to see the ramp up of the fertility

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services. That is the consci

ous decision we have taken. And we are optimistic about the growth of IVF services within the gamut of Rainbow.

Bansi: And in terms of investment, sir, if you can call out, what are the kind of investments are we looking at?

Dr. Ramesh Kancharla : I think IVF generally when people do it, it's a very cookie cutter model. It's about 1.25 Cr to 1.5 Cr per centre . But when we are doing within the hospitals, especially the new hospitals or existing hospitals, we have done it as an inclusive. And when you look at our centers, I mean, the quality -wise, they're very well taken care.

So, I would say that it's going to be kind of probably 1.5 Cr to 1.75 Cr. I don't have an accurate number at the moment of how much capex we have invested , because they always go t ogether along with the overall unit development rather than a separate capex (for IVF), a separate space. Because space is already there a nd we try to re -organize ourselves to fill the spaces within the existing facilities.

Bansi: Okay. And fair to assume this would be very profitable?

Dr. Ramesh Kancharla : Yes, I think so. You'll have to see that probably a year down the line when we'll have experience to let you know that what are the profitabilities in IVF. Because at this point of time, we are looking to build this like normally a full-time doctor engagement model . We are recruiting the doctors who are well -trained and can build their capabilit ies in IVF and with good results. That's what we're focusing on now. I think as we pass the year we'll know what is the kind of profitability is there in the IVF.

Bansi: Got it. I have more questions. I'll join back the queue .

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets, India Private Limited. Please go ahead.

Damayanti Kerai: Hi. Good morning and thank you for the opportunity. My first question is on your payor mix.

So now insurance is more than 50% of your total payer mix. So do you see more headroom to grow it? And if you can also update us i f you are done with tariff hike and rate negotiation for the cash as well as insurance channel?

Vikas Maheshwari: Damayanti, it's a good question. F or the quarter, we have seen the uptick of the insurance business, which is around 53%. And for the full yea r, it's almost 50 -50 with cash and insurance.

But since our penetration is increasing in Bangalore, Chennai and Hyderabad, and which are more of, you know, formal economy is very great here and a lot of overseas companies and the domestic companies which are setting up their service centers, we expect it to keep on increasing.

Also, the awareness about the insurance in particular we have seen is going up.

So we expect that, yes. It should see some uptick going forward. But let us wait and see how it progres ses quarter -on-quarter.. As far as the renewal of GIPSA and the cash insurance tariff is

concerned , it is an ongoing process. And then as and when those tariff are due for renewal, we
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keep taking it up with the insurance companies and finalize the tariff. So this is what the status
as on date , Damayanti .

Damayanti Kerai: So your renewal cycle is only two years, right ?

Vikas Maheshwari: Correct.

Damayanti Kerai: that's when you renegotiate your tariff, etcetera?

Vikas Maheshwari: Correct.

Damayanti Kerai: Okay.

Vikas Maheshwari: Generally, it is taking longer period for the finalization nowadays because between both parties ,
there is a push on the tariff side. So, it is taking a little longer cycle though it (tariff renewal) is
due for two years. But our team is working on that, and we are finalizing as and when we feel
that it is a good tariff to close.

Damayanti Kerai: Okay. And this cash part, do you take first of April every year or there is any other cycle? How
does that cash

division work?

Vikas Maheshwari: On cash tariff, we keep evaluating our cost structure and the market pricing and based on both
the components, we take a call. It is not necessary that it should be the year beginning exercise.
It can be a mid-year exercise. As and when the demand requires to react on that, we react on
that. We have a tariff committee who keeps looking at the cost and the pricing and the pricing
which our competitors are charging. And based on that assessment, the committee takes a
decision on that.

Damayanti Kerai: Sure. And then I guess we have seen very strong growth in the ARPOB numbers across the
quarter for last fiscal. And I believe the majority of this is driven by this improving penetration
in the insurance channel. Do you attribute case mix to be a big contributor there?

Vikas Maheshwari: Mostly the case mix , we have witnessed that our quaternary and tertiary care business has gone
up where the A RPP is higher. Obviously, ALOS also plays a role in deciding the ARPOB . But
what we have seen on an average, the quaternary care business has gone up in the last year. And
that is why we are seeing the uptick on the ARPOB . Obviously, there is some impact of price
increase which we have taken in between, but mostly from the tertiary and the quaternary care which has gone up.

Damayanti Kerai : Okay. And just to confirm, this price hike which you take is in single digit, right? Maybe a lower
single digit which you might have taken. And then next year...

Vikas Maheshwari: Actually, the inflation adjusted between 5% to 7% Damayanti .

Damayanti Kerai: 5% to 7%. Okay. And my final question is on your strategy for international patient segment, so around
3.4% which you mentioned for FY '24. So do you have any target for next two to three
years which you would like to achieve?

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Dr. Ramesh Kancharla : Damayanti, this (International Business) is a fairly nascent initiative. However, we have done
fairly well ~3.6% of revenue last year. We expect to do about 5% for the current year FY '24-25.
This is how we look at it. So , what we are trying to do is to engage more with the government s
the African and neighbouring countries and those things.

Second initiative we want to do is to get connected to more and more doctors. Children's business
is always better to connect with the doctors. If the doctor starts helping you, that becomes more
organic.

So, this is what we have been trying to do rather than trying to go and get the business through
the intermediaries. This is what our long -term focus is for now. Being a super specialty children's
hospital, we will try building our reputation and brand across the Indian neighborhood and also
in Africa and the Middle East and parts of the country. It's a long journey, but I think that's going
to be more sustainable and going to be more organic. That's our view of the international
business. However, we expect about 5% business in FY24 -25.

Moderator: Thank you. The next question is from the line of Nitesh Dutt from Burman Capital. Please go
ahead.

Nitesh Dutt : Hi, good morning. My first question is on the new hospitals operationalized this quarter. So what
was the EBITDA loss from these new hospitals? And also, what was the occupancy and
ARPOB ? And how soon do you expect breakeven from this?

Vikas Maheshwari: Hi, Nitesh. The three units which we have opened at three cities respectively, were opened in
the last quarter. And the EBITDA losses was in the range of roughly INR 5 Crores to INR 6
Crores from these units. And as far as the occupancy and the revenue, I think this is very early

stage to judge and give any comment on that.

From our side, whatever the clinical side and the doctor's side is required, the recruitment, etcetera, we have done that. We are also seeing some progress on the revenue ramp-up

across

all the three hospitals. But let us wait for another quarter before we give any judgment on that, how we feel about these three hospitals, ramp-up and guidance on that.

Nitesh Dutt : Sure. Second question is the mature hospitals in Q4 had a higher occupancy versus in Q3. So what was the reason for this occupancy increase? Because overall the occupancy has gone down?

Dr. Ramesh Kancharla : Yes, that's correct. As you know, our business is a little cyclical business. And the quarter second is generally going to be the best quarter. And quarter three and quarter four, successively comes down.

But if you look at our peak quarter, which was the September quarter, and the March quarter is more or less a flattish one. So that is what we are seeing. And second, we have added a few beds, right? We have in our unit, which is the matured hospital, we have added 50 beds there. So, the impact of that is also reflecting on the occupancy.

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Nitesh Dutt : Just asking that Q4 should be a lower quarter in terms of occupancy. But actually for mature hospitals, occupancy in Q3 was 57%. In Q4 it is 59%. So just trying to understand the seasonality pattern. And it's not repeating for mature hospitals, but occurring for new hospitals?

Dr. Ramesh Kancharla : Yes, the variability is not that significant (this year) to be honest. I think that, it's probably that occupancy is slightly more because of the Hyderabad and the Andhra cluster as they have clocked a little more occupancy, which is why we are seeing a little uptick in the occupancy compared to the last quarter.

But overall when you look at it (Q4 FY24 Vs Q4 FY23), the last year (Q4 FY23) was a completely different year with a lot of viral infections and the occupancy was kind of all over, it was very high in the last year at a group level.

Nitesh Dutt : Got it. And so when this normalizes, assuming occupancy has come back in FY '25 and seasonality pattern also comes back, so will our case mix be getting back to earlier levels? So meaning the ARPOB that we are seeing currently, do we expect it to come down for FY '25 if occupancy comes back?

Dr. Ramesh Kancharla : See what happens in our business, we expect our case mix to improve year-on-year in terms of the specialty services, obstetrics, intensive care services as well as quaternary care. That is the trajectory we would like to see. The occupancies will actually vary because of the seasonal business, which is secondary care, which is more of pediatric admissions, the season related admissions and those things.

That (Secondary admissions) will give a bump up of the occupancy. Usually you see in the Q2, Q3 with some variability. When you have this blend, what results in is higher outpatient turnover and higher overall occupancy levels and you may have noted that. If you see we had a 14% ARPOB growth because there's not much of a seasonal business this year. When you have a lot of seasonal business, ARPOBs will get kind of marginalized to an extent.

Our regular business was pretty good last year but the seasonal business was muted. As for expectation this year, we expect normal rainfall and more of a regular year. We will have higher occupancies, we would expect occupancy of about 55% at the year level. That will give us a both a blend of good complex business as well as a seasonal business and also higher outpatient volumes. Overall results in higher occupancies, higher revenues and a better EBITDA.

Nitesh Dutt : Correct. Thanks for that, sir. Any expectations of guidance for ARPOB for FY '25?

Dr. Ramesh Kancharla : I think last year, when we look at it, we have taken most of the insurance price hikes. (negotiations have been done and the price optimization, the price hike has been taken). Last year significant complex business has happened. We expect a lot of

seasonal business (this year),

then our ARPOB maybe may stay the same or may actually be a kind of a plus or minus 1%.

I'm not looking at the ARPOB growth this year. We would like to see the growth in numbers, both in IPs and OPDs and overall occupancies. That's what I would look at the trajectory this year.

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Moderator: Thank you. The next question is from the line of Karan Gupta from Varanium Capital. Please go ahead.

Karan Gupta: So my first question is regarding seasonality. I'm not able to understand the seasonality part because last year, first quarter, we said there was the holiday of doctors. So we were down in the quarter one. In the second quarter, we were seeing that there is a heavy rain in the southern part of India. So we were down.

In the third quarter, we had festivals. So that's why the seasonality part will come. And the fourth quarter, so what is the seasonality part? You said quarter two and quarter three were down last year because of the seasonality. And now you're seeing quarter three and quarter four. So just say the seasonality and what's the reason behind that?

Dr. Ramesh Kancharla : Yes, Mr. Gupta. Actually, we are a children's hospital. We don't do government business, CGHS, state government programs, any of those things. We are purely a private hospital. Therefore, most of our business is very organic. In the children's hospitals, we have three different types of businesses.

One is the regular pediatric business and outpatients plus regular inpatient admissions. Number two is the intensive care services. Number three is specialties and the quaternary care services, which includes all the super specialties, neurology, gastroenterology, nephrology, plus organ transplantation, all that stuff. And number four is the other part that is significant is the Obstetrics and gynaecology.

What affects the seasonality to a large extent is the pediatric admissions which are kind of pneumonias, fevers, viral illnesses, dengue, whatever is a whole bucket of season, monsoon-related or winter-related viruses, which makes the community become sick.

Children are more vulnerable and prone so they require a greater number of admissions. That's what it is. As a result, sometimes because of the heavy load of seasonal illnesses, some of the children become more and more sicker. They require intensive care services. Therefore, the intensive care services also become busier because of some encephalitis, meningitis, this kind of serious illnesses. So that usually happens from July till December, which is Q2 and Q3. (Q3 was inadvertently stated as Q4 over call. Regret the error)

Q1 is actually April, May, June which is the holidays for children and parents and everyone.

People go out of cities, and it's fairly healthy season as well. You don't see people falling sick.

What do we do in that kind of time? During those times, we did a lot of children's surgeries, ENT surgeries, pediatric surgeries, and obstetrics and gynecology. They take the lead, and also cardiac work and transplant work both take the lead.

So occupancies will be lower in these things because of the absence of seasonal illness and also people going out of the town. And the Q4, again, is going to be kind of January to March.

Generally, this period is marked with a fairly healthy season. And in March, exam starts, people start preparing for exams. In Q4 also not so much of higher outpatients or inpatient admissions will be there, there are routine admissions. This is how overall our business takes a varied pattern. The Q1 and Q4 are lower. Q2 and Q3 are higher.

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Karan Gupta: Okay. Second, as related to our patient acquisition part, what's the update you were seeing in the last quarter that small mother

and care children hospitals, we were approaching that those hospitals don't have much specialty treatment, so they will refer to us. So what the steps you are taking in and any advertisement or this kind of thing you are doing for the patient acquisition that we are providing the best services for the children care so patients will attract to us? Anything like that?

Dr. Ramesh Kancharla : See, our positioning is still different. We are not a mother and baby hospital or mother and child hospital. We are children's hospital. We also have a significant maternity. That bucket is Birthright, by Rainbow. It is perinatal service. So our face is children's hospital. Our drive is children's hospital. We have 24/7 children's services, 24/7 emergency services and 24/7 transport services.

That's why people know us as a children's hospital. I think that's what our positioning is. We don't conflict with the boutique centers like delivery centers and the birthing centers. I think there is absolute clarity in the southern part of the country and also Delhi people know very well what the rainbow children's hospital is for.

Karan Gupta: Okay. My question was if we are approaching to small mother care hospitals that don't have much specialty treatment. So if the hospitals, those who are just giving birth to the babies? So if there are any critical illness happening to the children that they don't care.

Dr. Ramesh Kancharla : We do get referrals. We do get referrals from mother and baby hospitals. We do get referrals from small children's hospitals. The services we offer are level three, level four, newborn and pediatric intensive care services. Obviously, if someone is short of it, they will refer the patients. Our patients themselves are very clever nowadays. They know where to go. When a child is sick beyond the point, when the baby is sick particularly when it's a smaller baby, less than one

kilogram, they know that what is kind of a way to go.

So, there are two ways it happens. The hospital refers and also a lot of times the parents themselves decide to come over. They come and take opinions. They come and talk to us. They come and see. Then they make a decision.

Karan Gupta: Okay. Yes, thanks for the answers.

Moderator: Thank you. The next question is from the line of Abhay Sahukar, an individual investor. Please go ahead.

Abhay Sahukar: Hello. Good morning, My question is on the growth side of the business. Let's say in FY '22 and 21, our base was very small, and we were growing at all 20, 20 plus growth in the top line. But after Q2 of FY '23, our revenues are really growing at 8 % to 9%. So what growth can we expect in the top line and the bottom line for the coming years?

Dr. Ramesh Kancharla : I think we need to see that our growth is probably going to be in mid-teens. This is what we would expect. If we do mid-teens revenue growth, then automatically the bottom line will follow that.

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Abhay Sahukar: Okay. Can you quantify the number in the future growth?

Dr. Ramesh Kancharla : I think, see, as we move forward the organization is becoming larger, and we continue to add new beds. We have added almost about 400 beds or something in the last two years' time, and we have about another 600 in the pipeline for the next three years' time. The organization, the hospitals are in the building stage and therefore growth is an important factor and also the margins and we are fairly cognizant about these.

We have always given our guidance of where our margins are going to be. But it depends on sometimes how much new capacity is coming in. That (Capacity addition) will definitely moderate to an extent the margins.

So therefore, we'll continue to grow about 15%, 18% on revenues and the bottom line closely follows it. This is how we have been very disciplined in

terms of our growth versus the top line

and bottom line. This is the discipline we've always followed. We continue to follow that but at the same time the nation needs a larger footprint of children's hospitals and building that is also very important.

Abhay Sahukar: Okay, great. So my next question is on the network of our brand record in the Chennai. So what are we going to improve or make our framework strategies to improve our brand record in the Chennai and Andhra Pradesh?

Dr. Ramesh Kancharla : I think in South we are a pretty household name in Telangana, Andhra Pradesh and in Chennai. I won't say that entire Tamil Nadu but in Chennai and Bangalore, in these areas the brand recall is pretty high. There are two ways of going about it. One is inclusivity with the doctors. Being a children's hospital, our inclusivity is mostly with the pediatricians. That's one group of that. But, generally for the public awareness, I think nowadays whatever you do ADL, BTL, they're not really going to work very well. It's more of digital now. And one of the things, recently we started doing more and more is suppose in Chennai, the digital media in the local language Tamil is very strong.

So, we are trying to be more regional, speak in the regional language than at a national level.

When you go in Chennai, we should do the Tamil more. And similarly, do more of Kannada in Karnataka. So, this is a lever which we are actually taking it more strongly now.

Abhay Sahukar: Thank you so much. That's all from my side.

Moderator: Thank you. The next question is from the line of Dhara Patwa from SMIF Limited. Please go ahead.

Dhara Patwa: Thank you for the opportunity. Just one question, sir. My question was an IVF unit. Usually it is a non-invasive procedure and a daycare procedure. So do you think the capex of INR 1.5 crores to INR 1.7 crores per bed is quite high? So is the cost higher on the clinical talent side or on the infrastructure side? Thank you.

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Dr. Ramesh Kancharla : We look at the board. We are a quality hospital. We always look at our services to be futuristic.

I mean whatever the hospitals we build, whatever we do, the standards are going to be looked at very differently. So as I told earlier, I don't know what is the exact amount for the IVF specific.

It is difficult to delineate also because we are doing it as a part of our existing space. If I do it standalone then I know exactly the capex. It is difficult to pinpoint the exact amount for IVF.

When you look at the capex for the equipment, yes, you can do that.

When you do an interior environment, it has to be a Pari-passu for the rest of the hospital. It can't be downgraded. The standalone IVF centers are very different. They can take whatever the approach they want to take it, but unfortunately being a hospital and also the level of the significance of building a good hospital for children and women's care has always been a higher standard for us.

Dhara Patwa: Sure, sir. Thanks.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Hi, good morning, everyone. Sir, can you provide some qualitative comments on the performance across Bangalore, Chennai and the Madhukar Hospital in FY24?

Vikas Maheshwari: Hi, Alankar. As far as Bangalore and Chennai is concerned, I will say compared to the Hyderabad standards, the Bangalore has picked up well. All the three units are working fine, doing good. The new unit which we have opened at Sarjapur has seen a very good traction. It is growing as per our expectations. So that cluster is doing very well. And we expect that as we ramp up our operations there and grow more units in the future, I think the brand recall and the growth will be similar to what we are seeing in Hyderabad

or slightly less than what we are seeing in Hyderabad.

As far as Chennai is concerned, that is the cluster which is slightly below our expectations. But the team is working hard on the ground. We have seen some change in the leadership there, putting up more effort on the marketing and sales there. And we expect that this year it should see the performance as per expectations. So this the brief commentary on the cluster.

As far as Delhi is concerned, again, we have put up a lot of effort on that. And for the March quarter, I will say the benefits are not being seen much there. But hopefully in this year, we will see the good traction coming up for whatever the seeds we have shown in the last one year, the efforts which we have taken, that should start showing some results.

Alankar Garude: Thank you for that, Vikas. And just to follow up on Madhukar, what is the outstanding amount now? And safer to assume that the hospital is making EBITDA margins in the range of early double digits even today?

Vikas Maheshwari: It is a high single digit, Alankar on the EBITDA margins and is improving. If I say whether the performance has improved versus last year, the answer is yes. And they have been able to pay the loan. So as on date 31st March, the loan outstanding was roughly INR 20 crores -21 crores .

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So they have paid some INR 5-6 Crores in the last year. Whatever the EBITDA they have generated, we have taken it back.

Alankar Garude: Understood. The second question is, so we have two land parcels in Gurgaon. I know it's a bit early, but do you broadly plan to commence operations at both the hub and spoke around the same time? And if it's yes, would that be a slight deviation from your usual strategy of opening a larger hospital first and then adding spokes?

Vikas Maheshwari: Alankar, it's a good question. Now we are seeing the light at the end of the tunnel as we have got the possession letter. We have paid for the registration fee and we expect it to get registered in a couple of weeks. Hopefully, we'll get it done. At the same time, the management has also appointed the architect, the conceptual design, etcetera, work in progress. And we expect it to have a ground breaking in three to four months' time.

The effort is that the first our smaller hospital which is the spoke hospital be up and running. So the effort is that we should start that. And within six months of that time period, the hub hospital should also be ready up and running. And so that is what we are planning as on date for this. As far as your questions regarding the first start hub and then a spoke, I think that this spoke hospital will be also be a large hospital. And then it is good to go if it is ready ahead of the time (hub), right? We already have some presence in Delhi market.

Dr. Ramesh Kancharla: Well, Alankar, as we earlier guided the large (hub) hospital is going to be 300 plus beds and is going to be a standalone pediatric super specialty hospital largely focused on the tertiary and quaternary care. This is how we want to look at it. Like how you look at the proper adult multi-specialty. Maybe a little more focused one. That's what as a doctor, what I've been running the healthcare 25 years. I see that opportunity. I see that the country requires. That's number one. The hub hospital is not going to be focused for Gurgaon alone. That's going to be focused for the north, perhaps for India, and also for the international. So the positioning of the hospital is going to be very, very different. The second thing is that, you know, the spoke hospital, another one on the Gulf course road, which is going to be about 125 beds hospital. So that will be for large intensive care and obstetrics, large obstetrics, and large newborn intensive care, It's mainly a place here, significant role for the

e Gurgaon and also expanding portions of the Gurgaon and Faridabad and those areas. So this is how we would like to position that. We don't want to have too much of a confusion between the children's hospital of high standard or children's super specialty hospital of more focused approach. This is how we envisage these two hospitals. Alankar Garude: And one, finally, one suggestion, given that we now have a sizable number of hospitals, 19 in number, which are largely across four major clusters, it would be really helpful if we can start providing the financial and operating metrics across clusters. This mature and new hospital, that number keeps on changing and that issue will only get exaggerated as we keep on adding more hospitals. So for a like-to-like comparison, it would be really helpful if we get more details on each of these clusters?

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Dr. Ramesh Kancharla : We will review and see how it can be done. Because one of the reasons what we see is the clusters which are maturing.

In Chennai's we have added one more spoke recently. There's going to be one more spoke as we move forward. Bangalore requires more spokes . we are continuously building our capacity . I always believe in this , that in the Hub and spoke model for children's hospital , capacity building is important. That is how actually you're able to cover the city at large and also provide wider coverage of the city for children's emergencies. That is the vision of Rainbow.

So, which is why we always felt that mature and new hospitals bucket probably makes more sense in terms of presenting to the investors and analysts. We will review once again Mr. Alankar and see what's right at the end of the day.

Alankar Garude: Sure, sir. Thank you.

Dr. Ramesh Kancharla : Thank you.

Moderator: The next question is from the line of Harsh from Bandhan AMC. Please go ahead.

Harsh: Just two quick questions. One, between matured and new hospitals, just from a fourth quarter perspective, the ARPOB improvement, and this goes back to the comment that you made, that there's some level of case mix improvement as well as the pricing. So you have the quaternary and specialized care mix improving.

I'm just trying to understand whether only new hospitals are seeing a significant level of case mix improvement and the pricing component is substantial for matured hospitals or it goes both ways. As in matured hospitals are also seeing, continue to see a lot of case mix improvement as well as pricing. I'm just trying to understand the split of that ARPOB improvement?

Dr. Ramesh Kancharla : I think what's important is what category of hospitals . Is there difference between a hub hospital and the spokes?

The spokes as they mature definitely will have better realization because you have a lot of outpatients and it keep building up, which is a significant part of our revenue, number one.

Number two is that as we move forward, the ARPOB may not increase significantly as there is limited variability in the spoke hospitals.

When it comes to the hub hospitals, ARPOB may play some role because the case mix in the higher category improves.

That's how we look at it. So it varies between the hub and spokes , but

at the same time, when you see ARPOBs between the hub and spoke, the variability may be about 5%, not very significant because the spokes does a lot of turnover.

The ALOS will be shorter much shorter (at spoke hospitals) than hub hospital. And the outpatients are one and a half fold higher than hub hospital. So these are the factors which actually make sense. It creates a nice blend between the hub and spoke. We wouldn't have so much of variability of the ARPOB (between hub and spoke hospitals) .

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Harsh: And on previous comment on insurance pricing, just

to clarify, you are expecting some insurance

contracts repricing to pick into FY'25. I understand that there will be multiple moving parts to

this, but you are awaiting insurance pricing in certain contracts and certain regions. Is that correct for FY'25?

Vikas Maheshwari : Yes. It's a time cycle when it lands for the renewal. When the se contracts keep coming for renewal, we keep doing it. So it's an ongoing process. Every quarter there will be something for the renewal.

Harsh: Sure, sir. Thank you.

Moderator: The next follow-up question is from the line of Nitesh Dutt from Burman Capital. Please go ahead. Hello. Nitesh, your line has been unmuted. Please go ahead with your question.

Nitesh Dutt: Hi. Thanks for the opportunity again. So my question is on, I am trying to deconstruct our revenue into ARPOB and occupied bed days. So when I look at our occupied bed days, basically a multiplication of the total operational beds and occupancy for FY'23 and FY'24, the occupied

bed days have gone down by 3%.

But at the same time, our inpatient volume has gone up by 1%. So just trying to understand what's the discrepancy here. Am I missing something?

Vikas Maheshwari: No. Your calculation is 100% correct, Nitesh. What is the function? See, the ARPOB is a function of multiple things. What you have to see What is our payor mix, case mix, and what is operational efficiency, which is ALOS . So, if you consider this ALOS also into the picture, I think that will give you a better picture .

Our ALOS has slightly come down from last financial years from 2.76 days to I think, 2.65 days.

So that is one major portion which plays on that part.

Nitesh Dutt: So ARPOB is clearer. It's trying to understand on the volume aspect, right? So revenue is a function of ARPOB and occupied bed days. So ARPOB has gone up by 14%. Our revenue has gone up by 10-odd percent, and bed days have gone down by 3%, 3.5%. But at the same time, the volume of patients have increased. So inpatient volume has increased by 1%. So just trying to understand on that?

Vikas Maheshwari: So what we have seen that our ARPP per patient was gone up by roughly 8%. So that is also one kicker, meaning the high quaternary care work and the tertiary care which we are doing .. So if you look, I think we are not publishing that information, but our IP-ARPP was roughly INR 94,000 last year, which has gone to roughly INR1,01,000 in this year.

So which reflects off, as we are saying is a high quality and tertiary care. 8% increase in IP - ARPP . Though the volume was down (IP volume is up 1% in FY24 compared to FY23) , but the revenue growth has come because of the higher ARPP .

Nitesh Dutt: So I think what you're talking about average revenue per patient, that gets taken care into ARPOB?

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Vikas Maheshwari: So there is an OP -ARPP also which comes. That has also seen some uptake last year by 9%. So that is also one factor which comes on that factor.

Nitesh Dutt: Got it. Thanks.

Moderator: Thank you. We'll take this as a last question. And I'll hand the conference over to the management for closing comments.

Vikas Maheshwari: Thank you very much for asking right questions and picking up a few insights for us. We expect to connect with all of you for the quarter one FY'25 conference shortly. Thank you everybody for joining us. Thank you very much.

Moderator: Thank you. On behalf of IIFL Securities Limited that concludes this conference. Thank you for joining us. And you may now disconnect your lines. Thank you.

Vikas Maheshwari: Thank you.

This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy. Please also note that this document has been edited without changing much of the content, for e

nhancing the clarity of the discussion. No unpublished price sensitive information was shared/discussed on the call.

Call: Aug 2024

August 21 , 2024

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.
Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.
Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q1 FY25.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015 ("SEBI Listing Regulations") .

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on August 14, 2024, with regard to Financial Results of the Company for Q1 FY25.

The said Transcript is also available on the website of the Company and can be accessed through the following link :

<https://d2sn5i18drfi94.cloudfront.net/investor-relations/reports/Analyst-Concall-Transcript-Q1-FY25.pdf>

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited

Ashish Kapil
Company Secretary and Compliance Officer
Encl.: As above
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"Rainbow Children's Medicare Limited
Q1 FY '25 Earnings Conference Call "
August 14, 2024

Call Duration • 55 Minutes

Management Speakers • Dr. Ramesh Kancharla – Chairman and Managing Director

- Mr. Sanjeev Sukumaran – Group Chief Operating Officer
- Mr. Vikas Maheshwari – Group Chief Financial Officer
- Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions • Mr. Pritesh Chedda , Lucky Investment

- Ms. Damayanti Kerai, HSBC Securities and Capital Markets
- Mr. Mahesh, Atal Investment Managers
- Mr. Harsh, Bandhan AMC
- Ms. Bansi Desai , JP Morgan

- Mr. Rahul Jeewani, IIFL Securities Limited
 - Mr. Alankar Garude, Kotak Institutional Equities
 - Anand Mishra from Philip Capital
- Moderator • Mr. Rahul Jeewani - IIFL Securities Limited

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Moderator: Ladies and gentlemen, good day and welcome to Rainbow Children's Medicare Limited, Q1 FY '25 Earnings Conference Call hosted by IIFL Securities Limited. As a reminder, all participants will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeevani from IIFL Securities Ltd. Thank you and over to you, sir.

Rahul Jeevani: Hi. Good morning, everyone. I welcome you all to the first quarter earnings conference call of Rainbow Hospitals hosted by IIFL Institutional Equities. From Rainbow, we have with us Dr. Ramesh Kancharla, Chairman and Managing Director, Mr. Sanjeev Sukumaran, Group Chief Operating Officer, Mr. Vikas Maheshwari, Group CFO and Mr. Saurabh Bhandari, Head Investor Relations and Group Business Analyst. Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla: Thank you, Rahul. Good morning, everyone. I'm delighted to extend a warm welcome to the earnings call for the first quarter of financial year FY25.

Our quarterly performance reflects our commitment to expanding footprint, operational excellence and our continued investment in technologies that enhance patient care and drive efficiency. This quarter, we have delivered a robust financial and operational performance. We have remained focused on optimizing our processes, ensuring that our services run seamlessly and efficiently to enhance the network effect of our hub-and-spoke model.

- In the last quarter, we marked a significant milestone by commissioning three new hospitals, one each in Hyderabad, Bangalore and Chennai, adding a total of 280 beds.

This brings our total capacity to over 2,000 beds, including managed beds in New Delhi.

- We delivered a credible 15% revenue growth in a seasonally weak quarter. We have seen progressive growth across all the operational metrics compared to the previous year. Our outpatient footfalls, inpatient admissions and deliveries are all showing significant improvement. We are optimistic about maintaining and surpassing this growth in the coming quarters.

- Our EBITDA growth has been slightly muted primarily due to the costs associated with our newly opened hospitals. The trajectory of the operations at our new hospitals is as per our expectations and showing growth month on month.

- For the Q1 FY25, our revenue registered a growth of 15%, amounting to INR 330.2 crores. Similarly, our EBITDA increased by 7%, reaching to INR 93.7 crores, while the PAT was down by 4.2% to INR 39.7 crores due to higher depreciation and finance costs.

- Our overall occupancy rate for the quarter was 42.4%, with the matured hospitals achieving 50% occupancy, and the new hospitals, including the recently commissioned hospitals, recording a 27.6% occupancy rate. These figures indicate that we have sufficient headroom for growth in the coming quarters.

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- During the quarter, we have launched a pilot project, Adult Vaccination Outreach Program (AVON), with the top five vaccine manufacturing companies to cater to the immunization needs of the families. AVON initiative will cover vaccines including Influenza, HPV, Pneumococcal and Shingles.

- The progress on IVF has been promising and has garnered good traction. We expect to keep up the momentum on the IVF side and grow in the coming quarters.

- Our ongoing investment in technology has achieved tangible results. By leveraging advanced digital tools, we have been able to significantly impact the patient experience, from streamlining the appointment

processes to the faster and more accurate diagnostics. These innovations not only enhance the patient's satisfaction, but also contribute to the operational efficiencies.

- In addition to this enhancing operational capabilities, we have made significant strides in data and cyber security. Recognizing the clinical importance of the safeguarding patient data, we have invested in robust security measures that keep us ahead of potential threats. This proactive approach ensures that our patients' information is secure and reinforces their trust in our services.

Highlighting the project's status :

- The development of our new regional hub in Rajahmundry (~100 beds), and the spoke hospitals at Hen nur, Bangalore (~60 beds) is progressing well. Both are expected to commence operations in Q4 FY25.

- The project work for the regional hub hospital in Coimbatore (~ 130 beds) has just started and is expected to complete in 24 months' time.

- We have registered the land parcels in G urugram at sector 44 and 56. For sector 44, we have submitted the drawings to HS VP for building sanction approvals. Further, for the sector 56, we will be submitting the drawings to HS VP in the coming week.

Beyond our growth plans, I would like to highlight some significant achievements that underscore our dedication to pediatric and maternal healthcare excellence.

- A 15-year-old child was being treated elsewhere for a seizure disorder for the last 12 years , with normal neuroimaging and EEG on several occasions by various doctors, including neurologists. When the child finally reached Rainbow , our pediatric neurologist carefully evaluated and discovered the child is having a very low heart rate and also has a partial hearing loss. So , he referred the child to our pediatric cardiologist suspecting a genetic disorder and there may be a heart involvement. The cardiologist performed the ECG and echocardiogram and detected there is a long QT interval on the ECG, which is known to cause bradycardias and arrhythmias. The genetic test confirmed the long QT intervals as part of Jervell and Lange -Nielsen Syndrome , which is a very, very rare condition. And in this particular condition, these children will have a long QT on ECG. They have proved to have bradycardias and sometimes arrhythmias and death. In this case, the child was being treated, unfortunately, for

12 long years with antiepileptics without diagnosis . I think the great clinical achievement of our pediatric neurologists and cardiologist was correct diagnosis . The Rainbow Children's Medicare Limited

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child was then treated with very simple medicines called beta blockers and mexiletine to treat for long-term basis.

- In another case, a 32-year-old lady with a complex medical history of ischemic heart disease and a stent in place with a chronic renal failure and diabetes was advised not to become pregnant because of her multiple comorbidities. However, the lady accidentally became pregnant and sought our help to continue the pregnancy. Our perinatal team responded very positively and included an adult cardiologist and the renal physician's team and followed up pregnancy very closely and optimized her care time to time and ensured the pregnancy sails as smooth as possible. Eventually, an elective cesarean section was planned and the lady delivered a healthy baby, and mother was so happy with that outcome .

These cases emphasize that importance of a multidisciplinary approach for tertiary and quaternary care patients to achieve better outcomes.

As we celebrate the Silver Jubilee of Rainbow , in providing world-class children's health care in India, I extend my heartfelt gratitude to our doctors, nurses and the entire Rainbow family, along with our valued stakeholders and the young parents who have contributed to this incredible journey.

As we look ahead, we remain committed

to maintaining our leadership in the pediatric and perinatal health care through continuous innovation, leadership development, operational excellence and an unwavering focus on patient care.

We are confident that our strategic investments will continue to drive our growth and deliver long-term value to our stakeholders.

With this, I will now pass the mic to our group CFO, Mr. Vikas Maheshwari to take you through the financial update. Thank you once again for joining us today. We look forward to your questions and insights as we move forward.

Vikas Maheshwari: Thank you sir. A very good morning to all of you and thank you for attending this teleconference. I am pleased to brief you on the financial performance and the key developments of the Rainbow Hospitals for the first quarter of FY25.

- Our operating revenue for the quarter stood at INR 330 Crores, reflecting a 15% growth compared to the corresponding quarter of the previous financial year.

- Our EBITDA for the first quarter amounted to INR 93.7 Crores marking a 6.9% growth compared to the same period last year with an EBITDA margin of 28.4%.
- The profit after tax for the quarter is INR39.7 crores, representing a 4.2% decline compared to the corresponding quarter of the last financial year, mainly due to increased depreciation and interest expenses related to our new units.
- In terms of operational performance, outpatient and inpatients growth was very healthy at 12% and 10% respectively for the quarter compared to the last year. Delivery volume also showed an impressive growth of 10% year -on-year growth.

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- For the quarter, our non-annualized ROC is at 5.2% and is expected to rise as the new units ramp up their operations.
- Our pay mix continues to be robust and balanced with 52% of the revenue coming from the insurance segment and the remaining 48% from the cash patients.
- Additionally, our international business accounted for 2.9% of our total revenue for the first quarter amounting to INR 9.4 crores.
- Our balance sheet remains strong with a net cash position of INR518 crores as of June 30, 2024 which will support our ongoing capital expenditure plans. Given our current cash flow and anticipated internal accruals in the coming years, we are confident in our ability to complete all planned capital expenditures through internal accruals without acquiring debt financing.
- During the quarter, the company invested roughly INR56.7 crores in ongoing capital expenditures.
- I am happy to share that we successfully conducted our AGM on July 30, 2024, with all the resolutions passed by the shareholders and the company has paid the dividend on the same day.

With these insights, I conclude my financial update. I will now invite questions and suggestions from the participants. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Pritesh Chedda from Lucky Investment. Please go ahead, sir.

Pritesh Chedda: Sir on the existing hospitals what was the revenue growth and the margin profile on a Y-o-Y basis? If you could share that because I was looking at the presentation, you've given all the other data points except the like-to-like comparison?

Vikas Maheshwari: On the matured units the revenue growth has been healthy. It has been at around 7% to 8% growth year-on-year basis and we expect this growth to go further up as we enter into the season.

Typically, the quarter first remains very muted. Still, we could show growth of 7.3% year-on-year basis and as we enter into the season which is September and December, you'll see more healthy growth in the double digits going forward.

Pritesh Chedda: But when we are comparing on a Y-o-Y basis which is a like-to-like quarter, why shouldn't the growth be, why is the growth single digit?

Vikas Maheshwari: Your point is correct. We have working on this. The last quarter has been quite hot due to unprecedented heat waves and people have taken leaves and have been travelling due to the (national general) elections (& state election in Andhra Pradesh). There may be other reasons as well, where we have seen this slightly less growth in terms of revenue growth.

Pritesh Chedda: And at the 7% growth what is the margin profile there? Is there any erosion in margin?

Vikas Maheshwari: We are not seeing any margin erosion. In fact, in the matured hospitals they operate on the operating leverage. So any incremental revenue growth will always improve the EBITDA or EBITDA margins.

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Pritesh Chedda: But at 7% growth, will the margin be a similar number only, is that the fair question?

Vikas Maheshwari: It should be the same. It's not changed. It has slightly improved only. We are not seeing any sign of EBITDA margin impact on a year-on-year basis.

Pritesh Chedda: And my second question is that on the newer hospitals, usually what is the trajectory of occupancy that year 1, year 2, year 3 usually you budget at while making these investments. And at what level of occupancy usually these hospitals break even?

Dr. Ramesh Kancharla: It depends on where these hospitals are. Usually in Hyderabad, we expect it to break even within the first year itself.

Pritesh Chedda: And at what occupancy, sir?

Dr. Ramesh Kancharla : Usually about 35% occupancy we break even. And at a place like Bangalore, it will take about 12 to 15 months time. Chennai might take about 15 to 18 months time. That's how we look at it. Based on the maturity of the hub and spoke model, we tend to clock that way.

Pritesh Chedda: So your prime market Hyderabad has year 1 of break even and everywhere else is less than 2 years basically, anywhere between 1.5, 2. And at the same time over of 35% or less?

Dr. Ramesh Kancharla : I think more or less 35% occupancy.

Pritesh Chedda: Okay, done. Thank you very much.

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: Hi, good morning and thank you for the opportunity. My first question is on costs related to new units. So just want to understand whether the first quarter numbers reflect most of the costs which you have incurred or you expect to incur on an ongoing basis for the new units or do you have a plan to add on more doctors, more people, etc there? So some color on that will be helpful?

Vikas Maheshwari: Damayanti, I think the quarter first whatever the cost structure we have seen for our new units, (which we started), I think more or less settles there. Obviously, we started these units at the beginning of the March quarter. The doctors have started joining April, May and June. You will see some incremental cost might come for the new units, but more or less I think 90%-95% the cost is settled there. As we grow these units and new clinical teams join, there may be some impact but not major. So what you are seeing is more or less settled there.

Damayanti Kerai: Understood. So in terms of margin, we understand your September and December quarter very healthy, but on an annual basis, how should we think about improvement or change from the FY24 level?

Dr. Ramesh Kancharla : I think, Damayanti just to anticipate that, our growth (Revenue) probably for this year will be kind of late teens. When you do a late teens revenue growth, I think overall, most of the operating metrics and financial metrics will be pretty healthy.

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Damayanti Kerai: Understood. And my next question is on the Hyderabad market, your competitors are getting more aggressive in mother and child care segment. So what are your

thoughts on the competitive

scenario and have you seen any impact in terms of a loss of clinical talent, etc?

Dr. Ramesh Kancharla : Rainbow is very large and has strong forte. I mean, I don't think we will not be kind of impacted directly through the loss of clinical talent because the brand drives the business here and not individuals. Second important thing is, of course, when a lot of doctors, hospitals have started coming in the similar space, certainly some competition will come.

But I mean with our credibility in Hyderabad city, we may not be impacted on the long-term basis. It's (competition) not something new to us. We have been seeing this competition almost more than a decade now.

Damayanti Kerai: Sure. And my last question is, you mentioned your IVF program is picking up momentum. So if you can just update us on like how do you plan to take it up and very broadly like over the next two to three years, how do you want to grow this business? And how critical this is for your existing offerings?

Dr. Ramesh Kancharla : Okay. Well, we had three in the past, one in Bangalore, Vijayawada and Hyderabad. Now we've got to about 11 total. Now I think all of them are commissioned, all the embryology services have started, the doctors have been recruited, the business plan is laid out. And I think whatever the early footfalls that we are seeing are pretty impressive and are pretty organic. What we are seeing is what we expected of Rainbow - to drive the organic footfalls, which is happening. And they are quite impressive numbers.

And as I think, we'll have to see that how this number is going to translate to the cycles. And it will take probably at least a couple of quarters for us to have a better comprehension on that.

Damayanti Kerai: So how much you have invested so far?

Vikas Maheshwari: The investment is not too much because we have expanded into our own hospitals only. It is the equipment and the furniture, fixtures, that's all. That is not very significant. It's very small, minimal.

Dr. Ramesh Kancharla : They're not standalone. They're all within the existing hospital, either new hospitals or existing hospitals. We have included them under the birthright perinatal services.

Damayanti Kerai: Okay. Thank you. I'll get back in the queue.

Moderator: Thank you very much. The next question is from the line of Mr. Mahesh from Atal Investment Managers. Please go ahead.

Mahesh: Sir, just wanted to know the new hospitals that we have just added, like I could see that last few quarters we are still at that occupancy levels of 27%, 28% there. So what are we doing? I mean, the ARPOB in the new hospitals is somewhere around 52,000 something. Is it because of high cost that these hospitals are not showing the occupancy of these areas? I mean, it is going as per

your plan. How exactly do you see this new hospitals? Because the earlier guy, you told that
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35% it will be a breakeven. So we are still at 27% for the new hospitals. So when do you see this break-even levels reaching if you can please throw some light?

Dr. Ramesh Kancharla : Mahesh, I've earlier already stated about the timelines of the break-even. We are in children's healthcare space and also the maternal services. These are organic businesses. These are not aggregation businesses like multi-specialties, like a lot of sales driven, those things. These services will take its own time because we do recruit doctors on a full-time basis. I mean, medical teams have to settle and people have to recognize, and then it starts growing organically. Why we say, we probably are doing the first year break-even in Hyderabad, because of the redistribution of the patients and also the huge brand reputation. You say there is a Rainbow, then people automatically go there. To an extent, that also happens in Ban

galore. In Chennai the traction will take a little longer time.

This is how we work. Generally, the acquisition of the practices is much less in pediatrics and is more of an organic drive. The timelines what we see, we're quite happy with our timelines of about 12 months to 18 months' time for the spoke hospitals to break-even at 35% occupancy. And one other thing is that we will take at least ~ 6 months to get all the insurance sorted out (when we start a hospital). So that's again, they're (Insurance Companies / TPA's) are not going to give you the insurance approval as soon as you start the hospital. It's always challenging. So, we are still waiting for some more permissions, some more insurance tie-ups. They'll all happen in 6 to 9 months' time. These are all the things which actually takes its own time to settle.

Mahesh: All right. So just for my general knowledge, just wanted to know what do we do to attract this polyclinic that you have in the Hyderabad region, but the spread of polyclinics is huge. I mean, the clinics where they don't have this beds and all, but they are doing it on a very lower scale. How do you - how are you reaching out to those people? And I mean, in terms of working from their site, how do you see that thing coping up in the future? Because I see Hyderabad is, I mean, currently in Hyderabad there are a lot of polyclinics operating. So how do you see that particular thing?

Dr. Ramesh Kancharla : I mean, you see, the polyclinics for children's health care we can do that since we are already there at most locations with seven hospitals in Hyderabad. And we are envisaging to do one more. We are very close to most of the geographies (within Hyderabad). In addition, recently we are contemplating to do one more polyclinic in Attapur area.

I think the polyclinics are important, but at the same time they have to be driven by our own doctors. We can't recruit doctors who are non-aligned to do polyclinics that convert patients to you. This is our experience when we look at the other hospitals, multi-specialty hospitals when they run the polyclinics. Whatever we do, it has to be in our control. It has to be within the network.

Mahesh: I was asking more about the polyclinics which are not in the Rainbow's umbrella. Polyclinics which are operating independently, how you are reaching out to them? How do you are getting referrals from their side?

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Dr. Ramesh Kancharla : Of course we have a sales team they will coordinate with them. You have a lot of doctors' networks who refer patients to us and the patient reference happens only when there's complexity and when they require them to send to Rainbow Hospital for a particular intensive care or specialty. I mean there's a huge connection with our referring doctors across Hyderabad, Telangana and Andhra Pradesh.

Mahesh: All right. My last question would be on this Haryana thing, sir. I mean, it's been like taking us a very long time because it's almost more than a year that we kind of, I mean, we went ahead with that project and it's taking a long time. Can you please throw some light on what exactly is happening on ground there and why it's taking so much time? Is it because of bureaucracy or some change of government or something like that I mean, what exactly

Dr. Ramesh Kancharla : I think, it's just a phase of government nothing else, actually. We know the CEO and despite knowing the CEO of HSVP, it still takes that much time because this is how the systems work.

Also, for Haryana, it has got two offices, one is in Gurugram and other is in Panchkula. Between these two offices it takes its own time. And, yeah, finally, We have got the permissions, we got land registered which is more important. Now, we have applied for the permissions. Hopefully, that won't take much longer. There're

some more clearances that need to be done. Then post that, the land and drawings would be ours, post that it'll be our project's team, how aggressively we're going to do that. Once you have all the permissions on board, I think it shouldn't take more than two years, plus or minus a few months.

Mahesh: Okay. So, 26, 27 as we reported in your presentation that's when we are going to get back?

Dr. Ramesh Kancharla : Yes maybe six months. We have to give some additional time.

Mahesh: Thank you so much.

Moderator: Thank you very much. The next question is from the line of Harsh Bhatia from Bandhan AMC . Please go ahead.

Harsh Bhatia: Thank you. So, just one clarification. Apologies if this is repetitive, but on this ARPOB move on a year -on-year basis, I understand the ALOS has also moved up, which is why there could be a case of ARPOB moving down. But this has been the case for both the new hospitals as well as the old hospitals. So, if you could help us understand like what exactly is happening across both the units.

Dr. Ramesh Kancharla : Yes. See, even I was wondering of the dynamics of this actually. I was trying to look at it. What I could actually look into and conclude is that on ARPOB guidance, whatever we have given during the last three, four earnings calls, our ARPOB guidance has always been 55,000 to 60,000. This is probably the right ARPOB for our business.

And sometimes it (the ARPOB) goes up because of the change in the case mix. When you have the season, we do a lot of pediatric business, then ARPOB s tend to go down. However at the same time you clock higher revenues and better profitability also.

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Now in the current quarter , when you look at the ARPOB s today it is slightly lower and the ALOS is slightly higher when compared on year-to-year basis , that's because of the change of the business (case- mix).

If you look at ALOS , it has increased by only 4 hours and that could be because of a little change in the case mix than anything else. Last year dominantly, the business was obstetrics and lots of pediatric surgeries that took the lead (in Q1FY24). Therefore ARPOB s are higher and ALOS have gone lower (in Q1FY24) .

If you go back and see the last four quarters, our ALOS was about 2.65 to 2.7 days. Somewhere around that is usually the ALOS. I'm not very sure if ALOS of less than 2.5 days could be there. It may happen occasionally, but not on a regular basis . Our ALOS will probably stand somewhere between 2.6 to 2.8 days. This is actually a proper business (case- mix) resulting in higher ALOS. This is how I look at it.

Vikas Maheshwari: Mahesh, if I can add, since that has explained about the ARPOB and ALOS. If you just multiply this, you will get the average revenue per patient, right, irrespective of their ALOS, etc. If you see that, that has gone up. That shows that we are getting the high-end complex cases at our hospital.

And that has gone up on year -on-year basis by 6% and quarter -on-quarter basis also. So, I think 1,07,000 is ARPP , which has gone up by 7%, which is a healthy sign. ALOS is the factor which depends on, critical cases, which you get, whether you are getting quaternary care, more cases or tertiary care.

Based on that, the ALOS keeps changing. But being an industry, we feel that what is more important is the ARPOB , what type of cases we are getting. So, we are happy with the ARPP growth of 6%, 7%, which we have seen in our case, both year -on-year and quarter -on-quarter.

Harsh Bhatia: Okay, so simply it is just about the cases, overall case mix becoming more-and -more complex to a certain extent.

Vikas Maheshwari: Correct. So, the more complex cases, the patients tend to stay, longer period at the hospital. So, that may impact the ARPOB . But what is more important is ARPP . Obviously, ARPOB is very

important because it is the key matrix, how we drive our occupancy and then revenue. But that is also one of the key matrix (ARPP) , which we should look at.

Harsh Bhatia: Just in terms of this volume growth, the double -digit volume growth that we have delivered, again, my understanding might be very limited and might be inaccurate in a certain way. But would we continue to see this double -digit volume growth going for the next few quarters on a year-on-year basis? Because FY '24 was also the year where we saw first quarter doing a double -digit growth on a year -on-year basis.

But then the volume growth was very mixed picture for the next 9 months of FY '24. So, would that be the case this year as well, or would we expect to see FY '25 remaining quarters to be a double -digit volume growth?

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Dr. Ramesh Kancharla : This year largely, many things remain very positive. There is nothing major. All the elections have been over and the monsoons are right on time. Things are going on right on track. I think the overall, even the first quarter itself, we sensed that there is a positive trajectory for the year.

So, I would expect this year to be a kind of a late -teens growth for Rainbow .

Harsh Bhatia: Sure, and you would continue to feel that would entirely flow through at the bottom line as well, given the incremental cost that we have seen in the first quarter?

Dr. Ramesh Kancharla : Yes. Let's see how the case-mix is and what kind of pathology we see and how we're going to deal with that. The revenue is a derivative. It depends on how and what we are going to do in the next 9 months' time as a hospital, that results in revenue generation. And I'm sure when you do about late -teens growth, you will definitely do an impressive show on the financial side.

Harsh Bhatia: Sure. Thank you.

Moderator: Thank you very much. The next question is from the line of Banshi Desai from JP Morgan. Please go ahead.

Banshi Desai: Hi. Thanks for the opportunity. So, just philosophically checking, is demand in this industry sensitive to pricing?

So, I assume that probably for our newer units, we would have competitive pricing and probably that also could have had some bearing on our ARPOB . So, maybe just your thoughts on these.

Dr. Ramesh Kancharla : You're absolutely right, Banshi. I think there are two -three things which play up in the newer units. One is that you want to be attractive for the patients. Generally, in a new hospital, people see how expensive it is. First thing is you don't want to get that in major name. So, you will definitely price it attractively in the beginning.

Second important thing is, you may not have the insurance to solve those things. So, that some of the patients may not come into the hospital because there's no cash left, which will take about

6 - 9 months' time to settle down and all the insurance tie-ups to come in. So, these factors certainly will play in. You're absolutely right.

Banshi Desai: Okay. That's helpful, sir. And from a longer term perspective, when we think about expansion, besides whatever you have shared, where do you think you have opportunities, if you think from

slightly longer term perspective? Would it be these existing cities, locations, or you think there is promise, probably newer markets, newer cities? And if at all, you want to call out where you probably see that opportunity for us?

Dr. Ramesh Kancharla : Yes, Banshi. One, we have absolute clarity about what Rainbow's plan is for the next 3 , 4 years' time. So, what we have done in Hyderabad, I think we're going to be closer to 1,000 beds . And definitely, we are in the business with the expansion in Bangalore to becoming the largest player in Bangalore. We are already there. And Chennai is the same way, and also regional spokes in Andhra Pradesh and Tamil Nadu. This will

I continue. I think this is where the Rainbow is a household name. This is where the demand continues to be there. This is where the patronage of the brand is very significant. We'll continue to play our dominance in the south, within the cities, and also connecting regional spokes. That will continue to expand.

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And while we do that, and also we are exploring other opportunities. Of course, Delhi is up, NCR is a larger play. And also, the opportunities we're looking at seriously are acquisitions, which we are quite optimistic about. If something comes which is philosophically aligned to us, we'll definitely go for it. This is how we look at it.

There is a constant research going on what are the new cities? How do we enter? One thing we are very sure of , we are not going to do about 50 beds or 100 beds into one large city and going around putting dots in multiple places. I don't think that's efficient way. If we get into the city, we will address the city needs of children's health care in a larger, on a long term basis. And also we do hub and spoke model to include the larger part of the city into our network. This is our game plan.

Banshi Desai: Thank you. So that's very clear. And just one clarification, on the cost side, for this quarter, the other expenses have declined sequentially. So anything to read there, I see, increase in professional fees, and you highlighted that's probably because of the newer units coming in. But why would we have other expenses decline? And should we see this as a sustainable trend?

Vikas Maheshwari: So there is a, accounting standard, this ECL provisions, which keeps coming. Based on that the other expenditure, which is the one part there gets a little bit adjusted. I think, whatever the numbers we have shown, should stand there right now, because it's a clean quarter, there is no ECL impact, etc. , in this quarter.

Banshi Desai: Okay, thanks a lot.

Vikas Maheshwari: If you look at the quarter 3, and the quarter 2, the number is in the same range, but the quarter 4

got slightly impacted because of the ECL. I think the numbers which we are showing for the quarter first is a clean quarter. That is the number which we should see. Is it clear, Bansi?

Bansi Desai: Yes, understood. Thanks.

Moderator: Thank you very much. The next question is from the line of Rahul Jeewani from IIFL Securities Limited. Please go ahead, sir.

Rahul Jeewani : Yes. Sir, you indicated in terms of the break -even timelines for new hospitals in Hyderabad being within 12 months, and for other hospitals being 18 months to 24 months. So can you please also talk about the timelines for break -even for these four new hospitals, which we commissioned in fourth quarter of last year?

Because one of those was a brownfield expansion at Hyderabad. And I'm assuming that the brownfield hospital would achieve EBITDA break -even faster than some of your other newer hospitals.

Dr. Ramesh Kancharla : That is actually, Rahul, is an additional block which we built adjacent to the Hyderabad hospital. Earlier it used to be about 85, 80 plus beds. Now that's become 110-120 beds. It's a new block.

Actually, we have dedicated this block to the perinatal services because we do a large number of deliveries in that hospital.

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We just made it completely the woman's block. So these are additional beds which we have done for the demand. That unit does very well anyway and it deserved that expansion. We have done it. We are very happy with that. The doctors are happy. Businesses also is actually going to be very well.

And the next one is that we have done in Himayat Nagar in Hyderabad city, which is actually the central part of Hyderabad city, the competitive area of Hyderabad city. I think it was

commissioned about 5 months ago. It's tracking pretty well. And it's all a new team completely, and they're doing well. It's going well. It is within the first year itself that we'll break even. In Bangalore, Sarjapura, I think it's also going very well. It will take probably ~ 12 to 15 months to break even. I think we're quite happy with the way it's going.

Chennai (Anna Nagar) is a large facility, a new area with a very new set of doctors. It will take ~15-18 months' time for break even because it's a little high cost unit and we have put in a lot of money into that. It is one of the areas where it is difficult to get a land or building or anything, which is called Anna Nagar area. We managed to get it and do a hospital, and it will take a long term. That's a great area to be in.

Rahul Jeewani : Sure, sir. So then potentially the Hyderabad, Himayat Nagar and the Sarjapura Hospital should achieve EBITDA break even by end of this year. And Chennai, you are possibly targeting break even in first half of next year.

Dr. Ramesh Kancharla: Yes, exactly. Possible next year's season, we should be fine.

Rahul Jeewani: Sure, sir. And given that this year, obviously, with the drag from these four new units, which would impact our margins this year, by end of this year, we would again be commissioning, let's say 160 incremental beds across Rajahmundry and Chennai.

So next year, do you think that given that this year already, we would have seen the impact of these new hospital's FY '26 should see a margin improvement for us despite incremental capacity addition in fourth quarter of FY '25?

Dr. Ramesh Kancharla : You're right, Rahul. That's what actually our strategy is.

Rahul Jeewani: Sure. That's it from my side. Thank you.

Vikas Maheshwari: Rahul, to add one point it shows the resilience of our business model. We have opened three new hospitals and one extension. Still we have been able to deliver a very healthy EBITDA margin.

And the trajectory of the break -even, as CMD sir has explained, I think is well poised for the next leg of the growth. And by the year end, once these hospitals get opened, I think two and a half of our hospitals including this extension which we did, I think they will be doing very well.

So I think we are well poised for the next leg of growth in terms of both margins and the revenue growth and EBITDA.

Rahul Jeewani: Sure, sir. Thank you.

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Vikas Maheshwari: Thanks, Rahul.

Moderator: Thank you very much. Now, the next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead, sir.

Alankar Garude: Yes. Hi. Good morning, everyone. So first question, we have eight hospitals in Hyderabad now

with 110 new beds commencing operations in the past few months alone. You also spoke about possibly adding another hospital in the coming years. So just wanted to understand, how are you thinking about the impact of cannibalization in Hyderabad?

Dr. Ramesh Kancharla : Yes, it's interesting. We always see some degree of cannibalization because the patients which are on the border between the two hospitals will settle down to the new hospital. But we have always seen that growth opportunity is much larger.

This cannibalization would not be really significant because the other hospitals have always been in demand for the beds and those things. So in a way that works better when you have a new hospital network, some degree of adjustments between both the units of the occupancies would help us in the long term to create more beds for that geography.

Alankar Garude: Understood, sir. The second question was, any color on the performance of the Madhukar Hospital in this quarter? You had mentioned about a pickup in performance in this scale. So any thoughts on how's the performance been in the first quarter?

Dr. Rame

sh Kancharla : Madhukar definitely there's an uptick in the performance and overall I shape of the Madhukar Hospital and traction seems to be well. Perinatal services are pretty good and the intensive services are picking up. I think the number of footfalls have increased significantly.

These are all very positive signs and we've been constantly working on the cost structures and we're seeing some tangible results from our operational teams working on the cost side and I think towards the end of the year we'll probably have a much better opportunity to post good EBITDAs for Madhukar as well as Rosewalk .

Alankar Garude: So I think FY '24 we were at high single EBITDA margins in Madhukar . So would it be fair to assume maybe a mid-teens kind of a low to mid-teens kind of a number in FY '25 and possibly higher going forward in FY '26?

Dr. Ramesh Kancharla : I think what we are looking at Alankar is a double-digit EBITDA. That's what I'm aiming at. I'm not sure where I would end up whether it is early teens or just teens or mid-teens. I'm not very sure at the moment. So definitely I think achieving double-digit EBITDA in that kind of a structure is quite satisfying to be honest with you.

Alankar Garude: Understood and maybe a quick follow-up there. What's the outstanding amount from the trust?

Vikas Maheshwari: It remains same from the March whatever we have seen. It is same. So as the year progresses we expect that the INR3 crores to INR4 crores to further come down Alankar. So it's more or less same status.

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Alankar Garude: Okay understood and one final question from my side. When it comes to Gurgaon, construction has not started yet at both the hub as well as the spoke hospitals. Just wanted to get a sense are we seeing any delays in both these hospitals?

Dr. Ramesh Kancharla : I'm not saying (delay), I mean we have still said about FY 27 right , mid of FY 27. I think there may be a four to six months delay because we didn't expect that it will take about a year to get the land parcels registered and now we applied for the permissions. Hopefully this phase of the permissions if we can cut short to two - three months' time then we should be right on the track. Maybe with a three four months leeway.

We have already boarded the head of the projects and we are actually working on the drawing and various other things. I think we are also trying to see some innovative ways to cut short the timelines in terms of a core construction or in terms of going the hybrid way. There are multiple things that we are exploring and I think from our side we are gearing up to try to bridge some gaps.

Alankar Garude: If we assume say you get an approval in the next say couple of months how much time do you think would it need to construct the spoke as well as the hub hospital?

Dr. Ramesh Kancharla : Once you start groundbreaking it will probably take about two years' time in the modern way of construction. If you have got everything else finalized and go on the turnkey basis or if you try to work with a great executive team .

Alankar Garude: Understood sir. That's it from my side. Thank you and all the best.

Moderator: Thank you very much. The next question is from the line of Anand Mishra from Philip . Please go ahead sir.

Anand Mishra: Yes. So I had just a couple of questions on your Gurugram expansion. So do we have any broad capex number for the whole project?

Dr. Ramesh Kancharla : I think we do have a broadly but once we are at BOQ stages and those things , we'll have a lot more clarity about it.

Anand Mishra: Okay and just quickly so I mean on the competitive side right so I think the Gurugram market is fairly competitive given the density of hospitals there. So I mean what is our go-to market strategy there? So are we looking at maybe being a premium player or are

we looking at being

a mid -market player? If you could give us some color on that.

Dr. Ramesh Kancharla : I think our Gurugram flagship hospital is going to be a national hospital for entire North India.

That's the positioning that we're going to base on this hospital. The spoke hospital of 100 plus beds is going to be a local Gurugram high-end hospital and these hospitals are actually are going to be very well built and will be very futuristic.

Anand Mishra: But is it fair to assume a capex per bed in the range of INR 1.5 crores to INR2 crores? I mean that be on the higher end or something?

Dr. Ramesh Kancharla: I think it will be around 1.5 Cr. Yes , for sure.

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Anand Mishra: Okay . Great, thanks that answers my question. Thank you.

Moderator: Thank you very much. Ladies and gentlemen that was the last question. I now hand the conference over to the management for closing comments.

Vikas Maheshwari: Yes. Thanks. We would like to thank you all for joining today's conference call. Your participation and insightful questions are invaluable as we continue to navigate our strategic path forward. The support and engagement are crucial and we appreciate the time and efforts you have invested in understanding our business and the future plans.

We look forward to our continued partnership and are excited about opportunities ahead. For any queries please reach out to investors@rainbowhospitals.in

or Mr. Saurabh Bhandari . Thank

you very much.

Moderator: Thank you. On behalf of IIFL Securities Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Vikas Maheshwari: Thank you.

This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy. Please also note that this document has been edited without changing much of the content, for enhancing the clarity of the discussion. No unpublished price sensitive information was shared/discussed on the call.

Call: Nov 2024

November 4, 2024

National Stock Exchange of India Limited Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.

Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.

Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q2 & H1 FY25.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and
Disclosure Requirements) Regulations 2015 ("SEBI Listing Regulations").

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on October 28, 2024, with regard
to Financial Results of the Company for Q2 & H1 FY25.

The said Transcript is also available on the website of the Company and can be accessed
through the following link:

[https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-
_Q2__H1_FY25.pdf](https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q2__H1_FY25.pdf)

We request you to kindly take the same on record.

Thanking You, Yours Faithfully,

For Rainbow Children's Medicare Limited

Ashish Kapil Company Secretary and Compliance Officer Encl.: As above

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"Rainbow Children's Medicare Limited
Q2 FY25 Earnings Conference Call"
October 28, 2024

Call Duration ■ 1 Hour and 1 Minute

Management Speakers ■ Dr. Ramesh Kancharla – Chairman and Managing Director

■ Mr. Vikas Maheshwari – Group Chief Financial Officer

■ Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions ■ Ms. Damayanti Kerai, HSBC Securities and Capital Markets

■ Mr. Bino Pathiparampil, Elara Capital

■ Mr. Sumit Gupta, Centrum Broking

■ Mr. Rahul Jeewani, IIFL Securities Limited

■ Mr. Ritesh Shah, Lucky Investment

■ Mr. Alankar Garude, Kotak Institutional Equities

Moderator ■ Mr. Rahul Jeewani - IIFL Securities Limited

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Moderator: Ladies and gentlemen, good day and welcome to the Rainbow Children's Medicare Q2 FY25 Earnings Conference Call hosted by IIFL Securities Limited. As a reminder, all participant lines will be in listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Rahul Jeewani from IIFL Securities Limited. Thank you

and over to you, sir.

Rahul Jeewani: Yes, thanks Sejal. Hi, good morning everyone. I welcome you all to the second quarter earnings conference call of Rainbow Hospitals hosted by IIFL Institutional Equities. From Rainbow, we have with us today Dr. Ramesh Kancharla, Chairman and Managing Director, Mr. Vikas Maheshwari, Group CFO and Mr. Saurabh Bhandari, Head Investor Relations and Group Business Analyst. Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla: Thank you, Rahul. Good morning, everyone and a warm welcome. It's a privilege to address such a distinguished group to discuss the earnings for the second quarter and the first half of the financial year 2024-25, reflecting on our company's progress, vision and exciting opportunities ahead.

In a rapidly evolving landscape, our commitment to innovation and excellence remains steadfast.

Today, I will share insights into our recent achievements and the strategic initiatives. I believe that transparency and open dialogue are key to fostering trust and collaboration with our stakeholders.

As we reflect on the second quarter, I'm pleased to report that we have made a significant stride in several key areas. Here are the highlights of the second quarter and the first half of the year.

■ We have strengthened our hub-and-spoke model further with the addition of the new hospitals commissioned in the past year. They have progressed impressively, reaching operational stabilization, standardization and integration into the Rainbow operating model.

We believe this will not only enhance our operational effectiveness as a group, but also strengthen our position as a leader in children's health care.

■ As expected, we have come up with a strong performance in the current quarter with a growth in all key operating metrics across the group including newly opened hospitals in Hyderabad, Bangalore and Chennai.

■ The progress in our IVF services has been notably promising, gaining significant traction. We are optimistic about sustaining this momentum and achieve further growth in the IVF segment in the coming quarters.

■ We have maintained a strong emphasis on the cost management, ensuring that we optimize our expenses wherever possible. This disciplined approach has been crucial as we manage the additional cost of our newly operating hospitals while maintaining our group's profitability. As a result, we achieved our highest quarterly revenue, EBITDA and PAT figures along with the positive patient feedback.

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■ We are facing some challenges in international business, particularly in some countries such as Bangladesh, Oman, Kenya and Sudan because of geopolitical situations. These countries have significantly decreased the issuance of health permits for patients pursuing medical travel. In response, we are actively pursuing with the health authorities of these countries and simultaneously we are also exploring opportunities in other potential markets.

■ We have largely a retail brand named Butterfly Essentials, a specialized retail store designed to cater to the unique needs of women and children. Butterfly products aim to transform the retail experience within our hospital facilities for customers by of

fering a

wide selection of items that prioritize organic and natural ingredients for mothers, babies and children. In the first phase, we plan to open Butterfly Essentials stores in most of our existing facilities.

■ I would like to bring something different now with most fascinating sequence of events in human life is in early childhood development. Any aberration would result in long-term consequences in neurodevelopmental milestones. We are witnessing a significant increase in childhood developmental and behavioral problems. We recognized the need for a comprehensive child development center to support these children. We have set up an 8,000-square-foot state-of-the-art comprehensive child development center adjacent to a cardiac center in Banjara Hills Hyderabad is expected to launch on 14 November, Children's Day aligning with our Foundation Day.

■ Our steadfast commitment to quality has allowed us to provide outstanding clinical and patient care. In line with this dedication, our Vishakhapatnam Rainbow Children's Hospital has recently achieved NABH accreditation. With this, 13 of our hospitals are NABH Accredited and three have earned JCI Accreditation. Notably, we are the only Paediatric hospital chain in the country with three JCI Accredited hospitals facilities.

Now, delving into the numbers for Q2 FY25, our revenue registered a growth of 25.5% amounting to INR417.5 crores. Simultaneously, our EBITDA increased by 25.1%, reaching INR147.1 crore while the PAT registered a growth of 25.1% to INR79 crores. I'm happy to share that all these growth numbers of 25% realign with our Silver Jubilee Year.

Our overall occupancy rate for the quarter was 59.9% with the mature hospitals achieving 68.6%

occupancy and the new hospitals including the recently commissioned hospitals in Hyderabad, Bangalore and Chennai recording a 43.2% occupancy rate.

Coming to the update on upcoming projects,

■ The project work of our regional hub hospital in Rajahmundry Hospital, 100 beds is progressing quite well and is poised to commence operations by March of 25.

■ The spoke hospital in Hennur, Bengaluru which is a 60-bed hospital is expected to be delayed due to the conflicts between the landlord and the builder. However, all these issues have been sorted and now the hospital looks to be commissioned by end of a H1 FY26.

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■ The project work for our regional hub hospital Coimbatore of 130-bed hospital has just started and is expected to be completed in 24 months' time.

■ On Gurgaon projects, we have submitted the drawings to HSVP for building sanction approvals for sectors 44 and 56. We are waiting for the approvals from HSVP to start the project work.

■ We signed a lease agreement for a 90-bed Brownfield Spoke Hospital, a rapidly growing population in Electronic City, Bangalore City. This building is in the final stages of construction. We shall commence the interiors very soon and we expect this hospital to commence operations by Q3 FY26. With this, it will be the sixth hospital of Rainbow Group in Bangalore City.

Beyond our growth plans, I would like to highlight some of our significant achievements that underscore our dedication to pediatric and perinatal healthcare excellence. During the last six months, we have treated a lot of sick children in pediatric intensive care and as well as pediatric subspecialty children. I would like to discuss a couple of such cases.

■ A 21-month-old child was diagnosed with a spinal muscular atrophy. It's called SMA. It's a rare genetic condition, which carries a significant developmental delay. It delays motor milestones, especially. The treatment to this genetic condition includes administering ZOLGENSMA. It's an expensive gene therapy drug, which costs up to INR14.5 crores. This child was very fortunate to have financial support from a large Indian public sector company and its employees to raise

a full amount to support this child. Our pediatric neurology team at Rainbow Children's Hospital, Secunderabad, led by Dr.

Ramesh Konanki, one of the top experts in the country with a large experience in treating spinal muscular atrophy. This child received gene therapy under special clinical circumstances. The follow-up of this child following therapy indicates the child is gradually improving motor skills and approaching independent walking.

■ In another case, there is a two-month-old child who has been having breathing difficulties since birth. A 2D echocardiogram revealed a right ventricular outflow obstruction, a mass which is adjacent to the pulmonary walls causing the obstruction. This baby required an open-heart surgery at two months, discovered a large mass occupying almost 90% of the right lower heart, which is one of the chambers, right chambers, and obstructing the outflow of the blood flow. The cardiac surgical team could meticulously extract and remove the entire mass, and the baby made a full recovery in a week's time. The histopathology of the mass demonstrated is rhabdomyoma. Rhabdomyoma in children is fairly benign, and we expect this child to lead a normal life.

These cases emphasize the importance of a multidisciplinary approach for tertiary and quaternary care for children to achieve better outcomes.

As we celebrate the synergy of Rainbow in providing world-class children's care in India, I extend my heartfelt gratitude to our doctors, nurses, managerial team, and the entire Rainbow family, along with our valued stakeholders and young parents who have contributed to this incredible journey.

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With that, I will now pass the mic to Group CFO, Mr. Vikas Maheshwari, to take you through the financial update. Thank you once again for joining us today. We look forward to your questions and insights as we move forward with us.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you, and thank you for attending this investor's conference. I'm pleased to brief you on the financial performance and the key developments of Rainbow Hospitals for the second quarter and first half of financial year FY '25.

■ Our operating revenue for the quarter stood at INR417.5 crores, reflecting a growth of 25.5%

when compared to the corresponding quarter of the previous financial year. For H1, our revenue stood at INR747.6 crores, reflecting a growth of 20.6% when compared to the H1 of the previous financial year.

■ Our EBITDA for the second quarter amounted to INR147.1 crore, marking a 25.1% growth compared to the same period last year. For the H1, our EBITDA stood at INR240.8 crores, reflecting a growth of 17.3% when compared to H1 of the previous financial year. Our EBITDA margin for the current quarter is 35.2%, while for H1 our EBITDA margin stands at 32.2%.

■ The profit after tax for the quarter is at INR79 crores, marking a growth of 25.1% in comparison to the corresponding quarter of the last financial year. For H1, our PAT stood at INR118.7 crores, reflecting a growth of 13.5% when compared to H1 of the previous financial year.

■ In terms of the operational performance, both outpatient and inpatient volume increased by 20% and 22%, respectively, when compared to the corresponding period of the last financial year. Deliveries grew by 12% compared to the corresponding period of the last year.

■ Our payor mix continued to remain robust and balanced with 52.8% of the revenue coming from the insurance and the balance 47.2% coming from the cash patients. For the H1, the payer mix stands at 47.7% cash patients and 52.3% as insurance patients.

■ Furthermore, I would like to inform you that our international business now constitutes approximately 2% of our total business for the second quarter. As highlighted earlier by our CMD, we are fa

cing some challenges in the international business and working on this simultaneously.

■ I am pleased to inform that our company's balance sheet remains very robust with a net cash position of approximately INR580 crores as of September 30th of this year and will support our ongoing capital expenditure plan. Given our current cash and anticipated internal accruals in the coming years, we remain very confident in our ability to complete all the capital expenditures through internal accruals without any debt financing.

■ During the quarter, the company has invested approximately INR23.4 crores in the capital expenditures.

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With these insights, I conclude my financial update. I now invite questions and suggestions from the participants. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Damayanti from HSBC. Please go ahead.

Damayanti: Hi, good morning and thank you for the opportunity. So my first question is on your plan for child development center. So can you talk a bit more about this new initiative in terms of what kind of demand you are seeing in the market? Say, out of 1,000 children, how many might require this sort of treatment? So any color on this new initiative will be very helpful.

Dr. Ramesh Kancharla: So, Damayanti, in modern society—particularly in a rapidly growing one—we're increasingly observing behavioral and developmental issues in children. Developmental problems often arise from various neurological and other pathological causes. Early childhood infections form a significant part of this group. Another major area is the behavioral spectrum, which includes conditions like attention deficit disorders, autism, and autism spectrum disorders, as well as other early childhood psychological issues. These issues collectively represent a substantial portion of the population affected. For example, autism alone affects about 1 in 80 children, with recent reports suggesting a rise to almost 1 in 60. This means a large number of children require long-term therapy, which necessitates a specific approach. Currently, in Hyderabad, we have about six pediatric neurologists in the Rainbow Group, each of whom sees a patient population where around 20% present with behavioral and developmental issues. So, this definitely requires not only the Rainbow Group, the country definitely requires to address this problem very seriously, because there's increasing numbers year-on-year.

What we observe is that while children are now coming to hospitals, they require a specialized environment with specialized therapies and specialized professionals. This includes a multidisciplinary team consisting of educational psychologists, physiotherapists, occupational therapists, and developmental physicians. We already have about 80% of this team in place. We're segregating, collecting all of them together in one place and trying to deliver excellence to these children. The behavioral problems for children are a serious curse to the mother, who are actively seeking these kinds of facilities.

Damayanti: Okay. So, you mentioned - you have already say 80% of doctors and other medical people who

are available in this segment, but it will be in a different unit altogether dedicated for this particular center?

Dr. Ramesh Kancharla: Exactly.

Damayanti: And doctor, did you mention all the neurologists, top neurologists in your hospital, almost 20% of patients which they receive face some or other such issue? Did I hear that correctly?

Dr. Ramesh Kancharla: "Yes, in the outpatient department, about 20% of volumes involve some form of neurodevelopmental or behavioral issues. This segment of neurodevelopmental care differs Rainbow Children's Medicare Limited

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significantly from mainstream neurology, requiring s

pecific focus and specialized attention. A

multidisciplinary team approach is essential, rather than one single individual.

Additionally, many therapies are now available for these children, often delivered in sessions.

For instance, behavioral modification for attention deficit disorder differs greatly from the approach needed for an autistic child. And this is something which is a need of the hour for the country"

Damayanti: Sure. And again, I think similarly, I want to hear some more update on your new initiative, Butterfly Essentials. So, I understand your hospital already have pharmacies, right, which are like supplying these baby and mother related products.

So, why like this new initiative, which is coming up when you already have pharmacies? And then just want to check here, products will be all private level or it will be a multi-brand store sort of facility in the hospital?

Dr. Ramesh Kancharla: "Okay. So, as a pharmacy, we can only cater to a small segment. In terms of visibility, mothers and families often expect a pharmacy to offer a comprehensive range of products for pregnant women, babies, and children.

What we aim to do is develop a specialized vertical within this segment. Currently, the number of people buying outside the hospital is relatively low because people perceive the pharmacy differently than a retail store. We are working to create a niche retail space focused specifically on medically aligned products, enhancing both comfort and accessibility for patients.

As a hospital-affiliated pharmacy, we take certain degree of ownership in selecting the products and take responsibility for offering a reliable product range. By aligning more closely with patient needs, we can provide a wider selection of items—from soaps and shampoos to nappies and everything else. We can curate it better and offer them a wider range of the products within the hospitals —so patients have everything they need without having to go elsewhere."

Damayanti: Sure, that's clear. And my last question is on the IVF services, which you initiated a quarter back or so. So you mentioned you have seen good momentum. So, can you put some number, like how things are picking up on that vertical?

Dr. Ramesh Kancharla: As I mentioned in the previous call, we are focusing on organic growth in the B2C segment for our IVF services, and it's unlikely that significant growth will come from B2B, given our primary focus on mainstream obstetric service delivery. Within our group, word of mouth and B2C channels will play a dominant role in building our IVF services.

So far, we've seen positive results: previously, we offered IVF in three centers, and compared to the first half of last year, we've seen about 50% growth, which is significant. Additionally, our revenue has grown by approximately 40%

Vikas Maheshwari: Yes, 73%.

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Dr. Ramesh Kancharla: 73% growth rate. We're in the early phases, but we're quite happy, and the results are very encouraging. The doctors within the hospital are also very optimistic, particularly regarding, first, our service delivery for these patients and, second, the overall satisfaction of both doctors and patients

Damayanti: Sure, doctor. Thank you very much for your response. Thank you.

Dr. Ramesh Kancharla: Thanks so much.

Moderator: Thank you. The next question is from the line of Bino from Elara Capital. Please go ahead.

Bino: Hi, good morning. I wanted to understand the ramp-up in the newly opened centers over the last few months. Last year, we have opened a couple of centers. What is the occupancy level there, and are we profitable there already, or what is the profitability?

Dr. Ramesh Kancharla: We opened toward the end of the last quarter of the financial year, and the centers are tracking fairly well. For example, the Hyderabad center is performing very well since it's our

primary

location. Other centers are also seeing good traction, both in peak and off-peak periods. I think we're close to reaching break-even. Of course, it's a busy season now, so we need to be careful. But overall, the trajectory is very encouraging. Things are going pretty well

Bino: Okay. So we are almost profitability. So in the next couple of quarters, probably they should break-even and then onwards slowly improve. Is that the right assessment?

Dr. Ramesh Kancharla: Yes.

Bino: Okay, got it. Thank you.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum Broking. Please go ahead.

Sumit Gupta: Thank you for the opportunity. Sorry, I joined the call a bit late. So sorry for the repeat question, if it may. So for the matured hospital, the revenue growth is tremendous, I understand, because of the occupancy. But what is driving that majorly? I understand 2Q is relatively strong, but still I just want to understand more on the fundamental aspect of it.

Dr. Ramesh Kancharla: Vikas.

Vikas Maheshwari: Yes, Sumit. For our established hospitals, the busy season typically begins in the second quarter for most, including both adult and pediatric care, leading to increased activity. Since these mature hospitals have been serving the community for over five years, many patients have prior experience with us. As a result, for any pediatric or maternal care needs, they continue returning to our hospitals, which drives our growth.

We're seeing growth in both inpatient (IP) and outpatient (OP) volumes at our mature hospitals.

This is steady, organic growth, across all our matured hospitals.

Sumit Gupta: Understood. And so what kind of margins are there for matured and new?

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Vikas Maheshwari: We don't disclose the margin based on the maturity.

Overall, at the company, we show as margins. But obviously matured hospitals will have a better EBITDA margins than the new ones because every year we are adding capacity. Matured hospital will have always some impact of the new hospital which is getting opened. So, obviously, the matured hospital will always have the better output which is the industry phenomena.

Sumit Gupta: So like this is a super 35% kind of margins and in 2Q obviously do this kind of margins. So in the second half also, we can expect around 30% to 33% kind of margin?

Vikas Maheshwari: What we have guided Sumit is that over a longer period we will be able to sustain this margin whatever we have shown in H1. So because of a little bit of seasonality. In the quarter first, the margins will be always a little bit impacted because in the hospital there is an operating leverage which gets kicks in, but the business is generally the quarter 1 is the weakest quarter for us and the quarter second is the strongest quarter.

So, if you look at the H1 figure whatever the EBITDA margins we are showing which is again is a post INDAS numbers, we should be able to sustain those numbers plus minus 1% depending upon what type of capacities we are adding, based on that we should be able to maintain that EBITDA margin.

Sumit Gupta: And lastly, on the EBITDA per bed, so what kind of EBITDA per bed do you target? So as of now, so this quarter it was a decline of nearly 9% to 10% on a Y-o-Y basis. So just on an annualized basis or for let's say over the next two to three years what kind of EBITDA per bed we can target?

Vikas Maheshwari: If we are keeping the EBITDA margin same Sumit, I think per bed EBITDA will also not change. What you have to factor in is that whatever the H1 number has come, you have to take it as a realistic basis and then project that accordingly.

Sumit Gupta: Understood. Thank you.

Moderator: Thank you. The next question is from the line of Karan Gupta from Invest Portfolio Management. Please go head.

Karan Gupta: Hi. Good morning. So my first question on the A

RPOB side is continuously from the last quarter decline...

Vikas Maheshwari: Karan there is a lot of disturbance on the voice, if you can just be very close to your handset, please.

Karan Gupta: Is that audible now?

Vikas Maheshwari: Same, a little bit. But okay, we can continue.

Karan Gupta: So my first question on the ARPOB side was the case mix in this quarter is continuously on the decline side. And the second one on the bottom slide retail stores what kind of products we are...

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Moderator: Mr. Karan, I would request you to please use your handset. Mr. Karan, we are not able to hear you.

Karan Gupta: Hello. Now it is audible?

Moderator: Yes sir.

Dr. Ramesh Kancharla: Yes, you are better now.

Karan Gupta: So first on the ARPOB side what is the case mix this quarter and the second one on the bottom slide side, what is your...

Moderator: Ladies and gentlemen, we have lost the connection of the current participant. We will move on to the next participant. The next question is from the line of Rahul Jeewani from IIFL Securities Limited. Please go ahead.

Rahul Jeewani: Yes. Hi, sir. Sir in Bangalore now with, let's say, the Hanur and Electronic City expansions coming up next year, we will have almost close to 600 beds in Bangalore and Hyderabad capacity right now is around 950 beds. So what kind of a potential do you see in the Bangalore market over the next 5 years to 6 years period? So can we get to, let's say, the kind of capacities which we have in Hyderabad and maybe potentially overtake in terms of the capacities which Bangalore could have versus Hyderabad 4 years to 5 years down the line?

Dr. Ramesh Kancharla: The story in Hyderabad spans about 25 years, whereas in Bangalore, it's only about 8 to 9 years, Rahul. In Hyderabad, we only expand when there is clear demand. In Bangalore, however, we're taking a more opportunistic approach since it's a city driven by micro-markets. Currently, we have four hospitals in key areas, and we're planning to add two more, one in Hennur and another in Electronic City.

With this, we'll have a well-distributed network across Bangalore. Over the next 2 to 4 years, we'll evaluate the performance of these hospitals and consider further expansion as demand arises. For instance, central areas like Rajajinagar may be potential locations in the future. However, we don't want to rush into expansion without careful consideration.

Once these two new centers are established, my focus will be on building their profitability and strengthening our hub before pursuing additional expansion. This demand-driven approach makes future growth more manageable and strategic. This will be our approach for Bangalore.

Rahul Jeewani: Sure, sir. So wherever you see demand, let's say in Hyderabad and Bangalore, you would be keen to add incremental capacities in these markets?

Dr. Ramesh Kancharla: Exactly, yes.

Rahul Jeewani: Sure, sir. And, sir, in the Hyderabad market one of your peers is entering into the Hyderabad market with 300-bed women's and child hospitals. So how do you see the competitive intensity in your core Hyderabad market?

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Dr. Ramesh Kancharla: No, I mean, we welcome anyone who wants to do high-quality work in the pediatrics domain, anywhere in this country or whichever is the city. That's not a problem. However, I don't anticipate any issues because we always faced competition in Hyderabad. This competition has consistently made us better, improved our performance, and helped us realize our potential. That's how I've always viewed it.

I always welcome competition, but one thing to consider is that a multi-specialty practice trying to focus on a single specialty will not find it easy. My focus is solely on one agenda, 24/7. How effectively they can maintain that focus while being a multi-specialty

practice is a big question.

I'll leave that for others to answer, but if someone is as disciplined as we are, I'm sure they'll be able to succeed."

Rahul Jeewani: Sure, sir. So while we appreciate sir then maybe some of these multi-specialty hospitals haven't been as successful in scaling up children-dedicated hospitals, but with increasing competitive intensity in Hyderabad would we see some sort of an impact in terms of our doctor talent and that maybe these other competitors become more aggressive in poaching doctor talent from us?

Dr. Ramesh Kancharla: What happens is that, unlike adult hospitals, in pediatric hospitals doctors cannot easily take the business away because everything is very interlinked. The brand and overall service system are more important than any individual doctor. This is a medical specialty, not like a surgical specialty where a surgeon can leave and take 70% of their surgical patients with them. Somebody needs to understand that.

Another key point is that we train a lot of doctors internally for our needs. As long as we maintain a balance and are supported by doctors in Hyderabad, we don't foresee any problems. I wouldn't expect people to make decisions that would lead to patient movement because, in the pediatric domain, patients typically do not switch doctors as easily. This is a critical factor.

We experienced this in the past, around 2009-2010, when some of our doctors left to start their own ventures, yet they couldn't take even 5% of the patients with them. This illustrates the

strength of our brand, the robustness of our operating model, and the trust that people have in both the brand and the model."

Rahul Jeewani: And, sir, last question from my end. We are looking at M&A opportunities in, let's say, Northeast and West India market. So any progress in terms of inorganic research?

Dr. Ramesh Kancharla: It's in early phase to discuss. I think when companies are consolidating, when it's (M&A) happening, definitely I would come forward and let you all know.

Rahul Jeewani: But, sir, anything which you can call out in terms of, let's say, the potential sizes of these assets in terms of which you are evaluating, either on bed number or what kind of, let's say, hurdle ratios would you be using for evaluating these assets?

Dr. Ramesh Kancharla: Well, sir, we are in a very early phase, sir. I think let's give some more time to discuss on those things.

Moderator: The next question is from the line of Ritesh Shah from Lucky Investments. Please go ahead.

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Ritesh Shah: I have a few questions. One, can you tell me, on the slide, I couldn't see the margins that you would make in a new hospital and margin that you would make in a matured hospital. Other details are there, but if you could just share at this stage of occupancy ratio that you have put in a new hospital, what are the margins?

Dr. Ramesh Kancharla: New hospitals, Ritesh, are a mix of very newly started facilities and those that are up to four or five years old, right? This makes it complex to categorize them by individual units, such as one-year-olds or two-year-olds. That's why we always group them into two buckets for consistency and legibility: mature hospitals and new hospitals (those under five years old).

Ritesh Shah: So, in the new hospital bucket, whatever bucket you have today, irrespective of the vintage of that bucket, what will be the margin there?

Vikas Maheshwari: Ritesh, let me take this question. As a company, we don't and would not like to share the margins on the matured hospitals and the new hospitals.

Ritesh Shah: No problem. Can I ask you differently then? At what utilization level does a hospital start becoming profitable?

Dr. Ramesh Kancharla: So, it depends on geography. If I do a hospital in Hyderabad, very likely in the end of the year, it will be either break-even or profit

able. In Bangalore, it will take about 12 to 15 months' time.

Chennai maybe takes 18 to 24 months' time. So, depending on your brand, recognition. And also, on micro-market where you are in and the doctors you're able to curate for that particular center. So, these are all things that go hand in hand together.

Ritesh Shah: And what utilization will it be, sir? And that utilization level will be 20%-25%?

Dr. Ramesh Kancharla: I think 30%-35%.

Ritesh Shah: The other question is, in your focus markets of Hyderabad, Bangalore and Chennai, if you could tell us the ratio of a pediatric hospital to a normal specialty hospital, what will be the ratio of beds or ratio is it one is to...

Vikas Maheshwari: So, more or less, in all of our hospitals, pediatrics beds and the mother care beds, the ratios will not change actually. It will be more or less the same.

Ritesh Shah: No, my question is, if, let's say, 100 is the total pool of beds in, let's say, Hyderabad, across all hospitals, across all types, in that, how many beds obviously, how many beds will be a pediatric bed? One bed, 200, two beds, 200, 10 beds, 200?

Dr. Ramesh Kancharla: Okay, here's how it works. We have a different structure for our hospitals. Typically, 30% of the beds are dedicated to obstetrics, and in some facilities, this can go up to 40% or 50%. It depends on the micro-market and specific requirements. We position the bed allocation based on these needs.

For our hub, the distribution is around 70% pediatrics and 25% obstetrics, which reflects the separation of the beds.

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In terms of business, the numbers are consistent. When you look at the overall figures, approximately 30% of our business and patient volume is in obstetrics/gynecology compared to pediatrics."

Ritesh Shah: Sir, I was actually asking for the market. I know for your company, but for the market, if there were 100 beds treating all types of patients, your place pediatric beds will be what? 1%, 2% of that? See, because in US, we have for every 10 or for every 20 hospitals, we have one dedicated pediatric hospital. So, I was just figuring out that, what is the ratio that we have reached in India?

Dr. Ramesh Kancharla: Sir, we don't have a knowledge about other hospitals or anyone else. People like you should do this diligence, frankly. I wouldn't be able to tell you about other hospitals.

Ritesh Shah: Maybe you can have a paid consultant to probably do it. And last question is, sir, in the Hyderabad market, Bangalore and Chennai, in your opinion what can be a pediatric bed potential in terms of number of beds?

Dr. Ramesh Kancharla: There is a growing market, particularly in Bangalore and Hyderabad, especially for the young population. If you look at the JLL report on the consumption of retail space, IT space, and the number of residential units being developed in these two cities and Pune, you can see that population growth is definitely on the rise, with an increase of about 15%. Most of this addition consists of young individuals. This is why we can make this assessment. However, I don't think it's a precise estimate; it's a rough estimate

Ritesh Shah: Sorry, I didn't get it, sir.

Dr. Ramesh Kancharla: When you look at the growth in real estate, including office space consumption, IT consumption, and residential properties like flats and villas, the year-on-year growth in the markets of Hyderabad, Pune, and Bangalore is definitely not just 15%. While I can't say that the number of buyers of new houses is absolutely at that figure, there is a noticeable increase of around 12% to 15% in the younger population in each of these cities' year-on-year.

Most of the population moving into the cities consists of young people, which is significant.

This is why I believe growth is not an issue in vibrant cities like Hyderabad, Bangalore, Pune, and Gurgaon

Ritesh Shah: And after the

se three markets, if there can be another focused market which can eventually be a target market for you, what it should be?

Dr. Ramesh Kancharla: I think right now that's where we are, our next focus is in NCR. NCR in itself is huge. And I think if you focus NCR and when we start our Gurgaon hospitals, then obviously we're going to look around more micro-markets in the NCR.

Ritesh Shah: And lastly, in the next 24 months, what is the beds which will come on screen for us, the number of beds?

Dr. Ramesh Kancharla: 24 months, I think we have about...

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Vikas Maheshwari: So Ritesh, so roughly 380 beds will come which will be Rajahmundry, 100 beds, which we are starting by March 25. And then two hospitals which is coming at Bangalore, which is Hennur and Electronic City roughly 150 beds there. And Coimbatore which will be the fag end of FY26, 130 beds. So total 380 beds are expected in next 24 month's time.

Ritesh Shah: Okay. So, when I look at your Occupancy at about 69% in the quarter for a mature hospital, looks like that the annual Occupancy is about 60% there. I remember last calls, historical calls used to always say that Pediatrics usually on an annual basis is about 60% Occupancy business because we have a seasonality element in it?

So is it fair to assume that the matured hospital for you, which is like this 1200, 1300 beds is peaking out in terms of Occupancy -- in terms of Occupancy you may have an ARPOB that increase, that's different. But on Occupancy is this comment correct or you want to modify this?

Vikas Maheshwari: In the peak season, we have multiple levels currently to accommodate the patients, which we have done, all of our units have done to accommodate the patients with the faster turnaround, adding few additional beds on a temporary basis. We try to manage. I think we will continue to do this and what you are saying is that, 60% on the matured is peaking it, which I partially agree.

It has another 5% delta, it can come.

Ritesh Shah: Another 5% delta. So, which means that the matured hospital for once on a volume basis can have this 5%, 10% delta and whatever is a case mix price led increases which will flow in your matured hospital?

Vikas Maheshwari: So, that's what happen in a medical hospital. So, any matured hospital which is growing, what the company or the hospital should do is keep adding the other super specialties and...

Ritesh Shah: Yes, that's why I said case mix and price

Vikas Maheshwari: So, that will continue. Some price corrections will continue. And obviously, if the demand is high, we will not be shying away from adding the beds. Which we have done in one of our hospital at Hydernagar in Hyderabad. We have added capacity, if we see the robust demand. So we will keep on adding the beds on the existing hospital if the demand is sustainable and we continue to grow.

Ritesh Shah: So, you have the structure is it a warm shell type structure where you can add beds, but there will be a limit to what you can add, right?

Dr. Ramesh Kancharla: What we do is take multiple approaches. Sometimes, we move out of certain services, add more beds, and enhance our facilities for diagnostics and related services. We create additional beds, and if there's available land, we might build a small block to accommodate more capacity. For instance, we recently added 50 beds to the Hyderabad Nagar facility, adjacent to the existing building. Additionally, we have added about 12 beds at LB Nagar to meet demand by creating space from

our existing facilities. We also secured space for the obstetric outpatient and IVF center in an adjacent building. We continue to explore these opportunities as they arise. When business demands it, we discover how to capitalize on those opportunities

Ritesh Shah: And lastly, sir,

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adjacent building. We continue to explore these opportunities as they arise. When business demands it, we discover how to capitalize on those opportunities

Ritesh Shah: And lastly, sir,

Dr. Ramesh Kancharla: Again, we don't share this. Just figure it out by multiplying Beds, AR POB and occupancy. We have given you all the information.

Ritesh Shah: No problem. Thank you, sir, and all the best.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Sir, we had mentioned once that if there is demand, we might look to add another hub in Hyderabad at the appropriate time. Now given incremental supply coming up from competition, do you think there is enough demand to add another hub in Hyderabad over the medium term?

Dr. Ramesh Kancharla: I can't speak for others, but I definitely feel that another hub will be required. I'm not sure if this will happen within a two-, three-, or five-year horizon, but I believe we will establish another hub in Hyderabad at some point in the next five years. That is my thought process.

It hasn't been fully consolidated yet, but we would pursue it because this is a place where people trust us and believe in our services. It's not just Hyderabad; we have support from the entire Telugu-speaking population across two states, as well as from nearby regions like northern

Karnataka and parts of Maharashtra. This is a large market, and we have established ourselves as a choice hospital for these areas.

We also need to focus on improvement as we look to the future. We plan to build more strength in specialties and expand our services based on how things develop over the next three to five years. For instance, we currently have 100 beds in the financial district, and it is doing very well. I don't anticipate needing more beds in the financial district of Hyderabad at this time; however, I may decide to create larger hub hospitals elsewhere. Building new facilities is another consideration. I could potentially utilize a Brownfield site that has been constructed but remains vacant. Ultimately, this decision will be made based on careful analysis of our needs and opportunities."

Alankar Garude: Sir, and if you look at the last decade, we have entered into several new markets. So Bangalore, Chennai, then the two hospitals in Delhi now. I mean, maybe we can also include the upcoming Gurgaon there is Vizag as well. So if you look at the next 5, 10 years, you mentioned about Delhi NCR and the plans over there. But apart from that, apart from these existing markets, which other markets can we potentially look to target?

Dr. Ramesh Kancharla: I believe we have always been very disciplined in our approach. Wherever we expand, we aim to establish a position of strength. I anticipate that Bangalore will develop similarly to Hyderabad

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over the next five to seven years. Chennai will also see a significant increase in the number of beds, as will Andhra Pradesh. This is the South story.

Now we are looking at Gurgaon. Once we start operations there, I will certainly consider expanding into the NCR region. This includes assessing areas like Noida, Greater Noida, Ghaziabad, and Faridabad, as there are many micro-markets to explore. Our understanding of these areas will improve significantly once we establish hospitals there, and we will definitely view NCR as a key area for our expansion.

If we explore other geographies, whether in the West or Northeast, we will apply a similar strategy to what we have done in these four regions. I think that is our approach. It is very unlikely that we will pursue sporadic small hospitals of around 100 beds in one city; that is not something we are keen to do."

Alankar Garude: Fair enough, sir. And one final question, possible to qualitatively comment on the performance of Bangalore, Chennai and Madhukar particularly Chennai and Madhu

kar, if you can comment?

Dr. Ramesh Kancharla: I think that Bangalore is performing quite well, and in Chennai, there are many new beds available. We have been operating in Chennai for about five to six years, and there are two new hospitals—one of which is less than two years old and other is recently opened. I see Chennai as a very good market, although it takes a little time to progress and build. It has good doctors

and a solid reputation, and I believe we have done well there. Last year experienced a bit of slowness, but we addressed all the issues, and things are going fairly well now. I think that we have done quite well in the last quarter.

Even Madhukar has seen positive movement; the top lines are strong there as well. However, Madhukar will not generate the same EBITDA margins as our other Rainbow hospitals because it simply isn't our model. We manage the beds, and the structure is very different. If I can achieve about 12% EBITDA at Madhukar, that would be good. For me, Madhukar is about establishing our brand, service model, and understanding the Delhi market, which will be helpful. As long as we maintain an EBITDA of about 12% to 15% over the next one to two years, that would be satisfactory. We are not expecting anything more than that, as we cannot manage more. For instance, I provide about INR 70 to 75 lakhs in free treatment every month, which otherwise would contribute to EBITDA. Additionally, we primarily manage beds, which means a significant amount of money flows back to the owners in the form of rent and other costs. Our capital expenditure for beds is low. When we consider all these factors together, our costs are not high. We have seen other hospital groups, such as Narayana, which run a similar model. For example, they operate a hospital in East Delhi and also struggle in the same way—while the hospital performs well, generating EBITDA remains challenging.

Alankar Garude: Just one follow-up there on Delhi NCR. If you look at the two sectors wherein we are opening our hospitals in Gurgaon, clearly there are a lot many more multi-specialty hospitals coming up for expansion over the next three to five years in that same micro market, if I take a certain radius. I understand that our model is different, but just can you share your thoughts on whether

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incremental multi-specialty capacity coming up, significant capacity coming up in that same micro market.

Is it something which worries us? Is it something which is something which we are actively considering? And also, I mean, when I look at our future plans, do we also consider upcoming capacity of multi-specialty beds coming up in any of the newer markets for us?

Dr. Ramesh Kancharla: No, what happens is that Gurgaon is perceived differently by people. You can talk to other promoters as well, but I don't think people are looking at Gurgaon as just a micro-market. Instead, they view Gurgaon as part of a larger health city overall. From the perspective of NCR and the North, everyone sees it as not just for Gurgaon, but for the entire NCR and even internationally. Every hospital seems to be aligned with this vision, so if you are in Gurgaon, you want to seize the opportunities.

In my view, the long-term network effect of Gurgaon will create a large ecosystem for medical care in the country. I'm not sure about other promoters, they may have more knowledge about it because they've already been running the hospitals. You might see it become one of the healthcare hubs in the world. This is how I envision Gurgaon shaping up. If you observe the investments and the quality of hospitals being built, you can see that every promoter is proud of what they are contributing to Gurgaon. That is how the network effect benefits everyone. I don't think the competition will be just about competing with one another; while there will be competition, the

opportunities will also be greater. People will come from wider parts of the country and even internationally, flocking to Gurgaon. That's how I see it unfolding. For me, when I'm looking at building 300 to 400 beds in Gurgaon, I'm not just considering Gurgaon; I'm thinking about NCR, the North, and the international market. That is the positioning I have for that hospital. This is, I believe, the way forward. I understand what other promoters and hospital groups are doing; everyone is looking at Gurgaon as a major healthcare hub.

Vikas Maheshwari: And also, we have gone through your report, right? I think that gives you some comfort. If you look at Bangalore, Chennai, Hyderabad, and even Mumbai, they have more beds per million than NCR, based on the projections you provided. It is expected to have roughly 2,700 beds per million, whereas Bangalore has 4,300 beds per million, Hyderabad has 3,600, and Chennai has 4,000 beds. So, I think there is a lot of scope.

As the doctor is also saying, we should not project NCR's healthcare needs based solely on its population. The road connectivity to nearby states, which have limited healthcare infrastructure, should also be considered. Therefore, Gurgaon is going to emerge as a healthcare center for all of North India, and even internationally.

Alankar Garude: Understood. That's helpful. Thank you and all the best.

Dr. Ramesh Kancharla: Maybe you should tell me, because you interact with all the promoters, so you can give me a better view about it. Next time when we meet together.

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Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments.

Dr. Ramesh Kancharla: Thank you. We appreciate your participation in today's conference and thoughtful questions. Your support is integral to our strategic progress and we value the time invested by all of you in understanding our business model and future plans. For further information, please reach out to Mr. Saurabh Bhandari, Head, Investor Relations , or investorrelations@rainbowhospitals.in. Thank you.

Vikas Maheshwari: Thank you.

Dr. Ramesh Kancharla: Thank you everybody.

Moderator: On behalf of IIFL Securities Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Note:

■ This is a transcript and may contain transcription errors . The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy. Please also note that this document has been edited without changing much of the content, for enhancing the clarity of the discussion. No unpublished price sensitive information was shared/discussed on the call.

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Call: Feb 2025

February 15, 2025

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.
Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.
Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q3 & 9M FY25.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015 ("SEBI Listing Regulations").

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on February 10, 2025, with regard to Financial Results of the Company for Q3 & 9M FY25.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q3__9M_FY_25.pdf

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited

Shreya Mitra
Company Secretary and Compliance Officer

Encl.: As above 5 1/~

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"Rainbow Children's Medicare Limited
Q3 FY '25 Earnings Conference Call"
February 10, 2025

Call Duration ■ 1 Hour and 5 Minute

Management Speakers ■ Dr. Ramesh Kancharla – Chairman and Managing Director

■ Mr. Vikas Maheshwari – Group Chief Financial Officer

■ Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions ■ Mr. R Sen, MAS Capital

■ Ms. Damayanti Kerai, HSBC Securities and Capital Markets

■ Mr. Sumit Gupta, Centrum Broking

■ Mr. Alankar Garude, Kotak Institutional Equities

■ Mr. Nitesh Dutt, Burman Capital

■ Mr. Nathan Subramanian, Individual Investor

■ Mr. Anshul Agrawal, Emkay Global

■ Mr. Pritesh Chheda, Lucky Securities

■ Mr. Deven Kulkarni, Marcellus Investment Managers

Moderator ■ Mr. Rahul Jeewani - IIFL Securities Limited

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Nurturing Smiles, Shining Bright

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Moderator: Ladies and gentlemen, good day, and welcome to the Rainbow Children Medicare Q3 FY '25. Earnings Conference Call, hosted by IIFL Capital. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask

questions after the

presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, sir.

Rahul Jeewani: Yes. Good morning, everyone. I'm Rahul from IIFL Capital. I welcome you all to the third quarter earnings conference call of Rainbow Hospitals. From Rainbow, we have with us today, Dr. Ramesh Kancharla, Chairman and Managing Director; Mr. Vikas Maheshwari, Group CFO; and Mr. Saurabh Bhandari, Head Investor Relations and Group business Analyst. Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla: Thank you, Rahul. Good morning.

Wishing you all a very happy new year, and a very warm welcome to our earnings call for the Q3 and the 9 months of the FY '25. I will discuss the company's progress, vision and exciting opportunities ahead.

Before that, I would like to take a couple of minutes to share that we have successfully concluded the silver jubilee celebrations of Rainbow Children's Hospital. Being an young organization with an average age of 32, the events were energetic and electrifying, featuring outstanding performance from employees. It was an immense joy to celebrate this milestone with an entire range of family, and I was deeply touched for the love and strong bonding the team shares with the Rainbow.

We have recognized and honored hundreds of employees for their outstanding contributions. Over the last 25 years, this incredible journey has touched millions of children for their health care needs and saved thousands of lives, hundreds of doctors being trained and creating value

I would like to take this opportunity to thank all of our medical, paramedical and support teams for their unwavering commitment in delivering exceptional medical care for children and women positioning Rainbow Children's Hospital as the India's leading children's hospital group.

■ We had a strong Q3 performance with our growth across all operating metrics, including our new hospitals launched in Hyderabad, Bangalore and Chennai, 3 quarters ago. These new facilities have now well integrated within our Rainbow hub-and-spoke model. ~

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■ Our IVF services have shown good progress and gaining traction, reinforcing the potential as a key growth driver for future. Recently, we have added a new IVF clinic close to Rainbow unit Bannerghatta Road, Bangalore bringing our total number of IVF clinics to 12.

■ Rainbow opened a state-of-art Child Development Center in the last quarter at Banjara Hills, Hyderabad, setting a new benchmark in a pediatric developmental care. This facility now serves as a central hub, consolidating child development service system all the Rainbow Hospitals in Hyderabad to ensure comprehensive and efficient care for children in one location.

h care permits for patients seeking medical travel.

■ Now delving on to those numbers, for the Q3 FY '25, our revenue has registered growth of 18.5% amounting to INR398 crores. Similarly, our EBITDA increased by 14%, reaching to INR134.3 crores, while PAT registered a growth of 10.2% to INR68.9 crores.

Coming to projects update.

■ We opened new outpatient clinics in the busy residential area of Attapur in Hyderabad. This clinic operates in conjunction with the Banjara Hills facility, ensuring a wider coverage for the hub hospital.

■ The regional hub hospital in Rajahmundry, Andhra Pradesh of 100 beds is nearing completion and is expected to commence operations by May '25 of the calendar year.

■ The project work at the spoke hospital in Electronic City of 90 beds and the Hennur, Bangalore of 60 beds are progressing well. Both hospitals are expected to commence operations by end of the Q2 FY '26.

■ The project work at -- commenced with a regional hub hospital in Coimbatore of 130 beds. The project is running slightly delayed with our operations anticipated to commence in about 24 months' time.

■ The company has obtained official building plan approvals for the land parcels in Sector 56 in Gurugram and -- while approval for Sector 44 is probably a few weeks away. Our project team is busy in the tendering process. ~

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In addition to our growth plans, I would like to highlight some key achievements that reflect our -- the commitment to delivering the high-quality pediatric and perinatal care. Over the past 9 months, we have successfully treated a number of children in intensive care services and also at the group level and managed complex pediatric specialty cases.

Here are a couple of notable examples, which I would like to present.

Case 1: World's First fetal balloon aortic valvuloplasty using a pioneering closure device

■ Our cardiac team at Rainbow Children's Heart Institute performed the world's first fetal balloon aortic valvuloplasty using a pioneering closure device.

■ I'll expand a little on this case—it's quite interesting. A 35-year-old woman was carrying a 27-week fetus with severe aortic stenosis, left ventricular dysfunction, and reversed flow in the aorta.

■ Our multidisciplinary team, including cardiologists led by Dr. Nageswara Rao, fetal medicine specialists, and obstetricians, successfully performed a fetal balloon aortic valvuloplasty on this 27-week, 700-gram baby while still in the womb. The ground-breaking aspect of this procedure was the use of a closure device to seal the puncture site—making it the first known case globally using this innovative approach .

■ To put it in perspective, the heart of a 700-gram foetus is about the size of a large grape. Normally, we approach the heart by puncturing the ventricle at the bottom and then performing the balloon valvuloplasty. However, in this case, the baby was so small that a conventional small puncture wouldn't allow the balloon to pass through. Therefore, our team decided to use a large bore, creating a wide opening to facilitate the procedure.

■ This approach led to massive blood leakage around the heart, which could have been life-threatening. To prevent this, our team successfully sealed the puncture site using a closure device—an unprecedented feat in fetal cardiology. This is what makes this case truly ground-breaking and the first of its kind in the world .

■ This pioneeri

ng success marks a significant milestone in fetal cardiology, expanding the possibilities of in-utero interventions and bringing hope to families worldwide facing severe fetal heart conditions.

■ The case gained widespread attention in national media, with coverage in all major national publications. Even Honourable Prime Minister Modi acknowledged this ground-breaking procedure."

Case 2: Successful Keyhole Surgery for a Rare Brain Tumor in a Child

■ The second case is successful keyhole surgery for a rare brain tumor, a 7-year-old child, who was experiencing persistent headaches for 10 months and gradually losing the peripheral parts of the vision along with the 2 seizure episodes.

■ The family consulted our pediatric neurology team, the MRI revealed a cystic tumor in the sellar and suprasellar parts of the brain, extending into the frontal lobe. A biopsy confirmed it to be adamantinomatous craniopharyngioma, a rare type of surgically treatable brain tumor. .~

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■ The child underwent minimally invasive keyhole surgery and our surgeons managed to excise completely the tumor.

■ Doing a keyhole surgery in this child reduced the complications, ensured a short stay and also provided a better cosmetic outcome.

■ 2 weeks post-surgery, the child was discharged with a complete neurological recovery.

These cases underscore the critical role of a multidisciplinary team in managing tertiary and quaternary care patients.

With that, I will now pass the mic to our Group CFO, Mr. Vikas Maheshwari: to take us through

the financial update. Thank you once again for joining us today. We look forward to your questions and insights as we move forward. Thank you.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you, and thank you for attending this investor conference. I'm pleased to brief you on the financial performance and the key developments of Rainbow Hospitals for the third quarter and the first 9 months of current financial year.

■ Our operating revenue for the quarter stood at INR398 crores, reflecting a growth of 18.5% when compared to the corresponding quarter of the previous financial year. For the first 9 months, our revenues stood at INR1,146 crores, reflecting a growth of approximately 20% when compared to the 9 months of the previous financial year.

■ Our EBITDA for the third quarter amounted to INR134 crores, marking up 14% growth compared to the same period last year. For the first 9 months, our EBITDA stood at INR375 crores, reflecting a growth of 16% when compared to the 9 months of the previous financial year.

■ For the 9 months of the current financial year, EBITDA is slightly impacted by close to INR7 crores due to one-off events related to 25th year anniversary celebrations.

■ The EBITDA margin for the current quarter is 33.8%, while for the first 9 months, our EBITDA margin is 32.7%.

■ The profit after tax for the quarter is INR69 crores, marking a growth of 10.2% in comparison to the corresponding quarter of the last financial year. For the first 9 months, our PAT stood at INR188 crores, reflecting a growth of 12.2% when compared to the 9 months of the previous financial year.

■ In terms of the operational performance, both outpatient and inpatient volumes witnessed a growth of 12% each when compared to the corresponding period in the last financial year. Our payer mix continued to remain robust and balanced with 51.3% of the revenue coming from the insurance and the balance, 48.7% coming from the cash patients.

■ For the first 9 months, the payer mix stands at 48% cash and 52% insurance. Furthermore, international business consti

tutes now approximately 2% of our total

business for the third quarter. As highlighted earlier by our Chairman, we are facing some headwinds in the international business, and we are working to mitigate the impact.

■ I'm pleased to inform that our company's balance sheet remains very robust with a net cash position of INR667 crores as of December 31 of the last year -- 31st December --

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of last year and will support our ongoing capital expenditure plan. Given our current cash and anticipated internal accruals in the coming quarters and the years, we remain confident in our ability to complete all planned capital expenditures through internal accruals without any debt financing.

■ During the quarter, the company has invested approximately INR22 crores in the capital expenditure.

With these insights, I conclude my financial updates. I now invite questions and suggestions from the participants. Thank you very much.

Moderator: Thank you very much. The first question is from the line of R Sen from MAS Capital. Please go ahead.

R Sen: Happy to see a good set of numbers that you've kind of reported. So just wanted to understand, we spoke about the largest pediatric training program. If you can just share some light about the opportunity size? And what kind of market do we see this kind of turning out to be?

Dr. Ramesh Kancharla: Yes. We have a total of about 200 DNB seats across the group for training. This is full-scale training for Pediatrics, and it is competitively selected through the NEET examination. Rainbow, being a premium institute, fills its seats in the first tranche itself, well within the top few thousand rankings.

We also offer extensive super-specialty training in neonatology, pediatric intensive care, pediatric hepatology, hemato-oncology, and cardiology. Additionally, we have a neurology program. Today, we have large training centres dedicated to training professionals."

R Sen: Okay. Okay. Sure. Sir, just drawing parallel with some of the other hospital chains, not in the pediatric region, I mean, space, but especially in the eye space, I see, there's a ultra-asset-light model that they have kind of used, especially for penetration into Tier 2 cities. Now the question

was, is there a school of thought with the management to kind of explore this model to enter Tier 2 cities with the ultra-asset-light model, which acts as a catchment area and feeds into the Tier 1 branches that we have?

Dr. Ramesh Kancharla: For a Rainbow operating model, ultralight may not be suitable, whether we go to the main cities, spoke hospitals, or even districts. That's because we are a complete healthcare model, not a part of a segmental healthcare model. Especially in children's healthcare, and when combined with maternity, we require space for both women and children, along with emergency services, an outpatient department, surgical procedures, intensive care, and birthing facilities.

Since it is a large setup, no matter how much we try to compromise, 50 beds is the minimum. Around 35,000 to 40,000 square feet is the lowest space requirement. That is what we always consider. Going ultra-small means actually compromising some of our offerings to patients. As an emergency hospital, this would make it difficult for us to operate, satisfy people, and gain traction. ~

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R Sen: Got it. Got it. Sir, if I may just ask one last question. This is at a macro level, 1,935 beds as on 2025. Can you share your aspirations till 2030, where do we see how many beds and at a long-term goal?

Dr. Ramesh Kancharla: We have a trajectory, which has already been discussed, of about 1,000 beds in the next 3.5 years, Sen. About 400 beds are planned for the NCR, with two hospitals already planned in Gurgaon. The remaining 600 beds will be in regional spokes as well as spoke hospitals in the south. Additionally, some regional spokes in new geographies, such as Rajahmundry and Coimbatore, are also planned.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Sir, my first question is on your Gurugram site. So can you update us like what are the pending approvals? And as you said, like some work has already started, but -- when are you expecting to start the construction work and now like what should be the time line for completion for the Gurugram site?

Dr. Ramesh Kancharla: We are almost there. The permissions for one of the sites have been granted, and we have heard that the other site's permission is expected in about a couple of weeks. We are actively engaged with our projects team, which is busy with the tendering process—floating tenders and inviting participants for the base building construction. We are quite active now and will likely start site construction in about 4 to 6 weeks.

Damayanti Kerai: 4 to 6 weeks?

Dr. Ramesh Kancharla: Yes.

Damayanti Kerai: And you mentioned approval for Sector 44 is a few weeks away. What about the other site?

Dr. Ramesh Kancharla: Other one is already there. We have received the permission for the Sector 56.

Damayanti Kerai: Okay, okay. So very broadly, say, you start on the construction, etcetera, in another 4 to 6 weeks.

So reasonably, we should be looking at FY '28 as the launch time line for this Gurugram facility?

Dr. Ramesh Kancharla: Yes. We are also taking steps to speed up construction, including using more steel structures and similar approaches. We are exploring various ways to accelerate the process of constructing the base building. Once the base building is completed within a year, we can set a more aggressive pace to complete the project. We expect to start in about 2.5 years from now.

Damayanti Kerai: Okay. That's helpful. My second question is your international business. So right now, it's small, like, 2% of your total business. But what are your aspirations here and especially in light of what we heard about this Heal in India program for promoting medical tourism. So I just want to hear your thoughts on this -- this part of the business.

Dr. Ramesh Kancharla: We started our international business post-COVID, and it was going pretty well. We had budgeted about 4% of the top line to come from international business this year (FY25). Last year, we clocked about INR 44 crores and were quite ambitious about its growth. ~

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Unfortunately, geopolitical situations have impacted key regions. Bangladesh and some areas like Oman have faced challenges, and Somalia—our largest contributor—along with Sudan, is experiencing serious internal problems. As a result, there has been a significant reduction in the healthcare business across these regions.

I'm not sure about other groups, but we have seen almost a 40% decline in this business compared to last year.

Damayanti Kerai: So you said -- you earlier budgeted for 4% of revenue from this segment, but due to macro uncertainties, there has been some 40% reduction from your initial anticipation. So where sh

ould

we end for FY '25 in terms of contribution?

Dr. Ramesh Kancharla: I think by the end of this year, we are expecting to close at around INR 34 crores. Last year, we did about INR 44 crores. We are waiting for things to open up. Our international journey is still in a very early phase, which is why it remains a building story.

Once things open up, we are also actively exploring opportunities in other countries, such as the Philippines, Mauritius, Uganda, and Zimbabwe. Since this is still an early-phase building story, we aim to establish a closer connection with doctors in African nations and neighboring Indian countries. This will help strengthen our relationships and build a better referral system (for medical tourism).

Damayanti Kerai: Okay, sir. And my last question is, in this quarter, the staff costs, we have seen around 8% sequential decline. I understand there is a bit of seasonality related factor as well. But in your staff costs, like what percentage will be variable in nature and which can just be adjusted according to the top line performance?

Vikas Maheshwari: Damayanti, that's a good question and a good observation. If you go back to Quarter 1, our staff cost was around INR 49 crores. In Quarter 2, which is seasonally very strong, we had additional nurses, paramedic staff, and contractual workers for cleaning, etc. So the costs slightly went up, along with some OT payments, as nurses had to work harder on patient handling, etc. I believe this is now normalized and has returned to its usual level. This should be the expected cost at the current bed capacity at which we are operating."

Damayanti Kerai: Around INR49 crores, INR50 crores kind of staff cost...

Vikas Maheshwari: INR 50 crores is what it should be. In the quarter 4, obviously, there will be some slight adjustment on the gratuity, etcetera, but will not impact much of the things. (HR cost)

Moderator: Next question is from the line of Sumit Gupta: from Centrum Broking.

Sumit Gupta: Sir, just now -- so on the capex part, like now that this Delhi-NCR project, Gurgaon hospital has now moved to FY '28. So I just want to understand on the overall capacity plans for the next 2, 3 years. You said, the same INR550 crores that you guided for? ~

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Vikas Maheshwari: We have already listed out our capex plan in our presentation. So, in FY '25-26, we are coming with 250 of the beds, and these are all asset-light, means these are leased assets. And similarly, on FY '26-27 is 130 beds. So roughly 380 beds to 400 beds, which will come in next 2 years. Since these are on the leased assets, roughly INR60 - 65 lakh per bed, you should assume on that as a capex.

As far as Gurugram hospitals are concerned, we have already spent around close to INR180 - 190 crores on the land acquisitions and related registration and permissions, etcetera. Now you should budget another INR400 crores in the next 3 years' time, starting from FY '25-26 to FY '27-28. But most of this capex will happen post 1 year because right now, once we have the approval, the ground breaking and then foundations, etcetera, will not take much of the cost. So most of the costs will come after 1 year. So that is what is the trajectory. The exact trajectory of Gurgaon, once we start the project, we will come to know. But that is the more or the less INR400 cores in the next 3 years for the Gurgaon.

Sumit Gupta: Understood, sir. And sir, lastly, on the -- like how the overall new centers, which were opened in the last 2 to 3 quarters, they are trending in terms of occupancy and profitability. Like what has been the trend that you're seeing?

Vikas

Maheshwari: For the past nine months, if you look at it—since we opened our units around March last year, about nine months ago—our new hospitals have an occupancy rate of around 37%. I think that

is a good level. On a blended basis, including some units that were opened two years ago and so on, the overall occupancy level is strong.

The new units are performing well and are on the expected trajectory. We have already provided guidance on breakeven timelines: for Hyderabad, we expect to breakeven in 12 months, for Bangalore in 15 months, and for Chennai in about 18 months. These projections remain on track with no changes

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, at the beginning of the fiscal, you had alluded to focusing more on volume growth in FY '25, especially with all the new hospitals, which became operational in 2024. Now given that we are seeing decent volume growth across both these newer centers as well as the existing centers, how should we look at ARPOB growth in FY '26?

Vikas Maheshwari: Okay, Alankar, this is a good question. See, we have been guiding on this for at least the last 2 to 3 quarters. ARPOB is a somewhat complex subject because it has two key variables: Seasonality and ALOS.

If ALOS increases, ARPOB gets suppressed, even though occupancy goes up. That's why we are guiding and requesting all analysts and investors to focus on our ARPP growth instead.

ARPP growth has been consistently between 5% to 8%, depending on the quarter.

We expect this trajectory to continue for two reasons. First, as new centres mature, we gain some pricing power. Second, mature centres start handling more complex cases and clinically challenging work, where ARPP is naturally higher. ~

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So, when looking at ARPP, you should consider a growth range of around 5% to 9%, as this reflects real growth. It also eliminates one major variable—ALOS. In our case, ALOS has increased by 12%, which has impacted ARPOB. If not for this, ARPOB would have been higher.

Alankar Garude: Actually, that was my other question. I mean, ALOS, both in new and existing, has increased meaningfully in 9 months. So let us know the reasons, at least for the matured hospitals, what is the reason for the sharp increase?

Vikas Maheshwari: Due to operational issues and the fact that the insurance business accounts for 50% of our revenue, there are times when insurance approvals come in late, causing delays of 4 to 5 hours.

Most of these delays are related to operational efficiency.

Additionally, more complex cases lead to patients staying longer in the hospital.

So, it's a combination of both factors. However, when it comes to operational delays, those are areas where we can definitely improve.

Alankar Garude: So how should we look at ALOS then going forward? Should it come back to the 2.6, 2.7 number?

Vikas Maheshwari: A 9-month median is a good average to consider. Right now, it stands at 2.8 to 2.9 days, and the 9-month average should be a reliable benchmark. We expect to improve from here, but that should be the average to consider.

Dr. Ramesh Kancharla: Yes, traditionally, Alankar, our ALOS ranges between 2.6 to 2.8 days. Depending on seasonality—if the seasonal demand is higher and more on the optimistic side—ALOS tends to come down. However, with greater complexity, more specialty cases, and increased NICU admissions, ALOS shifts slightly higher. This is a dynamic that continues to fluctuate.

Typically, in Q2 and Q3, ALOS is lower. But this quarter, seasonal demand was not as high, which led to a slight increase

se. I will definitely review whether any other factors contributed to this rise in ALOS.

That said, for a mature pediatric hospital, ALOS generally settles around 2.7 to 2.8 days, which is what I would expect.

Alankar Garude: Got it, sir. And one last question. You had spoken about evaluating certain M&A opportunities in the past. Can you update us on the progress on that front?

Dr. Ramesh Kancharla: Alankar, we continue to explore these opportunities and will keep working on them. Once they reach a certain stage of conclusion, we will be sure to update you.

Moderator: The next question is from the line of Nitesh Dutt from Burman Capital.

Nitesh Dutt: My question is related to the new hospitals that we added in Q4 of last year and Q1 this year. I think we have added close to 230 new operational beds. So what was the EBITDA drag because of these new beds this quarter? ~

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Vikas Maheshwari: Yes, that's a good question. The EBITDA drag is in the high single digits, close to INR 8-9 crores for the three hospitals combined over the first nine months.

Nitesh Dutt: Understood. Got it. Second question, the delays in 3 hospitals that you mentioned. Just wanted to understand the reasons for the delays. Gurgaon one, I think, you mentioned due to approvals. But for the remaining ones? And also any chances of further delay? Or do you think the stated time lines -- by those time lines, the hospitals should become active?

Dr. Ramesh Kancharla: I think there's just a slight delay of a couple of months in Rajahmundry—not a significant one, just a slower pace. Since it's a Tier 2 city, challenges are always expected, mainly because our teams, including project teams and vendors, are based in larger cities.

In Chennai and Coimbatore, there was a redesign of plans due to changes in government rules and regulations related to offsets and other factors. As a result, we had to resubmit the plans and go through a re-approval process. Since Coimbatore's approvals are handled in Chennai, this caused some degree of delay.

Otherwise, everything is ready for construction, and execution is already underway. Now, we are looking to accelerate the pace of work.

Nitesh Dutt: Understood. Great. One more question. Can you just give some sort of outlook for FY '26, both on occupancies and ARPOB?

Dr. Ramesh Kancharla: I think it is a bit early to kind of look at it, still we are in the current financial year. So probably we'll discuss in the next earnings call.

Moderator: The next question is from the line of Nathan Subramanian an Individual Investor.

Nathan Subramanian : First of all, wishes for your excellent performance. Okay. I just want to know -- I am a new investor. I just want to know how do you differentiate a new hospital and mature hospital? And that means how many months after you consider a new hospital as a mature hospital? And my second question is, what are the likely growth drivers?

Vikas Maheshwari: Okay. We classify matured and new hospitals based on the tenure from the start of operations. A new hospital is categorized as one that has been operational for less than 60 months (5 years), while a matured hospital is one that has been in operation for more than 5 years. This is how we differentiate them.

There are three key growth drivers:

■ Our matured hospitals continue to expand by adding doctors and new specialties, contributing to growth.

■ Our new hospitals (operational for less than 5 years) grow at a faster pace compared to matured hospitals, adding another source of growth.

■ The third growth driver comes from the new hospitals we are adding. In FY '25-'26, we will be

adding three hospitals, increasing our capacity by approximately 12.5% to ~

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13%, or close to 250 beds in the next financial year. This expansion will also drive growth in the coming quarters and years.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: Great. Sir, any reason that you would want to call out for the different gross margins in the current quarter? I believe this quarter would have lower surgical mix.

Vikas Maheshwari: Yes, Anshul. If you compare it with the second quarter, the mix in the third quarter will be slightly better. The second quarter is seasonally strong, with a higher volume of low-ticket, routine cases due to seasonal factors. In contrast, the third quarter sees a better mix of surgical and clinical cases compared to the second quarter.

Anshul Agrawal: No, I was trying to look at it, sir, on a Y-o-Y basis as well, we've seen a gross margin decline of almost 80 bps. And I thought considering that there are more medical cases in this, there should not be any reason for gross margins to sort of dip.

Vikas Maheshwari: On a year-on-year basis, Anshul, what has happened is that in the last quarter (March), we added three facilities, which have contributed to the drag in the P&L. This is one factor.

The second factor, as I mentioned in our opening remarks, is a one-off expense of close to INR 7 crores, which we incurred for our 25th anniversary celebrations.

If you exclude these two factors, we are essentially at the same EBITDA margins as before.

Anshul Agrawal: Got it. Got it. My second question, sir, is on EBITDA margins going ahead. And now I believe we'll about start to see breakeven on certain new facilities that we added in Q4. And at the same time, these new facilities should also start coming up in Coimbatore, et cetera. So would our margins sort of remain stable? Or do you see that -- do you feel that it will dip for probably about 2, 3 quarters in FY '26 as well?

Vikas Maheshwari: As a business, in the hospital sector, whenever a new hospital opens, there will always be some drag. The key question is how much the drag is and how much capacity is being added, right?

If you look at last March, we opened three hospitals, meaning the drag from these hospitals has been reflected from April to December. Despite this, our EBITDA margin has remained at 32%-33%, and for the nine months, it stands at 32.7%.

Going forward, with the three new hospitals we are adding, there will naturally be some drag. However, our effort is to keep the range within plus/minus 1% of our current level. As the newly opened hospitals progress, they will start contributing to EBITDA—albeit on the lower side—but they will move from negative to positive. Meanwhile, the new hospitals will have some initial drag.

Overall, we expect to balance this within a plus/minus 1% range, maintaining a strong margin of around 32.7%, which we believe is a good level to sustain. ~

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Anshul Agrawal: Got it. Just one last question, sir. Sir, once the Gurgaon facility comes online or in the runup to that, I believe our return profile would sort of get hit because of the asset-heavy nature of this facility. Any insights around how do we see this? Do we intend to get back to our 30% plus ROCE profile for, say, 1 or 2

years of this Gurgaon facility commencing? Any thoughts around this, sir?

Vikas Maheshwari: Any large capacity addition, by nature and arithmetically, will impact ROCE. The key question is the extent of that impact.

To give some perspective—while the exact numbers can be worked out—right now, we have ~2000 beds. By the time the Gurgaon facility starts, we should have around 2,500 - 2,600 beds, considering the additions we have planned and potential acquisition opportunities that may be integrated with us. This means we will be 25% to 30% larger than our current base. These additions will be EBITDA-generating and contribute to returns on the capital we have invested, which will help offset some of the ROCE impact from Gurgaon.

However, as Gurgaon begins operations, there will naturally be some drag on ROCE. This is inherent in the nature of the business—we must continue investing capital for future growth while driving efficiencies to ensure Gurgaon matures early and starts contributing positively. That remains our focus.

Moderator: The next question is from the line of Pritesh from Lucky Securities.

Pritesh: I have 2 questions. So one on the mature hospitals at 60% Occupancy Rate in 9 months, what is the further room in this occupancy ratio?

Dr. Ramesh Kancharla: Yes, we can actually reach occupancy levels of up to 68% to 70% in our mature sites, and there is still room for further growth in these hospitals. Within the mature hospital segment, some facilities are currently operating at around 50% to 55% occupancy, while others have already reached 68% to 70%. This indicates that there is still potential for growth within the mature hospitals, along with significant headroom in our newer hospitals.

Pritesh: Okay. And wasn't that versus a typical multi-specialty tertiary hospital, our children's hospital - in your earlier calls, you had always mentioned children's hospital have lower occupancy. So am I confusing something here or there is some revision in this OR number?

Dr. Ramesh Kancharla: No, sir, let's not confuse ourselves by comparing with multi-specialty hospitals, as they operate differently. A multi-specialty hospital primarily deals with wear-and-tear problems over a lifetime, whereas a children's hospital is a medical facility focused on treating pediatric patients requiring hospitalization, admissions, and acute care. While some cases involve chronic illnesses, the proportion is inverse compared to a multi-specialty hospital.

As a result, children's hospitals experience seasonal variations and different patient dynamics.

Additionally, we do not have a fixed government business consistently occupying our beds.

Instead, our revenue mix is approximately 50% insurance-based and 50% out-of-pocket

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Pritesh: Yes. So, sir, then by that logic, versus 70% occupancy of multi-specialty adult hospital, shouldn't the children's hospital be a lower occupancy?

Dr. Ramesh Kancharla: Yes, definitely, our occupancy levels are relatively lower. Even within our matured hospital segment, we maintain over 60% occupancy, but it's unlikely that we will exceed 70%. This is due to a combination of factors, including the varied nature of pediatric bed utilization and seasonality, as well as the absence of fixed government business.

Given these dynamics, a blended occupancy of around 60% is sufficient for us to deliver strong results. While we always aim to improve occupancy levels, children's hospitals naturally operate at lower occupancy rates compared to adult hospitals. This is why direct comparisons with multi-specialty or adult hospitals aren't appropriate—they

function in entirely different domain.

Pritesh: So is the -- so then that number is still 70 or the number is not -- will be less than 70 for mature?

Dr. Ramesh Kancharla: Sir, it's important to note that we are the first dedicated children's hospital in the country operating at this level—it's a continuous learning journey. If you ask me about the maximum occupancy we can realistically achieve, I'd say around 65% to 68%. Beyond that, it becomes challenging due to the inherent factors of pediatric care, including seasonal variations and specialized bed utilizations.

Pritesh: Okay. And my second question is on the Gurugram capex. So you mentioned that INR180 crores is spent on land plus INR1 crores per bed incremental. That's how it is?

Vikas Maheshwari: Yes, that's correct. Another INR 400 crore. (it was inadvertently mentioned as 200 Cr)

Pritesh: So, then it becomes INR180 crores, even INR200 crores. So then your capex is -- it's INR145 lakhs per bed, right?

Vikas Maheshwari: Yes, close to INR 1.5 crores per bed. That's correct.

Pritesh: So then the ROCE in this hospital will be less than 20%. So any observation there, the ROCE profile will be really different here?

Vikas Maheshwari: This facility will be slightly different as it is a heavy-asset model and a large-scale project. When evaluating ROCE, we should take a long-term perspective on its impact. This will be a state-of-the-art hospital designed to serve the entire country, particularly North India and international patients.

If we compare it to other multi-specialty hospitals being developed in the region, the cost structure is similar, ranging from INR 1.5 crores to INR 1.75 crores per bed. However, since we own the land and building, it is a more capital-intensive project. That said, from day one, it will operate as a full-fledged super-specialty hospital, equipped with all necessary medical infrastructure and staffed by top-tier specialists

Dr. Ramesh Kancharla: Our upcoming hospitals will be more aligned with a multi-specialty model, as we are positioning them as pediatric multi-specialty hospital of the highest standards in the country. This greenfield ~

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project is being built for future of the country. The cost per bed, including medical equipment, will be on par with leading multi-specialty hospitals of today.

Pritesh: So how is it different from your Hyderabad cluster, where you have the large hospitals there? So you must be having a multi-specialty hospital even in Hyderabad cluster, right?

Dr. Ramesh Kancharla: That's true, sir. That's the...

Pritesh: So how different is it by a capex, let's say, you have a Banjara Hills hub, how different it will be from that hub?

Dr. Ramesh Kancharla: So we have spent about INR75 lakhs, INR80 lakhs per bed Banjara Hills about 7 years ago. Today, we...

Pritesh: Including land building or excluding?

Dr. Ramesh Kancharla: Excluding base building and land.

In Gurugram, the land and the building are our own, and we're going to spend the capex on top of it. Over the past seven years, cost escalations have risen by nearly 30% to 40%. Factoring in these increases, the projected cost per bed stands at approximately INR 1.5 crores, aligning with current industry benchmark.

Pritesh: Okay. So excluding land building, Banjara was INR65 lakh. Today, that same INR65 lakh is about INR1 crores per bed for a hub. And over and above that, in Gurugram, you have the land and building also coming in. That's how we should interpret it?

Dr. Ramesh Kancharla: Yes.

Moderator: The next question is from the line of Sumit Gupta from Centrum Broking.

Sumit Gupta: Sir, 2 questions.

First is on the ARPOB. So how do you plan to increase or optimize the ARPOB over the near and the medium term? And secondly, on the Gurgaon facility, like once the Gurgaon facility gets opened, I hope I expect that international patient mix will change for the better. So how will we expect that to improve the overall case mix as well as an improvement of overall ARPOB?

Dr. Ramesh Kancharla: Yes, it's a pretty long shot when it comes to Gurgaon. Obviously, this will be in a different class of hospitals, with a better price point and the ability to treat more complex cases. We will definitely position it very differently. Closer to its launch, we can discuss how ARPOBs and revenues will play out, but it's too early to talk about that now.

Overall, as mentioned earlier in the call by Mr. Vikas, ARPOB is influenced by many variables.

That's why we focus more on ARPP for internal purposes. Over the last 8 to 9 quarters, our ARPP has been growing at about 6% to 7%, which is how we track progress. ~

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ARPOB can fluctuate significantly due to ALOS and seasonality—two key factors that keep changing in pediatric healthcare, particularly in Q2 and Q3. Additionally, business mix variations make it difficult to anticipate a consistent pattern over 360 days.

So while we monitor ARPOB, we primarily look at ARPP. As long as ARPP is growing, we know revenue generation is strong, pricing is effective, and overall business quality is improving

Sumit Gupta: So sir, just on the ARPP point only, so like your insurance has been consistently around 52% -- hovering around nearly 52%. So how -- like regarding the price hike, what kind of -- what is the time frequency in which you take price hike to improve the ARPP? And what are the overall drivers driving that?

Dr. Ramesh Kancharla: The two key drivers of ARPP are price hikes and case mix, provided there is consistency in the patient set. If these factors remain stable, ALOS will not see much variation.

Pricing adjustments are made across the board to account for inflation, and every hospital continuously strives to enhance quality year after year. Additionally, hospitals focus on handling more complex cases, further contributing to business growth and improved ARPP.

Sumit Gupta: So, what is the magnitude of the price hike that you have taken over the last 2 to 3 years?

Vikas Maheshwari: The price hike needs to be considered on a blended basis across two fronts. On the cash side, last year, we did not implement a price hike; we only made corrections where competition was higher and adjusted accordingly. This year, we plan to introduce some price hikes, and the working is currently in progress. We are also benchmarking against competition. By March, once we finalize our budgets, we will make a decision and communicate the price adjustments. As for insurance, they typically set prices based on a two-year cycle, though in some cases, adjustments can take three years or even longer. On average, we anticipate a 12% to 15% adjustment, which translates to approximately a 4% to 5% price hike from insurance.

For cash pricing, we need to carefully balance competitiveness and affordability while ensuring that we do not overburden our patients. As a healthcare provider, we must remain accessible while also accounting for cost inflation. Our goal is to strike a balance that protects our EBITDA margins while maintaining fair pricing.

Sumit Gupta: Understood. And sir, lastly, so in the 9 months FY '25, so just -- so your ARPOB growth has been like -- there's been a decline of 6%. So what was the major reason? Like did you take any price hike or was it n

ot taken and subsequently, there was an inferior case mix. So what really happened in that?

Vikas Maheshwari: What has fallen 6%, sorry, I could not understand.

Sumit Gupta: ARPOB, 6% Y-o-Y growth.

Vikas Maheshwari: If you adjust the ALOS, you will not get it because ALOS has gone up, right? Ex of ALOS, our ARPOB would have gone up. Or if you look at ARPP, it has gone by 7%, 8%, basically. ~

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Sumit Gupta: So we should look at you saying ARPP and then adjust through ALOS?

Vikas Maheshwari: Yes, We are disclosing ARPOB and ALOS. If you multiply, you will get the ARPP.

Consistently, we have seen year-on-year basis, seasonal adjustments leaving up on. So you have to see the year-on-year basis. We have seen the ARPP growing actually.

Moderator: The next question is from the line of Deven from Marcellus Investment Managers.

Deven: Sir, what's the extent of price correction that we have taken? You just referred to it while answering the previous question.

Vikas Maheshwari: Last year, for cash patients, we did not implement a broad price hike. Instead, we selectively adjusted prices where we benchmarked ourselves against the competition. The net impact of these adjustments was minimal, likely around 1% to 2%.

For insurance, some clusters come up for renewal each year. As mentioned earlier, when we revise prices for a particular cluster or hospital, the price hike typically falls in the range of 12% to 15%, and these adjustments usually remain in place for two to three years. On a blended basis, this results in an overall price increase of approximately 4% to 5%.

This year, as we finalize our budget, we will review pricing strategies in consultation with management, considering industry trends and competitive benchmarks. Additionally, we will factor in expected cost inflation, particularly since manpower costs—including doctors, paramedical staff, and corporate staff—constitute around 40% of our expenses. Based on these evaluations, we will make appropriate pricing decisions to maintain financial sustainability while ensuring affordability for our patients.

Deven: Got it. And this 1% to 2% price correction that you have taken, in which cities or which cohort of hospitals have you done it?

Dr. Ramesh Kancharla: It is across. Once we take, we take across clusters, sometimes some were higher, some were lower. So that way.

Deven: Okay. So like this is, let's say, even in Hyderabad, Bangalore, which are your, let's say, core or old markets as well as new markets across the board, you have taken a price correction?

Dr. Ramesh Kancharla: Correct.

Deven: And it's mainly OPD or IPD or both?

Dr. Ramesh Kancharla: Both, across all.

Deven: Okay. Got it. And my second question, sir, while answering an earlier question, you said that the mature hospital can do around 65%, 68% occupancy. I remember that when we used to discuss this a year ago, you used to say that a mature hospital can do 60% occupancy at peak. Now today, that number seems to have gone up to 65%, 68%. So has anything changed in the last 1 year that we have increased the occupancy cap earlier we used to expect 60% and now 65%? ~

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Dr. Ramesh Kancharla: The overall trajectory shows that as more hospitals mature and some enter their second decade of operations, occupancy rates naturally increase. Ho

wever, while some hospitals can reach up to 68% occupancy, it is unlikely to exceed that level.

For instance, in Hyderabad, I have a few hospitals consistently operating at high occupancy levels, often facing bed shortages. That's why I mentioned 68% as the potential upper limit. However, at a group level, achieving a consistent 60% occupancy is unlikely, as new hospitals are continuously being added to the mature category. As a result, blended occupancy will likely stabilize around 60%, possibly reaching 62% or 63%.

It's also important to recognize that pediatric healthcare operates differently from adult hospitals. Parents prefer to minimize their child's hospital stay, even by a few hours, whereas in

adult hospitals, patients are often more willing to stay longer. This is a key reason we don't compare ourselves to adult hospitals.

We are still in an evolving phase, continuously setting new benchmarks. To answer your question, while individual hospitals may reach 68% occupancy, achieving that across the group on an annual or quarterly basis would be extremely challenging. However, from a results perspective, if matured hospitals maintain occupancy above 60% and blended occupancy is around 55%, we are delivering strong performance. If we reach 60% blended occupancy, that would be exceptional.

Ultimately, in pediatric healthcare, hospital stays are naturally shorter, and the focus is on delivering results efficiently rather than maximizing occupancy beyond what is necessary. Deven: Understood. Understood. And finally, sir, when I'm looking at your mature hospitals performance for Q3, it seems like the IP volume growth is 0%. In Q2, this number was 8% positive. So that's like slowdown from 8% to 0. Any reasons behind it?

Dr. Ramesh Kancharla: In our matured sites, you must have noticed that occupancy has increased overall. However, on a sequential basis, it has come down—from 68.6% in the second quarter to 60% in the third quarter. This is expected, as the second quarter is seasonally strong, with a higher influx of patients. Given this seasonal trend, maintaining a similar trajectory in the third quarter indicates that we have done a good job.

Deven: No. So I'm actually looking at year-on-year. So let's say, the occupancy has increased from last year. So last year, it was -- no, just Q3 to Q3. So then last year, it was 56.5% occupancy, and this year, it's 60%, but at the same time, ALOS has gone up from 2.6 to 2.9. So net-net, inpatient volume seems to be flattish.

Dr. Ramesh Kancharla: That's true.

Deven: Yes. So -- so the exact number, if you look at Q2, that number had grown at around 8% Y-o-Y, and Q3 is low percent growth. So what has happened that the growth has come off?

Vikas Maheshwari: At any given point in time, there is a continuous transition from non-matured hospitals to matured hospitals. So, the trajectory you are observing is a moving one. However, if you --

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compare on a like-to-like basis, we have seen inpatient (IP) number growth of approximately 9%.

Deven: Okay. Okay. Yes. Okay. Good. So...

Vikas Maheshwari: There is a continuous movement of beds, which impacts the way the data is perceived. However, if you look at the growth of matured hospitals on a stand-alone basis, it is approximately 9%.

The data might seem a bit confusing because we are comparing both moving data and static data from the last quarter, which creates a difference in interpretation.

Moderator: The next question is from the line of Nitesh Dutt from Burman Capital.

Nitesh Dutt: Just a quick clarification. The INR7 crores onetime impact that you mentioned, was it for Q3 of

this year or was it during previous quarters?

Vikas Maheshwari: Quarter 2 and quarter 3 is evenly distributed, you can say, almost evenly. INR3.5 crores roughly in this quarter and last quarter is the similar amount. Total INR7 crores in 2 quarters.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments.

Vikas Maheshwari: Yes. Thank you. We appreciate your participation in today's conference call and the insightful questions. Your support plays a vital role in our strategic progress, and we truly value the time each of you has taken to understand our business in the future plans. For the further information, if any, please reach out to Mr. Saurabh Bhandari, Head, Investor Relationship at [investorrelations@rainbowhospitals.in](mailto:investorrelations@rainbowhospitals.in).

With this, I close the conference. Thank you for participation. Thank you.

Vikas Maheshwari: Thank you.

Moderator: Thank you. On behalf of IIFL Capital, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Note:

■ This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

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Call: May 2025

May 31, 2025

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.
Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.
Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q4 & FY25.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on May 26, 2025, with regard to Financial Results of the Company for Q4 & FY25.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q4_&_FY_2025.pdf

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited

Shreya Mitra
Company Secretary and Compliance Officer

Encl.: As above

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May 26, 2025

Call Duration ■55 Minutes

Management Speakers ■Dr. Ramesh Kancharla – Chairman and Managing Director

■Mr. Vikas Maheshwari – Group Chief Financial Officer

■Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions ■Mr. Mahesh Atal, Attal Investment Advisors

■Mr. Anshul Agrawal, Emkay Global

■Ms. Damayanti Kerai, HSBC Securities and Capital Markets

■Mr. Alankar Garude, Kotak Institutional Equities

■Mr. Rahul Jeewani, IIFL Securities Limited

■Mr. Manik from Findoc Investment

■Mr. Nitesh Dutt, Burman Capital

Moderator ■Mr. Rahul Jeewani - IIFL Securities Limited

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Moderator: Ladies and gentlemen, good day, and welcome to the Q4 FY '25 Earnings Conference Call of Rainbow Children's Medicare Limited hosted by IIFL Capital. As a reminder, all participant

lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, sir.

Rahul Jeewani: Good morning, everyone. I welcome you all to the fourth quarter earnings conference call of Rainbow Hospitals hosted by IIFL Capital. From Rainbow, we have with us today, Dr. Ramesh Kancharla, Chairman and Managing Director; Mr. Vikas Maheshwari, Group CFO; and Mr. Saurabh Bhandari, Head Investor Relations and Group Business Analyst.

Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla: Thank you, Rahul. Good morning, everyone. It's a pleasure to speak with you all today as we discuss the performance for the fourth quarter and the financial year 2024–25. It has been a transformative year for Rainbow, marked by key operational milestones and a continued focus on delivering high-quality pediatric and perinatal care.

As I reflect on the fourth quarter and the year gone by, I'm pleased to report that we've made significant progress across several areas. Here are some of the key highlights:

■ On the operational front, the performance in Q4 was modest compared to the strong second and third quarters. We witnessed an unusually quiet season. Approximately 35% of our business—including pediatric outpatient department visits, pediatric admissions, and pediatric intensive care—remained subdued during this period. However, certain segments performed strongly, particularly pediatric surgery, pediatric multi-specialty care, and obstetrics. All the new beds added during the last year have been seamlessly integrated into the Rainbow network and are performing well. This is evident from the performance of our new units in both Q4 and the full year.

■ I'm pleased to share that we successfully performed our first pediatric liver transplant at our Chennai hub hospital. Additionally, we secured a liver transplant license for our Bengaluru hub—a significant step in building Chennai and Bengaluru into tertiary and quaternary care hubs.

■ It gives me immense pride to share that Rainbow has received several prestigious accolades.

- o We were honoured with the 'Great Place to Work' award for the fifth time, achieving a 90%+ score. This recognition is a testament to our ongoing efforts to build a culture where our people feel valued, empowered, and inspired.
- o We were also recognized as an 'Amazing Workplace for Excellence' in People Practices and named the 'Best Place to Work for Women' by The Economic Times.

- o Furthermore, Newsweek recognized Rainbow as the 'Best Pediatric and Women's Hospital' of the year.

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■ Our fertility center in Kondapur, Hyderabad, received reaccreditation from the Joint Commission International (JCI). With this, Rainbow now has three JCI-accredited hospitals and thirteen NABH-accredited hospitals in its network.

■ Our IVF services have shown strong progress, reinforcing their potential as a key growth driver.

■ Our retail initiative, "Product Financials," offering a range of baby and women's care products, has performed well. We have recently launched a pilot to offer these products online as well.

■ We continue to face challenges in our international business, particularly in countries such as Bangladesh, Oman, Kenya, and Sudan, where there has been a significant decline in the issuance of healthcare permits for patients

seeking medical travel. For the year,

international revenue stood at INR 30.7 crores, compared to INR 44 crores last year.

■ We continue to prioritize cost management and actively seek opportunities to optimize expenses. This disciplined approach is especially important as new hospital beds become operational to maintain profitability.

■ Now, moving on to the financials: For Q4 FY25, revenue grew by 8.5% to INR 370.1 crores. EBITDA increased by 8.7% to INR 114.7 crores, and PAT grew by 10.7% to INR 56.5 crores.

■ Our overall occupancy rate for the quarter was 46.5%. Mature hospitals recorded 52.2% occupancy, while new hospitals posted a 35.6% rate.

Project Updates:

■ Our 100-bed regional hospital in Rajahmundry, Andhra Pradesh, is in the final stages and is expected to commence operations by the end of Q1 FY26.

■ Two new spoke hospitals in Bengaluru—Electronic City (90 beds) and Hennur (60 beds)—

are expected to begin operations by the end of Q2 FY26.

■ The 130-bed regional hub hospital in Coimbatore has commenced project work and is expected to take 20–24 months to complete.

■ We performed Bhoomi Puja (groundbreaking ceremonies) at both our land parcels in Sector 44 and 56, Gurugram, in late April. Project work is progressing well at both sites.

Patient Care Highlights:

Let me share a couple of powerful patient stories that highlight our commitment to pediatric and perinatal care.

■ A couple from Myanmar, after several failed fertility treatments back home, travelled to our Chennai facility. They conceived through IVF, but the mother experienced premature delivery, giving birth to a 472-gram baby with barely any signs of life. Thanks to our neonatology team's 24/7 dedication and advanced care infrastructure, the baby, despite multiple challenges—extremely low birth weight, respiratory issues, infections, gut problems—has made a remarkable recovery. Now at six months, the child is thriving with healthy development milestones.

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■ In another case, a 5-year-old presented repeatedly with symptoms of intestinal obstruction. Initial tests showed no clear issues, but further evaluation revealed severe constipation and a non-functioning colon—diagnosed as a rare intestinal motility disorder. Additionally, the child had unusually high blood pressure. A PET scan revealed a mediastinal mass, later diagnosed as undifferentiated neuroblastoma. The mass was removed, and the child began chemotherapy. Our oncology team is optimistic about a positive outcome. These cases underscore the importance of a multidisciplinary approach in managing complex cases and improving outcomes.

With that, I'll now hand over to our Group CFO, Mr. Vikas Maheshwari, for the financial update.

Vikas Maheshwari: Yes. Thank you, sir. A very good morning to all of you, and thanks for attending this investors conference call. I'm pleased to brief you on the financial performance and key developments of Rainbow Children's for the fourth quarter and for the full year FY '24-'25.

■ Our operating revenue for the quarter stood at INR370.1 crores, reflecting a growth of 8.5% when compared to the corresponding quarter of the previous financial year. For the full year, our revenue stood at INR1,515.9 crores, reflecting a growth of 16.9% when compared to the previous financial year.

■ Our EBITDA for the quarter stood at INR114.7 crores, marking an 8.7% growth compared to the same period last year. For full year, our EBITDA stood at INR489.9 crores, reflecting a growth of 14.2% when compared to the previous financial year.

■ Our EBITDA margin for the current quarter is 31%, while for the full year, our EBITDA margin is 32.3%.

■ The profit after tax for the quarter is INR56.5 crores, marking a growth of 10.7% in comparison to the corresponding quarter of the last financial year. For the full year, our PAT stood at INR244 crores, reflecting a growth of 11.9% when compared to the previ-

ous

financial year.

■ The main highlight for the year is that our cash generation from the operation for the full year remains very, very robust. Cash generation from the operations was INR481.2 crores versus EBITDA of INR489.8 crores, resulting in 90% conversion into the cash generation from the operations.

■ In terms of operational performance, outpatient and inpatient volumes witnessed a growth of 3% and 4%, respectively, when compared to the corresponding quarter in the last financial year. Deliveries grew by 6% compared to the corresponding period of last year. For the full year, outpatient, inpatient and delivery volume witnessed a robust growth of 12% and 10%, respectively, when compared to the last financial year.

■ Our payor mix continued to remain robust and balanced with 52.9% of the revenue coming from the insurance and the balance 47.1% coming from the cash patients. For full year, the payer mix stands at 48% cash and 52% insurance.

■ I'm pleased to inform that our company's balance sheet remains very robust with cash positions of close to INR700 crores as of March 31, 2025, and will support our ongoing capital expenditure plan.

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■ Given our current cash and anticipated internal accruals in the coming years, we remain confident in our ability to complete all planned capital expenditures through internal accruals without any debt financing.

■ During the quarter, the company has invested approximately INR43.7 crores in the capital expenditure. For the full year, the capex is INR145.7 crores.

With these insights, I conclude my financial update. I now invite questions and suggestions from the participants. Thank you.

Moderator: The first question is from the line of Mahesh Atal from Attal Investment Advisors.

Mahesh Atal: My first question is more on how many beds are going to mature in the FY '26 out of our new hospitals? How much would be shifted to our mature hospital space? And how much new would be added to the new hospitals? That is what my first question would be.

Saurabh Bhandari: There are two hospitals, which would move from the new hospital category to mature hospital, one is the Children's Heart Institute and the second is the Rosewalk Hospital. And the number of beds, which will basically move is 100 beds of Rainbow Children's Heart Institute and 24 beds of Rosewalk. So total 124 capacity beds will move from new to mature category.

Mahesh Atal: And how many new would be added to the new?

Dr. Ramesh Kancharla: ~ 250 beds are going to be added to the new beds. ~150 beds in Bangalore across the 2 spokes and ~100 beds at regional spoke in Rajahmundry, totalling to 250 that are going to add to the network.

Mahesh Atal: Okay. Fair enough. My second question would be more on -- we could see that you are -- I mean, I'm not -- I'm trying to understand this. We have actually our new hospitals has come up, the new beds have come up, but our -- one is that ARPOB is down in the new hospital space and -- but the occupancy is quite good.

So, what is that we are doing? We are like trying to focus more on occupancy levels right now, so that -- what is the strategy for the new hospitals going in FY '26? How do we increase the ARPOB to the mature hospital thing?

Dr. Ramesh Kancharla: The ARPOB comes over a period. Whenever there's a new hospital, a new geography, or a micro market, one factor is that insurance will take some time. Also, the tariff is usually set a little more attractively to draw more patients to your hospital. This is a very common practice for every hospital. As the hospital's occupancy picks up and institutions are all onboarded, then in the second or third year, the pricing power increases. This is very natural for most new hospitals across the industry

Mahesh Atal: All right. Is there any change in the insurance rate that you have taken?

Dr. Ramesh Kancharla: I think there's

a renewal cycle that typically happens once every two years. Sometimes, it can take longer. So, periodically, as they come in at various stages, each year another cluster comes up for renewal. We negotiate and handle it accordingly. Nowadays, what we've been doing is Rainbow Children's Medicare Limited

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managing this at the group level — for example, as a Bangalore cluster, Hyderabad cluster, and Chennai cluster. Chennai was done last year. Bangalore is due now, and Hyderabad is also going to be done now.

Mahesh Atal: Any guidance you'd like to give on the broader margins for the FY '26?

Dr. Ramesh Kancharla: See, as we are a growing franchise — like last year, if you recall — what we've always guided is a premise EBITDA of around 25% (Pre INDAS), which is based on our operations and our ability to drive growth. Whenever a significant number of beds are added, there may be around half a percent to 1% EBITDA margin pressure.

But that's temporary. If you look at last year, we were exactly in line with our guidance — with expected growth in the high teens and around 25% pre-Ind AS EBITDA. The margin pressure was roughly 1%, which is exactly how it played out after adding 270-280 beds. We saw about a 0.9% margin impact compared to the previous year.

Vikas Maheshwari: Just to add to what Dr. Ramesh mentioned — as he has also highlighted in previous calls — whenever we add new beds, there will be some pressure on margins. However, we have threshold limits in place, and we always anticipate that our pre-Ind AS EBITDA margin will be higher than 25% (Pre INDAS). We're happy to share that for FY '25, despite adding 280 beds last year, our EBITDA margin was shaved off by only 0.9%. It stands at 26.6% pre-Ind AS and 32.3% post-Ind AS.

As we enter this year and continue adding beds, there will understandably be some cost pressure from the new units. However, some of this may be offset by the performance of recently opened units. As our CMD Dr. Ramesh has guided, it is safe to assume a minimum EBITDA margin of 25%. And, as we've done this year with 26.6%, we will aim to exceed that (threshold of 25%).

Mahesh Atal: Do you foresee any competition coming in your main scope, I mean, main Hyderabad market?

Dr. Ramesh Kancharla: Well, there's always been a competition for the last 15 years' time. And city is growing and also population is growing, the more new hospitals are coming up. That's a natural history.

Mahesh Atal: Sorry to interrupt you, sir, any inorganic thing that you are looking at in Hyderabad?

Dr. Ramesh Kancharla: In Hyderabad, no, very unlikely because we don't need to do inorganic in Hyderabad because if something opportunity is there, we also will do it. That's probably more efficient than going for inorganic.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: Sir, first question is on the mature portfolio. Any comment on this performance, sir? Have occupancy been weak in a certain cluster? Or is it across cluster?

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Vikas Maheshwari: Yes, occupancy at our mature hospitals — the ones we've listed — has been slightly muted. As mentioned in the opening remarks, pediatric volumes were a bit soft in the March quarter. On the other hand, segments like mother care performed well.

Anshul, as we've consistently guided, if you look at our 3-year or 5-year CAGR, it has been very robust — around 16%. And when you look at a 3- or 5-year horizon, that translates to 12 or 20 quarters. Naturally, in that period, there may be one, two, or even three quarters where performance is somewhat subdued or below market expectations. But that's where we, as a management team, catch up — and over the long term, the trend has remained strong. So, while this quarter may appear slightly subdued, it's not a cause for concern, nor does it signal any issu

e

with our long-term growth trajectory. It's more of a one-off. As we look at how the June quarter is shaping up, we expect things to stabilize and return to our long-term growth path of 15–16%. This year is going to be very busy. Over the next six months — as outlined in our presentation — we're adding significant capacity, including 250 beds, starting with Rajahmundry, Hennur, and Electronic City hospitals. This will further strengthen the foundation for growth in the coming quarters

Anshul Agrawal: Noted, sir. Sir, just a follow-up on this. So, are we saying our mature portfolio should also clock, say, around about mid-teens growth going forward, this is just a one-off kind of quarter?

Dr. Ramesh Kancharla: I think for mature hospitals, typically, when you factor in pricing power and other efficiencies, a growth of around 8% to 10% is considered excellent. That's what we usually see.

Take, for example, a place like Hyderabad — 8% to 10% growth is fantastic, and we deliver excellent results at those levels. These hospitals are fairly mature, and most of them are clocking around 70% occupancy.

There's still room to grow, and mature clusters generally have better pricing power. So, it's a combination of factors we need to look at — pricing, occupancy, and even case mix evolution, which tends to happen in mature hospitals. All of these contribute to their performance.

Anshul Agrawal: My second question is, sir, out of the new units that you have commissioned in Q4 last year, could you quantify the drag in margins from these new units in FY '25? And the ones in Hyderabad broken even?

Vikas Maheshwari: Yes, Anshul. The Hyderabad unit has broken even, in line with our usual timeline of 12 months. For Bangalore and Chennai, the trajectory is similar — we usually target break-even within 15 to 18 months depending on the cluster.

As for the margin drag, the total impact from the three new hospitals — one each in Hyderabad, Bangalore, and Chennai — was around ₹12 to ₹13 crores for the full year.

Anshul Agrawal: From all the 4 new hospitals, right?

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Vikas Maheshwari: Yes, correct — the ₹12 to ₹13 crore drag was from the three hospitals that opened last year, one each in Hyderabad, Bangalore, and Chennai.

Anshul Agrawal: Got it. And sir, any guidance on the Gurgaon project commissioning timeline? I heard your comments in your opening remarks, but any definitive guideline of whether it should open by FY '28 end?

Dr. Ramesh Kancharla: Yes, originally, we guided for a calendar year 2027 launch — likely in the second half — give or take 2 to 3 months. We've now taken some additional steps to accelerate the project by adopting a hybrid construction approach using steel column structures. This should help us speed up structural work significantly. Once the structure is completed, we typically need another 12 to 14 months to finish the rest.

So overall, we're targeting completion in around 26 to 28 months from now. That's the current

estimate from our project team — barring any unforeseen headwinds.

Anshul Agrawal: All right. Very clear. Just one last question, if I can squeeze that in. So, for the next 3 years, sir, what would be our capex plan, sir? Would it be on par with what we had guided last quarter, around INR650-odd crores in the next 3 years?

Dr. Ramesh Kancharla: Yes, it remains the same. There's no change to the guidance or the timelines at this point.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Just wanted to understand for the upcoming units at Rajahmundry and 2 units in Bengaluru, have you started doctor recruitment, etcetera?

Dr. Ramesh Kancharla: Yes, we have. For Rajahmundry, doctor recruitment is almost complete. For Bengaluru, we are currently in the process — it's well underway..

Damayanti Kerai: Okay. So, for all the 3 units, it's underway. And Rajah

mundry, as you mentioned, is broadly completed?

Dr. Ramesh Kancharla: Yes, that's correct. Recruitment for Rajahmundry is nearly closed. Across the network, we've recruited over 70 doctors in the last year, spanning various specialties, surgical areas, and subspecialties.

Damayanti Kerai: Okay. But 70 new additions were across the network, not specific to...

Dr. Ramesh Kancharla: Yes — they were across our Hyderabad, Bangalore, Chennai, and Delhi clusters.

Damayanti Kerai: Okay. That's helpful. And my second question is on your 2 recent initiatives. If you can update us on the Child Development Centre and the IVF business, how these 2 units are picking up?

And where do you see these scaling up in next, say, 3 years or so?

Dr. Ramesh Kancharla: Sure. The Child Development Centre is a service that naturally takes time to scale. We're working on building a care model based on standard operating protocols (SOPs), with a focus

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on quality. It's also a very price-sensitive service since it involves long-term care rather than acute interventions.

We're taking a cautious, phased approach — figuring out how to effectively address the community's needs related to developmental delays and behavioral issues. One key challenge is the limited availability of trained developmental pediatricians. We currently have two, and we're looking to add two more.

It's going to take some time before this becomes a significant revenue contributor, but the focus right now is on building a high-quality, sustainable service. In about six months, I should be able to provide more concrete projections for the CDC.

Damayanti Kerai: Okay. And on the IVF business?

Vikas Maheshwari: Yes, Damayanti — the IVF business has performed very well. It's now operational in 12-13 of our hospitals, and it's growing rapidly. Given the low base, we've seen strong year-on-year growth — close to 70%.

In terms of contribution to overall revenue, it's still relatively modest — around 2.9% — but the trajectory is clearly upward.

Damayanti Kerai: Okay. So right now, like under 3% of your total revenue, but it's growing obviously much faster?

Vikas Maheshwari: Correct.

Damayanti Kerai: Okay. And my last question is on your ARPOB trajectory. So earlier, we talked about like somewhere in high single-digit growth. So, will that continue?

Vikas Maheshwari: Yes, that trend continues. As we've also mentioned in earlier investor calls and meetings, when it comes to ARPOB (Average Revenue Per Occupied Bed), one component that affects it is ALOS (Average Length of Stay), which can vary for operational or clinical reasons.

We suggest that a better metric to track is ARPP — the Average Revenue Per Patient. On a year-on-year basis, IP ARPP (Inpatient ARPP) has consistently grown. For the full year, ARPP grew by 5.6%. While full-year ARPOB was down 3.4%, the ARPP increase shows strong underlying revenue efficiency. So, ARPP is a more stable and relevant indicator, especially since ALOS is often outside our control. It's more of operational driven and case driven.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, you mentioned 2.9% contribution from fertility in FY '25. Similarly, can you comment on the broad sales split between paediatrics and maternity and fertility for the quarter?

Vikas Maheshwari: Sure, Alankar. Broadly, the mix hasn't changed much. Approximately 30% of our business comes from mother care — which includes maternity and fertility — while the remaining 70% is paediatrics.

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Alankar Garude: Sir, but you mentioned about mother care doing well in this quarter. So just trying to understand from a fiscal standpoint and specifically for the fourth quarter standpoint, what was the contribution of paediatrics for us?

Vikas

Maheshwari: Yes, for Q4, the mother care business — including fertility — contributed around 32%.

However, on a full-year basis, it averages back to the 30:70 split, so the quarterly deviation is just a few percentage points.

Alankar Garude: Okay. So basically, a few percentage points here and there?

Vikas Maheshwari: Yes. Plus/minus here because both the facilities are growing, right?

Alankar Garude: Got it. Sir, the question then is, I mean, if we assume that over the next 3, 5 years, the share of fertility and maternity inches up further, should we expect the seasonality in the business coming from paediatrics to gradually reduce over the years?

Vikas Maheshwari: In theory, yes, it should. However, we are equally focused on scaling up our paediatrics business. For instance, we're expanding super-specialty programs such as transplants in hubs like Bangalore and Chennai. These initiatives will help elevate the value of our paediatrics portfolio.

Dr. Ramesh Kancharla: Alankar, I'm sitting here. paediatrics remains our core strength and strategic priority. That doesn't imply we're deprioritizing obstetrics, but rather that our primary focus and expertise lie in paediatric healthcare. We see ourselves as the leading organisation committed to solving paediatric health challenges in India.

Obstetrics, for us, is an integral part of that continuum — particularly the perinatal aspect, which directly aligns with paediatric outcomes. That's why we don't focus extensively on expanding into general gynaecology. Our interest lies more in perinatal care, which is critical for both maternal and newborn health.

That said, we do maintain gynaecology capabilities to meet the broader surgical and clinical needs of our network patients. However, our identity is very clear: we are a multispecialty children's hospital, with a dedicated focus on perinatal services as an extension of paediatrics. Unlike others, we have absolute clarity in our positioning — both in what we stand for today and where we are headed in the future.

Alankar Garude: Got it, sir. The second question is we saw a 3% decline in ARPOB in FY '25. How should we look at ARPOB growth in FY '26? And maybe a second one related to fertility itself. Does fertility have a positive impact on ARPOB?

Vikas Maheshwari: Fertility does have a positive impact, but given its current scale, the overall contribution to ARPOB is still limited. As we've explained earlier, ARPOB can fluctuate due to several factors — particularly ALOS (Average Length of Stay), which is influenced by case mix and clinical complexity and is not always in our control.

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That's why we recommend focusing on ARPP — Average Revenue Per Patient — as the more reliable metric. For FY25, ARPOB declined 3.4%, but ARPP grew by 5.6%. This increase in ARPP came despite a 7.7% rise in ALOS, which naturally affects ARPOB. So, ARPP is a clearer reflection of underlying growth.

Alankar Garude: Understood, sir. So broadly, a 5%, 6% number on ARPP should be something which we can pencil in?

Vikas Maheshwari: Yes, that's a reasonable assumption. And if ALOS stabilizes or comes down, we may see ARPOB improve accordingly as well.

Alankar Garude: Got it. And one final thing, sir, one request from our end. Now that we have 19 hospitals and a sizable number in -- across Hyderabad, Bangalore, Chennai, we have a couple in AP. There's Rajahmundry also coming up. Can we please start sharing cluster-wise details? It would be really helpful if we can get some of those details to understand the numbers better, sir.

Dr. Ramesh Kancharla: Alankar, we understand the need and appreciate the suggestion. However, we want to be thoughtful about how we present that data. At the end of the day what matters for us is to see annual growth rate / CAGR of 4-5 important metrics. Each cluster is at a different stage — Hyderabad is mature, Ban

galore is evolving nicely, and Chennai is relatively new. Comparing them directly may not provide meaningful insights and could lead to misinterpretation.

That said, we are evaluating ways to present this in a useful and consistent format. Give us some time — perhaps about a year. As you know, we're possibly the only hospital group in India truly operating on a hub-and-spoke model, especially in an urban context. So, it's a bit complex, but we'll try to structure this in a way that adds value without creating confusion. Let me work on this and come back to you on this, okay? Otherwise, I will lose my hair completely.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital.

Rahul Jeewani: Sir, of the four new hospitals, which we have opened since fourth quarter of fiscal '24, you said that the Hyderabad hospitals have achieved breakeven. So, can you also talk about the Chennai and the Bangalore hospitals, which were opened up in, let's say, last year?

Dr. Ramesh Kancharla: Yes, the Bangalore hospital is performing well and is not very far from breakeven. Chennai, on the other hand, will take a bit longer. It's a slightly higher-capex hospital, and building a medical team in Chennai typically takes time. Overall, we expect that, when looked at collectively, these new hospitals should achieve a neutral or slightly positive EBITDA this year.

Rahul Jeewani: Okay. So, sir, this INR12 crores, INR13 crores EBITDA loss which you had from these hospitals in FY '25, that will, let's say, at a combined level will be neutral going into '26?

Dr. Ramesh Kancharla: Yes. Exactly.

Rahul Jeewani: Sure, sir. And sir, while you pointed out that the margins would, let's say, moderate by 50 to 100 basis points depending on the capacity expansion. But given that in FY '26 and FY '25, we already had this base of INR12 crores, INR13 crores hitting the EBITDA, which would, let's

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say, get to a neutral position this year. So, FY '26, is there a possibility that FY '26 margins would broadly be similar to FY '25 margins?

Dr. Ramesh Kancharla: I think so, yes — that's very much achievable. See, when we provide guidance, I don't want to go completely haywire. We try to remain conservative and realistic. If you look at our past commentary, we've always aimed for a pre-Ind AS EBITDA margin of over 25%. That's the internal benchmark we've set.

Of course, we always aspire to reach 26% or even 26.5% pre-Ind AS, and often, we've been successful in doing so. So that remains our ambition going forward as well. Now, with a larger number of beds in the maturing stage, strong operational focus, and a solid sales and marketing push — including the addition of 60–70 doctors this year — plus three more hospitals opening, we believe that if we can achieve high teens to 20% growth, all other issues will take care of themselves. There won't be any underperforming numbers.

Rahul Jeewani: Sure, sir. And sir, since you pointed out that, obviously, our business has some seasonality, can you also talk about, let's say, that how are we looking at the growth of the business from a next 3-year perspective before the 2 Delhi hospitals get commissioned?

Dr. Ramesh Kancharla: So, let's look at this broadly, Seasonality will always play a role in our business. We are primarily a medical hospital — around 75–80% of our work is medical, and only about 20% is surgical. We are not a wear-and-tear hospital; we are emergency-driven and community-based hospital. seasons always play out either positively or negatively.

Children's hospitals globally evolve in this manner. However, we are building long-term resilience by adding more specialties — paediatric super-specialties, intensive care, and eventually quaternary care. That's the long-term play.

As we mature, we may start to resemble a multispecialty hospital in some respects, but we will never beco

me a typical multispecialty hospital where procedures (like cardiac or whether it's orthopaedics or urology, neurosurgery and those things) dominate. Instead, our business mix — including ICU, newborn, or other pediatric treatments — can vary significantly year-on-year, which impacts ARPOB (Average Revenue Per Occupied Bed). Which is why your ARPOB also keeps changing with us because based on the tickets, the same ICU ticketing can be low in 1-year, higher next year. So newborn ticketing can be low in 1-year, high next year. So, there will be a kind of -- when you look at the CAGR of 3 years or 5 years, I can assure you that we are growing as much as any other hospital or better than other peers.

That's why we advise analysts and investors to look at our performance over a longer term, like 3- to 5-year CAGRs, instead of quarter-to-quarter numbers. Over the last 3 years, our CAGR is about 16%, and the same over 5 years. There will be a few standout or weaker quarters, but overall, things balance out.

Rahul Jeewani: Sure, sir. So, a mid-teens kind of a revenue CAGR is what we should expect going forward as well?

Dr. Ramesh Kancharla: I would expect it to be late teens to 20%.

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Rahul Jeewani: Okay. Sure, sir. And sir, obviously, our cash generation has been very good, and we have INR700 crores of cash lying on the books. We were also evaluating inorganic opportunities in some of the markets. So, can you update us in terms of how some of those M&A discussions have been ongoing?

Dr. Ramesh Kancharla: Yes, we're actively working on M&A opportunities. We hope to update you soon, but it's a bit premature to discuss details right now..

Rahul Jeewani: Okay, sir. Okay. And sir, one last question from my end. Within your core Hyderabad market, some of your peers have also announced stand-alone dedicated paediatric hospitals. So, some of your peers are now putting up dedicated blocks within their multispecialty hospitals for paediatrics. So how do you see the competitive intensity, let's say, in your 2 core markets of Hyderabad and Bangalore. And do you face any challenges in terms of onboarding doctor talent for, let's say, the spokes which you are adding in these markets?

Dr. Ramesh Kancharla: We'll have to wait and watch how those new entrants perform. Some large multispecialty players have tried this in Hyderabad before, and they haven't been very successful. And see, basically, it's not something any rights of proprietary or anything. There's no proprietary advantage here — anyone can try it. But unless someone has the patience, commitment, and deep understanding required to build a children's hospital, it's going to be very difficult to succeed.

You've asked about our cash reserves — yes, we have ₹700 crore, and theoretically, we could build 1,000 beds. But would we? No. It's not feasible to build and operationalize 1,000 pediatric beds efficiently. If I were a multispecialty player, building 500 beds might make sense. But pediatric hospitals are different — they're deeply rooted in communities and are heavily emergency-driven. You can't scale them the same way as multispecialty hospitals

Rahul Jeewani: Sure, sir. So -- and just a follow-up to that. So, in the core Hyderabad market, what is the potential to add more beds, let's say, over the next 3-year period? So, in the past, you have indicated that Bangalore market could potentially mirror the number of beds which we have in Hyderabad over a period of time. But within Hyderabad, let's say, do you see opportunities to further add capacities over the next 3-year period? Or are we sufficiently covered in Hyderabad?

Dr. Ramesh Kancharla: There's still a scope to add more beds in Hyderabad. We're exploring some opportunities and will look to expand capacity further in the city

Moderator: The next question is from the line of Manik from Findoc Investment.

Manik: Sir, I'm pretty new to this company. So, can you please help me with the OPD revenue and volume for FY '25?

Vikas Maheshwari: Sure. When we talk about OPD revenue — which includes consultations, lab and radiology services, plus pharmacy — around 30% of our total revenue comes from the OPD segment. In terms of OPD volumes, the number of outpatient visits was approximately 14.26 lakh for the full year. Does that answer your question??

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Manik: Yes, that answers my question. And how are we envisaging the growth in this segment? Is it somewhere linked to the number of beds that we have? Or how will it be shaped going forward?

Vikas Maheshwari: As far as OP revenue you are saying?

Manik: Yes, yes.

Vikas Maheshwari: Or overall revenue?

Manik: For OP revenue.

Vikas Maheshwari: Got it. So, OPD revenue generally grows in proportion to our overall revenue. If you look at historical trends over the last few years, the contribution from OPD has remained fairly stable — around 30%, give or take 1%. So, going forward, OPD revenue growth will largely mirror overall revenue growth, and its share should continue to hover around that 30% mark, plus or minus 1%

Moderator: The next question is from the line of Nitesh Dutt from Burman Capital.

Nitesh Dutt: Just one reconfirmation. An earlier participant had asked about Gurgaon timelines. I missed your response. Is it as per the schedule that you previously highlighted or any delays?

Dr. Ramesh Kancharla: Yes, as mentioned earlier, we are sticking to the previously guided timeline — which is the latter part of calendar year 2027, around October or November. That's what we had shared earlier, and that continues to be the plan. Unless something unexpected happens on the sales or execution side, we're on track.

Nitesh Dutt: Is it for both the hospitals, October, November of '27?

Dr. Ramesh Kancharla: Yes. There may be a couple of months' variation here and there, but nothing significant. These are structured projects, and our project teams and all other stakeholders are aligned and working in sync.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital.

Rahul Jeewani: Yes. Sir, just a clarification, this IVF revenue, which you called out as 2.9% of the revenue, was it for the quarter or for the full year?

Dr. Ramesh Kancharla: Full year.

Vikas Maheshwari: Actually, Rahul, for the full year, it was 2.6%. For Q4, it was 2.9%.

Rahul Jeewani: Okay. Sure, sir. And when you are talking about this, sir -- so this is of the overall company revenue or the IPD revenue?

Vikas Maheshwari: Overall revenue, Rahul.

Rahul Jeewani: Okay. Sure, sir. So, sir, the IVF business is scaling up pretty well given that, let's say, we started this business 1.5 years or 2 years back. And now this business has surpassed the international Rainbow Children's Medicare Limited

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revenues, which we are doing. So, what are your further plans in terms of trying to grow the IVF portfolio? So, is IVF now available across all your 19 hospitals or there are still some hospitals where we don't offer IVF services?

Dr. Ramesh Kancharla: We plan to integrate IVF wherever there's a viable opportunity within our existing facilities. Currently, IVF services are available in around 12 hospitals. We may be kind of adding some more, but it's not on my top of head at the moment

One important point — we do not intend to set up many stand-alone IVF centres outside our campuses. We've made a conscious decision to keep IVF services within our ecosystem. That way, we can ensure complete clinical oversight, which is crucial, especially considering the strong ethical framework required in this field.

Given that we are primarily a children's hospital with significant maternity services, we prefer organic growth and a B2C model. We're not exploring B2B in this segment, as it's typically n

ot

viable — obstetricians rarely refer patients to other obstetric hospitals

Moderator: As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Vikas Maheshwari: Thank you for joining today's conference call and for your thoughtful questions. Your continued support is instrumental in our strategic journey, and we sincerely appreciate the time you all have invested in engaging with our business and the future directions. For any further information or clarification, you may please reach out to Mr. Saurabh Bhandari, Head Investor relations at the e-mail ID being given in the presentation. Thank you very much.

Management: Thank you all.

Dr. Ramesh Kancharla: Thank you very much. Thank you.

Moderator: Thank you. On behalf of IIFL Capital, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Note:

■ This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

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Call: Aug 2025

August 4, 2025

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.

Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.

Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q1 FY26.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and
Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on July 28, 2025, with regard to
Financial Results of the Company for Q1 FY26.

The said Transcript is also available on the website of the Company and can be accessed
through the following link:

https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q1_FY2026.pdf

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited
Shreya Mitra
Company Secretary and Compliance Officer
Encl.: As above

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"Rainbow Children's Medicare Limited
Q1 FY '26 Earnings Conference Call"
July 28, 2025

Call Duration ■ 58 Minutes

Management Speakers ■ Dr. Ramesh Kancharla – Chairman and Managing Director

■ Mr. Vikas Maheshwari – Group Chief Financial Officer

■ Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked questions ■ Ms. Damayanti Kerai, HSBC Securities and Capital Markets

■ Mr. Amit Agicha, HG Hawa & Company

■ Mr. Anshul Agrawal, Emkay Global

■ Mr. Rahul Jeewani, IIFL Securities Limited

■ Mr. Naveen Baid, Nuvama Asset Management

■ Mr. Sumit Gupta, Centrum Broking Limited

■ Mr. Alankar Garude, Kotak Institutional Equities

■ Mr. Nitesh Dutt, Burman Capital

■ Ms. Varti Shah, Systematix Investments

■ Ms. Aadya Singh, Findoc Investment

Moderator ■ Mr. Rahul Jeewani - IIFL Securities Limited

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Moderator: Ladies and gentlemen, good day, and welcome to the Rainbow Children's Medicare Q1 FY '26
Earnings Conference Call hosted by IIFL Capital Services Limited. As a reminder, all participant
lines will be in the listen only mode and there will be an opportunity for you to ask questions
after the presentation concludes. Should you need assistance during this conference call, please
signal an operator by pressing star then zero on your touch tone phone. Please note that this

conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital Services Limited. Thank you, and over to you, sir.

Rahul Jeewani: Hi. Good morning, everyone. This is Rahul from IIFL Institutional Equities. I welcome you all to the first quarter earnings conference call of Rainbow Hospitals being hosted by IIFL. From Rainbow, we have with us today, Dr. Ramesh Kancharla, Chairman and Managing Director; Mr. Vikas Maheshwari, Group CFO; and Mr. Saurabh Bhandari, Head of Investor Relations. Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla: Thank you, Rahul. So good morning, everyone. Thank you for joining us today. We have entered a new financial year with a continued commitment to our long-term vision to deliver high-quality pediatric and perinatal care across the geographies, we are scalable, effective and deeply specialized network.

Let me start with a few key strategic and operational updates for the quarter.

■ Despite Q1 being a seasonally softer, especially in the general pediatric outpatient as well as inpatient volumes, we have delivered a steady financial performance. This growth was supported by strong momentum in the new hospitals and consistent demand for advanced services like pediatric specialty, multi-specialties and quaternary care.

■ The revenue registered a growth of 7% amounting to INR353 crores, EBITDA increased by 11% to INR103.6 crores, while PAT registered a growth of 35% to INR53.8 crores.

■ The overall occupancy rate for the quarter was 40.2%.

■ The newer hospitals added in the last 2 years are contributing in line with our expectations.

■ During this quarter, we have achieved a significant milestone by entering Warangal, a Tri-City with a population of 1.1 million with a catchment area of 3 million through the acquisition of 76% majority stake in Prashanthi Hospital Warangal. This is a strategic move as we expand into Tier 2 cities with access to high-quality pediatric care limited. Warangal with now function as I spoke to our Hyderabad hub other strengthening our hub and spoke model in Telangana. We are excited about this opportunity and value it can create -- it can add to a long-term growth.

Let's talk about projects. Our project pipeline is pretty well intact, where we continue to maintain our disciplined execution.

■ Our regional hub hospital in Rajahmundry of 100 bed is completed and are waiting for the final permissions to commence operations from government.

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■ In Bengaluru, the construction continues at this pace at Electronic City of 90 beds and the Hennur with 60 beds, both projects are on track to become operational by the end of the Q2 FY '26.

■ At Coimbatore the project work for 130 beds hospital is in progress.

■ In the national capital region, the exploration work is progressing quite well at both sectors, Sector 44, which is for 325 beds, on Sector 56 for 125 beds capacity in Gurugram.

■ We are in advanced discussions to finalize an asset-light build-to-suit gre

enfield hospital in

Pune with 150 bed capacity.

This calibrated approach reflects our ambition to scale bed capacity responsibly in existing cities as well as new geographies.

While challenges continue to persist international business due to higher tighter regulations in markets like Bangladesh, Oman and Sudan, we're actively monitoring the situation, adapting to our strategies to focus on other countries.

Clinically, we are very encouraged to see continued growth in the pediatric specialties, quaternary care, including organ transplantation. These services continue to be strong drivers of case mix enhancement and the revenue contribution.

We are also pleased to see the steady ramp-up in our newer hospitals, confirming both the scalability of our model and the operational capability of our teams on the ground.

I would like to take a moment to highlight 2 remarkable achievements that underscore the strength and the expertise within the Rainbow Group.

■ And inspiring patient outcome from our Bengaluru Bone Marrow Transplant Unit, a 15-year-old girl from Iraq with a sickle beta-thalassemia, a complex and debilitating conditions since infancy was going through a miserable complications like sickle cell crisis, a vascular necrosis of the hip joints and multiple transfusions. She underwent a life-saving bone marrow transplantation with the donor being the sibling. Despite facing a rare serious post-transplant complications because of the blood group

incompatibility, our team managed these complications with exceptional skill and care, resulting in a successful outcome that truly reflects our multidisciplinary approach.

■ In another breakthrough milestone, Dr. Nageswara Rao Koneti, Director of our pediatric heart institute was granted U.S. FDA patent for discovering the innovative multifunctional cardiac device named KONAR-MF Occluder. This device is used to close the ventricular septal defects in the bottom of the chambers of heart and several other cardiac conditions like aortopulmonary windows and the patent ductus arteriosus and other fistula within the heart. So far, it's been used successfully in about 18,000 patients across 90 countries.

These achievements clearly demonstrate that our clinical capabilities and the dedication of our teams, reinforcing our position at the forefront of pediatric and prenatal health care in India.

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Looking ahead, our priorities are very clear,

■ Ensuring the timely commissioning of the project of Rajahmundry and also 2 spoke hospitals in Bengaluru in this year,

■ Deepening clinical capabilities by strengthening our strength of excellences.

■ Strengthening our sales and marketing function by bringing in experienced leaders and expanding the teams across key markets to deliver sustainable and scalable growth

■ To support the future growth through inorganic acquisitions as well as greenfield and brownfield opportunities.

I want to take this opportunity to thank our medical teams and leadership teams for their dedication and the hard work. We are grateful to our stakeholders for their continued trust in building this much needed health care for women and children. And I'm confident of creating a significant value for our shareholders.

With that, I now hand over to Group CFO, Mr. Vikas Maheshwari to take you through the financial details for the quarter.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you and thank you for attending this investors conference call. I'm pleased to brief you on financial performance and key developments of Rainbow Hospitals for the first quarter, FY '25, '26.

■ Our operating revenue for the quarter stood at INR353 crores, reflecting a growth of 7% when compared to a c

orresponding quarter of the previous financial year.

■ Our EBITDA for the quarter stood at INR103.6 crores marking an 11% growth compared to the same period last year.

■ The EBITDA margin for the current quarter is 29.4%, which is an improvement of close to 1% from Q1 FY '25 due to effective cost discipline.

■ The profit after tax for the quarter is INR53.8 crores, marking a growth of 35% in comparison to corresponding quarter of the last financial year.

■ I'm happy to share that Prashanthi Hospital, Warangal has been seamlessly integrated with Rainbow Hospitals - all systems, including our IT, which is HIS, SAP and HRMS have been integrated to the Rainbow network.

■ In terms of operational performance, outpatient volumes for the quarter witnessed a growth of 6%, while inpatient and the deliveries de-grew by 1% and 2%, respectively, when compared to corresponding period of last year.

■ For the quarter, our payer mix continued to remain steady and balanced with close to 52% of the revenue coming from the insurance and the balance, 48% coming from the cash patients.

■ I am pleased to inform that our company balance sheet remains robust with net cash position of INR735 crores as of 30th June 2025 and will support our ongoing capital expenditure plan. Given our current cash and anticipated internal accruals in the coming years, we remain confident in our ability to complete all planned capital expenditures through internal accruals without any debt financing.

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■ During the quarter, the company has invested approximately INR41.5 crores in the capital expenditure.

■ I am happy to share that we successfully concluded the AGM on 5th of July 2025 and the dividend to all the eligible shareholders has been credited on the same day. With these insights, I conclude my financial update.

I now invite questions and suggestions from the participants. Thank you very much.

Moderator: Thank you very much. The first question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: Sir, can you talk a bit more about the newer market where you're planning to enter? So, Pune and Warangal, which you mentioned. So just in terms of the market opportunities, whether we should look at this newer market more similar to what you see in the Hyderabad market? Or these will be more like Chennai or Bengaluru market? So, a bit more colour on that would be helpful.

Dr. Ramesh Kancharla: Yes, we now have a very clear approach to expansion strategy. While we continue to add beds in our established markets like Hyderabad, Bengaluru, Chennai, and Andhra Pradesh, we are also exploring newer geographies.

One such region is NCR. Though we already manage a hospital in South Delhi, the rest of NCR is still a new geography where we have now commenced work. Another region is the Northeast. And most recently, we have started evaluating Pune.

When we look at pediatric markets and identify cities where a children's hospital could do well, we focus on places with strong IT and knowledge-based job ecosystems. These create a good population segment for our model. So, NCR and Pune — along with Hyderabad and Bengaluru — are cities where we see good opportunity to build a full-fledged pediatric and perinatal model.

Damayanti Kerai: Okay. So, Pune, you said you are in advanced discussion. When will this discussion likely result in concluding the project?

Dr. Ramesh Kancharla: Yes, we've agreed on commercials and other aspects before signing. I think the legal work is almost concluded, and we should be going ahead with the signing in a couple of weeks.

Damayanti Kerai: Okay. So, this year, it's most likely you'll see the Pune project to go ahead?

Dr. Ramesh Kancharla: Yes. It would

take about 2.5 years' time to commence the operations.

Damayanti Kerai: My next question is on your -- the newer hospitals where you mentioned pickup is broadly in line with expectations. So just specifically on Sarjapur and Anna Nagar, if you can share whether these are nearing cost breakeven or it will take some more time?

Dr. Ramesh Kancharla: Sarjapur has breakeven, and Anna Nagar is close to it. It will probably take another 6 months. During the current season, we should reach breakeven.

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Damayanti Kerai: Okay. And my last question is, in the previous call, I guess you mentioned ARPP will be the better metric to look at the operational performance, but you haven't shared it. Maybe if you can share that on a regular basis in your presentation, that will be helpful. Well, you mentioned ARPOB might not be the right metric to really look at your business. But for this quarter, if you can just -- if you can just share how things have moved in terms of ARPP metrics?

Vikas Maheshwari: Damayanti, this is Vikas Maheshwari. Actually, ARPP is a derivative — if you multiply ARPOB by ALOS, you'll arrive at the number. But yes, it's a good suggestion, and we will start publishing ARPP going forward.

On a year-on-year basis, ARPP has increased by roughly 3%. As we've shared earlier, ARPP is on an upward trend — we've seen 6% to 7% growth over a period of time. We remain on that trajectory, though there could be fluctuations of a quarter or two.

Damayanti Kerai: So broadly 6% to 7% growth is a normal range, maybe the 3% number which you mentioned for 1Q, is because this is a seasonally low quarter?

Vikas Maheshwari: Yes, we can attribute this to that.

Moderator: The next question is from the line of Amit Agicha from HG Hawa.

Amit Agicha: My question is only connected to doctor engagement, could you elaborate on the retention strategy for full-time doctors, especially in Tier 2 cities? And what percentage of your doctors are on retailer model versus revenue share, like the basic category, with such a high training output? Like are you able to absorb trained doctors into your system effectively?

Dr. Ramesh Kancharla: Yes. One of the key aspects for us is our doctor engagement model. We broadly look at two different categories of doctors. In the pediatric space, you typically don't find too many generalist doctors — we focus on those with the right qualifications and specialized skill sets. We invest in them from the start. For the first 2 to 3 years, they are usually on a retainer model.

Once they start performing well, they transition to a variable fee structure. This is how we've been operating across all geographies.

The model has been quite stable and successful. Once doctors settle down and begin earning well through the variable fee, they tend to stay. One of the key reasons for strong doctor stickiness to our model is that Rainbow functions as a medical institution — it's not just about individual performance but about service delivery as a team. Doctors here require strong support from the hospital infrastructure and from their colleagues. It's not a solo practice — it's a collaborative system. That's why the retention is strong, and the engagement model works well. It's a truly institutional model.

Amit Agicha: Any percentage-wise, can you give like retainer model as a revenue share?

Dr. Ramesh Kancharla: Sorry? So, the majority of doctors are in various stages — that's why I mentioned it's difficult to give a fixed percentage. As the hospital matures, we gradually transition doctors to the revenue share model. This keeps them engaged and allows them to grow both clinically and financially.

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For example, most of the doctors in Hyderabad are already on the revenue share model. In Bangalore too, a significant portion have moved to this model. Chennai is still largely on the retainer model since it's in the early phase of operations. So that's how the progression works.

Moderator: The next question is from the line of Anshul Agrawal from Emkay.

Anshul Agrawal: My first question is on the mature hospitals. Any comment on why volume growth has been -- volumes have been muted for this portfolio?

Dr. Ramesh Kancharla: Yes, Anshul. We observed this trend in the last quarter as well — both Q4 and Q1 have been quite muted, seasonally speaking, in the current year. We haven't gone into deep analysis yet, but from what we've seen, there aren't any specific reasons driving this. It's just generally been a very healthy period. The patients who are coming in are mostly visiting for vaccinations, specialty care, or nutritional advice.

There hasn't been any significant disease activity over the last 5 to 6 months. That's what we've observed. When you look at pediatric volumes, they're the key contributor to our occupancies — and those have been soft, leading to a plateau or even some degrowth in occupancy compared to last year.

But this seems to be more of an external factor — we don't believe it's due to cannibalism, competition, or any internal issues. We're pretty confident that we'll come back when once things start settling up and operating side, absolutely, we're on top of the things.

Anshul Agrawal: Sure. A follow-up on this, sir. So, our surgical mix would have actually deteriorated in the current quarter. And so what would explain the jump in ARPOB or ARPPs, given that surgical mix would have actually reduced?

Vikas Maheshwari: Anshul, let me add here before Dr. Kancharla steps in. For mature hospitals, overall revenue grew by 2%. What came down was the seasonal, lower-value footfall — typically a large number of pediatric walk-ins, which were softer this quarter. However, our critical care segment remained strong and steady. Since the low-ticket pediatric cases reduced, the higher-quality, higher-value segments continued performing well — and that's what's driving the ARPOB and ARPP improvement.

So yes, while the icing on the cake — the seasonal pediatric volumes — were missing, the core high-value business remained intact, which explains the growth in ARPOB.

Dr. Ramesh Kancharla: No, Anshul — I wouldn't say the surgical mix deteriorated. In fact, our surgical mix has been quite healthy over the last 4 to 5 months. The volume of surgeries has been good. That's also why you see a lower ALOS, as most pediatric surgeries today are minimally invasive.

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There are two or three key contributors here: pediatric specialties, quaternary care, and pediatric surgeries — all of which reduce ALOS and enhance ARPOB. So, in the absence of the seasonal, lower-value business, these factors drove a 13% increase in ARPOB.

Anshul Agrawal: So there has been no price hikes in the insurance compartment or cash tariff at all, this is purely

pediatric...

Dr. Ramesh Kancharla: For this year, not yet. We are expecting these revisions to close — at least for Hyderabad, Bangalore, and Chennai — probably sometime in the next couple of months.

Anshul Agrawal: And any update on the IVF business vertical? How is it faring, have you added IVF vertical in any newer hospitals?

Dr. Ramesh Kancharla: No, it's the same — we have 12 centers currently, most of them within our main hospitals. Growth has been quite impressive. We are on

track to perform around 2,000 cycles by the end of the year. Revenue has grown by 50% year-on-year, which is very encouraging.

Anshul Agrawal: Just one last question. We'll be maintaining our mid-teens revenue guidance for the next couple of years? Or we'll see any upward plans for this, given the fact that they're adding hospital beds aggressively?

Dr. Ramesh Kancharla: Yes, we're adding beds now — with the recent acquisition and around 250 beds already in the pipeline expected to come into operations over the next 2 to 3 months. So, between those 250 beds and an additional 100 from recent acquisition (Warangal), we're looking at roughly 350 beds getting added this year, not counting any further additions from possible acquisitions.

There are enough number of beds that are going to add this year. For the guidance, we continue to anticipate revenue growth in the late teens to around 20%. With three quarters still to go, we remain quite positive.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital Services Limited.

Rahul Jeewani: Sir, on this Pune greenfield hospital, which we have, let's say, focused on now, what led to you deciding Pune as a market within the West India market? So can you talk about in terms of the market potential within Pune and are you choosing Pune for your West India foray?

Dr. Ramesh Kancharla: Pune, geographically, is a natural westward extension of our Hyderabad cluster. It has a strong catchment area. Additionally, we already see patients coming in from regions like Solapur and Parbhani districts. So, it effectively becomes an extension of the Deccan Plateau.

Pune is also a young people's city, with a significant presence of IT and ITES sectors, and high insurance penetration. While there is competition on the maternity side, the large population base and strong maternity demand naturally create a need for advanced pediatric capabilities — which aligns well with our strengths.

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We see Pune as a strong long-term addition to the Rainbow network. It also offers the potential to develop a hub-and-spoke model, given the city's size and ongoing expansion.

Rahul Jeewani: Sure, sir. And sir, what kind of capex would we be incurring for this 150-bed greenfield hospital?

Dr. Ramesh Kancharla: This is going to be a greenfield hospital, and the base building will be handed over to us. Based on our current understanding with them, the cost is expected to be well under INR 1 crore per bed. While we're still finalizing the exact numbers, I would expect it to be somewhere around INR 80 lakhs per bed.

Rahul Jeewani: Okay. So largely in line with what we have done for our asset-light hospitals previously?

Dr. Ramesh Kancharla: Absolutely, yes.

Rahul Jeewani: Sure, sir. And sir, second question on while the IP volumes were impacted because of seasonality, what we see is that the delivery volumes were also muted this quarter. So, a 2% Y-o-Y decline. So, what led to, let's say, this muted or sluggishness in terms of the delivery volumes?

Dr. Ramesh Kancharla: Well, we're not entirely sure. We've been closely looking at the data from multiple angles. Sometimes, delivery volumes are influenced by earlier wedding patterns, which can create cyclical trends — that could be one factor here.

When we look at peer groups, especially in Hyderabad, the two major players in deliveries are Rainbow and Fernandez. Even Fernandez has shown slightly muted growth — about 2–3% down in the last quarter. A similar trend is visible in Bangalore and Chennai as well.

So, overall, it seems like there's a general dip in deliveries across regions, not just specific to Rainbow. We don't see any strong or structural reasons behind this — it appears to

be more of a temporary, cyclical phenomenon rather than anything concerning.

Rahul Jeewani: And this, let's say, what this muted growth in delivery, was it across clusters or was specific to certain clusters?

Dr. Ramesh Kancharla: No, we've actually seen this trend more in our larger hub hospitals, where there's naturally more competition. The micro-market hospitals, which we've developed over the last 2–3 years, are

doing fairly well — the growth opportunities for these new hospitals remain strong. So, the slowdown is mostly visible in areas where our hospitals are in highly competitive zones. In such areas, everyone takes a bit of the share, and even then, the overall delivery numbers aren't very high for anyone.

That said, we've been present in these high-density areas across the three cities for quite some time, but we haven't seen this kind of dip before. We're exploring the issue more deeply to understand it better and identify ways to rebuild delivery volumes. Possibly, we need to step up our marketing and outreach efforts, especially to connect more with the younger population. We also need to reinforce our positioning around natural birthing and related services. It's time to re-energize and recommit ourselves to get back on track with double-digit growth.

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Rahul Jeewani: Sure, sir. And sir, on the inorganic opportunity, Northeast is what you are evaluating and let's say, with the entry into the Pune market, would you be evaluating inorganic opportunities in other, let's say, Tier 2, Tier 3 markets in Maharashtra as well?

Dr. Ramesh Kancharla: Well, I think if there are any suitable opportunities that align with our approach, we will certainly consider them. By 'suitable,' I mean hospitals that are built with quality infrastructure, hold necessary accreditations, and have a long-term vision where we can build scale and embed our philosophy. In such cases, we would definitely be open to exploring them.

Rahul Jeewani: And sir, can you update us on the opportunity which you were evaluating in Northeast?

Dr. Ramesh Kancharla: Yes, we'll probably come back to you very soon with more details on that. It's still a bit premature at this stage...

Rahul Jeewani: Okay. Sure, sir. And sir, just one request from me here, given that our business is slightly seasonal in nature and you talked about the fact that the general pediatrics business is what was impacted and tertiary and quaternary care where you continue to see healthy growth. So, if you can, going forward, split the business in terms of these different segmental, so that will help us to better track the operating metrics in terms of how some of these dynamics are playing. So yes, I would request if you can share that data going forward?

Dr. Ramesh Kancharla: Rahul, the challenge is that children's healthcare is inherently a very medicalized and team-based model. A child may be admitted under one department but receive interventions or support from multiple specialties. It's rarely linear. It's very difficult to kind of segmentalize the departmental wise in those things.

The child usually presents as a sick patient, and only after evaluation do we determine the underlying condition and assign them to the appropriate specialty or intensive care team. So, segmenting the pediatric business in a clean departmental format is quite complex and may not meaningfully reflect the clinical reality. But we'll certainly think about it and see what's feasible

Rahul Jeewani: If not that way, maybe the split of the business between seasonal and non-seasonal, obviously, there also you would have to take your judgment. But yes, even that will be helpful.

Moderator: The next question is from the line of Naveen Baid from N

uvama Asset Management.

Naveen Baid: I'm not sure if this has been answered before. But if I look at your mature hospitals despite poor inpatient volume, what kind of explains the sharp rise in ARPOB, especially in the mature hospitals? Just a mix as Rahul was referring to.

Dr. Ramesh Kancharla: Yes, what typically happens in Q4 and especially in Q1 is that it's generally a holiday season. Many people travel abroad, and cities like Bangalore and Hyderabad — especially in the IT zones — see a 40% to 50% drop in population during this period.

However, what remains consistent is pediatric surgeries — particularly specialized ones such as pediatric urology, neurosurgery, and ENT procedures. These are often scheduled during the

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summer because it's the preferred time for parents to have their children undergo planned surgeries.

As a result, we see a significant uptick in surgical volumes during this time. It's a combination of the holiday season and it being a relatively healthy period with lower infection risks — making it ideal for elective procedures. This includes not just pediatric surgeries but also cardiac procedures, transplants, and other elective surgeries that people tend to schedule during this

quieter, safer season.

So, while Q1 may reflect lower overall volumes, the business we do is of higher quality — with a richer case mix. That's why ARPOBs are typically higher during this period

Naveen Baid: Just one more question. So, you have 800 bed addition kind of a pipeline over the next 3 years.

How much of that is going to be on the asset-light model?

Dr. Ramesh Kancharla: Other than 450 beds, which are going to be in Gurgaon, rest are for all asset-light, at least 50% (of the total bed addition) are asset light.

Moderator: The next question is from the line of Sumit Gupta from Centrum.

Sumit Gupta: Just wanted to know is there any EBITDA drag due to the new facilities this quarter?

Dr. Ramesh Kancharla: Sorry, your voice was not very clear.

Sumit Gupta: Yes. So, was there any EBITDA drag due to the new facilities in this quarter?

Vikas Maheshwari: No, Sumit — we haven't opened any new facilities in the last 12 months. The three facilities we did open were in Q1 of FY '25, prior to this period. So, the drag from those new openings has surely come down, and that's also contributing positively now

Sumit Gupta: And with respect to the IVF, like it contributes around 3%.

Vikas Maheshwari: It has gone up to 3.2% type.

Sumit Gupta: And just one clarity on ARPP, average revenue per patient. So, you were referring 3% for -- on Q-on-Q basis?

Vikas Maheshwari: Year-on-year basis.

Sumit Gupta: So just want to get clarification. So, like with respect to my calculation, it is coming around 9% basically with -- so just want to understand.

Vikas Maheshwari: The slight variation is due to rounding off, particularly with the ALOS figure, which may have been rounded. In terms of year-on-year growth, IP ARPP increased by 3%, and OP ARPP grew by around 8%. So, the overall ARPP impact for the full year should be in the range of 3% to 3.5%.

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Sumit Gupta: Okay. So, for, let's say, FY '26 year-end FY '27, how much ALOS should we expect? Around 2.65, so nearly this level or what?

Vikas Maheshwari: Yes. We have a very stable ALOS of 2.6 to 2.8 days. I think in this range, it should be. The ALOS prediction is a very difficult job, Sumit.

Sumit Gupta: Right. Yes. I understand that, like what can be the like minimum ALOS which you are like building on o

r...

Vikas Maheshwari: The 2.6 to 2.8 days, that should be the right range that you can take.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, can you help us understand the seasonality patterns seen by Rainbow over a slightly longer time frame, say, over the past 10, 15 years, and particularly Hyderabad? The reason for the question is you've seen a fair amount of ups and downs over the past few years. For instance, FY '23 was a strong season, then '24 was a bit slow. First half CY '25, again has been a bit weak on the pediatric side, so the question is prior to FY '23, were we seeing similar kind of fluctuations across alternate years?

Dr. Ramesh Kancharla: We do observe these stretches of 1 to 1.5 years that are relatively healthy — what we could call low-incidence seasons. Looking back over my 20 years of experience, we've seen this at least twice before. Typically, once every 5 to 6 years, there's a noticeable slump in pediatric admissions, which eventually corrects itself and picks up again.

However, it's important to recognize that pediatric healthcare is undergoing significant change.

Unlike earlier, we no longer see diarrhoea cases spiking in the summer, and respiratory infections have reduced thanks to better flu vaccine coverage, improved hygiene, and nutrition.

The profile of pediatric illnesses is evolving.

At Rainbow, we're not running mom-and-pop pediatric setups — our intent is to build depth and strength in pediatrics. We want to focus on addressing critical care needs, building specialty-led capabilities, and managing more complex disease patterns, rather than depending solely on seasonal patient inflows.

The challenge with pediatric care is that when the season does peak — say for 4 to 5 months — if you don't have the bed capacity to meet that demand, you're in a tough spot. This is an emotional healthcare model, especially when children are involved, so it's essential to maintain capacity that can handle demand when it arises.

Of course, the flip side is: what do you do with that capacity during the off-season? That's where our strategic shift comes in — we aim to strengthen our pediatric specialties further so that our services remain relevant and impactful year-round, beyond just seasonal needs.

The broader narrative is that pediatric healthcare in India is evolving. It's no longer what it was 20 years ago — not even what it was a decade ago. That said, seasonal fluctuations still play a role. It's too early to draw firm conclusions from just six months of downturn, but we're watching it closely.

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role. It's too early to draw firm conclusions from just six months of downturn, but we're watching it closely.

Even in adult and multispecialty hospitals, you've likely observed how seasonal factors have significantly influenced volumes and growth over the past 4 to 5 years. So, this isn't unique to pediatrics. Over the next 3 to 4 years, we'll need to reflect deeply, understand the trends, and reposition our business model for a more sustainable and future-ready growth path.

Alankar Garude: So maybe a couple of follow-ups here. The first one is pharma companies have been commenting about seeing some pickup in the season over the past few weeks? Are we also seeing some increase in the disease burden over the past few weeks?

Dr. Ramesh Kancharla: Yes, there has definitely been some increase, and we're also seeing a shift in seasonal patterns. Unlike in the past, a few days of rain no longer trigger the start of the season. Now, it takes a more significant change in the environment to lead to illness — factors like viral breeding and other infections play a bigger role.

Of course, during the monsoon, we do see a rise in illnesses. But people have also become more resilient compared

to before. They no longer rush to the hospital for a simple runny nose or a one-day fever. Many now know how to manage mild symptoms at home with basic medication.

Alankar Garude: And secondly, sir, broadly from 1/3 currently coming -- of our revenue is coming from critical care or beds being allocated for critical care. Very broadly, say, 8, 10 years back, what would that number have been?

Dr. Ramesh Kancharla: I think the ratios and proportions tend to vary season to season. Broadly, they've remained consistent, though it's hard to put an exact number or percentage on the variations. We've seen exceptional seasons in the past — for example, during severe swine flu outbreaks or prolonged dengue seasons that we have seen 7 - 8 years ago, when our intensive care units were consistently full across the board.

Barring such extraordinary circumstances, the intensive care season typically lasts around 4 to 6 months. During the rest of the year, our ICU beds are occupied by cases involving rare disorders or complex specialty conditions like liver failure, renal failure, multiple organ dysfunction, and similar issues.

That said, we've experienced a relatively stable and healthy period over the past year. Even last year's peak season wasn't as intense compared to the two post-COVID years, which were exceptional — with a surge in illnesses, admissions, and demand for intensive care services. That intensity has now subsided. I believe we're seeing a healthier trend overall, and yes, we probably all deserve a better experience and healthier lifestyle — something that has become more evident over the last year.

Alankar Garude: And one final question on Tamil Nadu, can the margins of the mature hospitals in Tamil Nadu reach, say, the margin levels of the mature hospitals in Bangalore over the next 2 to 3 years?

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Dr. Ramesh Kancharla: I'm sure it will reach Bangalore-level performance in about 3 to 4 years — it will scale up to that level over time, yes.

Moderator: Our next follow-up question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Just wanted some clarity on Warangal unit. So, it's not part of 1Q numbers, right? It will be reflecting from 2Q onwards?

Vikas Maheshwari: Yes, that's correct, Damayanti, so we have closed the transition on 2nd of July. So, it will reflect from this quarter. (September quarter)

Damayanti Kerai: Okay. And can you also tell like what is the maturity profile, whether it will be part of the mature hospital cluster or new hospitals? And of the 100 beds how many are operations currently?

Vikas Maheshwari: Damayanti is a new hospital and will be categorized under our new hospitals segment. There's still a lot of setup and development work required which hasn't been done yet. Currently, the hospital is operating with an occupancy of around 30 to 35 beds. As we continue to add doctors and build out clinical facilities, we expect occupancy to grow gradually over time.

Damayanti Kerai: Okay. So, say, like over the next few years, it has potential to move up to the corporate level occupancy like 50% or so or it could be better?

Vikas Maheshwari: I think we should keep it at the corporate average.

Moderator: The next question is from the line of Nitesh Dutt from Burman Capital.

Nitesh Dutt: I had a question on other income. This quarter, it has increased by 33% quarter-on-quarter. Is there a one-off component because basis the INR735 crores of cash, if I calculate basis, the cash yield, it should have been slightly lower.

Vikas Maheshwari: No. The treasury income was slightly better because of the debt funds where we have taken the positioning of interest rates coming down, probably that got captured and you are seeing this MTM gain

on that. But yes, going forward, I think that type of performance may not be there.

Nitesh Dutt: Understood. And so, second question is on the matured hospital occupancy. There's been quite a fall of 11% Y-o-Y. How do you see it? Given that in the past, we have cited weather patterns, rainfall, et cetera, as reasons for the fluctuation. So was it one of the reasons or does higher competitive intensity or some degree of cannibalization were also the factors for the reduced occupancy Y-o-Y?

Vikas Maheshwari: We've already discussed the occupancy for our mature hospitals. The primary reason for the dip is the delayed onset of the pediatric season. Apart from that, our tertiary and quaternary care business continues to perform strongly. So, while occupancy has declined slightly, revenue from mature units still grew by 2%. This reflects the resilience of our quaternary and tertiary care-focused business model

Moderator: Our next question is from the line of Viri Shah from Systematix.

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Viri Shah: My question is, was the ARPOB improvement this quarter a one-off thing, or we can expect this to sustain for full year?

Vikas Maheshwari: As we mentioned in response to earlier questions, the correct way to assess performance is through ARPP, as ALOS is one of the factors that slightly affects ARPOB — along with case mix. In this quarter, the low-ticket pediatric business was slightly muted, which is why overall inpatient numbers and occupancy appear lower.

However, the tertiary and quaternary care segments remained strong, which supported a robust ARPOB. When we look at the overall ARPP — which includes both IP and OP income — it grew by approximately 4.5%. So, ARPP is the right metric to focus on

Moderator: Next follow-up question is from the line of Rahul Jeewani from IIFL Capital Services Limited.

Rahul Jeewani: Sir, can you also talk about the competitive intensity across your core markets of the Hyderabad, Bangalore and Chennai, you pointed that the delivery volumes were muted in some of the larger hub hospitals because of some higher competitive intensity. So just in terms of how you are looking at the competitive scenario across your key markets?

Dr. Ramesh Kancharla: Yes. I mean, birthing is always a competitive market, especially in cities like Bangalore where the intensity is very high. Hyderabad has also become quite competitive over the last 4 to 6 years. Despite this, we've managed to do well in a challenging landscape. However, the plateauing of deliveries isn't necessarily due to competition alone.

We follow a full-time doctor engagement model and focus on clinical excellence, natural births, and other quality parameters. Some of the VC (visiting consultants) deliveries, which tend to be more price-sensitive, may shift to other hospitals due to competition. But as an organization, we are not dependent on VC deliveries. Our focus is on consistent, high-quality deliveries, and that's where our strength lies.

That said, I believe we need to bring a greater degree of inclusivity and sharpen our marketing strategies to regain some lost ground. I see strong potential for a comeback.

In Hyderabad, over the past 15 to 18 years, two large organizations — including us — have consistently done high volumes. We've never viewed each other as direct competitors; both have their unique strengths and have coexisted respectfully. While many boutique centres have emerged, most haven't made a significant impact. Some VC deliveries may temporarily shift to these smaller centres, but the reputation that the two leading institutions enjoy remains very strong.

We do, however, need to reposition ourselves slightly. When I look at the other large player in our space, they've

also seen some degree of degrowth. So, we're asking our business analysts to dig deeper — for example, to look at the number of weddings last year and other indicators. Perhaps uncertain socio-economic conditions, job instability, or other factors are causing people

to delay family planning.
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It's important to understand these broader dynamics. We haven't seen anyone in the market significantly grow either, based on the data we've reviewed. So, we're not overly concerned at this point. We'll reassess toward the end of the year and see where we stand in terms of growth. For now, this seems to be one of those quarters where we've seen some degrowth.

Rahul Jeewani: Sure, sir. So whatever competition is there is on the birthing market. On your core, let's say, tertiary and quaternary care business, you don't see much of a competitive threat there.

Dr. Ramesh Kancharla: No.

Rahul Jeewani: And sir, just a clarification. What is the VC delivery?

Dr. Ramesh Kancharla: Visiting consultants.

Moderator: Our next question is from the line of Aadya Singh from Findoc.

Aadya Singh: Sir, question to you is about the upcoming hospitals in Gurugram, when do you expect them to turn operation? What's the status there?

Dr. Ramesh Kancharla: We are targeting around September 2027 (in the calendar year 2027) — that's the timeline we've set for ourselves. I believe our project is on track, and I also receive regular updates from my project teams. They indicate that completion could be expected by June or July, and operations may begin around September or October. That's the guidance I've received from the project team

Moderator: Ladies and gentlemen, this was the last question for today. I now hand the conference over to the management for closing comments. Over to you, sir.

Vikas Maheshwari: Thank you to everyone who joined the call and for the insightful questions — much appreciated. We look forward to having you all join us for our second quarter results. Thank you once again

Moderator: Thank you. On behalf of IIFL Capital Services Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Note:

- This is a transcript and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.
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Call: Nov 2025

November 20, 2025

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.

Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.

Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q2 & H1 FY26.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and
Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on November 14, 2025, with
regard to Financial Results of the Company for Q2 & H1 FY26.

The said Transcript is also available on the website of the Company and can be accessed
through the following link:

https://d31scedd5hpupb.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q2_&_H1_FY26.pdf

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited
Shreya Mitra
Company Secretary and Compliance Officer
Encl.: As above

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"Rainbow Children's Medicare Limited
Q2 FY '26 Earnings Conference Call"
November 14, 2025

Call Duration ■ 1 Hour

Management Speakers ■ Dr. Ramesh Kancharla – Chairman and Managing Director

■ Mr. Vikas Maheshwari – Group Chief Financial Officer

■ Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions ■ Ms. Damayanti Kerai - HSBC

■ Ms. Kritika - Prospera Financial Solutions

■ Ms. Bansi Desai - JPMorgan

■ Mr. Rahul Jeewani - IIFL Capital

■ Ms. Isha - Individual Investor

■ Mr. Vishal Manchanda - Systematix

■ Mr. Anshul Agrawal - Emkay Global

■ Mr. Dhananjai - Alchemy Capital

Moderator ■ Mr. Rahul Jeewani - IIFL Securities Limited

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Moderator: Ladies and gentlemen, good day and welcome to the Rainbow Children's Medicare Limited Q2
FY '26 Earnings Conference Call hosted by IIFL Services Limited. As a reminder, all participant
lines will be in the listen-only mode and there will be an opportunity for you to ask questions
after the presentation concludes. Should you need assistance during the conference call, please
signal an operator by pressing star then zero on your touch-tone phone.

I now hand the conference over to Mr. Rahul Jeewani. Thank you and over to you, sir.

Rahul Jeewani: Hi, good morning everyone. This is Rahul from IIFL Capital. I welcome you all to the second quarter earnings conference call of Rainbow Hospitals being hosted by IIFL. From Rainbow, we have with us today Dr. Ramesh Kancharla, Chairman and Managing Director, Mr. Vikas Maheshwari, Group CFO and Mr. Saurabh Bhandari, Head of Investor Relations. Over to you, sir, for your opening comments.

Ramesh Kancharla: Thank you, Rahul.

Good morning, everyone and thank you for joining us for Rainbow Children's Medicare Ltd earnings call for Q2 FY26 and first half of the year.

I would like to start with a few key strategic and operational updates for the quarter.

Operational Performance commentary:

■ During the quarter, our operational performance showed some softness, largely influenced by low incidence of seasonal illnesses resulting in lower patient volumes in general Paediatrics and Paediatrics intensive care.

■ In addition, early festive season (Ganesh Chaturthi and Dussehra in Q2) compressed the typical peak period for admissions.

■ These factors collectively led to lower occupancy levels and a transient impact on financial performance. However, the underlying business fundamentals remain strong — with continued growth in specialty services, disciplined cost management and steady progress in operational efficiency.

■ However, these challenges are temporary, and we expect normalization in the upcoming quarters.

Strategic Acquisitions & Expansion:

■ I am pleased to inform you that the acquisition of Prashanthi hospital in Warangal and Pratiksha hospital in Guwahati, marking Rainbow strategic entry into the Northeast region.

■ Both hospitals have now been fully integrated into the Rainbow network, and initial traction has been encouraging and in line with our expectations.

■ Further, we commissioned our new hospital in Rajahmundry in east Godavari district of AP. Having a regional hub in Vishakhapatnam, Rajahmundry and Vijayawada would cover most of the affluent parts of coastal Andhra Pradesh.

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■ We are now gearing up for the commencement of two new spoke hospitals in Bengaluru — Electronic City and Hennur — further expanding our presence in this key metropolitan city.

■ With these additions, RCML will conclude its current high-bed addition phase, having added 780 beds over the past two years through expansions and acquisitions.

■ To dwell on the financials for FY26 Q2, revenue registered a growth of 6.5%, amounting to ₹445 Crore, EBITDA increased by 1%, to ₹149 Crore, while there was a de-growth in PAT of 4% registering 76 Cr.

■ The overall occupancy rate for the quarter was 52%

Project Updates:

I am pleased to share the Project updates as below:

■ The Electronic City (Bengaluru) facility (~90 beds) is fully ready for clinical commencement, pending requisite government approvals.

■ The Hennur (Bengaluru) facility (~60 beds) is in its final stages of c

ompletion and is

expected to commence operations by January 2026, subject to approvals.

■ The regional hub hospital at Coimbatore (~130 beds) has entered the development phase and is expected to be ready by the end of FY27.

■ In Gurugram (NCR), construction work was progressing rapidly at both Sector 44 (~325 beds) and Sector 56 (~125 beds) but the work has been stalled by the government temporarily because of dangerously high pollution hospitals.

■ The regional hub hospital in Pune (~150 beds) is currently in the project design phase while the permissions are awaited.

Leadership Strengthening

■ I am pleased to announce the appointment of Mr. Abrarali Dalal as Group Chief Executive Officer for Rainbow Children's Medicare Ltd, effective January 20, 2026.

This appointment marks an important milestone as we fortify the leadership team to drive the next phase of growth and strengthening our business model.

International Business Update

■ Though some of the issues regarding the medical visas persists, we have seen progress on the international business front with the monthly revenue now touching ~3 Cr. We see the business returning to normalcy in coming quarters.

Clinical & Operational Highlights:

Clinically, we are continued to see good growth in our Pediatric specialities & quaternary care with improved case mix and revenue contributions. I would like to share one of our remarkable clinical achievements during the quarter.

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■ A 27-year-old expectant mother with an underlying complex congenital heart disease having only a single ventricle went through a difficult course as a child and had a shunt surgery called Glenn procedure to improve her oxygenation and quality of life.

Though the definitive procedure was planned family was never turned up because of decent quality of life. She got married and planned for pregnancy. Unfortunately, she lost 3 pregnancies in her 6th and 7th month of pregnancies.

■ She had reached to Rainbow Children's Heart Institute with lot of hope to save the baby from current pregnancy. During evaluation, she was found to have oxygen saturations around 75%. This clearly indicates continuation of pregnancy may result in Intrauterine death of the baby because of poor oxygenation. Scientifically this could have happened to her previous pregnancies resulted in Intrauterine deaths.

■ Since there was no option of definitive cardiac surgery to correct the underlying anomaly during pregnancy which obviously carries very high maternal and foetal mortality. Our cardiac and perinatal team had extensive deliberations and finally decided to rescue the mother and baby non-invasively. Therefore, our Pediatric cardiology team had discovered the way to rescue both mother and baby was to place a stent through femoral catheterization from single ventricle through the Atrietic pulmonary wall into pulmonary artery. During this procedure the baby was taken care from any exposure to radiation. Post cardiac stenting the oxygen saturations have come up to 94%. The mother's cardiac assessment post-procedure was extremely satisfactory and foetal viability was very good.

■ The perinatal team had taken care of the pregnancy and delivered the full-term healthy baby.

■ I was so excited and went down to Rainbow Children's Heart Institute to celebrate this success with the children's cardiac team and perinatal team.

Looking Ahead for second half of FY26

■ Ensure the timely commissioning of our 2 spokes in Bangalore

■ Focus on strengthening business operations and delivering sustainable growth across its network.

■ Strengthening our sales and marketing function across key markets to deliver sustainable and scalable growth

Let me take thi

s opportunity to thank our medical teams and leadership team for their dedication and hard work. We are grateful to our stakeholders for their continued trust in building this incredible healthcare for Children and Perinatal care. With that, I now hand over to our Group CFO, Mr. Vikas Maheshwari, to take you through the financial details for the quarter.

Vikas Maheshwari: Thank you, sir. A very good morning to all of you and thank you for attending this investor's conference. I am pleased to brief you on the financial performance and key developments of Rainbow Hospitals for the second quarter and first half of FY2025-26.

■ Our operating revenue for the quarter stood at Rs.444.8 Crores reflecting a growth of 6.5% when compared to the corresponding quarter of the previous financial year. For Rainbow Children's Medicare Limited, November 14, 2025

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H1, our revenue stood at Rs.797.7 Crores reflecting a growth of 6.7% when compared

to the H1 of the previous financial year.

■ Our EBITDA for second quarter amounted to Rs. 148.8 Crores, marking a 1.2% growth compared to the same period last year. For H1, our EBITDA stood at Rs. 252.5 Crores reflecting a growth of 4.9% when compared to the H1 of the previous financial year.

■ The EBITDA margins for the current quarter is 33.5%. While for H1, our EBITDA margins is 31.7%.

■ The profit after tax for the quarter is Rs. 75.6 Crores a de-growth 4.3% in comparison to the corresponding quarter of the last financial year. For H1, our PAT stood at Rs.129.4 Crores reflecting a growth of 9% when compared to the H1 of the previous financial year.

■ In terms of the operational performance, outpatient volumes witnessed a growth of 5.7% when compared to the corresponding period in last financial year. Deliveries grew by 6.8% compared to corresponding period of last year. Inpatient volumes witnessed a de-growth of 1% when compared to the corresponding period in last financial year due to the reasons outlined by Dr. Ramesh, CMD, earlier.

■ Our payor mix continued to remain robust and balanced with 52.8% of the revenue coming from the insurance and the balance 47.2% coming from the cash patients. For H1 the payor mix stands at 47.7% Cash and 52.3% insurance.

■ I am pleased to inform that our company's balance sheet remains robust with net cash position of Rs. 555.8 Cr as of September 30th of this year and will support our ongoing capital expenditure plan. Given our current cash and anticipated internal accruals in the coming years, we remain confident in our ability to complete all planned capital expenditures through internal accruals without any debt financing.

■ During the quarter, the company invested Rs. 260.6Cr in capital expenditures towards expanding and enhancing our services at existing / upcoming hospitals and acquisition of Guwahati and Warangal hospitals.

With these insights, I conclude my financial update. I now invite questions and suggestions from the participants. Thank you.

Moderator: Thank you very much. The first question is on the line of Damayanti from HSBC. Please go ahead.

Damayanti: Hi, good morning and thank you for the opportunity. My first question is for Dr. Ramesh. So, with the appointment of CEO, how your involvement or how your role will change in the company?

Ramesh Kancharla: Yes. Damayanti, I have been on the front line for a long time, and the time has come for the company to move to a professional management structure. We have expanded significantly across multiple cities, and it is important that, as Chairman, I align with the CEO and the strategic leadership team. Having a professional CEO who independently drives operations and guides the functional heads and Directors is essential for an organization of this scale.

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We did an extensive evaluation process and ultimately selected Mr. Abrarali as CEO.

Damayanti: Okay. So, obviously, I think you will be there to, handhold and advise the company as we progress ahead. Thanks for that. My second question is on your new unit, Prashanthi and Pratiksha, both integrated during second quarter, as you mentioned. So, just some color on the revenue contribution which came through these new units and your plan and expectation for scaling up operations in these two units.

Ramesh Kancharla: Yes. These units are still in the early phase. Pratiksha was doing about INR 8–8.5 crores before we took over. After the acquisition, the entire management changed, and all insurance empanelment had to be redone. We have now obtained the required permissions and insurance approvals, so we expect to see an uptick from December onwards.

Warangal had similar issues—being a Tier-2 city, insurance onboarding took time, which temporarily impacted deliveries. Now all insurance partners are on board.

We are happy with both acquisitions; integration has been seamless across doctors, nursing, and staff. Our operating model is now fully implemented, and we are optimistic about their growth.

Damayanti: Sure. So, you are largely done with integrating the entire two units and maybe from March quarter, we will see meaningful pickup in revenues. So, just a point, I think Pratiksha, you mentioned INR8 crores, INR8.5 crores kind of number which came in 2Q or what was that number?

Ramesh Kancharla: It was slightly lower due to the insurance transition. Integration also required migrating SAP, supply chain, HRMS, and HIS. The hospital earlier operated under multiple entities — They were operating on three, four companies each one for pharmacy, operations, etc. — so consolidation was a challenge, but the team has done a great job. It now operates fully like a Rainbow Children's Hospital.

Damayanti: You got it. That's helpful. And my last question is on your international business. Can you remind us, you mentioned there are some issues which should be normalized in coming quarters and then you expect things to pick up?

Ramesh Kancharla: Yes, issues continue primarily with Bangladesh — visas for patients have not been issued. However, African markets have started picking up again. We have reached back to the revenue levels of about 1.5 years ago, ~back to around INR 3 crores per month, which is what we used to do earlier. It is improving month on month and should normalize soon. It's not going to be too long time.

Damayanti: Okay, thank you for your response. I'll get back in the queue. All the best.

Moderator: Thank you. The next question is in the line of Kritika from Prospera Financial Solutions. Please go ahead.

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Kritika: Yes, congratulations on a strong quarter. I actually want to know like you mentioned earlier, that a payor mix was roughly 52% insurance and 40% cash in Q1. Has that ratio changed meaningfully in Q2 and more importantly, where do you stand on pricing renegotiation with issuers in your key market because with rising specialty payor mix and inflation in chemical inputs, are you confident that the price revision in Hyderabad, Bangalore and Chennai will come through with the timeline you had guided earlier?

Vikas Maheshwari: Yes, so the payor mix for the quarter 1 and the quarter 2 is more than the same line. There is not much of the change. This is a very balanced mix which we are driving. The good part is that we

don't do any government related schemes businesses since it is a pediatrics. So our payor mix is only cash and insurance.

As far as the insurance renegotiation is concerned, I think our team has already concluded for the Hyderabad. For the Bangalore cluster is under negotiation and it's happening. So it is a routine business for any healthcare operator for the hospital company because the insurance keeps falling on some multiple timelines of the life cycle.

So as and when it comes, we keep negotiating and so far we have been very successful in terms of the pricing and hedging against the inflation costs and the type of facility we have built and that type of complex work we do. We have been able to negotiate it properly with the insurance company so far. So we expect that, yes, we will be able to manage that going forward also.

Kritika: And my second question is one of Rainbows biggest differentiator is your strength in neonatal and the pediatric intensive care. Could you share whether MICU utilization improved in Q2 relative to Q1 and whether that was a meaningful driver for the ARPOB recovery and do you see that MICU capacity approaching constraints in any of your hubs which may require for the expansion?

Vikas Maheshwari: As far as the capacity constraints of the MICU is concerned, I think whatever the facilities we have is very well built keeping in the future requirements. So we have a headroom available in all of most of our hospitals to accommodate these type of patients in our MICU and PICU. So we don't see as of now any constraint on such capacity utilization.

Ramesh Kancharla: With our hub-and-spoke model, accommodating patients is not an issue. We always try to adjust on the occupancy basis and then when we do spoke, should we increase the bed capacity or reduce the bed capacity? We always adjust among the units between them. So that optimization is always done

I think as we move forward in future, what we had an earlier number of beds in ICUs, may be a kind of numbers wise, we probably need to optimize a little bit down because of a number of the units are distributed within Rainbow network itself. Because in the past, we used to think about the hospitals to have a larger unit. Now because of lot of micro market-driven business, we try to optimize number of beds to see that the efficiency is improved.

Kritika: Thank you. That was really helpful.

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Moderator: Thank you. The next question is from the line of Bansai Desai from JPMorgan. Please go ahead.

Bansai Desai: Yes, hi. Firstly, congratulations on the CEO appointment and we look forward to speaking with him. Sir, my first question is that, if you just step back and look at the last three to four years, while Rainbow has done very well operationally, we have seen seasons impacting our business probably twice in the span of last three to four years.

I am just trying to understand that, is this impact felt similarly across all your units or is this more pronounced in one region over other? And the second question in continuation to that is that as we look ahead and as we diversify our presence in north and northeast, should that in any way lessen the impact of seasonality that we are feeling?

Ramesh Kancharla: Seasonality is similar across southern states — Hyderabad, Bangalore, Andhra Pradesh — they behave like one cluster. Chennai is slightly different as monsoons arrive later.

The Northeast follows a distinctly different pattern. NCR also behaves differently, with peak season from October–January driven by pollution. So diversification will bring some variation, but the South largely behaves as one unit.

Bansai Desai: And sir, I was just saying that we have entered northeast with

Prashanthi's acquisition, but do

we have plans to scale up our presence there?

Ramesh Kancharla: Yes. First, we are building a strong specialty footprint. We already started some very interesting things in the northeast. We have started road transport services — we have done ~14 transports in the last 1.5 months, which never happened earlier in the region. We've gone far off distances to Shillong and all the areas and done the transports of critically sick children. We're adding speciality one after the other. We are adding pediatric neurology, nephrology, and respiratory medicine.

We want to build Guwahati as the hub for the Northeast. We're trying to see that how early, how fast we can do a periodic cardiology, cardiac surgery in our Guwahati hospital

We want to ensure that the base business is very strong, with a solid reputation. Our aim is to establish this hospital as the center for pediatric services in the Northeast. Once that is achieved, we can explore opportunities in smaller cities such as Silchar and Shillong, which are promising greenfield markets.

For now, we want to strengthen our position here over the near term before expanding further.

Bansai Desai: Thanks for that, sir. And secondly, if you could also update us on how our IVF centers are performing. It's been a year. If you could comment on how they're doing operationally and also in terms of patient volumes?

Ramesh Kancharla: Yes. As expected, our IVF growth is coming mostly through our own network and through B2C channels. B2B remains very negligible because obstetricians often see us as competitors, so they

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hesitate to refer their infertility patients to Rainbow, given our strong obstetrics network. We had a sales team for this, but it did not work.

What is working for us is, first, the in-house growth, and second, the B2C business. Both have been performing well. Collectively, we are growing quite impressively over last year. On a same-quarter basis, growth is around 40%, although it is on a low base. We would now like to sustain an impressive growth trajectory of about 25% going forward

Bansai Desai: All right. I have questions. I'll join back in the queue.

Ramesh Kancharla: Sure. Thanks. Thank you, Bansai.

Moderator: Thank you. A reminder to all participants, you may press star and one to ask a question. The next question is from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: Yes. Thanks, sir. Sir, you did talk about seasonality in terms of obviously impacting our performance this quarter. Now, if we look at the IP volume growth between the mature and the new hospitals, it seems that the volume -- the IP volumes in mature hospitals have declined high-single-digit kind of a number between 8%-9% Y-o-Y. So, why is the impact of seasonality more pronounced in the mature hospitals, given that for some of these mature hospitals, we would be doing other services as well, like at the Banjara Hospital?

Ramesh Kancharla: Yes. So, the mature hospitals have a standard, regular base business of about 50% occupancy. We expect them to reach around 60% plus occupancy in Q2 and Q3. If you look back at last year, we were at about 65%–66% occupancy. When we are at around 66%, the additional 10% growth typically comes from seasonal business. So, when we evaluate our occupancies, that

10% uplift is due to seasonality.

And the corresponding revenue increase usually comes in at around 12%–14%. This is why we typically guide that our occupancies go up by about 8% and our revenue grows by around 12% in Q2 and Q3. This pattern occurs in most years, though sometimes it is cyclical. In years with no seasons, we see the impact clearly.

This year, our base business

is strong, and our EBITDA is still robust, but occupancies are down

by about 8% and revenue is down by around 10%–12%. If seasonality had been present, we would likely have delivered 16%–17% growth. That's the clear explanation.

Mature hospitals are more affected because, during non-seasonal periods, fewer patients come to the larger, more popular hospitals. The second important factor is Pediatric Intensive Care.

During seasons, a significant number of children become sick due to pneumonia and related illnesses, and some of them require intensive care. These two segments – general pediatrics and pediatric intensive care – work hand-in-hand during seasonal illness periods. The absence of this seasonal load is what resulted in an 8%–9% dip in overall occupancy.

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In contrast, the new units operate on a low base, at around 40%–42% occupancy, so the impact of seasonality is minimal for them. In mature units, where regular fixed business is around 50%–54%, the variation is much more visible.

Rahul Jeewani: So this 8%-10% point occupancy contribution and 12%-14% revenue contribution from the seasonal business, you seem to indicate that's the contribution at the mature hospital level for 2Q and 3Q generally.

Ramesh Kancharla: Yes, exactly.

Rahul Jeewani: And sir, with respect to let's say the IVF revenue, so can you talk about in terms of some of the levers which could help us to reduce the impact of this seasonality in the overall business? So obviously, North East expansion would help. Can IVF scale up also help us in reducing the impact of this seasonality? So what is our IVF revenue contribution right now? And where do you see that number settling over the next, let's say, three to five year period?

Ramesh Kancharla: I think we are at about 3.2% of the total topline this year. It will go up, but I am not sure how much it will increase as a percentage. Overall, we expect to do around INR40 crores plus in the current year, which reflects almost 50% year-on-year growth over the last three years. This growth is driven by our specialty and super-specialty services, as well as some of the advanced specialized services we offer, including rare disease treatments. These areas will continue to expand as we mature more hospitals.

Rahul Jeewani: Sure, sir. Sure, sure. And sir, the next question I have is, as you indicated that now most of our expansion plans would get completed by FY26. And the next round of expansions would come online in FY29 only. So now as some of these newer hospitals scale up over the next two year period, can we see a margin expansion playing out for Rainbow over the next two years as some of these newer capacities scale up?

Ramesh Kancharla: No, certainly that I think what's more important for us is to drive our operations. And also, as I told you in the last earnings call that now we have a leadership in the sales and marketing, and we're doing very well. And also, now the CEO joining, I think, is strengthening the overall leaderships.

And also, what we have is the next couple of years' time is a very pure play of driving the growth and driving the revenue growth. We'll have to see that. We always guide about, 25% is our base EBITDA, pre and early EBITDA. We'll stick to that. We'll continue to drive more top-line growth.

Rahul Jeewani: Sure, sir. And any comments in terms of what kind of a top-line growth are you looking at from a next two year perspective?

Ramesh Kancharla: I think, moving forward, we should be able to deliver around 20% growth. That is what we are aiming for over the next two years. We have completed all the expansions and added plenty of beds in the growth areas. As long as we stabilize our mature units at aro

und 8%–10% growth,

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and the rest of the units grow at about 25%, we should be able to achieve a consolidated growth rate of 20%.

Rahul Jeewani: Sure, sir. So, this 20% growth includes the contribution from M&A or you are talking about the organic growth for the business?

Ramesh Kancharla: Including M&A. They are integrated now. So, they are part of the business now.

Rahul Jeewani: Sure, sir. Last question before I join back the queue. We continue to generate very healthy cash.

Right now, also, we have around INR560 crore of net cash on board. So, with the Guwahati and the Warangal acquisitions being done, are you looking at more M&A or would you slow down, let's say, from a two year perspective, scale up these newer assets and then only evaluate fresh M&A? So, any color there would be helpful, sir.

Ramesh Kancharla: Surely, if there are appropriate M&A opportunities, we would be interested. If such opportunities arise, we would certainly evaluate them. We have cash on the balance sheet and strong execution capabilities. What we look for are M&As where we can add scale, integrate the assets into our operating model, and further expand that scale. We would consider such opportunities.

Rahul Jeewani: Sure, sir. I will join back the queue.

Moderator: The next question is supplied of Isha, an individual investor. Please go ahead.

Isha: Hi, good morning. So, I am fairly new in analysing the hospital industry. So, sir, my first question is, how is the company able to maintain such high gross margins of 86%-87% when the industry average is around 73%-74%? How are you able to maintain such high gross margins? And my second question is...

Vikas Maheshwari: Yes.

Isha: Yes, sir. You can go ahead.

Vikas Maheshwari: With respect to gross margin, there is a difference between our specialties and the multi-specialty hospitals that are mostly listed. In the pediatrics and maternity space, we are the only company of scale. In our industry, the cost of consumables typically ranges between 12%-14%, depending on the mix of services being provided. That is why our gross margins are higher.

Saurabh Bhandari: See, most adult hospitals are surgically driven, whereas we are a medical hospital. In a medical hospital, the cost of implants and related items is much lower. Hence, you see higher gross margins.

Isha: Okay, sure. So, sir, my second question is, what capex do we expect to incur in H2 FY26? And what prediction should we make for the hub and spoke models bed? What capex curve bed should we project going forward?

Vikas Maheshwari: I think the projects that are still pending are Hennur and Electronic City in Bangalore, where work is ongoing. These involve the remaining payments, along with the Gurugram project, Rainbow Children's Medicare Limited, November 14, 2025

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which is also progressing. For the second half, we expect to spend close to INR100 crores. And for our Pune and Coimbatore projects over the next three years, you should budget roughly around INR600 crores.

Isha: And so, my third question is, how do we see competition affecting the occupancy rates and especially the hub region?

Ramesh Kancharla: So, competition will always be there, and the market is also expanding. It is a question of how we navigate and continue to grow. In Hyderabad, competition has existed for the last 15-18 years, and it will continue to remain. That is the reality. We simply have to work around it, play to our strengths, expand into more micro-markets, and remain a dominant

player. I think that is the only way forward.

Isha: Okay. Thank you, sir. Thank you for taking my questions and all the best for your future endeavors. Thank you.

Moderator: Thank you. The next question is from the line of Vishal from Systematix. Please go ahead.

Vishal: Thanks for the opportunity and good morning, everyone. I heard that you talked about expanding in Pune. Can you kind of give some color as to what size of a hospital you would, you're looking there and any competitive, are there any large pediatric hospitals already there in Pune?

Ramesh Kancharla: You see, in Pune, we are looking at a 150-bed children's hospital with perinatal services, with a much stronger focus on Pediatrics. This will be located on the Bangalore Highway, closer to Baner. When you look at the landscape in Pune, it is becoming similar to Hyderabad and Bangalore today. There are many mom-and-pop setups that are doing well because there are a

large number of deliveries and a significant child population in Pune. There are also fairly strong catchment areas feeding into Pune.

This is the competitive landscape. The hospitals doing significant pediatric work are largely Trust-run institutions and medical colleges. There is not much in terms of standalone comprehensive pediatric facilities. For example, Deenanath Mangeshkar has a fairly large department, and a couple of medical colleges also have sizeable pediatric departments.

There are some smaller pediatric units — about 50 to 60 beds — doing a certain degree of ICU work and related services, but they are not comprehensive children's hospitals. There is both a need and a demand for such a facility. Ultimately, it depends on the kind of professionals you bring in and how you drive clinical outcomes.

Vishal: Understood, sir. Probably this would be the first pediatric hospital that would have the most broadest suite of services for pediatrics.

Ramesh Kancharla: Exactly, that's exactly.

Vishal: And with respect to your existing business, if you could share what percentage of our business is generated from high-end services like transplant, oncology?

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Ramesh Kancharla: We provide tertiary and quaternary care as an integrated offering — it is very difficult to separate the two in a children's hospital setting. Collectively, these services, including transplants, contribute approximately 40% of our overall business.

Vishal: Okay, and is this part of the business growing faster than the rest of the business?

Ramesh Kancharla: In a hub-and-spoke model, the case mix naturally gets diluted because spokes typically do not handle high-end tertiary or quaternary care. As a result, the overall mix tends to stay in similar proportions.

For example, as we continue adding more spokes, we have seen obstetrics grow from about 20–25% earlier to nearly 30% now. The dynamics keep evolving, and such shifts are expected when you operate a broad hub-and-spoke network.

Vishal: Okay, and like within our large hub hospitals, Hyderabad, Bangalore, Chennai, would large part of the high-end care be delivered from Hyderabad and while Chennai and Bangalore are yet to catch up?

Ramesh Kancharla: Yes, it doesn't catch up quickly. It actually takes a long time to build high-end pediatric services. First, you need the right expertise; second, you need awareness; and third, you need strong outcomes. All of these take their own time to develop.

It took us many years in Hyderabad. We have now started doing transplants — bone marrow transplants, and liver transplants in both Chennai and Bangalore. But reaching a mature scale and depth in these services will still take time.

Vishal: Okay, okay. And just one final one on international patients. So tw

o things, one is getting

international patients in pediatric care, is that more difficult than probably a general hospital?

And second, what is the current state like, do we have a minor contribution coming from the international patients?

Ramesh Kancharla: See, it's a long shot, but we are working on it. We began developing our international business, and it was progressing well until the recent geopolitical developments. Because of these issues, we have seen a decline in international business from Africa, Bangladesh, and a few other regions.

However, we remain optimistic. As we move forward, international business — even in pediatrics — will become a significant component for us. That is my belief. We will need to invest more in building strong networks with doctors in these markets. For example, engaging with pediatricians, pediatric cardiologists, and other relevant specialists internationally will help us deepen relationships and drive more business.

Vishal: Got it, sir. But any sense on how large this can become, 10% of the top line, maybe next five years or so?

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Ramesh Kancharla: I won't put a specific number, but I think we should aim for at least 10%.

Vishal: Okay, thank you very much. Yes, thank you.

Moderator: Thank you. The next question is from the line of Anshul Agrawal from Emkay Global. Please go ahead.

Anshul Agrawal: Great, sir. First, a clarification. I think I mentioned, sir, saying that the top line that we're aiming or targeting is at about 20%. Now, considering that H1 was at 7%, is that any indication around H2 being stronger than the usual H2 versus H1 kind of growth?

Vikas Maheshwari: So, Anshul, let me take this question. What sir mentioned is that the 20% CAGR growth is over a period of time. The basis is twofold. First, the capacity addition we have undertaken—which is substantial—represents around a 47% increase from a low base, with nearly 780 beds added. This includes all capacity expansion and the super-specialty enhancements in our hub hospitals at Bangalore and Chennai, as well as the ramp-up of the two acquisitions we have completed, Guwahati and Warangal. Therefore, over the next two and a half to three years, you should expect revenue growth of roughly 20% CAGR. That's the first point.

Second, how the second half will pan out is difficult to predict. But our approach is not to focus on quarter-on-quarter variations, but instead on the longer trajectory. So far, if you look at the last three- to five-year CAGR, it has been roughly 16%–17%. With the significant bed capacity additions now behind us, we are optimistic that we should be able to deliver around 20% growth CAGR over the next two and a half to three years.

Anshul Agrawal: All right. So, growth could be back-ended is what you are suggesting on back of expanded capacity.

Vikas Maheshwari: Yes, acquisition integration.

Anshul Agrawal: Got it. Sort of a follow-up on this only. So, our depreciation number has not increased on an H1-H basis. Would this also sort of grow in line considering that they have added almost 800 beds in the last two years?

Vikas Maheshwari: So, the commercialization of the Rajahmundry unit and the two hospitals in Bangalore, which will happen in the December quarter, will lead to some uptick in depreciation. In our case, depreciation also includes a portion related to Ind AS accounting for leases, which gets split between finance cost and depreciation.

But as a percentage, you should expect a similar trend overall, except for one or two quarters where the opening of these three facilities will create a slight bump. Otherwise, it should remain in the same range.

Anshul Agrawal: Got it. Third question I had was, any update on

the break-even of the Bangalore and the Chennai

hospitals that will be opened in 2024, the Anna Nagar and the Sarjapur hospitals? Are we closer to break-even in Bangalore?

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Ramesh Kancharla: The Sarjapur hospital reached break-even in about 15 months. Anna Nagar took around 18 months, exactly as guided. Over the last few months, it has achieved cash break-even. Overall, the facilities are performing in line with the timelines we had indicated.

Anshul Agrawal: Got it. So, despite seasonality impact in the current quarter, the ramp-up on these hospitals has been in line with what we have guided?

Vikas Maheshwari: Yes.

Moderator: Got it. That's it from me. Thank you very much.

Moderator: Thank you. The next question is from the line of Dhananjai from Alchemy. Please go ahead.

Vikas Maheshwari: Dhananjai, we are not able to hear you at all. Dhananjai?

Moderator: As there is no response, I take the next question from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: Yes, sir. Just a few clarifications. You indicated a capex of INR 100 crores in the second half, but I missed the number you mentioned for Pune, Coimbatore, and Delhi NCR combined. Did you say INR 600 crores?

Vikas Maheshwari: So, including Coimbatore, Pune, and Gurgaon, we should be doing capex of roughly close to INR 600 crores, Rahul.

Rahul Jeewani: Sure, sir. And are you including the initial capex—let's say, what you would have already incurred for the two NCR hospitals—or is this the incremental capex over the next two to three years?

Vikas Maheshwari: Rahul, let us talk about Gurgaon. We are adding roughly close to 450 beds, and here the land and building are owned by the company, and we are constructing the entire facility. So it will

cost close to INR 1.5 billion plus for the construction of the whole facility. The capex is large. For that, we have already spent close to INR 180 crores on the land and roughly INR 20–30 crores on other work that has already been carried out.

The balance construction, which is progressing over the next two and a half to three years, will account for the remaining spend. So, out of the amount I mentioned, the maximum pertains to the Gurgaon project.

Rahul Jeewani: Sure, sir. So INR 600 crores over FY '27 to '29?

Vikas Maheshwari: Yes, over three years.

Rahul Jeewani: Sure, sir. And with respect to breakeven for the spoke hospitals—your spoke hospitals in Bangalore and Chennai achieve breakeven within 12–18 months. Can you similarly talk about the timelines for some of the regional hospitals you are setting up in the Telangana market? How

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much time would Rajahmundry and Coimbatore, once commissioned, take to achieve breakeven?

Ramesh Kancharla: Yes, sure. Rajahmundry would achieve breakeven in about 15–18 months, because brand visibility in Rajahmundry is very high. Across coastal Andhra Pradesh, every household knows Rainbow Hospital; it has a huge reputation.

Rajahmundry was started about a month and a half ago, and traction is already pretty good.

Rajahmundry integrates very quickly. The network across Rajahmundry, Visakhapatnam, and Vijayawada covers a vast stretch from Srikakulam right up to Bengal—one of the most potential areas of Andhra Pradesh. Nellore district will be covered by the Chennai hospital.

So, we are present across the entire Andhra Pradesh coast till Chennai. Warangal is an acquisition, so we need to build more pediatric business there.

For the two B

angalore hospitals—Electronic City and Hennur—Electronic City might take a little longer, around 15 months. Hennur would break even in about one year.

Rahul Jeewani: Sure, sir. And with respect to these regional hospitals in Tier 2 markets, can you talk about how your pricing varies between Tier 1 and Tier 2 markets?

Ramesh Kancharla: In Tier 2 markets, for example, Vijayawada is a matured hospital with almost 15 years of operations. Pricing there is about 75%–80% of Hyderabad levels.

In Vizag and Rajahmundry, pricing would be about 65%–70% of Hyderabad levels. These markets are more cash-oriented, with less insurance business, so you drive more occupancy with popular pricing for some time, then it stabilizes after 3–5 years as reputation builds further.

Rahul Jeewani: Sure, sir. So pricing in these Tier 2 markets could be 25%–30% lower than Tier 1?

Vikas Maheshwari: Around 30%–35%, Rahul.

Rahul Jeewani: But at steady state, these hospitals should make margins similar to Tier 1 hospitals, given that cost structures are also lower?

Ramesh Kancharla: That's true. For example, Vijayawada has margins similar to Hyderabad. Vizag may be 4%–5% lower because of the market's characteristics—more public-sector and district populations that are price sensitive. But Rajahmundry is quite good; it is an affluent part of Andhra Pradesh.

Rahul Jeewani: Sure, sir. And my last question is on the Delhi hospital—the Madhukar Trust Hospital. Can you talk about the ramp-up at Madhukar in terms of EBITDA margins? And what is the outstanding receivable from the Madhukar Trust in terms of the loan we have extended?

Ramesh Kancharla: Yes. This year we are encouraged to see that it is finally doing double-digit EBITDA. Over the last four to five months, it has consistently delivered double-digit EBITDA. We expect the business to grow well over the next couple of years. However, EBITDA margins in this hospital

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will not be very high—best case 15%–16%—because of the free beds. In a children's hospital, when sick patients occupy ICUs and free beds, it dilutes revenue significantly.

That is why it is a lower-EBITDA hospital.

Rahul Jeewani: Sure. Vikas, the outstanding receivable from the Madhukar Trust?

Vikas Maheshwari: The principal outstanding remains the same, Rahul—around INR 23–24 crores, plus other items

mentioned in our financials. Last year, we restructured their repayment schedule, offering a moratorium and revised timelines. As of now, the status is the same as on 31st March.

Rahul Jeewani: Sure, sir. And we were also supposed to convert the Rosewalk Hospital from a premium birthing centre to a regular birthing centre. Has that been completed?

Ramesh Kancharla: Yes, it has been done. It is doing well—much better now. Though it is difficult to erase the perception of luxury from people's minds, it is tracking well.

Rahul Jeewani : Sure, sir. That's it from my side. Thank you.

Moderator: Thank you. As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Vikas Maheshwari: Thank you for joining today's conference call and for your thoughtful and thought-provoking questions. It is always a pleasure to speak with all of you and take all the questions to clarify.

Your continued support is instrumental to our strategic journey. If there are any other further questions, please do connect with Mr. Saurabh Bhandari, who heads our investors relationship, at the email which is given in our press release. Thank you very much.

Moderator: On behalf of IIFL Capital, that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Note:

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ipt and may contain transcription errors. The Company or the sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

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Call: Feb 2026

February 3, 2026

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.
Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.
Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q3 & 9M FY26.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on January 29, 2026, with regard to Financial Results of the Company for Q3 & 9M FY26.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q3_&_9M_FY26.pdf

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited

Shreya Mitra
Company Secretary and Compliance Officer
Encl.: As above

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"Rainbow Children's Medicare Limited
Q3 FY '26 Earnings Conference Call"
January 29, 2026

Call Duration ■ 1 Hour

Management Speakers ■ Dr. Ramesh Kancharla – Chairman and Managing Director

■ Mr. Abrarali Dalal – Group Chief Executive Officer

■ Mr. Vikas Maheshwari – Group Chief Financial Officer

■ Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions ■ Mr. Dhananjai Bagrodia - Alchemy Capital

■ Mr. Prithvi Raj - Unifi Capital

■ Ms. Nancy Yadav - Allegro Capital

■ Mr. Anshul Agrawal - Emkay Global

■ Mr. Rishi Modi – RDM Advisory

■ Ms. Banshi Desai - JPMorgan

■ Mr. Venkata Sivaram – Individual Investor

■ Mr. Rahul Jeewani - IIFL Capital

■ Mr. Alankar Garude – Kotak Securities
Moderator ■ Mr. Rahul Jeewani - IIFL Securities Limited

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January 29, 2026

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Moderator: Ladies and gentlemen, good day, and welcome to the Rainbow Hospitals Q3 FY '26 Earnings Conference Call hosted by IIFL Capital. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is now being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, sir.

Rahul Jeewani: Yes. Hi. Good morning, everyone. This is Rahul from IIFL Capital. I welcome you all to the third quarter earnings conference call of Rainbow Hospitals being hosted by IIFL.

From Rainbow, we have with us today Dr. Ramesh Kancharla, Chairman and Managing Director; Mr. Abrarali Dalal, Group CEO; Mr. Vikas Maheshwari, Group CFO; and Mr. Saurabh Bhandari, Head of Investor Relations. Over to you, sir, for your opening comments.

Ramesh Kancharla: Thank you, Rahul.

Good morning, everyone and thank you for joining us for Rainbow Children's Medicare Ltd earnings call for Q3 FY26 and the nine months ended December 31, 2025.

I would like to start with a few key strategic and operational updates for the quarter.

■ Our operational performance showed steady improvement across all the key performance indicators, including outpatients, footfalls, IP discharges and deliveries. Growth has been quite satisfactory, considering the festivities, year-end holidays and ongoing seasonal impact.

■ The new units performed well with a steady improvement both in IP and the OP volumes. The acquired hospitals at Warangal and Guwahati have operationally well integrated into Rainbow network. Both these hospitals are performing well and in line with our expectations.

■ During the quarter, we commissioned a 100-bed hospital at Rajahmundry, strengthening our presence in Coastal Andhra Pradesh. The performance of this hospital so far has been exceptional and very close to breakeven in a few months' time.

■ The 90-bed spoke hospital in Electronic City, Bangalore, it just commenced the operations, reinforcing our footprint and advancing the hub-and-spoke model in Bengaluru. With these additions, the company has largely completed the planned expansion for the current cycle and now transitioning from the phase of capacity addition to the execution.

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■ For Q3 FY '26, the company reported revenue of INR445.4 crores, growing at a 12% and the EBITDA of INR147 crores with a growth of 9% and a PAT of INR73.9 crores, a growth of 7% compared to the Q3 of the last financial year.

■ Occupancy for the quarter stood at 47.2%.

Coming to projects:

■ The 60-bed spoke hospital in Hennur in Bengaluru city is in concluding stages, and we're awaiting for the final governmental approvals. We expect to commence operations in a few weeks' time.

■ The construction has started at the regional hub hospital at Coimbatore of 130 beds. We are targeting to commence operations by end of the '27.

■ Our upcoming Gurgaon hospital's expansion in Sector 44 and Sector 56 NCR, right now we are at the basement stage slab, and we are hoping to complete the basement slabs in 3 months' time.

■ The greenfield regional hub hospital in Pune of 150 beds has received project plan approvals from government and excavation work has just started.

On the clinical side:

I would like to present our experience of first liver transplant in Bengaluru hub hospital.

■ An 11-year-old child was referred to us with chronic liver disease, presenting with persistent jaundice and progressive weight loss. Detailed evaluation by our pediatric liver team confirmed advanced liver disease, with living-related liver transplantation identified as the only viable long-term life-saving option.

■ Following extensive discussions with the family, both parents were evaluated as potential living donors. The mother was found to be the most suitable donor. A comprehensive pre-transplant workup was completed for both the donor and the recipient, and a successful living donor liver transplantation was performed using the mother's left lobe.

■ The transplant procedure was completed smoothly, and the post-operative course was uneventful. The transplanted liver demonstrated immediate and satisfactory function. The donor was discharged on post-operative day 7, and the child was discharged on day 18 with normalized liver function parameters.

■ Although this was the first liver transplant performed at this hospital, the robustness of clinical systems and multidisciplinary coordination enabled a seamless execution

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of the program. We are deeply grateful to the mother, who not only gave birth to her child but also gave a second lease of life through her liver donation.

■ Today, we are proud to state that pediatric liver transplantation is being successfully performed across all our hub hospitals in Hyderabad, Bengaluru, and Chennai. As a pediatric liver specialist, I take immense pride in our teams. Over the past six years, we have successfully established three pediatric liver transplant programs, achieving an overall survival rate of 94%.

Clinically, we continue to see strong traction across pediatric specialties and the quaternary care, with improving case mix and the growing contribution from complex and high-end procedures.

Our teams across the network remain focused on delivering high-quality outcomes supported by full-time consultant-led model with a strong multidisciplinary collaboration.

I'm pleased to welcome Mr. Abrarali Dalal as a Chief Executive Officer. I'm sure his leadership and experience will be a strong addition to the organization.

As I look forward, I would like to reiterate that the major capacity addition in the last 2 years have almost concluded, and we are now focusing on operational excellence, patient experience and strengthening our sales and marketing and to improve our occupancy across the group level.

I'm sure with our focused executive plan, we will be able to deliver great outcomes and value for our stakeholders. With that, I now hand over the mic to Mr. Abrarali to introduce himself before passing the mic to Mr. Vikas Maheshwari for the financial update.

Abrarali Dalal: Thank you, sir. Good morning, everyone. I come with over 25 years of experience across the health care, telecommunications and FMCG industry. My career spans leadership roles in operations, strategy and business development.

My previous track record has largely been in managing turnaround, brownfield expansion, driving clinical excellence and brand repositioning mostly in the health care industry.

In my previous role, I led the growth journey of Sahyadri Hospitals for 6 years and successfully completed 2 transitions from Everstone Capital to OTPP in October '22 and then from OTPP to Manipal recently in October '25. I look forward to contributing to Rainbow at this important juncture, with a clear focus on improving occupancies, enhancing service levels, and building

scalable, sustainable operations for a longtime growth

h.

Thank you so much, and over to you, Vikas.

Vikas Maheshwari: Thank you, Mr. Abrar. A very good morning to all of you, and thank you for attending this investors' conference. I'm pleased to brief you on the financials performance and the key developments of Rainbow Hospitals for the third quarter and the first 9 months of FY '25-'26.

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■ Operating revenue for the quarter stood at INR 445 crores, reflecting a growth of 12% when compared to the corresponding quarter of the previous financial year. For, the first 9 months, our revenues stood at INR1,243 crores, reflecting a growth of 9% when compared to the same period of the previous financial year.

■ Our EBITDA for the third quarter amounted to INR 147 crores, marking a 9% growth compared to the same period last year. For the first 9 months, our EBITDA stood at close to INR400 crores, reflecting a growth of 6% when compared to the first 9 months of the previous financial year.

■ The EBITDA margin for the current quarter is at around 33%, while for the first 9 months, our EBITDA margin is 32.1%.

■ The profit after tax for the quarter is INR74 crores, a growth of 7.2% in comparison to the corresponding quarter of the last financial year. For, the first 9 months, our PAT stood at INR 203.3 crores, reflecting a growth of 8.3% when compared to the first 9 months of the previous financial year.

■ In terms of the operational performance, inpatient discharges, outpatient volume witnessed a growth of 9% and 18%, respectively, when compared to the corresponding period of the last financial year. Deliveries grew by an impressive 16% when compared to the corresponding period of last year.

■ Our payor mix continued to remain robust and the balance with 51% of the revenue coming from the insurance, and the balance 49% coming from the cash patients. For the first 9 months, the payor mix remains more or less the same, where the cash stands at 48% and 52% at insurance.

■ I'm pleased to inform that our company continued to maintain a robust balance sheet with a cash position of INR579 crores as of December 31, 2025. These funds will support our ongoing capital expenditure, merger and acquisitions plans. With our current cash and anticipated internal accruals in the coming quarters, we are well positioned to complete all our planned capital expenditures using internal resources.

■ During the quarter, company has invested INR57 crores in capital expenditures towards expanding and enhancing our services at existing and upcoming hospitals.

With these insights, I conclude my financial update. I now invite questions and suggestions from the participants. Thank you.

Moderator: Thank you very much. The first question is from the line of Dhananjai Bagrodia from Alchemy. Please go ahead.

Dhananjai Bagrodia: Congratulations on your new assignment. Just wanted to understand, sir, what are we thinking regarding occupancy, because now we have enough capacity, how are we thinking on growing occupancy?

Vikas Maheshwari: Question on occupancy, how do we build occupancy?

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Ramesh Kancharla: Currently, our overall occupancy stands at around 47.2%. This year, we have seen some muted seasonality across the group, more pronounced in mature hospitals such as Hyderabad, Andhra Pradesh, and Bangalore. This has impacted occupancy levels. Specifically, pediatric outpatient volumes, inpatients, and intensive care admissions have been lower than usual due to subdued seasonal trends

. These are the three segments that typically benefit the most from seasonality. Otherwise, the rest of the system remains well positioned. Typically, Q2 and Q3 are strong seasonal quarters. However, this year has been unusual, with a lack of seasonal illnesses that would otherwise drive footfalls and inpatient admissions.

Dhananjai Bagrodia: Sure sir. We get just add that in terms of longer term, we are now below 50% utilization. What can we see or what can we think of doing to grow that significantly?

Vikas Maheshwari: Dhananjai, your voice was not very clear, actually.

Dhananjai Bagrodia: Okay. Hello?

Ramesh Kancharla: Okay. Can you please repeat again? Not clear.

Dhananjai Bagrodia: I think right now we're below 50% utilization?

Ramesh Kancharla: Right.

Dhananjai Bagrodia: What are we thinking longer term on how we could move that? Because once we get that, then our margins and numbers could be increasing significantly and even our growth will be there.

So anything we're trying to pinpoint that on how to growth -- how to grow occupancy?

Ramesh Kancharla: Absolutely. We have a clear plan in place for the coming year to drive occupancies. We have significantly strengthened our sales and marketing function, including appointing senior leadership. Mr. Abrar Ali has also joined us, which further strengthens execution.

We are focused on more aggressive sales and marketing initiatives, along with exploring additional avenues to improve occupancy. We are also evaluating selective CGHS empanelment at revised rates, though no decision has been taken yet.

Our objective is to improve occupancy to around 55%–60%, which is the range where we deliver strong operational and financial outcomes.

Dhananjai Bagrodia: North of 65%?

Ramesh Kancharla: 55% to 60%.

Dhananjai Bagrodia: Okay. And sir, would we look at also increasing now more capacity in the near term? Or would we now just focus on occupancy, occupancy, occupancy?

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Ramesh Kancharla: Over the last two years, we have added close to 780 beds, which is a significant capacity expansion. Our pipeline remains strong, with the next phase of bed additions expected over the next 18 to 24 months.

This gives us a clear runway of about 1.5 years to focus on operational execution—driving occupancy, improving outcomes, and optimizing performance from the recently added capacity.

Dhananjai Bagrodia: Okay. And sir, the average length of stay for this 2.73, what do -- what are we targeting? Is this a fair number we should target going ahead? Or how should one look at this?

Vikas Maheshwari: Yes, this is broadly in line with our historical range. ALOS typically fluctuates between 2.6 and 2.8 days, with an average of around 2.7. This is appropriate for our business model.

Dhananjai Bagrodia: Okay, fine. Fantastic. And ARPOB growth at some stage, let's say, now we have already reached -- We're actually doing really well on ARPOB now. We've almost INR60,000. Would it be fair to assume that ARPOB has peaked out? Or do we see in that also improving from it?

Vikas Maheshwari: Inflation-adjusted ARPOB growth may continue, but the more relevant metric to track is ARPP, as ARPOB can fluctuate due to mix changes and seasonality.

Dhananjai Bagrodia: Okay. But is that your disclosure?

Vikas Maheshwari: We expect ARPP to grow at around 5%–7% CAGR over the long term. While there may be quarter-to-quarter volatility, over a longer period, ARPP growth has consistently been in the 6%–7% range.

Moderator: The next question is from the line of Prithvi Raj from Unifi Capital.

Prithvi Raj: First, on the new hospitals, is it possible to give more details about how Guwahati, Warangal, Rajahmundry are scaling up? What kind of patie

nts are we getting? And how is the insurance

empanelment? Also a follow-up on that, could you please give us the occupancy and margin details for each of these 3 hospitals during the quarter?

Vikas Maheshwari: That's a good question, Prithvi. As a company policy, we do not disclose geography-wise granular details. However, I can share directional colour.

At the time of acquisition, Guwahati was generating close to INR 8 crores per month in revenue.

Currently, it is doing slightly better than that on a monthly basis. Warangal, at the time of acquisition, was generating around INR 2 to 2.5 crores per month, depending on the month. We have broadly reached that trajectory again.

As insurance and other empanelments progress, performance is improving. For both Guwahati and Warangal, approximately 70% of insurance empanelment's are now complete. There are

two to three large insurers where empanelment is still pending, and our teams are actively engaged. We expect most of these to be in place by February or March.
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Overall, the company is fully focused on completing all business-enabling requirements, including insurance and government empanelment.

Prithvi Raj: And anything on the margins?

Vikas Maheshwari: Margins at Guwahati are broadly in line with company-level margins. Warangal is EBITDA positive and continues to improve, though it is still on a ramp-up trajectory.

Prithvi Raj: And you also made a point that Rajahmundry has broken even in the first quarter. Is it right?

Ramesh Kancharla: Yes, Rajahmundry is close to breakeven—specifically in the last month of the quarter. The traction there has been very good, across outpatient volumes, inpatients, and intensive care. Patient transfers are also strong.

Rainbow is a well-established and trusted brand in Coastal Andhra, and we have built a strong clinical team at Rajahmundry. Overall, performance has been encouraging.

Prithvi Raj: So even given the way these hospitals are scaling up, is it fair to assume that maybe in 2 years, these margins can be in line with company's margins?

Ramesh Kancharla: Yes. In some cases, yes—that is our expectation. However, it may not be uniform across all locations. Outcomes will depend on factors such as competitive intensity and local pricing dynamics.

From a consolidated perspective, our long-term margin aspiration remains around 24% to 25% EBITDA. That is where Rainbow should operate while continuing to grow.

Prithvi Raj: Okay. One last question from my side. I mean, let it be ARPOB or ARPP, we saw a high growth this year. It might be also because of lower seasonal business. Let's assume we get back the seasonal business next year. Will the ARPOB or ARPP growth rate will be lower than inflation rate just for next year if the seasonal business comes back?

Ramesh Kancharla: No. Not materially. When seasonal business forms a smaller proportion of the overall mix—relative to intensive care and complex inpatient treatments—the impact on ARPOB and ARPP is limited.

Seasonal admissions typically involve shorter stays—one or two bed-days—with lower billing.

When such volumes increase, average metrics can soften marginally. However, our core business—obstetrics, neonatal care, intensive care, pediatric specialties, and pediatric surgery—remains stable and forms the bulk of revenue.

Seasonal business, which usually peaks in Q2 and Q3, sits on top of this core and drives incremental growth in occupancies, outpatient volumes, diagnostics, and overall revenue. While quarter-to-quarter metrics may fluctuate, on an annual basis, an ARPP growth of 5%–6% is healthy and sustainable.

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Prithvi Raj: Okay. Got it. Just a bookkeeping question because you had recent hires, CEO and some senior level, do we expect an increase in the employee expenses or it's already in the books?

Vikas Maheshwari: With senior leadership hires now largely completed, and capex plans already in place, we expect revenue growth to come through and EBITDA margins to improve accordingly. While there is some incremental cost from leadership additions, as a percentage of revenue, we expect this to stabilize over the next two to three quarters.

Moderator: The next question is from the line of Nancy Yadav from Allegro.

Nancy Yadav: Congrats on a great set of numbers. I think some part of my question already got clarified in the earlier question. But I just wanted to understand why there has been some decline in the volumes and our revenues are flat as compared to the previous quarter, but there's been some decline in OP and IP volumes. And we started our Rajahmundry facility also this quarter. So how exactly should we look at a decline in volumes? Was it just because of seasonality or there was something else that impacted it?

Ramesh Kancharla: Oh, it's clearly seasonality what we have seen. The decline was primarily due to seasonality.

Last year, we had a very strong seasonal base, with high occupancies and outpatient volumes. This year, seasonal demand has been muted, particularly in mature units.

In markets such as Hyderabad, Bangalore, and Vijayawada, lower seasonal outpatient footfalls and admissions led to flatter performance. This explains the softer occupancy levels across the group, especially in mature hospitals

Nancy Yadav: Understood. And sir, just another thing about hospitals. So when we start a unit, like we started 3 units recently, how long does it take for them to start ramping up, for them to start contributing meaningfully to financials?

Ramesh Kancharla: Okay. For example, Rajahmundry is expected to generate positive EBITDA from the next financial year without any doubt. The Electronic City unit in Bangalore will likely take around 15 months to break even. The Hennur unit, which is expected to commence operations shortly, should take less than a year to reach breakeven.

Overall, by next year, most units launched over the last two years should be EBITDA positive.

Except for Electronic City during its ramp-up phase, most units in Chennai and Bangalore are expected to contribute positively to EBITDA.

Moderator: The next question is from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: First question, was while you have suggested the time lines of breakeven for the Bangalore units, would it be possible to guide or quantify the losses that we could expect in the coming quarter or any period as such till these units breakeven?

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Also a follow-up on that is, Rajahmundry, I think in the last call, if -- we mentioned that we're expecting it to break in within 12 to 15 months, but I think we are on track to breakeven within two quarters. So any particular reason that we would want to call out or any change that we have seen that has happened versus our earlier expectations which has led to this strong performance in that unit?

Ramesh Kancharla: On Rajahmundry, our brand equity in the Godavari district is very strong— almost every household is familiar with Rainbow. We have built a very strong clinical team there and have been able to scale intensive care services effectively. Demand has been fairly good. Additionally, Rajahmundry is strategically located between our Visakha

patnam and Vijayawada

units, roughly 200 kilometers on either side. It serves a large catchment area, including several nearby towns, and is part of a strong agricultural delta region. These factors have led to much better traction than initially expected

Anshul Agrawal: Got it. Would you be able to quantify the losses or guide towards what kind of losses can we build in for the Bangalore units in the upcoming quarters?

Vikas Maheshwari: For the two Bangalore units— Electronic City and Hennur, which is expected to commence operations in the coming weeks— a full-year EBITDA loss assumption of around INR 5–6 crores per unit for FY26–27 would be reasonable.

Anshul Agrawal: Got you. Both units combined, not per unit?

Vikas Maheshwari: 5 Cr loss for each hospital. So INR10 crores for both.

Ramesh Kancharla: I would about say about INR12 crores to INR15 crores.

Vikas Maheshwari: Yes, INR 12–15 crores would be a prudent assumption.

Anshul Agrawal: Got it. Second question what I had was on the mature portfolio. While I understand the seasonality impact in the current quarter as well, we are hearing incremental news flow around competition sort of in the single specialty ramping up and another multi-specialty hospital is sort of coming up with a new hospital in Hyderabad itself. While we obviously can't call out a season will return next year, but do you foresee mature hospitals to sort of lag given the fact that increasing competition, funding for other hospitals is on the rise, especially in our core markets?

Ramesh Kancharla: Oh, there's no doubt about it. Competition in Hyderabad has existed for the last 10–12 years, but it is certainly increasing. Multi-specialty hospitals that earlier lost pediatric and obstetric volumes are now attempting to re-enter mother-and-child care within their broader hospital frameworks. At the same time, smaller standalone hospitals and clinics are competing for outpatient footfalls and deliveries.

However, when it comes to children requiring high-end or complex care, patients continue to prefer Rainbow. That said, increased local competition does impact outpatient volumes. Cities are expanding, and more young families are moving in, so the overall market is also growing.

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Our focus is on capturing a larger share of this expanding market. One of the initiatives we are working on is setting up clinics in local geographies, particularly as traffic congestion increases and families prefer care closer to home. Competing effectively with smaller hospitals and clinics is an ongoing effort, and we are actively working on these challenges.

Anshul Agrawal: Got it. So again, would we be maintaining our sort of high teens top line growth aspiration/goal despite all these challenges in the mature hospitals or core geographies?

Ramesh Kancharla: I'm sure. Yes, we remain confident. Challenges exist for all hospital operators. There would be ~6, 7 multi-specialty hospital groups in each city, they all have challenges, but they're overcoming. The same way for us. Even large multi-specialty hospital groups face similar pressures and continue to adapt.

Importantly, there is no direct equivalent to Rainbow at our scale and level of specialization—most competitors are still 7–10 years behind us in terms of experience and capability. Where we do see some leakage is in outpatient footfalls, as patients sometimes opt for nearby facilities. Addressing this is a key focus area.

We currently operate close to 1,000 beds in Hyderabad and will continue expanding outpatient touchpoints through multiple formats, including clinics and additional units. This is an ongoing process. As a category leader with strong brand equity and reach, we int

end to continue driving growth proactively

Anshul Agrawal: Got it. Very clear, sir. Just one last question. Are there any immediate plans to increase the operational beds at the Guwahati hospital? I see that we only have 85 operational beds where the capacity is around 150 beds. Are there any immediate plans to increase the operational beds there?

Ramesh Kancharla: Yes. Over the next three to four months—likely in the early part of the next financial year—we plan to operationalize additional beds at the Guwahati hospital.

Anshul Agrawal: Thank you. That's it from my side. All the very best.

Moderator: Thank you. The next question is from the line of Rishi Dilip from RDM Advisory.

Rishi Dilip: A few questions. So a lot of questions have been asked on occupancy and the rationale has been seasonality-led occupancy decline. I just wanted to understand, are we exploring any other business models, maybe smaller clinics to bring in patient flow from slightly further distances, so that even if the seasonality doesn't work for us, yet our mature hospitals can rise up to 60% occupancy? Just what initiatives we are taking there?

Ramesh Kancharla: Yes, those plans are underway. We are exploring smaller clinic formats, and the work is currently in progress. These initiatives are moving into the execution phase, and we expect more clarity over the next six months.

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Rishi Dilip: Okay. All right. That's great to hear. Secondly, I wanted to understand the international patients was -- is the big rationale behind our Gurgaon hospital investment that we are doing. We want to make it a global hub for international patients for pediatrics. Given how we've seen a lot of volatility in geopolitical relations, what hedging mechanisms or what mechanisms are we implementing so that even if there is a disruption, we still don't suffer extremely highly when we end up operationalizing that hospital?

Ramesh Kancharla: Yes, it's a good question. The Gurgaon hospital is still about 18 to 24 months away from operations, and we will be starting detailed strategic planning well in advance. This will include decisions on hospital positioning, doctor mix, specialty mix, and overall market positioning. International patients will certainly be an important element for a super-specialty children's hospital in the Delhi region like multi-specialty hospitals. However, we are mindful of geopolitical volatility. We are already thinking through how to diversify international markets and identify geographies that are more stable and accessible.

In parallel, our teams are working on de-risking mechanisms to reduce dependence on any single geography, especially given the challenges we have seen in markets such as Bangladesh. This is a long-term strategic exercise, and we are fully cognizant of the geopolitical risks involved.

Rishi Dilip: All right. I understand, sir. Maybe pick this up once we are closer to the operationalization of the Gurgaon Hospital?

Ramesh Kancharla: Yes.

Rishi Dilip: Third question I had was on the hospital operationalization delays that you've experienced over the past 1 year, some because of construction time lines, some because of government delays and permissions. So just wanted to understand, is there -- has there been some sort of root cause analysis being done on how to ensure that we hit our time lines on hospital operationalization and change in the process that needs to be done? Any technology change that can help us reduce the time line for construction, just to get your view on that?

Ramesh Kancharla: Yes, it's a great question. Yes, this is an important area of focus for us. We are actively working on strengthening project management consultant

ts (PMCs) and assessing how technology can be better leveraged to improve execution and compress timelines.

Based on our experience, delays typically arise from three phases. The first is during planning and sanction approvals, which are dependent on government processes. The second is execution, which involves contractors and internal project management—this is largely within our control. The third phase is final statutory approvals before commissioning, which again depends on local authorities.

For example, in Bangalore, delays occurred due to administrative changes following the division of the Bengaluru metropolitan area into zones, which resulted in approval delays of two to three

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months. Historically, over the last six to seven years, delays have typically been limited to one or two quarters. However, in the recent cycle, some projects have seen delays of nine to twelve months.

In Coimbatore, delays were caused by the need for re-sanctioning of drawings, leading to an additional six months, which we have already communicated earlier. At this point, execution timelines are more predictable.

We believe execution-related delays are within our control, and we are strengthening processes there. Approval-related delays, especially around elections or administrative changes, are harder to control and vary significantly by state and city.

Rishi Dilip: Right. All right. It's good that we're working at least on the things that we can control, hopefully, through some technological construction equipment changes, we can ramp it up quicker. Finally, you mentioned you are exploring CGHS sign up to ramp up occupancy given the new rates. Just wanted to understand how much is the rate differential now versus our current rates for cash patients?

Ramesh Kancharla: No. There is a significant difference in pricing, which is why we are still in the evaluation phase. We are assessing this hospital by hospital and geography by geography, particularly where there is surplus bed capacity.

This would be the first time we are considering participation in a government scheme, so we want to be very cautious. If it makes economic sense without materially impacting ARPOBs or overall margins, we will consider it. Any adoption would be selective and not across the entire organization.

Rishi Dilip: And sir, what would be the percentage difference in the pricing?

Ramesh Kancharla: We are still working through the analysis, and it is premature to quantify the differential at this stage.

Moderator: The next question is from the line of Banshi Desai from JPMorgan. Please go ahead.

Banshi Desai: Just a follow-up on occupancy. I'm sorry to harp on it. If the seasonality continues to remain subdued going into the next year, do we see ourselves going past 50% occupancy levels just driven by the measures that we are undertaking?

Ramesh Kancharla: Yes, absolutely. Seasonality is one factor, but there are several action items we are actively executing to drive occupancy. Based on these measures, we are confident of achieving around 55% occupancy next year.

We have significantly strengthened our sales and marketing capabilities and are making meaningful investments in digital initiatives. These efforts should start yielding results. Over the last six months, I have been deeply analyzing our business to identify ways to drive volumes

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without being overly dependent on seasonality. That exercise is currently ongoing, and we are opt

imistic about the outcome.

That said, seasonality is inherent to any medical hospital, and illnesses cannot be entirely predicted or avoided. We see the current year as somewhat of an aberration, and we expect next year to be materially different.

Bansi Desai: Thanks for that, sir. And besides these marketing-led efforts, if you could illustrate the efforts that you are undertaking? I mean, are these in lines of adding more doctors or, again, doing more complex work? If you could illustrate what are these efforts?

Ramesh Kancharla: Certainly. One key focus area is strengthening our specialties and expanding advanced and tertiary care services. For example, we started our liver transplant program in Hyderabad five years ago, and it has performed very well. We now perform the second-highest number of liver transplants in the country at our Banjara Hills unit.

We subsequently launched the program in Chennai, where it has completed a successful first year, and Bangalore has now commenced operations as well. Over the next 16 to 18 months, we aim to scale up to around 100 liver transplants across centers.

In addition, we are investing across pediatric subspecialties, including neurosurgery and pediatric cardiac care. We are also focused on driving growth in intensive care and more complex case mix. Pediatric surgical specialties, orthopedics, and other subspecialties are all areas of active focus.

Our objective is for hub hospitals—such as Banjara Hills—to operate with stable volumes throughout the year, reducing dependence on seasonality. Over time, we would like more of our hub hospitals to follow a similar model.

Bansi Desai: All right. Noted. And sir, just a second bookkeeping one. If you could comment on how the IVF business has trended and what is it contributing in terms of revenues in this quarter? And on an annualized basis, what is it in terms of run rate? And also Butterfly Essentials, our retail endeavour?

Vikas Maheshwari: Sure. The IVF business currently contributes around 4% of overall revenue, translating to approximately INR 17–18 crores. We believe this represents a good scale-up so far.

As for Butterfly Essentials, it continues to grow. On a quarterly basis, it contributes roughly INR 4 crores, which is about 1% of total revenue. It is still in the growth phase. Between the two, IVF has greater potential for further scale.

Bansi Desai: All right. And are you dissecting margins for IVF or not at this point in time?

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Vikas Maheshwari: At this point, we evaluate margins at a broader, company-wide level. IVF is not an EBITDA-dilutive segment and is aligned with overall company margins.

Ramesh Kancharla: I agree. IVF does not dilute EBITDA and should remain in line with, or potentially better than, company-level EBITDA as it scales.

Moderator: The next question is from the line of Venkata Sivaram, an Individual Investor. Please go ahead.

Venkata Sivaram: Is the company actively investing in improving digital user experiences? And do you expect any meaningful revenue contribution from it in coming quarters or in coming financial years?

Ramesh Kancharla: Yes, absolutely. We have finalized our marketing and digital strategy, and we are also planning significant capital expenditure to evolve into a “digital front-door” hospital model. Given that we operate in the women's and children's healthcare space, our patient base is largely digital-native.

We believe digital initiatives will be a meaningful growth lever over time, and there is significant upside potential. We have firmed up the roadmap for our digital ecosystem and digital marketing initiatives and are in the process of executing them.

Venkata Sivaram: Yes. I mean apart

from marketing, I would like to know if -- the user experience kind of thing, like some patient and...

Ramesh Kancharla: Yes. We are revamping the entire digital stack, including our website, patient app, and doctor

interfaces. We are also moving towards a fully EMR-based ecosystem. While these initiatives take time to implement, our immediate focus is on enhancing the patient app, enabling online consultations, and integrating seamless payment gateways. Many of these initiatives are expected to go live in the near term.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital. Please go ahead.

Rahul Jeewani: So sir, can you also talk about in terms of the ramp-up, which we would have seen in the international business? Because I think for the past couple of quarters on the international business also, we had seen some sort of an impact because of what's happening geopolitically.

So where is the international business trending now? And what kind of a trajectory do you expect over the next, let's say, 2 years?

Ramesh Kancharla: Yes. We have gradually come back to around 2% of revenue from international business. While we had originally budgeted for about 4%, geopolitical situations significantly impacted some of our key source markets. We previously had a meaningful number of patients coming from Bangladesh, Sudan, and Kenya, two of which were severely affected. Somalia was another impacted market, including a ban affecting patient flow to Hyderabad.

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As a result, international revenues are currently around 2%, which is not very meaningful at this stage. We are revisiting our international strategy to include new countries and explore new markets, with the objective of bringing international revenue back to around 4% to 5% over time.

Rahul Jeewani: Sure, sir. And sir, with respect to the acquired assets, what was the contribution, let's say, in third quarter?

Vikas Maheshwari: Contribution in terms of what, Rahul?

Rahul Jeewani: In terms of top line?

Vikas Maheshwari: Guwahati contributed approximately INR 26–27 crores, and Warangal contributed around INR 7 crores during the quarter.

Rahul Jeewani: Okay. So INR34 crores kind of a contribution for the quarter?

Ramesh Kancharla: Yes.

Rahul Jeewani: Sure, sir. So if we adjust that, obviously, the organic growth has been mid-single digit only. And we are talking about a recovery going into '27, '28. But yes, what comfort do you have now that we would see that recovery playing out in '27 and '28?

Vikas Maheshwari: Rahul, there are two key points. Over the last 12 months, the company has made significant investments in senior leadership across sales and marketing, which we have already communicated. Mr. Srinath has joined, and Mr. Abrarali Dalal has come on board as CEO to drive the business.

We have also revamped unit heads and cluster leadership wherever required and taken corrective actions. As mentioned earlier, we are significantly increasing our focus on digital initiatives and brand repositioning. A lot of these actions are already underway.

Additionally, the seasonal impact started around Q4 of last year, and the resulting low base should also support recovery going forward..

Rahul Jeewani: Sure, sir. Now with -- just on seasonality, obviously, what happens for us is 1-year we see a very strong season followed by a weak season next year. Now let's say, if one had to evaluate the business over a medium to a long-term perspective, would you say that maybe the growth potential for our business is mid-teens kind of a number over the next 5-year period versus our earlier guidance of 18% to 20% CAGR?

Ramesh Kancharla: We should look at growth on a four-year CAGR ba

sis. Over a four-year period, we believe we

should be able to achieve around 17-18% CAGR. This year may be an exception, where growth could be closer to double digits. However, historically, we have consistently delivered 17%–18% growth, and that remains our target and comfort zone.

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Rahul Jeewani: Okay, sir. So you are still sticking to that, let's say, an 18% kind of a revenue CAGR over a

medium-term period?

Ramesh Kancharla: Absolutely.

Moderator: The next question is from the line of Prithvi Raj from Unifi Capital. Please go ahead.

Prithvi Raj: Sir, I just have one follow-up question. If you look at our margins, the margins are quite high. That's also because of the kind of treatment that we provide at the hospitals. But just given that the competition is increasing, are we seeing any signs of the pricing coming in? And is there a risk for margins to come down?

Ramesh Kancharla: Our margins are driven by the operating model we follow, which is largely a fixed-cost model. When we perform well— for example, at occupancy levels of 55% or higher— the margins are quite achievable.

Our hospitals are predominantly medical in nature, where performance depends on multiple specialties rather than just one or two. As a result, fixed costs are relatively high. When performance is strong, it translates into excellent outcomes and margin delivery. Conversely, if performance weakens, margins can decline meaningfully.

That said, the occupancy threshold is critical for us. Even at around 50% average occupancy over a full year, we are still able to sustain margins because our underlying core business remains strong. That is the key factor we continue to monitor..

Moderator: The next question is from the line of Alankar Garude from Kotak. Please go ahead.

Alankar Garude: Sir, you touched upon Telangana and Bangalore briefly. Similarly, can you touch upon your performance across some of the other clusters as well?

Ramesh Kancharla: Yes. On Chennai— as I mentioned in previous calls— Chennai is improving well and is on a good trajectory overall. All three hospitals are performing well. Guindy and Anna Nagar are doing very well, and Sholinganallur has also started showing a good ramp-up. We believe Chennai is being positioned very strongly for the next year.

The next cluster is Andhra Pradesh. Andhra Pradesh is very promising for us because we have strengthened the doctor team and leadership in Visakhapatnam, which should lead to positive outcomes. We are also planning to introduce additional programs in Visakhapatnam. Currently, we operate around 100 beds there and plan to operationalize another 50 beds.

Vijayawada continues to perform very well. Rajahmundry has joined the cluster and is moving closer to breakeven by the third month. Overall, the Andhra cluster has the potential to become another Hyderabad for us in the future.

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Coming to Delhi, performance has been okay, but I would not call it exceptional. The smaller hospital, Rosewalk, has turned around and is doing very well. Malviya Nagar, however, faces structural challenges. Given the high-cost structure and bed management issues, EBITDA margins are capped at around 8% to 10%, which has been a consistent challenge. That said, the hospital is doing well overall, especially in obstetrics. We perform the maximum number of deliveries in South Delhi, and obstetrics remains a strong area. However, in pediatric specialties, the hospital lacks sufficient sp

ace to build advanced care capabilities. It is a mid-sized hospital and combined with high fixed costs and significant payments to the society, this continues to be a challenge.

Despite this, the hospital has a strong brand and credibility, which we intend to continue strengthening. This is particularly important in light of our upcoming Gurgaon hospitals over the next two years.

Guwahati has been a great addition for us. We are actively recruiting specialty doctors, and we expect to launch a cardiac program there by mid-next year. It has been a strong acquisition, and our expectation is that Guwahati will eventually contribute to company-level EBITDA, similar to Hyderabad and other mature clusters.

Alankar Garude: Just a follow-up on Guwahati, given your experience so far -- I know it's early days, but just given your experience so far, do you get the same confidence about adding more spokes in the region? And if yes, by when would you be deciding on your growth plans in the region?

Ramesh Kancharla: Our partner is keen on expansion, but we want to take our time. Each state has its own challenges— Manipur has its own, Nagaland has its own. We want to carefully study the potential versus the challenges before taking a call. We expect to take about a year to evaluate and decide on further expansion.

Alankar Garude: Got it, sir. Sir, the second question is, would it be fair to say that we will be playing a bit of a catch-up to some of the leading multi-specialty hospital companies when it comes to some of

the sales and marketing activities, digital initiatives, etcetera?

Ramesh Kancharla: Yes, that would be fair. There is a significant ramp-up required. We have a gap, and we are actively working on closing it. This is a work in progress. Over the next one to two years, we expect substantial build-up on the digital, technology, and marketing fronts.

Alankar Garude: Maybe a crude way of asking the question. I mean, on a scale of, say, 1 to 100, if the leading multi-specialty players are at 100 when it comes to these marketing activities, digital initiatives, et cetera, where would we be at this point of time?

Ramesh Kancharla: I would place us at around 30 to 40. I would not rate us higher than that. Historically, Rainbow has grown in a very organic, doctor-driven, and community-inclusive manner. The organization was not built around sales, marketing, or digital platforms.

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However, the environment is changing. Word-of-mouth referrals are increasingly being replaced by online ratings and digital platforms. Cities are becoming more fragmented and cosmopolitan, and people now rely more on digital sources than neighborhood recommendations. That is the reality we operate in today, and we need to be at the forefront of this shift.

If you ask me where we should ideally be, we should certainly be higher, somewhere higher than multi-specialty rather than lower than multispecialty as we move forward our penetration.

Alankar Garude: Understood, sir. And one final question. See, there's been a lot of discussion on seasonality, and we've seen that impact on us for a few quarters now and maybe 2 quarters -- 2 years back as well. From your understanding, how do you see some of these global pediatric hospitals dealing with seasonality? Are they doing something different which we can try and do or this is just par for the course?

Ramesh Kancharla: Globally, there are two kinds of pediatric hospitals. One is highly specialized institutions like the Children's Hospital of Philadelphia, and the other is mid-sized hospitals such as those in Cincinnati. I recently had a discussion with the team at Cincinnati.

What they explained is that seasonality in pediatrics

is not just about coughs and colds leading

to admissions. During seasons with viral infections or unusual pathogens, it is not only otherwise healthy children who get affected. These seasonal triggers tend to destabilize children with chronic conditions— those with cardiac issues, neurological problems, or other underlying illnesses.

These children are more vulnerable during seasonal spikes and often require intensive care. As a result, volumes increase significantly in critical care units. While we often attribute seasonality to acute illnesses, the real driver is the exacerbation of chronic conditions triggered by seasonal factors, often requiring higher levels of care.

This phenomenon is consistent across geographies— in India as well as globally. While India has a higher proportion of secondary and tertiary pediatric care compared to super-specialty children's hospitals in the U.S., mid-sized pediatric hospitals there behave very similarly to us when it comes to seasonality.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments.

Vikas Maheshwari: Thank you to all the participants for joining today's conference call. We appreciate the insightful questions and the valuable suggestions shared during the discussion. Your continued support remains instrumental to our strategic journey. Should you have any further questions after the call, please feel free to write to us or reach out at Saurabh.bhandari@rainbowhospitals.in / investorrelations@rainbowhospitals.in.

Ramesh Kancharla: Thank you.

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Moderator: Thank you. On behalf of IIFL Capital, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Note:

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Call: Jun 2026

June 1, 2026

National Stock Exchange of India Limited
Exchange Plaza, Plot No. C/1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051.

Symbol: RAINBOW BSE Limited
Corporate Relationship Department,
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400001.

Scrip Code: 543524

Sub: Transcript of the Investor call - Financial Results for Q4 & FY26.

Ref: Disclosure pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations 2015.

Dear Sir/ Madam,

Please find enclosed the Transcript of the Investor call held on Monday, May 25, 2026, with regards to Financial Results of the Company for Q4 & FY26.

The said Transcript is also available on the website of the Company and can be accessed through the following link:

https://d31scedd5hpubp.cloudfront.net/investor_files/Analyst_Concall_Transcript_-_Q4_&_FY26.pdf

We request you to kindly take the same on record.

Thanking You,

Yours Faithfully,

For Rainbow Children's Medicare Limited

Shreya Mitra
Company Secretary and Compliance Officer

Encl.: As above

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VISAKHAPATNAM WARANGAL

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"Rainbow Children's Medicare Limited
Q4 FY26 Earnings Conference Call "
May 25, 202 6

Call Duration • 1 Hour

Management Speakers • Dr. Ramesh Kancharla – Chairman and Managing Director

• Mr. Abrarali Dalal – Group Chief Executive Officer

• Mr. Vikas Maheshwari – Group Chief Financial Officer

• Mr. Saurabh Bhandari – Head Investor Relations & Group Business Analyst

Participants who asked

questions • Mr. Sucrit D. Patil – Eyesight Fintrade Pvt. Ltd.

• Ms. Damayanti Kerai – HSBC

• Mr. Alankar Garude – Kotak Institutional Equities

• Mr. Rahul Jeewani – IIFL Capital Services

• Mr. Mahesh – Individual Investor

• Mr. Anubhav Sangal – Anand Rathi

• Mr. Vedant Nilekar – ICICI Securities

• Mr. Vishal Manchanda – Systematix Institutional Equities

• Mr. Uttam Purohit – VVD Asset Management

Moderator • Mr. Rahul Jeewani - IIFL Capital Services

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Moderator: Ladies and gentlemen, good day, and welcome to Rainbow Children's Medicare Limited's Q4

FY26 Earnings Conference Call hosted by IIFL Capital Services Limited. As a reminder, all

participant lines will be in the listen -only mode and there will be an opportunity for you to ask

questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator

by pressing star then zero on your touchtone phone. Please note

that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital Services.

Thank you,

and over to you, Mr. Jeewani.

Rahul Jeewani : Good morning, everyone. This is Rahul from IIFL Capital. I welcome you all to the fourth

quarter earnings conference call of Rainbow Hospitals being hosted by IIFL. From Rainbow, we

have with us today Dr. Ramesh Kancharla, Chairman and Managing Director; M r. Abrarali

Dalal, Group CEO; Mr. Vikas Maheshwari, Group CFO; and Mr. Saurabh Bhandari, Head of

Investor Relations. Over to you, sir, for your opening comments.

Dr. Ramesh Kancharla : Thank you, Rahul. Good morning, everyone, and thank you for joining us for Rainbow

Children's Medicare Limited earnings call for Q4 and financial year ending March 31, 2026. As

I reflect on the year gone by, I would like to start with a few key strategic and operational updates

for the quarter as well as the year.

- FY26 has been the landmark year for Rainbow with the company adding nearly 500 beds, the

highest annual capacity addition in its history, significantly strengthens our platform for long -

term growth across key markets.

- The investments made over the last 2 years are now beginning to reflect on our performance

with Rainbow delivering its highest ever quarterly revenue and achieving 20% year -on-year

growth.

- Our key operating metrics include outpatient footfalls, inpatient discharges and deliveries

continue to witness healthy growth across both mature as well as the new hospitals. Importantly,

the occupancies remain stable despite substantial bed addition.

- The acquired hospitals at Warangal and Guwahati have integrated seamlessly into the Rainbow

network and continue to scale up well operationally.

- In addition, the newly commissioned hospitals at Rajahmundry, HRBR, Electronic City and

Mahadevapura IVF center in Bangalore are witnessing good patient traction. We are pleased with the response in Bengaluru, where we managed to onboard highly reputed consultants across pediatrics, obstetrics, neonatal and fertility specialties. Today, Rainbow is the largest pediatric and perinatal care provider in Bangalore City.

- As we enter the next phase of growth, our focus firmly is centered on improving occupancies, clinical excellence and patient experience.

- Mr. Abrarali Dalal has transitioned seamlessly into the role of Chief Executive Officer position and is actively driving day-to-day and strategic operations across the organization. ,,~

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- I'm also extremely proud to share that Rainbow has once again been recognized as a Great Place to Work, making sixth recognition in the last 7 years with the outstanding employee satisfaction score of 90%.

- Coming to financials. For Q4 FY26, the company reported a revenue of INR459.9 crores, a growth of 24%, EBITDA of INR144.7 crores with a growth of 26% and a PAT of INR 78.2

- crores, a growth of 38% compared to the Q4 of the last financial year.

- Occupancy for the quarter stood at 45.3%.

Com in

g to projects :

- We have further strengthened our projects team with onboarding of a Director of the Projects for timely execution of our projects.

- The regional hub hospital in Coimbatore of 130 beds is in an active construction phase now and we expect to commence operations in H2 FY28.

- We continue to make significant good progress in our Gurugram projects, where we are trying to fast track the Sector 56 of 125 beds to commence operations in H2 FY28, and a large super specialty hub in Sector 44 of 325 beds is expected to commence operations in Q1 FY29.

- We have received all statutory approvals for regional greenfield hub hospital in Pune of 150 beds. Excavation work is in progress now.

- We have also signed a lease agreement for a greenfield

spoke hospital of 80 beds in Bangalore

City at a Micro market is called Seegehalli, which is very close to ITPL and Hoskote. This is a rapidly growing micro market. This will be our seventh hospital in the city.

- We have entered into an agreement for build-to-suit 100-bed hospital in Central Indore and it's in the permission stage.

- We have also entered into MoU to run pediatrics and obstetrics services in 200-bed multi-specialty hospital, which is owned by the same owner. This multi-specialty belongs to the same owner, and he would be building for us a 100-bed hospital in the neighboring site.

- Therefore, it would allow us to transfer our practice seamlessly in 2 years' time into the new premises.

With the current expansions, what we have nearly 900+ beds in the execution phase, which are all likely to be complete in about 2.5 years' time.

As I conclude, I want to reiterate the strategic priorities as we enter into the next phase of growth :

- My role will be working very closely with the CEO and the senior management personnel to drive growth, service excellence and consultant engagement, strengthening Rainbow's clinical offerings significantly in all our hub hospitals as well as regional spoke hospitals and focus on clinical outcomes.

- With the success of two acquisitions, which have been done in the last 2 years' time, we are very excited about looking at the acquisitions which are aligned to our philosophy and also where we can add scale.

- And we are looking at some new spoke hospital in existing cities and are firming up our plans for the new geographies as well. ,,~

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With that, I now hand over the mic to Mr. Abrarali Dalal before Mr. Vikas give us financial update. Thank you.

Abrarali Dalal : Thank you, Dr. Ramesh. Good morning, everyone, and thank you for joining us today. It has been a little over 4 months since I joined Rainbow and I am extremely pleased with the depth of clinical excellence, quality culture and strong organizational foundation that the company has built over the years. Rainbow today has a highly differentiated and scalable platform in women and children health care, supported by a strong brand, full-time consultant-led model and an outstanding clinical team across the network.

Over the last few months, I've spent good time engaging closely with our unit leadership teams, clinicians and functional heads across the organization. One of the key opportunities ahead of us is to further improve occupancies, consultant engagement, service mix, patient experience and operating leverage across the network, particularly as the recent capacity additions mature over the coming years.

I am now deeply involved in our operating review cadence and transformation initiatives and are building a more data-driven and execution-focused organization. This includes strengthening dashboarding and performance tracking systems, institutionalizing structured review mechanisms and improving accountability across units for sharper execution at hospital level. In parallel, we are making meaningful investments in sales and marketing, doctor engagement and patient conversion initiatives to accelerate growth momentum and improve asset utilization across the network. I'm looking forward to working closely with the leadership team to drive aspirational yet achievable growth of 20% in the current year while maintaining margins. Further, I remain highly optimistic about Rainbow's long-term growth opportunity and our ability to create a sustainable value for all stakeholders. Thank you so much and I'd like to hand over the call to Vikas. Thanks.

Vikas Maheshwari: Thank

you, Mr. Abrar. A very good morning to all of you and thank you for attending this investor conference. I'm pleased to brief on the financial performance and the key developments of Rainbow Hospital for the fourth quarter and the financial year ended 31st March, 2026.

- Our operating revenue for the quarter stood at INR 460 crores, reflecting a growth of 24% compared to the corresponding quarter of the previous financial year. For the full year FY26, our revenues stood at INR 1,703 crores, reflecting a growth of 12% compared to FY25.
  - Our EBITDA for the quarter amounted to INR 145 crores, marking a growth of 26% compared to the same period last year. For FY26, our EBITDA stood at INR 544 crores, reflecting a growth of 11% compared to the previous financial year. Our EBITDA margin for the current quarter stood at 31.5%, while for FY26, our EBITDA margin stood at 32%.
  - Profit after tax for the quarter stood at INR 78.2 crores, reflecting a growth of 38%. Ex of deferred tax adjustment, PAT growth is at around 17% over the corresponding quarter of the previous year.
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financial year. For the FY26, our PAT stood at INR 282 crores, registering a growth of 15.3% compared to FY25.

- In terms of operational performance, inpatient discharges, outpatient volumes and the deliveries witnessed a very healthy growth of 18%, 19% and 22%, respectively, during the quarter, reflecting a strong demand momentum across both mature and newly commissioned hospitals.
- Our payor mix continues to remain very robust and balanced with insurance and cash contributions remaining broadly stable during the quarter and for the full year. We had 47.5% cash business for the quarter, while the insurance business was at around 52%.
- I am pleased to inform that the company continued to maintain a strong balance sheet and healthy liquidity position of INR594 crores, which provides adequate flexibility to support our ongoing capital expenditure plans, growth initiatives and the potential inorganic opportunities.
- With our current cash position and anticipated internal accruals, we remain well positioned to fund all planned expansions

through the internal resources. Our operating cash flow to pre-Ind AS EBITDA is at around ~ 72%, reflecting a strong cash conversion post tax payment.

- During the quarter, we invested INR 61 crores on the ongoing capex plan. And for the full year, we have invested close to INR 217 crores capex towards expanding and strengthening capabilities across existing and upcoming hospitals, in line with our long-term growth strategy. With these insights, I conclude my financial update.

I now invite questions and suggestions from the participants. Thank you.

Moderator: The first question is from the line of Sucrit D. Patil from Eyesight Fintrade Pvt Ltd.

Sucrit D. Patil : I have two questions. My first question to Mr. Dalal is, looking ahead, how do you see Rainbow's operational model evolving to balance scale with efficiency? What measures are being taken to strengthen hospital network resilience, automation and patient delivery standards across the diverse geography? That's my first question. I'll ask my second question after.

Abrarali Dalal : To answer your first question, technology will play a central role in the evolution of our operating model. We are currently implementing a new CRM platform, which is a key part of this initiative.

Rainbow already generates a significant number of patients leads through various channels and activities. Our focus now is on strengthening our operational capabilities to improve lead conversion and patient engagement. Technology will be a

critical enabler in achieving this objective and enhancing overall operational efficiency across the network.

Sucrit D. Patil : My

second question to Mr. Vikas is, from a technical standpoint, how is the company optimizing its capital structure to balance growth investments in new hospitals and technologies with keeping the debt under control, especially given the capital-intensive nature of the business? Can

you elaborate in detail on the framework being used for cash flow forecasting, interest rate risk ,~

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management and working capital efficiency to ensure liquidity while still maintaining the profits above the green zone?

Vikas Maheshwari : That's a great question. That is a very good question.

If you look at our balance sheet, Rainbow has historically maintained a strong financial position and remains a cash-rich company. We currently have no debt on our books and hold close to INR ~ 700 (it is ~600 Cr, regret the error) crores in cash and cash equivalents.

To date, all our expansion and capital expenditure programs have been funded through internal accruals. Similarly, the development pipeline of approximately 900 beds that we recently outlined in our presentation is expected to be funded through internal cash generation.

At this stage, we do not foresee the need for incremental borrowing to support these growth plans, and we remain committed to maintaining a prudent and conservative capital structure while funding our expansion initiatives .

Moderator: We'll take the next question from the line of Damayanti Kerai from HSBC.

Damayanti Kerai : Two questions. First, can you bit elaborate on your M&A strategy? Now you mentioned you are looking to expand meaningfully with assets which are aligned to your philosophy. So, which will be the market where you are mainly looking to acquire assets? And what kind of hospital in terms of size or the ticket size which you intend to look for?

Dr. Ramesh Kancharla : Yes, Damayanti. As we have already discussed, we currently have around 900 beds in various stages of development and operationalization.

Looking ahead, our experience with the two acquisitions completed so far has been very positive.

As we evaluate future opportunities, we will focus on both the scale of the operations and the potential for further expansion and value creation.

Ideally, we would look at assets that have at least 50 operational beds with the ability to scale up to 100 beds or more. Smaller facilities are generally more difficult for us to integrate and scale effectively.

Our acquisition strategy will focus on opportunities where there is scalability, alignment with our clinical philosophy, and an attractive micro-market. We are also open to evaluating larger platforms or multi-unit operators where there is a strategic fit.

A second area of focus is entering new geographies. We are evaluating number of emerging cities with strong long-term growth potential. This work is currently underway, and we expect to share a more detailed growth plan over the next three months .

Damayanti Kerai : So, if I understand correctly, you intend to look assets pan-India. As you mentioned, wherever opportunities are good, where there is attractive micro market, you will look to add assets there?

Dr. Ramesh Kancharla :

Absolutely. ,,~

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Damayanti Kerai : Okay. That's good to hear. And then, sir, the Indo

re arrangement, so you mentioned 200 beds

which is existing hospital. It is an operational and management contract for you. Is that right?

Dr. Ramesh Kancharla : No. The existing 200-bed facility is a multi-specialty hospital being developed by a doctor - entrepreneur. Adjacent to this facility, he is also developing a dedicated 100-bed women's and children's hospital for Rainbow.

In the interim, since the 200-bed hospital is already operational, we have been invited to exclusively manage and operate the pediatric, obstetric, and neonatal services within that hospital. We viewed this as an attractive opportunity because it allows us to establish our presence in the market immediately, build patient volumes and brand recognition, and subsequently transition those services to the dedicated 100-bed Rainbow facility once it becomes operational in approximately two years.

That is the structure of the arrangement .

Damayanti Kerai : And under this arrangement, you will be booking revenue of -- what are the financial arrangement very broadly, if you can highlight?

Vikas Maheshwari : Damayanti, thanks for this question. Certainly. Under this arrangement, Rainbow will be allocated dedicated space within the existing operational hospital. Admissions, patient care, and discharges for the pediatric, obstetric, neonatal, and intensive care services will be managed by Rainbow. We will deploy our own doctors, nurses, and clinical teams to operate these services. The arrangement is intended as a transition phase until the dedicated facility becomes operational.

Saurabh Bhandari : The revenue will accrue to us, Damayanti.

Vikas Maheshwari : Yes, revenue will be in our name.

Saurabh Bhandari : It is not an O&M contract.

Damayanti Kerai : Okay. And these 200 beds is the total capacity. Out of that, how many will be for the pediatric care part, which you will manage?

Dr. Ramesh Kancharla : We have designated approximately 25 beds for neonatal, pediatric intensive care, and critical care services.

As for inpatient rooms and ward beds, the allocation is more flexible. Depending on demand, the effective capacity could range from 50 to 70 beds or even higher. Our objective is to

maximize utilization based on patient demand rather than operate within a fixed room allocation.

The key dedicated component is the approximately 25 intensive care beds .

Damayanti Kerai :

25 beds in the intensive care. Okay. ,,~

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Dr. Ramesh Kancharla : Yes.

Moderator: Sorry, sir, you were saying something.

Vikas Maheshwari : No, I was just checking, Damayanti, whether the clarity is there or any follow-up question is there on that.

Moderator: She has left the queue, sir. We'll take the next question from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, out of this 18% growth in IP discharges in the fourth quarter, how much was organic?

Vikas Maheshwari : Alankar, around 9% -10% of the growth came from the acquired and newly added units, while the balance was organic.

Alankar Garude: Okay. And if I strip off the acquired units only, how much would the growth be, the mature plus standalone unit?

Vikas Maheshwari : The acquired units, primarily Guwahati and Warangal, contributed around 7% –8% of revenue growth, depending on the month.

Saurabh Bhandari : This is revenue, Alankar, not volumes.

Alankar Garude: Okay. And will the mix in volumes be any significantly different?

Vikas Maheshwari : Almost it should be on the same line, Alankar.

Alankar Garude: Okay. So, then the question is, I mean, we have seen an improvement in the organic growth after a few quarters. So just wanted

to understand the sustainability of it, how much of this improvement can be attributed to the digital initiatives as well as the marketing initiatives you

had talked about in the previous call?

And if you can elaborate on, where are we now in this journey of enhancing -- I mean, you mentioned about technology in the opening remarks, but on digital, marketing, where are we in this journey right now?

Abrarali Dalal : Alankar, as I mentioned earlier, we are undertaking several technology initiatives focused on both operational efficiency and sales and marketing effectiveness.

Two key initiatives currently underway are the implementation of a new CRM platform and a new Hospital Information System (HIS). As we increase our investments in digital marketing and promotional activities, our objective is to enhance the visibility of our doctors.

The idea is

that as more patients engage with the Rainbow brand, technology will help us improve conversion rates throughout the patient journey.

We started this transformation a couple of months ago, and while there is still a long way to go,

we

are already seeing some early signs of improvement. A portion of the recent volume growth ,,,~

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may be attributable to these efforts, although I would not attribute all of it to them at this stage.

Over the next two to three months, we expect more visible results and should be able to provide

a more meaningful update at the end of the first quarter.

Alankar Garude: Got it, sir. The second question is if you look at ALOS for us, it's somewhere around 2.7, 2.75 days. Is there any scope to reduce ALOS further?

Vikas Maheshwari : Alankar, we are already among the best in the industry on this metric. Over the last three years, ALOS has consistently remained around 2.7 days, with only minor fluctuations of about 0.1 days. This is the natural range for our business model.

Alankar Garude: Okay. So difficult to go significantly beyond this number.

Dr. Ramesh Kancharla : Let me provide some additional perspective. In women's and children's hospitals, recovery tends to be faster because patients are generally younger. As a result, ALOS is largely driven by clinical realities rather than management decisions.

It is not something that can be materially increased or reduced at will. Changes in case mix can have some influence. For example, if we see more chronic cases, longer ICU stays, or highly complex treatments, ALOS could move higher. Conversely, if obstetrics becomes a larger proportion of the mix, ALOS may decline slightly.

Overall, we view an ALOS of 2.7 –2.8 days as very healthy and appropriate for our business .

Alankar Garude : Got it, sir. That's helpful. The other question was can you qualitatively talk about your performance across the key clusters, specifically Hyderabad, Bangalore, Chennai? And even if you can talk about Madhukar, how has been the performance, say, in FY26, fourth quarter? And what are your expectations over the coming years?

Dr. Ramesh Kancharla : We have already discussed the seasonal backdrop during the call. If you look at the year as a whole, performance was relatively stable, with some acceleration in Q4.

This improvement was largely driven by a better -quality mix of patients, higher case complexity, and the increasing contribution from fertility services. Obstetrics has also increased as a share of revenue, moving from around 28% –30% historically to approximately 32% today.

The business mix is gradually evolving. We are consciously working to reduce dependence on seasonal demand and move toward a more diversified and sustainable growth model. While seasonality will always play a role, our go

al is to become less reliant on it over time.

A key part of this strategy involves strengthening clinical capabilities across both hub hospitals and regional spokes, onboarding additional specialty doctors, and expanding our ability to handle more complex cases.

Alankar Garude: Sir,

if you can help, I mean, break your response across the 3 main clusters, Hyderabad, Bangalore, Chennai and even Madhukar. Any qualitative comments on the performance, where, ~

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are we in the journey of taking Bangalore closer to Hyderabad levels or even Chennai closer to Bangalore? And any color on that will be really helpful, sir.

Dr. Ramesh Kancharla: Certainly. Intensive care services have been significantly strengthened in both Bangalore and Chennai. Specialty services have also expanded, particularly at our Marathahalli hub in Bangalore.

The overall service mix has improved, and we are now performing a growing number of bone marrow transplants and liver transplants. In Chennai, for example, we completed approximately

10 liver transplants in the first year of the program. This creates opportunities for government tie-ups and should further strengthen both transplant volumes and intensive care services.

In Bangalore, we have now built a comprehensive specialty stack, and the next phase is to expand advanced and complex care programs. Pediatric specialty hospitals naturally take time

to mature, but I am very encouraged by the progress we are seeing in both Bangalore and Chennai.

The same approach that helped us build Hyderabad is now being successfully replicated in these markets.

Alankar Garude: Got it, sir. And one final question with your permission. We have seen some consolidation -- actually, consolidation of the top 2 players, at least on the maternity side, last week in the mother and child segment. From a competition standpoint, can it change anything at all for Rainbow?

Dr. Ramesh Kancharla: I would answer that in two parts.

First, in pediatric healthcare, Rainbow remains the largest player in the country, both in terms of scale and clinical capability. There is currently no other player that comes close to our breadth of specialties, clinical depth, and ability to handle complex pediatric cases. We remain the undisputed leader in this segment.

Second, in maternal and perinatal care, we perform approximately 20,000 deliveries annually. While we do not emphasize this fact frequently, we are likely among the second - or third-largest maternity chains in the country. Over the last two years, we have also built a rapidly growing fertility business.

I do not want to compare ourselves or match ourselves with any of the boutique chains, which are more of a mom-and-pop stores because their sizes are less than 50 beds or 25 to 30, 40 beds.

And we don't do any hospital which is less than -- lowest is 50 beds. Even the 50 beds are only 10% of the hospitals. Our model is fundamentally different. We focus on larger hospitals, broader service offerings, and stronger clinical capabilities.

Our strategy is centered on

building large, clinically differentiated institutions with

comprehensive offerings in women's and children's healthcare. As a physician-led organization, ~

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our primary focus has always been clinical excellence and advancing healthcare delivery in these segments, and that will continue to define our approach going forward.

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Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital.

Rahul Jeewani: Yes, sir. Just a few clarifications again on the Indore hospital. So, sir, this incremental, let's say, 100-bed new pediatric unit which is being built, I'm assuming that the capex for that unit will be done by the partner. So, what would be Rainbow's investment if we have to do anything there?

And if you can also talk about what kind of revenue sharing arrangement, we have with the

partner?

Dr. Ramesh Kancharla : The partner will undertake the base building and provide us with a semi -warm shell on a long - term lease basis. This is similar to the model we follow for several other leased hospitals, where we enter into lease agreements for 20 –30 years. Rainbow will invest in the interior fit-outs and medical equipment required for operations .

Vikas Maheshwari : It is lease rental payment only.

Rahul Jeewani : Okay. So as of now, the arrangement is for the 200- bed existing multi-specialty hospital, you are just managing the pediatric unit for them. And once you move to this new 100 unit, this would function as a normal leased hospital for you?

Dr. Ramesh Kancharla : Exactly. That is the intended structure .

Rahul Jeewani : Sure, sir. And then what kind of -- because obviously, this is not part of the expansion plan which you have put out in the PPT. So, what kind of spend do you envisage for this 100- bed Indore hospital?

Dr. Ramesh Kancharla : It is still at an early stage, but similar to our other regional spoke hospitals, the investment would likely be in the range of INR 65– 70 lakhs per bed .

Rahul Jeewani : Okay. Sure, sir. Sure, sir. And commissioning, you said over the next 2 years period?

Dr. Ramesh Kancharla : Yes, I think likely 2 yrs.

Rahul Jeewani : Sure, sir. And sir, my second question is with respect to performance across the mature and new hospitals. Now while on the new hospital side, we saw a very significant increase in capacity to the tune of 80%, despite that, we were able to hold on to occupancy in the new hospitals.

But if we look at the mature hospitals for the full year, we have seen a decline in occupancy, while on the ALOS as well on the mature hospitals, there has been some decline. But can you qualify in terms of what led to the decline in occupancies for the mature hospitals apart from, let's say, a weaker seasonal business which we might have seen last year?

Dr. Ramesh Kancharla :

We have discussed this over the last two quarterly earnings calls as well. The primary reason has been the absence of the seasonal volume uplift that typically benefits mature hospitals. This was particularly evident during Q2 and Q3. ,~

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In Q4, however, performance improved, and occupancy increased to approximately 52% .

Saurabh Bhandari : About 52% occupancy is there.

Dr. Ramesh Kancharla : As mentioned earlier, one of our key priorities is to reduce dependence on seasonal demand. We are working towards building a broader service portfolio and a deeper specialty stack. The contribution from obstetrics and fertility has increased, and the specialty mix will continue to improve.

Seasonality will always have an impact no doubt , but over time, we expect the business to become significantly less dependent on seasonal trends .

Rahul Jeewani : Sure, sir. So just on the seasonal business. In the past, the seasonal business has contributed around 10% to 12% of our overall top line. So, let's say, what would have been the contribution in FY26 for the seasonal business? And if you can also call out the contribution from the IVF segment and the international business for last

st year?

Dr. Ramesh Kancharla : If you look at the overall growth for FY26, we delivered around 12% growth. Had seasonal demand been at normal levels, growth could have been closer to 18% –20%.

As Abrar mentioned earlier, based on the current quarter's performance and the growth initiatives underway, we believe the business is positioned to deliver around 20% growth in the upcoming financial year.

International business, however, was impacted by geopolitical developments. Historically, we have received a meaningful number of patients from Bangladesh and other international markets. Dominantly, we used to get patients from Bangladesh and

Saurabh Bhandari : Somalia.

Dr. Ramesh Kancharla : Because a lot of deporting has happened, Somalis and Nigerians and those people. Yes. Disruptions in these markets affected patient flows, and in some cases local coordinators were not available, resulting in a significant shortfall versus our expectations for international business.

Saurabh Bhandari : Additionally, in Q4, the escalation of conflict in the Middle East affected air connectivity and travel, which also impacted international patient inflows .

Rahul Jeewani : Sure, sir. Can you call out the number just for bookkeeping purposes, both international patient and the IVF revenue contribution?

Vikas Maheshwari : International patient revenue remained broadly flat during FY26 at approximately INR 28.9 crores. As for IVF, it contributed 3.7% of total revenue for the full year, with the contribution increasing

quarter on quarter. In Q4, IVF accounted for approximately 4.1% of revenue. ,,~

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Saurabh Bhandari : And in absolute number terms, IVF business for full year was INR60 crores .

Vikas Maheshwari : INR61.4 crores .

Moderator: The next question is from the line of Mahesh, an Individual Investor .

Mahesh: I'm retail investor and also a customer of Rainbow. And I have seen the different experience between Hyderabad and Vizag branches. So, as we are entering into new cities, so how are we ensuring the same quality standards maintained at new branches or spoke hospitals?

Dr. Ramesh Kancharla: That is a very important point, Mahesh, and it is something we are actively addressing through our "One Rainbow" initiative.

We have dedicated teams working on standardization across the network, including clinical protocols, SOPs, and operating frameworks. Given that pediatric healthcare is highly clinical in nature, consistency in medical protocols and care delivery is critical.

Hospitals in Hyderabad, Vijayawada, and Bangalore already operate with a high degree of standardization. As our network has expanded to 25 hospitals, extending this consistency across all units has become a major strategic priority. It is an area that my senior clinical leadership team is actively focused on .

Mahesh: Yes, because I have observed this pattern. Like most of the jobs are in urban cities, and they come to hometowns and they prefer Rainbow because the same quality they prefer, but they are experiencing a little bit quality gap . So that's why...

Dr. Ramesh Kancharla : Thank you for bringing that to our attention. We take such feedback seriously and will continue working to ensure a more consistent experience across the entire Rainbow network .

Moderator: The next question is from the line of Anubhav Sangal from Anand Rathi.

Anubhav Sangal : Could you provide the revenue numbers for pharmacy for full year FY26 and Q4?

Saurabh Bhandari : Sorry, Anubhav, there was disturbance. Can you please repeat your question?

Anubhav Sangal : Sir, could you provide the pharmacy revenue numbers for Q4 as well as FY26?

Saurabh Bhandari

dari : Pharmacy numbers?

Vikas Maheshwari : Anubhav, as a matter of policy, we do not disclose pharmacy revenue separately. However, if you look at our pharmacy -related costs, they typically account for around 12% –13% of revenue.

Based on that and the associated margins, you may be able to estimate the pharmacy revenue contribution. At this stage, we do not disclose it separately .

Moderator: T

he next question is from the line of Vedant Nilekar from ICICI Securities. ,,~

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Vedant Nilekar : Am I audible?

Abrarali Dalal : Yes.

Moderator: Yes, you are.

Vedant Nilekar : Congratulations to the management for a great set of numbers. My first question is on the dedicated IVF center we have at Mahadevapura, Bangalore. Now that we have our full year revenues from IVF at around 3.7%, how are we envisaging the ramp-up? And what would be the drivers for it in the next 2 to 3 years?

Dr. Ramesh Kancharla : Historically, most of our fertility services have been housed within our hospitals. Currently, we

have a couple of standalone fertility centers in Bangalore and one in Hyderabad.

Our approach to building the fertility business is centered on the quality of doctors, adherence to a strong ethical framework, and the support we receive from our internal referral network, along with direct patient engagement. We have been very encouraged by the growth trajectory of the fertility business over the last two years.

Going forward, we expect the fertility business to continue growing at approximately 25% year-on-year for at least the next three years.

Vedant Nilekar : Okay. And my second question is on the capex. Can you please help us with the figure of capex for the next 2 years along with the bifurcation of maintenance and growth?

Vikas Maheshwari : Certainly. Our capital expenditure can broadly be divided into maintenance capex and growth capex. Maintenance capex is approximately INR 45 crores per year across our network of around 25 units.

Growth capex can be further divided into two categories. The largest component relates to our Gurugram projects, where we own the land and are constructing the hospitals ourselves. We

have already incurred part of the expenditure and expect an additional investment of around INR 400 crores towards the Gurugram facilities.

For the remaining projects—such as Coimbatore, Pune, and other facilities—you can assume a capex intensity of around INR 65–70 lakhs per bed.

Vedant Nilekar : INR 70 lakh per bed.

Vikas Maheshwari : Yes, around INR 65–70 lakhs per bed is a reasonable assumption.

Vedant Nilekar : Okay. And so, is there any capex included regarding Indore in this?

Vikas Maheshwari : Yes, it does. You can assume approximately 100 beds for Indore.

At present, our total development pipeline is around 900 beds, including the Gurugram projects. Excluding Gurugram, the pipeline is approximately 450 beds. Applying a capex intensity of ,~

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around INR 70 lakhs per bed, the investment requirement for Coimbatore, Pune, Indore, and other facilities would be roughly INR 300–350 crores.

In addition, we expect to invest approximately INR 400–500 crores in the Gurugram projects over the next two years.

Moderator: The next question is from the line of Vishal Manchanda from Systematix.

Vishal Manchanda : I have a question on the occupancy. Like in your opening remarks, you highlighted that im-

proving occupancy and service mix is one of the areas that you are working on. So, can you highlight some near-term and medium-term levers for the occupancy improvement and service mix improvement?

Dr. Ramesh Kancharla : When it comes to occupancy, our view is that a group-level occupancy of around 56%–58% is the desirable range for achieving our budgets and delivering the expected outcomes of our operating model.

Occupancy naturally fluctuates due to seasonality, holidays, and other cyclical factors. However, if we can maintain a blended occupancy of 56%–58% across the group, we believe we can deliver on all our key operating and financial metrics.

In terms of service mix, there is a clear distinction between hub hospitals and spoke hospitals. Spoke hospitals are primarily volume-driven businesses, whereas hub hospitals focus on both volume growth and enhancement of service mix. This includes expanding specialty services, increasing surgical volumes, driving more intensive care admissions, and treating a larger number of complex specialty cases.

For example, in super-specialty areas such as liver transplantation, we have planned a significant ramp-up during the current year. Initiatives such as these will play an important role in driving top-line growth.

Vishal Manchanda : Are there segments within health care, like within the pediatric care that you currently do not have presence in and are intending to build presence in those categories?

Dr. Ramesh Kancharla : In Hyderabad, we already have a comprehensive portfolio of pediatric specialties. In Bangalore, we have built approximately 60%–70% of that capability and are continuing to expand it. We are following a similar path in Chennai as well.

Pediatric super-specialties take time to develop, as many of them are relatively new disciplines in India and require specialized talent and infrastructure. We have successfully built these capabilities in Hyderabad and are now accelerating the development of similar capabilities in Bangalore and Chennai.

Our objective is to create a pediatric equivalent of a multi-specialty adult hospital—offering a

comprehensive range of pediatric specialties under one roof. This has been a core part of our strategy, and we believe we have executed it quite successfully so far . ,~

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Vishal Manchanda : Sir, would you be able to share some number as to like what -- in your kind of core hubs like Hyderabad, what would be our market share in the super specialty services? Like would we have a very dominant share, like most of the cases coming to Rainbow?

Dr. Ramesh Kancharla : We certainly believe we hold a leading position. However, it is difficult to accurately quantify market share because comprehensive market -level data is not readily available, and patient flows are often fragmented.

That said, among dedicated pediatric healthcare providers, there are very few players operating at a meaningful scale in these super -specialty segments .

Vikas Maheshwari : Vishal, we have recently engaged CRISIL to conduct an industry study covering the pediatric healthcare market and our positioning within it. That work is currently in progress.

Once the report is finalized, we will be able to share more authentic and data-backed insights. At present, market data remains highly fragmented, which is why we have sought CRISIL's support to build a more comprehensive assessment of the market and our position within it .

Moderator: The next question is from the line of Rahul Jeewani from IIFL Capital.

Rahul Jeewani : Yes, sir. Sir, just on the geographic expansion, which you are targeting now with the, let's say, the Delhi hosp

itals, we would have a presence in North. Pune provides us an entry into Maharashtra, and now we have announced this Indore hospital as well into Central India. So, let's say, M&A opportunity which you're looking at would largely be foc used in these 3 clusters? Or you are looking at outside as well?

I forgot about Guwahati as well you have Guwahati, Northeast. So, if you can talk about this geographic expansion which you are targeting in some of these newer markets.

Dr. Ramesh Kancharla : Rather than being focused on specific geographies, we look primarily at the quality of opportunities available and whether they fit our operating model. Our approach is driven more by the attractiveness of the opportunity than by a predetermined geographic preference.

We continuously evaluate potential opportunities across the country and assess where they can create value for us. At this stage, we are not restricted to any region and are open to pursuing opportunities wherever they make strategic and operational sense .

Rahul Jeewani : Sure, sir. Sir, why I was asking because, let's say, in our core clusters of Hyderabad, Bangalore and Chennai, we have adopted this hub-and -spoke model where we have a larger hub hospital and the spokes feed into the hub. So, in these -- obviously, Delhi has the hub and one hub and one spoke. But for some of these other markets, wouldn't you want to have a similar kind of a hub-and-spoke approach to expansion?

Dr. Ramesh Kancharla :

No. Eventually, wherever it is appropriate. For example, in Guwahati, we have acquired a hospital that has integrated seamlessly into the Rainbow network. We are expanding specialties there and evaluating how best to address the broader Northeast market. ,~

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Our long -term vision is to provide pediatric care through a hub-and -spoke model wherever it is suitable. However, the model depends on factors such as the size of the city, population density, accessibility, and local healthcare needs. It is not necessary that every market will require a hub - and-spoke structure.

For example, in a city like Indore, a spoke may not be necessary. Similarly, in smaller cities such as Raipur, a single hospital may be sufficient. In contrast, a market like Pune would likely be well suited for a hub-and -spoke approach.

Ultimately, decisions around hub -and-spoke expansion are influenced by city size, population, accessibility, traffic conditions, and several other local factors .

Rahul Jeewani : Sure, sir . So maybe within some of these smaller cities where you might expand, we might have just 1 or 2 hospitals without necessarily having a hub?

Dr. Ramesh Kancharla : Yes, absolutely.

Moderator: We'll take the next question from the line of Uttam Purohit from VVD Asset Management.

Uttam Purohit: Congratulations on a good set of numbers. My question was on the ARPOB. If you could give some color on the drivers of ARPOB this quarter and for the full year as well? How much of what -- was it price driven and how much of it was mix driven?

Dr. Ramesh Kancharla : ARPOB has grown by about 8%, right? In our case, ARPOB growth is predominantly driven by case mix. In addition, the increasing contribution from the fertility business has also supported ARPOB growth.

Vikas Maheshwari : Correct.

Dr. Ramesh Kancharla : These are the two primary drivers of ARPOB growth. Pricing increases also contribute to ARPOB growth on a year -on-year basis .

Uttam Purohit: And how do we see this number 2, 3 years down the line?

Dr. Ramesh Kancharla : I think it is important to look at both ARPOB and ARPP together. However, if we are evaluating growth trends, I would place greater emphasis

is on ARPP. ARPOB can fluctuate from quarter to

quarter due to changes in case mix, whereas ARPP provides better visibility over time. As we have consistently indicated, we expect ARPP to grow by around 5% –6% annually .

Uttam Purohit: Yes. And on the growth side, if I see this quarter, a good chunk of growth came from the maturing hospitals. So, if you could give some sense like which geographies were the key drivers and where are we seeing a quick ramp up other than Hyderabad.

Dr. Ramesh Kancharla :

We have seen growth across the group, including Hyderabad, Bangalore, Chennai, and Andhra Pradesh. These continue to be our key markets and primary growth drivers . ,,,~

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Uttam Purohit: And on the mature side, from a long time, we have seen the occupancy has been range-bound near the 54% -- 55%, 56% kind of number. Historically, it was around 60%, 61%. So, do you think we could get back to those figures? Because I think it's been long since we have got such figures in matured side ?

Dr. Ramesh Kancharla : Yes, certainly. That is our expectation. For the coming year, we believe mature hospitals should be able to operate at around 60% occupancy. To achieve our planned budgets and growth targets, mature hospitals will need to be around that level. On a blended basis across the network, we would expect occupancy to be in the range of 56% –57%.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments. Thank you, and over to you, sir.

Vikas Maheshwari : Yes. Thank you, investors, for joining the conference call and giving your views and asking the thoughtful questions. Thank you very much for this. If you have any questions, you can reach out to Investor Relations at Rainbow Hospitals or you can e -mail us directly.

Dr. Ramesh Kancharla : Thank you very much. Thank you.

Moderator: Thank you, sir. Thank you, members of the management. On behalf of IIFL Capital Services, that concludes this conference. We thank you for joining us, and you may now disconnect your lines. Thank you.

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